

Dr. Lal Path Labs Ltd. (LALPATHLAB)

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National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q 4 & FY23

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q4 & FY23 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
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Dr. Lal PathLabs
Q4 & FY23 Earnings Conference Call Transcript
May 11 , 2023

Call Duration ■ 1 hour 06 minutes
Management Speakers ■ (Hony) Brig. Dr. Arvind Lal – Executive Chairman
■ Dr. Om Prakash Manchanda - Managing Director
■ Mr. Bharath Uppiliappan - Chief Executive Officer
■ Mr. Ved Prakash Goel – Group Chief Financial Officer
■ Mr. Shankha Banerjee – CEO Suburban and other Group Companies
Participants who asked questions ■ Rahul Agarwal – InCred Capital
■ Prakash Kapadia - Anived Portfolio Managers
■ Bino Pathiparampil - Elara Capital
■ Lavanya Tottala - UBS Securities (India)
■ Prakash Agarwal - Axis Capital
■ Aneesh Deora - Nomura Financial Advisory and Securities
■ Nitin Agarwal - DAM Capital Advisors
■ Cyndrella Carvalho - JM Financial Institutional Securities
■ Tanmay Gandhi - Investec Capital Services
■ Yogesh Tiwary - Arihant Capital Markets
■ Praveen Kumar - Acuitas Capital Advisors
■ Zaid Munshi - Concept Investwell

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Moderator: Ladies and gentlemen, good day, and welcome to Dr. Lal PathLabs Q4 FY23 Earnings Conference Call. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Siddharth Rangnekar of CDR India. Thank you. And over to you, sir.

Siddharth Rangnekar: Good evening, everyone, and welcome to Dr. Lal PathLabs Q4 & FY23 Earnings Conference Call.

Today, we are joined by senior members of the Management Team, including (Hony) Brig. Dr. Arvind Lal – Executive Chairman, Dr. Om Prakash Manchanda – Managing Director; Mr. Bharath Uppiliappan – CEO; Mr. Ved Prakash Goel – Group CFO; and Mr. Shankha Banerjee – CEO, Suburban and Other Group Companies.

I would like to point out a standard disclaimer here. Some of the statements made on today's call could be forward-looking in nature and actual results could vary from these forward-looking statements. A detailed disclosure in this regard is available in the 'Results Presentation,' which is available on stock exchange websites.

I would now like to invite Dr. Lal to share his perspectives. Thank you. And over to you, sir.

Dr. Arvind Lal: Thank you, Siddharth. A very good evening and a warm welcome to everyone present on this call. We are here to discuss Dr. Lal PathLabs Q4 FY23 Earnings and I would like to take you all through the "Key Developments and Updates" during the period under review.

India requires quality diagnostics of scale. Over the years, we have tracked the gradual evolution of the way doctors and patients manage disease and health. Accurate diagnostics is increasingly contributing throughout the treatment process, including health management, disease detection, prognosis, diagnosis, treatment planning and post-treatment monitoring. We therefore continue to invest in creating an efficient technology-backed network that can align with the growth in sample volumes.

To start with, I'm delighted to share that Dr. Lal Path Labs has clocked 15.5% growth in the non-COVID revenue in FY 23. We have proudly served about 27 million patients in FY23. This has come on the back of a sharp focus on core operations across the country including Tier-2 and Tier-3 cities and investments in digital infrastructure.

We have continued to make progress across our three strategic pillars that is geographical expansion, creating a unique and fresh-size portfolio and augmenting the technology infrastructure for the future.

The highlight was the unveiling of our Mumbai Reference Lab at Vidyavihar in January this year. This has immediately resulted in the increased testing of the high-ended test and it is also the first private lab in West India to have a BSL Level -III biocontainment lab. This speaks leaps and bounds of our commitment to offer our best-in-class services to our patients by bringing accurate diagnosis at reasonable prices to the fore.

We also added a second electron microscope at our National Reference Lab in Rohini in Delhi.

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At this point, I would like to remind you that Dr. Lal Path Labs already have the world's largest histopathology testing center or load, processing up to 1,400 biopsies a day, and also the largest kidney biopsy testing center. We were the first private lab in South Asia and Southeast Asia to have an electron microscope way back in 2016. Furthermore, I am pleased to share that we continue to work towards enhancing our technological capabilities. We have begun implementing and upgraded lab information system and advanced tools like artificial intelligence and data mining. These have enabled us to achieve enhanced

operational efficiencies and quicker turnaround time or TAT.

We continue to add new tests to our test menu and diversify our reach to higher franchise share of patient service centers.

Our processes are tuned to drive trust towards our brand and therefore I think we are uniquely placed to both drive and benefit from the expansion in consumption of diagnostic services going forward.

Thank you very much and I would now like to hand over the floor to Dr. Om. Over to

you, Dr. Om.

Dr. Om P Manchanda: Thank you, Dr. Lal. Welcome everyone to Dr. Lal PathLabs Q4 & FY 23 earnings call.

I hope that you and your loved ones are keeping safe and healthy .

I'll share a few of the industry and business insights that I see emerging based on FY23 results. During the entire FY23 in quarterly commentaries, I've always been highlighting that we should annualize instead of quarterly trends. Now, since the financial year is over, the entire analysis becomes a bit more meaningful as we have full year numbers in front of us.

Let's have a quick look at FY23 :

While FY22 was operationally very challenging due to very high incidence of delta and subsequent overhang of travel -related test requirements and various waves like Omicron, etc.,

In contrast to FY22 , we experienced FY23 turned out to be financially very challenging for the entire industry due to the following reasons :

1. Nearly 80% to 90% decline in COVID and COVID -related revenues. The higher the contribution of COVID testing to the total revenue , bigger the challenge.

2. Inability to rollback capacities and manpower and reagent stocks due to unpredictability of the future COVID demand pattern.

On the other hand, FY23 also experienced an uneven quarterly base and coupled with aggressive competitive activities further made it difficult for all of us in understanding the underlying growth trends .

Now since FY23 is over , I'm clearly seeing few trends. So, my take is the following :

1. Bundled packages have become a way of life in our industry . Contribution of Swasth fit to LPL total revenue has further gone up to 22% in Q4. This means we are doing a higher number of tests per patients and that's very clearly visible in our sample volumes as well .

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2. CAGR trends based on this is our internal estimates of few large players.

Their data is in public domain. That also include their inorganic assets and residual FY23 COVID revenues . Over four years suggest that some of these players have experienced all put together about growth rate of 10 -11%. This may not be reflective of entire industry growth, but if I look at all the data put together in public domain , I'm seeing CAGR of 10 -11%.

3. After a long gap , we saw in Q4 some of the players have taken price increase and we also did a little bit in the month of February , we will talk about that .

4. As the pressure on margins buildup in the industry, especially on smaller and mid -sized players , we do see consolidation opportunities emerging in times to come ; however , valuation expectations and unavailability of quality assets will make this journey a little difficult.

All suburban diagnostics as a platform we are now set for the next phase of integration . As Dr. Lal mentioned that we built a state -of-the-art reference lab in Vidyavihar in the month of January this year .

Second is, as all of you know, that COVID contributed nearly 50% of the total revenue of the Suburban in FY22. It is now down to just 6% in FY23. The stage is definitely set for the next phase wherein we plan to bring in now back -end synergies between the parent company, LPL, as well as Suburban Diagnostics and go further aggressive in Mumbai and Western region .

In the end , I would say the brand that has the trust of the patients and medical fraternity , holistically invested in network creation , infrastructure scale up , patient services, etc. , will

sustain and grow in the long run .

With that now I would like to invite our CEO – Bharath to continue this conversation.

Thank you and over to you , Bharath .

Bharath Uppiliappan: Thank you , Om . A very good evening to everyone present here and I warmly welcome you all to our Q4 FY23 earnings conference call.

I will now take you through some of the operating and business highlights of our company :

In Q4 FY23, we registered 6.3 million patient visits, generating a total revenue of Rs.491 crore . COVID and allied test declined 83% and contributed to only Rs.11 crore , that is just 2% of the overall revenue.

During the full financial year, we registered 26.9 patient visits and generated a total revenue of Rs.2,017 crore .

You would remember last year, in Q4, we had a pronounced omicron wave of COVID with high testing volumes comparable to the end of delta wave. As a result, test mix profile shifted significantly. Hence for analysis purposes, one must look beyond YoY

growth and use other metrics like sequential and three -year CAGR numbers .
Our non-COVID revenue at Rs.480 crore registered a YoY growth of 14.4% and followed historical sequential trends between Q3 and Q4 .

The test per patients also moved up from 2.48 in Q4 FY22 to 2.76 in Q4 FY23, representing a 11.2% growth in non -COVID test conducted . Underlying non-COVID patient visits in the organic business grew near 8%.

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As you are aware we had taken some price corrections in the month of February '23. Same has got settled and has been absorbed by various segments of our customers. According to our estimate, this price correction has had a 1.7% impact in this quarter and 2.5% on an ongoing basis.

We have been focused on four strategic programs building bundle test portfolio , Swasth fit, super specialty business , enhancing digital capabilities and geographic expansion including serving Tier-3 and 4 cities .

Like Om mentioned , bundle test portfolio continues to do very well and it contributed nearly 22% of Q423 of non -COVID business.

Overall , Swasthfit portfolio generated a revenue of Rs.370 crore in FY23. Super specialty portfolio led by Genomic Testing Division, Genevolve , Center of Excellence for Reproductive Diagnostics, LCoRD and also center for autoimmunity excellence LACE is becoming similar to the Swast fit portfolio in size and growth rate.

Our expansion program aimed at reaching the interiors of the country in Tier-3 plus towns or Bharat, continues unabated. We now have 277 labs at the year end, including 70 labs at Tier-3 towns like Siwan, Murshidabad in East and Sultanpur in the North .

We continue to invest in scalable and profitable digital programs , combining medical excellence and leveraging digital and analytical capabilities to improve patient journey.

I would like to talk about three digital properties that were recently launched and scaled up . Today, namely Chips, recommendation, engine and patient wallet . Our Chips customer health improvement plans is designed to engage our patients digitally with personalized digital diagnostic interventions. Further, we have better recommendation engines that share these specific tests at the point of sales. All these programs combined with the patient wallet will provide personalized , scalable , digital customer relationship management program. We are receiving good customer traction on this program since their launch.

We believe we have the right set of building blocks, which we have put in place in the marketplace with a few more in the pipeline and these we believe will help us deliver industry -leading growth rates.

With that, I would like to invite Ved to take you all to the financial performance . Over to you , Ved .

Ved P. Goel: Thank you, Bharat h. Good evening, everyone and thank you for joining this call. I'm now sharing some of the "Key Financial Highlights " for Q4 & FY23 :

Our non -

COVID revenue for Q4 FY23 came in at Rs.480 crore versus Rs.420 crore last year same quarter , a growth of 14.4%, whereas FY23 non -COVID revenue came in at Rs.1,954 crore versus Rs.1,691 crore last year, a growth of 15.5%. Total revenue for Q4 FY23 came in at Rs.491 crore against Rs.486 crore last year same quarter, a growth of 1.1%. Total revenue for FY23 came in at Rs.2,017 crore versus Rs.2,087 crore in FY22.

Revenue realization per patient for Q4 FY23 is Rs.774 against Rs.728 last year same quarter, an increase of 6% , led by price increase, test mix and higher contribution of Swasth fit.

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Normalized EBITDA after eliminating the impact of RSU , CSR and one time exceptional expenses for Q4 FY23 came in at Rs.131 crore and Rs.528 crore for FY23. Normalized EBITDA margin for Q4 is at 26.6% and 26.2% for FY23 .

Normalized PBT after eliminating the impact of notional depreciation on account of consolidation of suburban financials and one time exceptional expenses for Q4 FY23 came in at Rs.102 crore versus Rs.94 crore in last year same quarter and Rs.400 crore for FY23 versus Rs.494 crore in FY22.

Normalized PBT margin is 20.8% for Q4 FY23 and 19.8% for FY23.

Normalized PAT for Q4 FY23 came in at Rs.76 crore versus Rs.73 crore in Q4 FY22 and Rs.297 crore for FY23 versus Rs.369 crore in FY22. Normalized PAT margin is at 15.5% for Q4 and 14.7% for FY23.

Cash and cash equivalents as on March '23 is Rs.838 crore. Net of borrowing, it is Rs.601 crore.

I'm pleased to share that our efforts toward optimizing cost and increasing the operational efficiency by using the technology and digital tools has resulted in reducing our debtor outstanding and inventory to 28 days now. So, one side, we are doing efforts on front-end patient side as Bharat mentioned that digital or technology has given the edge. Another side on the operational side, we are taking technology and tools to optimize the cost.

Finally, we are pleased to share that the Board of Directors has proposed a final dividend of Rs.6 per share. With this, the total dividend for FY23 is Rs.12 per share.

A final dividend will be paid after the shareholders' approval in the upcoming AGM.

This brings me to the conclusion of my opening remarks, and now I would request the moderator to open the forum for Q&A. Thank you.

Moderator: We will now begin the question-and-answer session. The first question is from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal: Two questions. Firstly, I noticed revenue mix largely shifting to outside Delhi NCR.

So, obviously other geographies are doing better. B2B is a bit higher YoY and Swasthfit is obviously growing faster. All of this combined, Om, the question for you that does this result in margin dilution for fiscal '24 YoY?

Dr. Om P Manchanda: We don't think so. Our margins, if you look at are nearing our pre-COVID level, so the COVID impact is out. So, even in FY24 also, we believe that we should be able to sustain a similar level of gross margin.

Rahul Agarwal: In spite of these three things gaining revenue share, outside Delhi NCR B2B and Swasthfit?

Dr. Om P Manchanda: Yes, you see, outside Delhi NCR, one must realize that as a scale picks up, there are markets which actually mimic like Delhi NCR. And if you notice in the last four to five years, our expansion through lab network is actually not that fast that it used to be earlier. So, because most of it is now building through hub labs where our tendency is now to open more collection points and labs. So, I think we are actually optimizing the lab cost, in such a way that our margins don't get diluted. So, that's the way we see it. And in any case, if you look at the earlier portfolio mix, we always had some markets which are operating on a lower margin, but our rest of north now

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has fairly scaled up, where margins are

pretty healthy there. There is some variation, it's not material enough, so that's the way I would look at it.

Rahul Agarwal: Secondly, on the growth side of it, obviously, starting first quarter of fiscal '24, the base is supported because we no more have very large COVID revenues. Life coming back to normal, obviously that helps in getting normal diseases and hence normal testing for non-COVID should pick up. Employee attrition is generally low now in favor of digital lab what I understand from my channel checks. And obviously growth outside Delhi NCR is faster than Delhi NCR. So, combined of all of this, do you see 13%,-14% revenue growth next year?

Dr. Om P Manchanda: So, I think instead of me putting a number, clearly I do believe that we entered in FY23, a much more difficult outside environment than in FY24. As you outlined it very well, pressure on attrition is very low, our ability to take price increase there we have demonstrated that COVID pressure, should we roll back capacity or not, should we buy some more stocks, etc., I think now that is also behind us. So, the good news is that we are now entering into FY24 with a cleaner slate with the less sort of headwinds compared to what we saw last year. I think overall it augurs well. Now coming to growth rate, it's very difficult for the number right now, but I definitely do believe that we should do better than what we've done in previous year.

Rahul Agarwal: Just a bookkeeping, 26.6 million patients non-COVID per full year, could you break down that into Dr. Lal and Suburban separately for FY23, 26.6 million patients.

Ved P. Goel: So, Rahul, this Suburban was not fully baked-in in this year. So, probably these numbers may not give the right picture. So, at this point I think this is the consolidated number we have –

Om P. Manchanda: For us, if you look at suburban, so far, our tendency was to show separately because in some quarters we didn't have a base. So, now, since that sort of issue is no more with us, we want to operationally see it as if like both parent and suburban they go hand-in-hand together. So, we would not want to actually not highlight suburban separately, that's the way we look at it.

Rahul Agarwal: Can I get it for fourth quarter in that case, that's a YoY comparison, right?

Ved P. Goel: I'll come back to you.

Om P. Manchanda: That number I think immediately not available with us. We'll come back to you.

Moderator: Thank you. The next question is on the line of Prakash Kapadia from Anived Portfolio Managers. Please go ahead.

Prakash Kapadia: A couple of questions from my end. There have been price hikes in the industry by

new age players. Any change in competitive landscape we are witnessing and given that growth rates in industry have generally come down post -COVID , so any acceleration or any trends we are witnessing or sensing from unorganized or shift from regional players for national players like us ? We've also taken some price hikes in our portfolio. So, if you could help us understand the impact of revenues for FY24 and is it in specialty, is it in packages, is it in specific geographies, if you could highlight that? And the third question is the endeavor to drive organic growth, what should be the key drivers for that in FY24 and beyond? So, obviously Suburban scale is one of the factors, but how is non-metro contribution shaping up , how is new patient addition happening , new test menu, so some of these points will be helpful if you can clarify ?

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Bharath Uppiliappan: Hi, Prakash, this is Bharath here. Let me start with the first question on how the competition intensity is shaping up in the industry, right? So, there are two, three ways to look at it. One is that the incumbent players and let's say some of the new traditional players , model players, let us say for sake of any other better word , continue

to intensify be competitive in the marketplace. So, there is no let down on that account. So, including new entrants, regional lab chains, etc ., all continue to compete very aggressively in the marketplace like in the past. What we are seeing is that some of this new age e-Com-based Google advertising, Facebook advertising -based companies because of maybe funding pressure or because they're not able to see the traction themselves are scaling down, let us say deep discounting because patients are also now realizing that the deep discounting are actually a clickbait operation whereas they finally end up paying the money , they pay similar money to what they pay to one of the incumbent players . So, I think it is all catching up and the industry is a bit of a transitional phase , everyone's trying to innovate and try and do the best they can. So , that is the way to look at the competitive landscape today , but yes, the advertising intensity has mildly come down, at least from a (ATL) above the line perspective. Digital media competition still continues very intensely. So, I think that's the way to -

Prakash Kapadia: So, Bharath , effectively, you're saying this would take time and there is no major shift. Maybe the pace has decreased, but it remains as competitive as it was.

Bharath Uppiliappan: Yes, . It is as if the world has come to a new age, new world wherein it's all less competition and the things back. It is still intensely competitive. All of us fighting for a share of the business. But yes, some segments of competition have kind of come down. So, the second question revolves around, just remind me on -

Prakash Kapadia: It was about price hikes, which we have taken. So, is it, for specialty portfolio , is it specific geographies and the impact of that if any on next year and going forward.

Bharath Uppiliappan: So, we have not taken any price increase on routine test , the most commonly ordered tests including packages. So, that is the first statement I'd like to make. We have taken price increase on what is really esoteric test, high end test done in reference lab only and so on and so forth. And I talked about that in the opening speech of 1.7% impact for this quarter and 2.5% estimated going forward, assuming the mix holds up. So, it is not geography -specific , it is pan India basis and really tests which are less frequently ordered , so consumers and patients don't feel the pinch of it. The good news is that all B2B partners somewhere the samples predominantly come from, have largely accepted the price increase. We are not finding any major impact or push back as a result of this , because it's been long overdue, and they also realize that they have also taken up prices. So, I think it has gone well how execution was planned for. And on the third question of growth levers going forward , Dr. Lal covered this, Dr. Om covered this , I also covered this , there are three or four ways to look at it. One is geographic expansion in Tier-3, Tier-4 cities, along with strengthening our metro city presence either in wherever you operate in North east or also including Mumbai. So, there is a geographic expansion plan. Second is differentiated channel management programs. We talked about that I think in the last quarter also on how do we manage B2B separately then B2C . So, a lot of work is going on channel management programs and driving growth through channel partners. The third obviously we spoke about today of high-end testing , either the genomic testing or the centers of excellence on reproductive diagnostics or autoimmunity. Those are all key ways to differentiate your business while delivering value to all stakeholders. The fourth is obviously getting the tech stack up to shape , so that we constantly innovate to provide a wow ! factor to the patients , saying something new has happened . So, we spoke about the chips program today

, we spoke about the recommendation

engine and also the wallet. So, it's a combination of what we have done traditionally,

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which is geographic expansion combining with obviously Swasthfit and super specialty portfolio and layering a tech component to that.

Moderator: The next question is from the line of Bino Pat hiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil I: Just a quick clarification. When you say the price increases that you took will have an impact of 2.5% , is that on your overall revenue or on just the portfolio on which you have taken price increases ?

Bharath Uppiliappan: On the overall revenue .

Bino Pathiparampil: What are your plans for addition of labs in FY 24 and FY '25?

Bharath Uppiliappan: So, our lab expansion program continues the way it has always been , about 10 to 15 labs predominantly in Tier-3, Tier-4 cities spread across northeast and some portions of south and west.

Moderator: The next question is from the line of Lavanya Tottala from UBS Securities (India) . Please go ahead.

Lavanya Tottala : I just wanted to understand the margins for suburban in th is quarter just because last quarter we have seen a meaningful hit ?

Ved P. Goel: So, normalize d EBITDA margins for Suburban for this quarter is about 11.2% and for full year it is 12%.

Lavanya Tottala : So, we are still seeing the impact of the new franchise opened on the margin fr om suburban ?

Dr. Om P Manchanda: I think on Suburban, if you go back pre-COVID times, that was the trajectory of this business in any case. I think during COVID times we experience d very high leverage because Suburban has e xperienced very sharp drop of COVID revenue this year from nearly Rs.113 crore , it has fallen to just Rs.9 crore . The P &L shape is very similar to let's say what it used to be pre-covid time. But the important thing is that this has a platform of nearly Rs.155 crore of revenue , 80% of that comes from city of Mumbai. And now with the sort of support of centralized lab and LPL parent support , we are basically hoping to drive this both revenue synergies as well as the cost side synergies. We realize higher m argins, but this is the margin which we used to have pre-COVID times.

Ved P. Goel: And these margins, please note that we have launched this big reference lab in Mumbai. So, this quarter all the impact of that reference lab has also come.

Lavanya Tottala : One more thing on the exceptional expense that you have seen in the current quarter . What is it related to ? I just wanted to understand the tax rate guidance that you give for next F Y24.

Ved P. Goel: So, this exceptional item is one large amount receivable from BMC , Mumbai Municipal Corporation. As per policy , we have provided , but we are hopeful that this money will come to us very soon. And second , a small amount which has been provided on account of COVID inventory, which is going to expire. So, these are the exception item s.

Lavanya Tottala : On the tax rate?

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Ved P. Goel: We have a normal tax rate which is about 25% for the corporate. There is nothing which is exceptional here.

Moderator: The next question is on the line of Prakash Agarwal from Axis Capital. Please go ahead.

Prakash Agarwal: First question is just trying to understand the price increase . You mentioned it is for specialized test. How has been the experience versus the competition , has the competition followed you or have you lost some business because of price hike or it is status quo and it is value -accretive ?

Bharath Uppiliappan: Two, three things . One is that this price increase has gone relatively well. We were skeptical in the b eginning, but I think execution was near perfect and communicating to customers on why this price increase and in the context of the overall tenure of the price increase, this seem to be logical and reasonable to them. Number two , w

e

explain ed the impact. That's about 2.5% on an ongoing basis on the overall portfolio given the mix of the portfolio on w hich this price increase has been taken. And we also have not taken price increase on the routine portfolio o r the most commonly ordered test portfolio or the health packages or the bundle test packages. Now, with competition follow, each company will follow its own policies, procedures, priorities, etc., But given the inflationary situation, the lowering of COVID volumes, I do expect that everyone will take pr ice increase as per their areas of strength and what they think they should be doing . So, yes, the industry price table should start to move up.

Prakash Agarwal: And the price increase would be only for your specialized, which is about 30 %- 35%

or lower in terms of total volume , value?

Bharath Uppiliappan: So, it's on value basis, it is about nearly 50% of the portfolios on which we have taken.

Prakash Agarwal: 50% you claim is on your specialized testing menu ?

Bharath Uppiliappan: It is not only specialize d, specialized plus I said less frequently ordered tests and so on.

Prakash Agarwal: And the second question was trying to understand the competitive environment and the B 2B segment. Now , we hear that a lot of new players are getting very aggressive especia lly on the B2B side. And your progress on the South, please?

Bharath Uppiliappan: On the B2B side, my response is that , yes, it is as competitive as ever. We have actually put in place multiple programs to better manage this channel and segment this chann el like we do for patients. So, we have a B 2B channel program, which is very intense and very well appreciated . And in fact, B2B is one of our fast growing channels today as well compared to historical averages. So , we're very pleased with the progress of this whole approach of channel management program on the overall revenue growth rates. On South also, our progress continues ; we grew South at a reasonable pace compared to the company growth rate , we grew ahead of the company growth rate significantly. So, I think , yes, we'll continue to build deeper penetration in the South and the geographies that we currently operate in.

Prakash Agarwal: So, B2B you would have grown is what you're saying ? You've done very well.

Bharath Uppiliappan: Good growth rates on B2B.

Prakash Agarwal: And margin accretion or flattish?

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Bharath Uppiliappan: So, if you manage the mix well and the cost structure well and there is also scope to negotiate the back -end cost, so I think it plays out well enough. Also, as the volume grows a s testing efficiencies come in, we're able to amortize lot of control, calibration cost, etc.,

Moderator: The next question is from the line of An eesh Deora from Nomura Financial Advisory and Securities . Please go ahead .

Aneesh Deora : Sir, I'm looking at the quarter -on-quarter sequential data of 4Q over 3Q. So, while the realization per patient has improved and even the number of tests per patients and the number of samples process have shown an upt ick, but if you see the number of patients , that is the patient volume, they have shown a decline over 3Q which was supposed to be a seasonally weaker quarter over a strong 4Q. So, can you share your thoughts around this decline and is it pointing out any particular trend that you're seeing that w e have a bearing in the coming periods?

Bharath Uppiliappan: So, on patient visits, I don't think we should get swung by quarter -on-quarter movements. There's lot of base issue last quarter, this quarter, last year, this year , etc., Like I mentioned in the opening comments , our underlying patient visit growth rate is near 8%, number of tests performed is at 11 .2% and revenue is obviously higher.

Moderator: The next question is on the line of Nitin Agarwal from DAM Capital Advisors . Please go ahead .

Nitin Ag arwal: Two que

stions. One is on the operational bit. We've seen some reduction in employee cost on a YoY and QoQ basis. Likewise, if I adjust for the one -off that you mentioned about Rs.7-odd crore , other expenses are down . So, what would we attribute to these reductions given the fact that business has been growing ?

Bharath Uppiliappan: So, while Ved can give you specific s at an overall level, Ved in this opening comment talked about use of technology in automating a lot of processes, etc. , So, that has given us a large headcount , what I would call leverage . But specifics around the rest of the equation , Ved, you want to take on?

Ved P. Goel: Nitin, so this is primarily because of RSU charge, which is lower than last year and that is the precise reason for compensation or employee cost. So, this is one . Other expenses obviously we had a lot of programs done in last year where efficiency has been created , as in my opening remarks also I've said lot of use of technology tools we have created and duplication of the repetition of the work which can be used by technology tools. So, those have given us some advantage and that's why we have got some advantage on other expenses.

Nitin Agarwal: And secondly, since we have the full year numbers and you have some more clarity on the post -COVID trends, what is a realistic ballpark range that one should work with in terms of the patient testing volume growth for us ?

Ved P. Goel: Volume means patient you are saying?

Nitin Agarwal: Yes, patie nt volume going forward, I mean on a directional basis, it's a late single digit, early double digit, mid double digit , I mean, what is the realistic number given the industry dynamics we can work with ?

Dr. Om P Manchanda: See, it's very difficult to put a figure, but I think the first question was asked is that we do believe that FY24 numbers should really be better than last year figures. I think all sort of variables are pointing towards that. We will not have that sort of

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competitive scenario as we experienced last year. And as you mentioned that since COVID is not there, actually in one-way, the good news is that we're done with it in one year itself, otherwise it would have been something which is base effect and now we can completely focus on non-COVID business. I think the only thing we are now carrying with us is I think Rs. 63 crore of COVID and allied business in our base. We don't know, it's highly unpredictable, it can actually be as good as zero as well, it could be same. But I think if I take this out, we definitely are hopeful of improving our performance in FY24.

Nitin Agarwal: Typically, from a seasonal perspective, is Q4 expect supposedly a better quarter for the diagnostic business than Q3?

Bharath Uppiliappan: No, no, I mentioned in the opening speech saying that sequentially Q3 and Q4 are similar for us.

Dr. Om P Manchanda: I think we've also made this comment in the past, but I think for the last two, three years, I'm seeing that these variations come due to infections. And for some reason I find that even though following the pattern because Q3 is no more a healthy season anymore, even in Q3, also we are having lot of infections. So, in some way Q3 actually has moved up while Q4 may have been similar. So, that's why the difference you see is much less than before. Let's see how it happens this year, because this COVID thing has always been there, right? In fact, this whole respiratory infection in general has been there in the population. I think let's just wait out for FY24, but you're right, Q3 is generally a softer quarter, but we have not seen that trend in the last couple of years.

Nitin Agarwal: Since you mentioned the point of infections we've seen very high intensity of incidence rather of infections. The whole of FY23 which is reflected in I guess the pharmaceutical sales of anti-infectives and all the c

old and cough preparations, I

mean in your assessment any tailwind for our business also?

Dr. Om P Manchanda: Actually, respiratory infections in general do not really directly lead to very high jump in testing. In fact, during this month of April and even in March also, we were talking about COVID, etc., We didn't see direct sort of jump in COVID testing. But I think one phase which normally tends to go up in situation like this is CRP which is inflammatory marker. So, that is one test we have seen a very significant jump in Q4, right? So, CRP is one test we saw a big jump.

Nitin Agarwal: And lastly, on the realization per patient, obviously with the bundling that we are doing, that's sort of going up. We are about 780 or there about for this quarter. I mean how does one look at this number, is there a significant scope for appreciation from these levels on this number?

Dr. Om P Manchanda: I don't think so. I think there are a few things that have gone into the base - I is of course bundle packages now contribution going up to 2.2%. I don't foresee a very sharp jump again in the contribution. I think it should stabilize around this number.

Suburban in general has a higher realization than Dr. Lal PathLabs. So, that also has gone in for the full year impact. I think whatever little will come now is due to price increase that we've taken in the month of February. Maybe that will have some marginal impact. Mix may not change. My gut says that we may not see a very sharp jump on this in FY24.

Moderator: The next question is from the line of Cyndrella Carvalho from JM Financial Institutional Securities. Please go ahead.

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Cyndrella Carvalho: So, if we are looking at the volume growth, you in your opening remark quite interestingly highlighted the Mumbai lab. We know our earlier labs whenever they have come, they have given us a good traction. So, what do you think about the Mumbai lab, how it will contribute to our overall patient growth, like, how should we relate this, if you can help us understand this?

Shankha Banerjee: So, the Mumbai lab has been launched in January. The thing is it could be a slightly longer wavelength compared to the other reference labs because from the Suburban point of view, it's kind of starting a completely new vertical so to say in terms of the specialized business, and also trying to get a completely new customer client profile to be built under the portfolio that we are currently doing. So, from that point of view, yes, from the last few quarters, it will definitely have better impact in terms of patient growth, but it may not be exactly comparable in terms of the things which we saw in East Calcutta or Bengaluru, it might take a slightly longer wavelength.

Cyndrella Carvalho: In your opening remarks, you also mentioned about a slightly higher cost base that

we carried for the entire FY23 . So, now, if we look at our cost base versus the growth that we have seen, at least the normalization that we have seen in F Y23, do you think the upcoming year F Y24, '25 this cost base will support us on the growth and you should see some operating leverage also ?

Ved P. Goel: That's the idea , because one is launching this reference lab, which will obviously give advantage and what Shank ha is saying on B2B side . Because all the infrastructure is in place and I think you also have visited that lab. Now we have to leverage this infra . In our view , the efforts is more on top line but similarly the operating leverage will also come on the bottom line. So , I believe that margins from here should improve.

Cyndrella Carvalho: If we try and understand the overall marketing scenario today, you highlighted that there is some sluggishness , there is intensity continues, we have taken specialized price increases . But the specialized price increase should reflect across the entire year, right, I mean

n, we just started in month of Feb, is that understanding correct?

Bharath Uppiliappan: Yes, indeed.

Moderator: The next question is from the line of Tanmay Gandhi from Investec Capital Services . Please go ahead.

Tanmay Gandhi: Sir, first question is on sequential growth, right . If we historically look at our sequential growth in 4Q , it had changed between 3 % to 5%, right , but this time it was flat. So , anything worth highlighting the re?

Bharath Uppiliappan: We couldn't hear your question properly. Could you repeat yourself if you don't mind ?

Tanmay Gandhi: So, my question is that, historically if we look at our sequential growth for 4Q, it has ranged between 3 % to 5%, right , but this year it is flat. So, anything particular would like to highlight here ?

Om P. Manchanda: That's what I actually mentioned. Your observation is right . It's not 3 % to 5% , normally it's about 2% , and the 2% actually is neither here nor there , so sometimes depending on the infection rate is flat . But I'm actually seeing this trend for the last 2 years , even I think if I remember in the previous year also we saw the same thing. So, my hypothesis is that now it's just evened out in terms of infection rates. Earlier Q3 was seen as a healthy quarter. I think now because of air pollution, etc ., all these things, Swasthfit which is more screening packages , it's become a little bit of even. So, looks like now the pattern that I'm seeing is the Q2 is the highest, all other three quarters are very similar, Q 1, Q3 and Q4. That's the observation that I have for the last couple of years. Let's see in FY24 what happens because at least we will not have this whole variable of COVID , at least it will be more an illness space. But your observation is right , this time we are not seeing a sequential jump which we normally see. This is not the first time I was noticing, even last year also , I did notice that.

Tanmay Gandhi: We understand that year -on-year growth for this year is not that relevant because of volatile base . But sir, if we try to extrapolate our sequential growth, which we have seen during the last four quarters to F Y24, then it roughly translates to a mid to high single digit growth. So , is that the right way to look at it ?

Dr. Om P Manchanda: No, no, I don't think . We have done 15 .5% in non-COVID, but it also has four quarters of Suburban compared to two quarters previous year, right? So, the normalized growth will be how much for this year?

Ved P. Goel: 10%-11%.

Dr. Om P Manchanda: Yes, we definitely hope to have better performance this year. It won't be single digit.

Tanmay Gandhi: On Swasthfit, so we have been saying that largely it has peaked out and you don't see any sharp jump from this level, right? And historically, our revenue growth was largely driven by our patient and sample growth . But this year it was also driven by higher realization per patient. Going forward , as we expect some moderation in Swasthfit expansion , do you see again our revenue growth would be driven by volume and volume growth has actually come down , right?

Bharath Uppiliappan: So, yes, our efforts will always be to drive the patient visit growth , and there are various techniques we are using to get better on that count . But as a result of bundling , the test per patient is also moving up significantly , about 11 %-odd in Q4 . And that trend, Swasthfit further grows, it will reflect in the numbers . But our intrinsic effort is to acquire new customers and service our existing customers in a better fashion.

Tanmay Gandhi: A related question , in Q4, did we have any one -off kind of growth in in tax -related packages for Swasthfit ?

Bharath Uppiliappan: In the last two, three years I think in Q4 Swasthfit picks up because of this tax advantage. It's a normal scenario. It's in the base as well. But this year growth has been

still better than the previous year led by better distribution, better understanding from patients, acceptance, etc., So, it is more than tax.

Moderator: The next question is from the line of Yogesh Tiwari from Arihant Capital Markets. Please go ahead.

Yogesh Tiwari: So, my first question is post-COVID, what would be the growth of the in-vitro diagnostic industry?

Om P. Manchanda: Normally, IVD industry, we refer to the reagent suppliers. I presume you're asking for pathology, people like us, right?

Yogesh Tiwari: Yes.

Om P. Manchanda: So, as I mentioned to you the data, which is available in public domain, I just added up at least four, five companies data. In the last four years CAGR I have removed this impact of COVID years which is FY21 and FY22. The growth rate that I'm seeing
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is about 10-11% CAGR over a three year or four-year period. I personally feel that it should sustain. One variable which probably is going to get added because in the last four years CAGR may not have very high pricing impact. But since our ability to take price increase is now coming back, let's see how other players do, but my reading is it will definitely be in this way going forward.

Yogesh Tiwari: And sir, what would be the total size of the industry if you can share your thoughts?

Om P. Manchanda: There is no published data available. Normally, we refer that large players contribute about 15% of the value. So, I think one crude method would be to divide the Rs.

5,000 crore by 0.15 and that gives Rs. 30,000-35,000-40,000 crore, which is essentially the private lab business and then you add this to our hospital lab business, which is there. So, my sense is it should go up to Rs. 60,000 crore, something like that. That's one of the ways to look at it.

Yogesh Tiwari: Which you mentioned, this is for the Indian domestic, right?

Om P. Manchanda: Yes, India domestic market.

Moderator: The next question is from the line of Praveen Kumar from Acuitas Capital Advisors. Please go ahead.

Praveen Kumar: I had a couple of questions. The first question was on a comment that Dr. Om had made in his opening remarks. He had mentioned that bundled packages have become a way of life. So, just thinking about the comment a little bit more, see, in terms of bundle packages, the decision-making point could be perceived to shift to the patient instead of the doctor. And since the Doctor Connect is a major complete advantage for Dr. Lal, how do you perceive this panning out in terms of your competitive advantage?

Dr. Om P. Manchanda: I think that's a great sort of a question. My reading is if it is only limited to preventive health checkup, yes, decision-making is with consumer or the patient. Since lot of chronic disease patients also are shifting to bundle pack... they actually upgrade to these packages you are going to see better value for money. So, even illness patients are also in bundle packages. So, the doctor's decision-making is still there. Only thing is that the doctors also have started prescribing now some of these packages as a part of the normal prescription.

Praveen Kumar: I had a second question, which was more on the investor presentation, the financials in particular. What I wanted to understand was the company declares normalized EBITDA margins where few items are excluded, one of which is the employee stock compensation or RSU based compensation. So, I just wanted to understand the rationale for excluding this. Is it that you perceive these to be more one-off kind of expenses because if not, if you perceive that these are more ongoing nature of compensation-based practices, then in one way or the other, the companies bear in the cost for that, right. So, shouldn't it be part of the overall EBITDA itself, not a specific normalized EBITDA?

Ved P. Goel: So, Praveen, there are two reasons. One is of course this

is non-cash item, it is not

really where the cash outflow from the Company. And second, this charge varies depending on the price of the share because if we are granting ESOP at a time where the price differentiates, so probably this fluctuates the charge. So, these are the reasons why we are showing separately.

Praveen Kumar: But just to understand that, if I look at your past annual reports and other financial disclosures, this cost has varied between one to one and a half percent kind of a
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number, right? So, just wanted to understand that it seems to vary in that range. So, shouldn't that be part of this non-normalized EBITDA?

Dr. Om P. Manchanda: You have both the figures in any case; you have a normalized and if you see EBITDA figures also you can see that. But you're right, there is a baseline number which will stay here, which could be, I think one, one and a half percent is really a good assumption.

Moderator: The next question is in the line of Zaid Munshi from Concept Invest well. Please go ahead.

Zaid Munshi : My question is related to the hospital base. Do we have any plans to expand in partnering with the hospitals for their diagnostic part , as we don't disclose the numbers how many of hospitals we operate ?

Bharath Uppiliappan: So, we have said in the past that we continue to look at HLM as and when we find a very strategic or a geographic fit available to us, because expanding into a hospital lab management like a normal retail business can lead to a lot of other issues like outstanding debtors and so on. So, it is best to do it with lot of consideration rather than do it on a mass scale. So, we are selective about the HLMs we enter into contracts with, but yes, we do HLM.

Zaid Munshi: So, currently main focus is on independent labs and not on hospitals?

Bharath Uppiliappan: Yes, yes. So, I think that is the main focus. Once in a while we get an opportunity, we'll do HLM, no problem at all.

Moderator: The next question is from the line of Sayantan Maji from Credit Suisse. Please go ahead.

Sayantan Maji: So, my first question is on home sample collection. So, I just want to check what are the current trends that you're seeing after normalization in post-COVID outbreak, do you see patients coming back to your centers and a percentage of revenue from home sample collection, has it come down and currently how much percentage of B2C revenues are coming from home sample collection? In case this is something which is sustaining even after COVID, then what are the steps that we're taking to be competitive especially versus the integrated digital health platforms?

Bharath Uppiliappan: So, two, three points. One is that during COVID I think the predominant mode of operation itself was home collection. So, the number was very, very high; I think close to 20%-odd those days. But pre-COVID this number of home collection was about 5%. Today, I think what we are seeing is let us say about 8%, 9% of home collection, which is a better trend than what we had in the pre-COVID days, but significantly down obviously from the COVID days. So, that is one way to look at the numbers. Second is, the underlying thing is about the patients choosing a lab of trust. It is not about convenience. Convenience is one of the factors involved in it, but they want to look at labs, etc., and we continue to enjoy a very favored position on that count. So, we are seeing our home collection revenues in line with this thought of us.

Sayantan Maji: This 8% to 9% is of B2C or of the total revenue?

Bharath Uppiliappan: Of I think the total revenues here.

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Sayantan Maji: In that you have mentioned of variable model. Can you just explain it a little bit like what is this variable model which grows with business volume?

Bharath Uppiliappan: When did we speak out variable model.

Sayantan Maji:

So, in slide number 25 of the investor deck, it says that in the home collection, it says that it's a variable model which grows with business.

Ved P. Goel: This is one way of hiring people, which is fixed pay and another way is you pay as per pickup. So, it is not a fixed charge, like our own center versus franchisee center, it is similar to that like, we instead of hiring people giving fixed contract, it is per pickup base the payment.

Sayantan Maji: Basically, the phlebotomist is entirely dependent on the number of samples he or she picks up in a day?

Ved P. Goel: Right.

Sayantan Maji: And the second question is a bookkeeping one. So, now that FY23 is over, what was the total Ind AS impact on EBITDA or Ind AS benefit to the EBITDA?

Ved P. Goel: So, it is about two and a half percent roughly which is on account of Ind AS.

Moderator: Ladies and gentlemen, that would be our last question for today. I now hand the conference back to the management for the closing remarks. Thank you and over to you.

Ved P. Goel: Thank you, everyone for being with us on this call today. I hope we were able to address your queries. If you have any more questions or queries, please feel free to reach out to us or our Investor Relations team, CDR India and we will be happy to clarify your thoughts. Thank you once again. I would now request the moderator to close the call.

Moderator: Ladies and gentlemen, on behalf of Dr. Lal PathLabs, that concludes this conference.

Thank you all for joining us and you may now disconnect your lines.

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Call: Aug 2023

August 02 , 2023

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q 1 FY24

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q 1 FY24 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 15:

Dr. Lal PathLabs Limited
Q1 FY24 Earnings Conference Call Transcript
July 27, 2023

Call Duration • 1 hour

Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman

- Dr. Om Prakash Manchanda - Managing Director
- Mr. Bharath Uppiliappan - Chief Executive Officer
- Mr. Ved Prakash Goel – Group Chief Financial Officer
- Mr. Shankha Banerjee – CEO – Group/ Associate Companies

Participants who asked

questions • Rahul Agarwal – InCred Capital

- Aneesh Deora - Nomura Financial Advisory and Securities
- Tanmay Gandhi - Investec Capital Services
- Mehul Sheth - Axis Capital
- Cyndrella Carvalho - JM Financial Institutional Securities
- Aashita Jain – Nuvama Securities
- Vignesh Iyer - Sequent Investments
- Omkar Kamtekar - Bonanza Portfolio
- Karan Vora - Goldman Sachs
- Dheeresh Pathak - White Oak Capital

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Moderator : Ladies and gentlemen , good day, and welcome to the Q1 FY 24 Earnings Conference Call of Dr. Lal PathLabs.

As a reminder all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call please signal the operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India.

Nishid Solanki : Good evening, everyone and welcome to Dr. Lal PathLabs ' Q1 FY24 Earnings Conference Call.

Today , we are joined by senior members of the Management Team including (Hony) Brig. Dr. Arvind Lal - Executive Chairman , Dr. Om Prakash Manchanda - Managing Director , Mr. Bharath Uppiliappan – CEO , Mr. Ved Prakash Goel - Group CFO along with Mr. Shankha Banerjee – CEO of Suburban and other Group Companies.

I would like to share our standard disclaimer here. Some of the statements made on today's call would be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the Results presentation which has been circulated earlier and also available on the stock exchange websites.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives.

(Hony) Brig. Dr. Arvind Lal : A very good evening today everyone. I welcome you all to Dr. Lal PathLabs ' Q1 FY 24 Earnings Conference Call. I would commence by shedding some light on the evolving market situation and the progress that we have made.

India's diagnostic industry remains vibrant presenting abundant opportunities in the future. In contrast to other nations India's healthcare expenditure relative to its GDP is reasonably low. However, a shift towards evidence -based medicine has come of age after the pandemic. The industry is undergoing a transformation from unorganized to organized more so since the onset of the pandemic. This positive shift continues and is attributed to multiple reasons including higher awareness and increasing preference of customers for branded players it offers superior quality and efficiency.

I am glad to share with you all that International Newsweek magazine has recognized Dr. Lal PathLabs as the 15th of the 100th Global Most Loved Workplaces 2023 across the world. There are only two other Indian companies on the list which were ranked much below us.

As a leader in the diagnostic industry, we have consistently thrived to deliver superior patient experience by expanding our test menu and penetrating deeper in to our core markets as well as foraying into newer markets. The synergy between Suburban Diagnostics and Dr. Lal PathLabs' brand has further enhanced , helping us garner incremental shares in our high potential Western market. We are constantly focused on expanding our presence to reach a larger number of customers , especially in the Tier-III and Tier-IV cities . While doing so we remain committed to continuously improving our testing capabilities, quality, and patient experience.

We have implemented Total Lab Automation or TLA which is supplemented with digital intelligence. We have also reduced the requirement of aliquots or what are sub-samples by 93% by using inbuilt advanced sorting logic and aliquoting

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algorithms which resulted in saving of more than 1.3 million aliquots and more than 5,200 tons of plastic waste in our National Reference Lab at Rohini, New Delhi. This underlines Dr. Lal PathLabs' strong commitment towards sustainability and creating a greener environment.

Our investments in strengthening the digital infrastructure has started yielding positive results as anticipated . This will help us in consistently meeting patient expectations on several parameters while

driving volume growth. Given our strong operating model and brand affinity in both B2C and B2B segments , I see the company continue to gain significant market share.

Thank you very much . I would now like to hand over the floor to Dr. Om Manchanda. Over to you, Om.

Dr. Om P. Manchanda : Thank you Dr. Lal. Welcome to Dr. Lal PathLabs' Q1 FY 24 earnings call. I shall share the broad industry trends.

The first one is at an overall industry level, the contribution from COVID testing has further declined. We are now mainly left with COVID allied testing and this sub -group within COVID sales has a very strong overlap with non -COVID testing. Therefore, tracking and reporting sales with and without COVID separately does not make sense anymore. Therefore, we have also removed this chart from the Investor Deck where we were showing these numbers separately. The good news is that in some

ways this reduces the complexity and helps us analyze business trends better. Second point is the Suburban Diagnostic now enters into Phase II of integration where we have started looking at common lab infrastructure between the two brands that is Dr. Lal PathLabs' and Suburban.

Swasthfit continues to ride the momentum of preventive health checkup sales. The contribution has further gone up in this quarter. That is showing a strong positive impact on test per patient as well as higher realization per patient. The added advantage of this increase in contribution is also seen in simplified operations. Widening our footprint both in terms of depth that is going deeper into Tier-II, Tier-III and Tier-IV towns as well as in terms of width which is both through inorganic as well as organic will continue to be the central theme of growth strategy of Dr. Lal PathLabs.

With that, now I handover to Bharath for further update on Q1 FY24 results. Over to you, Bharath.

Bharath Uppiliappan : A very good evening to everyone present here. I warmly welcome you all to Q1 FY 24 Earnings Conference Call.

Operating and Business Highlights of the Company

In Q1 FY 24, we recorded a revenue of Rs. 541 crore which is a growth of 7.6% over last year Q1, and the non-COVID revenue grew by 9.7% YoY. The COVID RT-PCR testing revenue declined by 81% in Q1 FY 24 to Rs. 2.3 crore from Rs. 12 crore in Q1 FY 23.

Revenue per patient for Q1 FY 24 was Rs. 789, this was sequentially higher versus the preceding quarter by 1.9% led by high contribution of Swasthfit, Delhi NCR region and full quarter flow through of price adjustments.

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In Q1 FY 24, we recorded 6.9 million patient visits, which is flattish at an overall level led by an unusually high base in Q1 last year. However, net of COVID-related testing we grew patient volumes sequentially by 8%. Also, on a four-year CAGR basis, the patient volume growth for Q1 FY 24 stood around 7% for the organic business. We also recorded a 6.1% growth in samples, which stood at 19.3 million in Q1 FY24. The tests per patient also moved up significantly from 2.6 in Q1 FY 23 to 2.8 in Q1 FY24, representing a 7.0% growth.

Update on Strategic Initiatives

The first one is on increase in marketing efforts. In this quarter, on the organic business side, we launched a new marketing campaign "Bharat Ka Vishwas" with quantifiable reasons to believe why millions of patients and doctors trust Dr. Lal PathLabs. We believe that this campaign over a longer period of time will help pivot the conversation to the underliners of this category and not just on pricing and discounting.

This combined with our digital led initiatives like chips, recommendation engines, supported with on-ground activation programs are driving growth in a strong hold markets of North and East. Delhi NCR region also recorded strong growth rates in Q1 FY 24. In the city of Mumbai and Pune, under Suburban, we have launched a strong direct-to-consumer program supported by Thematic above-the-line campaign.

Second strategic initiative revolves around our portfolio. Our bundled test portfolio Swasthfit continues to perform well and I am happy to share with you that we have registered the highest ever quarterly revenue from Swasthfit of Rs. 112 crore in this quarter with a contribution of 22% to the total revenue excluding Suburban.

Our Super Specialty business continues to do well with strong growth rates supported by new initiatives. We are also creating variants of common tests like Lipid Profile and making it medically more relevant to patient sub-segments like diabetics and heart related ailments. Higher Delhi NCR contribution and above average growth rate in Swasthfit and Super Specialty coupled with reinvigorating our routine portfolio have positively contributed to revenue per patient and margin profile for the quarter.

Number three, strategic initiative revolves around geographical expansions. As stated in our previous call's strategic expansions in Tier-III and Tier-IV cities of the country continues as per plan. In addition to the three labs, we opened new labs in the preceding quarter at Siwan, Sultanpur, and Murshidabad, we have commenced operations at four of our Greenfield labs in Jind in Haryana, Una in Himachal Pradesh, Tamluk, and Hooghly in West Bengal. These continue the right set of building blocks coupled with the Pan India presence and the digital infrastructure that will help us deliver industry-leading growth rates for a long period of time.

With that, I would like to invite Ved to take you all through the financial performance. Over to you, Ved.

Ved P. Goel : Hello everyone and thank you for joining this call. I trust each of you and your family

are safe and healthy.

Key Financial Highlight for the Quarter

Our non-COVID revenue for Q1 FY 24 came in at Rs. 528 crore versus Rs. 482 crore last year same quarter, a growth of 9.7%. Total revenue for Q1 FY 24 came in at Rs. 541 crore against Rs. 503 crore last year same quarter, a growth of 7.6%.

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Revenue realization per patient for Q1 FY 24 is Rs. 789 as against Rs. 727 last year same quarter, an increase of 8.6% led by price increase, test mix and higher contribution of Swasthfit.

Normalized EBITDA after eliminating the impact of RSU and CSR for Q1 FY 24 came in at Rs. 154 crore as compared to Rs. 125 crore in Q1 FY 23. Normalized EBITDA margin for Q1 FY 24 is 28.4%.

PBT for Q1 FY 24 came in at Rs. 117 crore versus Rs. 81 crore in last year same quarter. PBT margin is 21.7% for Q1 FY 24. PAT for Q1 FY 24 came in at Rs. 84 crore versus Rs. 58 crore in Q1 FY 23. The PAT margin is at 15.4% for Q1 FY 24. Cash and cash equivalent as on June 30th is Rs. 877 crore, net of borrowing it is Rs. 731 crore.

I am pleased to share that our continuous efforts towards optimizing material costs and improving operational efficiency has enabled us to invest more into A&P & Technology.

At last, we are pleased to share with you that the Board of Directors of the Company have approved an interim dividend of Rs. 6 per equity share.

This brings me to the conclusion of my opening remarks and now I would request the moderator to open the forum for Q&A. Thank you.

Moderator: The first question is from the line of Rahul Agarwal from Incred Capital. Please go ahead.

Rahul Agarwal: Firstly just your thoughts Dr. Om on the quarter, 10% revenue growth I think both YoY and four-year CAGR look similar. So, what went well and what did not versus what you expected?

Dr. Om P. Manchanda: So, let me first talk about what did not do well rather than what we did well, what did well. One is I think we would have loved to see a slightly higher volume growth, that's something we felt short. But I think what did well on the number of samples growth or test growth, because test per patient has gone up very sharply.

Second is contribution

from Swasthfit continues to do well and I think this is a very strategic sort of initiative from our side, we believe that trust is very important in diagnostics and more and more people do health checkups and as healthcare awareness goes up, this segment will continue to show growth and that's a very big positive sign. I think COVID part in any case was declining and we are happy that it's almost going away. Life becomes simple to analyze the trend so that I think is a positive part.

On the financial side I am sure you would have seen that our margins are much better than what we expected. So, I think these are all your takeaways as far as results are concerned.

Rahul Agarwal: Got it, yes because I was thinking 10%, I thought we should do better in Q1, not because it was a very strong season for us but just purely because Q-o-Q we were seeing improvements into non-COVID so it looks like last quarter versus this quarter there has hardly been any improvement on non-COVID that's what it feels like, is that correct understanding.

Dr. Om P. Manchanda: So, I personally think I have repeatedly been saying that these are gradual improvements that happen in our space. So, certainly, overnight jump one should

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not expect. But I think the good news is trajectory is moving upward because there was a time when we were a little concerned about hyper-competition and whether we can sustain this growth, but I think overall now signs are very positive as we go forward.

Rahul Agarwal: And secondly on the operating expenditure, looks like that's not grown in proportion to business growth which is good news and hence margins are up. I also understand that generally the first half of Dr. Lal has higher margins and then through the year it comes down second half. But any specific efforts you would like to highlight on cost savings with some details?

Ved Prakash Goel: Yes, so Rahul there are two major initiatives one is on the material cost side which obviously there is an impact of mix and portfolio change. And second is obviously we are looking at how to optimize this cost because this is the biggest cost in our P&L. So, this helped us in this quarter. And the second is last year in the same quarter we had some provision against debtors which is not there in this quarter. So,

that is another reason .

Rahul Agarwal : What was the quantum for that provision?

Ved Prakash Goel : It was about Rs. 5 crore , this was the BMC provision we had made against recovery for Suburban.

Rahul Agarwal : And lastly on Suburban , how did that do during the quarter , some color on whatever you could share net of IndAS revenue, patient volumes, our revenue per patient , and overall business , you are expanding into South Bombay, what's really happening on Suburban ?

Dr. Om P. Manchanda : So, I think Suburban what we are doing now is seeing what kind of synergies we can draw between the two companies and two brands. So, we are looking very carefully at some synergies that can come out of the lab infrastructure that exists between the two companies. So, I think that's the update we have.

Secondly in terms of marketing activities , we are looking more aggressive than before in Mumbai. We are trying to see how we can improve our B2C contribution in Mumbai market.

Rahul Agarwal : Any topline number and volume number you could share please.

Dr. Om P. Manchanda : Yes, I think right now I would say volume growth is slightly muted there, but we will probably share as we go along.

Moderator : Thank you. The next question is from the line of Aneesh Deora from Nomura Financial Advisory and Securities . Please go ahead.

Aneesh Deora : Firstly, on the margin , so it was 27% for this quarter , the EBITDA margin which is higher than the full year guidance of 25%. So, does this guidance level still hold and how should we think about margins for the coming

quarters?

Ved Prakash Goel : So, this is Ved , again as Rahul mentioned previously , generally we have first half where margins are higher than second half, so we must wait, but we are holding that statement that we should maintain our pre -COVID margins going forward.

Aneesh Deora : And what is the reason for the margins being higher in the first half of the year for Dr. Lal?

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Dr. Om P. Manchanda : So, the first half usually our revenue if you look at the trends historically, they are always higher. And we have also seen the mix of tests also drifted more toward certain kind of tests where gross margins are higher.

Aneesh Deora : And secondly , we have implemented the price increases which is being adequately reflected in the revenues per patient which have inched up quarter -on-quarter , but when we look at the revenue per test, that has largely remained flat. So, how do you think about this and what are the dynamics around it, like one would ideally expect that the revenue per test would also inch up proportionately , but that has not happened so any thoughts around this.

Bharath Uppiliappan : So, revenue per patient sequentially there was a positive impact of about 1.2% led by price flow. Revenue per test is a mix of multiple things I am not sure how I should respond , but give me some time if it comes up later in the queue I will

Dr. Om P. Manchanda : I will probably add to this. See the revenue per test will continue to be lower because as a contribution of Swasthfit keeps moving up, because Swasthfit has lot of tests in it you will always see that situation , but I think we have to look at economics from an overall standpoint because we don't mind revenue of Swasthfit going up because the marginal cost on additional test is very low. But we can pass on that benefit through bundle packages to the patient, in that case, you will always see not only for us I think the industry itself is seeing that the revenue per test is going down , but revenue per patient is going up.

Aneesh Deora :: And just a clarification you mentioned that four-year CAGR for the organic business was 7%, right if I got that correctly, just want to clarify.

Ved Prakash Goel : Yes, there was patient growth.

Dr. Om P. Manchanda : So, volume we were talking about.

Moderator : Thank you. The next question is from the line of Tanmay Gandhi from Investec. Please go ahead.

Tanmay Gandhi : My question is again on the operating expenses so if we look at the sequential growth, so revenues have grown by almost 10%, but still our other expenditures have degrown by 5%. So, is there any one-off in the last quarter base or is there any postponement of expenses which is there in this quarter.

Ved Prakash Goel : No, so Tanmay as I mentioned there was one provision, we took in the last year that was the only one item. And there is no postponement of the expenses in this quarter.

Tanmay Gandhi : Last year you mean Q4 FY23?

Dr. Om P. Manchanda : Q1 of last year.

Tanmay Gandhi : No, I am talking about sequentially, so even sequentially it has declined by 5%.

Ved Prakash Goel : Sequentially also.

Tanmay Gandhi : So, provision was there in Q1 as well as Q4, is that right?

Ved Prakash Goel : Yes in Q4 .

Tanmay Gandhi : And can you share margins for Suburban, have they also improved?

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Dr. Om P. Manchanda : Margin for Suburban also has improved , if you do just non -COVID versus non - COVID, right Yes margins have improved there also .

Tanmay Gandhi : Can you share the numbers?

Dr. Om P. Manchanda : I think the range is between about 11% to 13%, exact numbers we don't have , but it's in that range about 11% to 13%.

Moderator : Thank you. The next question is from the line of Mehul Sheth from Axis Capital.

Please go ahead.

Mehul Sheth : First question is around the gross margin so does your gross margin also had some benefit of that price increa

se that you have taken in few of your tests?

Dr. Om P. Manchanda : Yes, that's right , it has.

Mehul Sheth : Can you quantify, mean how much was because of that price increase?

Ved Prakash Goel : It is about 3% which is for the full year.

Dr. Om P. Manchanda : Yes, so what we are seeing is that there is a 3% upside to the revenue due to price increase. So, whatever benefit goes down to gross margin one can calculate that.

Mehul Sheth : And one more question on your expansion side so do you have any near-term target which you are focusing more on the West and the South now to expanding collection centers there . So, do you have any near -term target, how much center you want to add?

Dr. Om P. Manchanda : So, I think our near -term target for the West region is how do we create synergy between Dr. Lal PathLabs network as well as Suburban network. So, currently we are looking at , we have mapped the lab locations for both companies and identifying gap areas so we will gradually fill these gaps. On an overall company basis usually, we put about 10 to 15 labs so that effort will continue on the organic side. And we will continue to look at white spaces for our collection centers. So, in general , normally we have seen about 10% odd of infrastructure growth that happens in our system. So, that really turns out to be about 20-odd labs every year.

Mehul Sheth : And just a last question to Ved, can you give break up the Suburban sales number at least, how were total sales, COVID and non -COVID sales breakup if you have them handy ?

Ved Prakash Goel : So, Suburban revenue is Rs. 37 crore this quarter. And largely it is a non-COVID, very small portion was COVID .

Dr. Om P. Manchanda : Of which about Rs. 36 crore is non-COVID, Rs. 1 crore is COVID.

Moderator : Thank you. The next question is from the line of Cyndrella Thomas Carvalho from JM Financial Institutional Securities . Please go ahead.

Cyndrella Carvalho : Dr. Om you mention ed certain disappointments that you were experiencing largely on the volume side, could you elaborate more on that statement . I mean is it something to do geography wise or is it a certain segment of the business that you were expecting to be better?

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Dr. Om P. Manchanda : See the thing is if you look at th e patients that we report, technically they are not unique patient s, these are all patient visits . So, that's how historically we have been tracking this number. And we have always had a challenge of tracking unique patients because patients in our s ystem come from various channels. They come through walk -ins, they come through collection centers, they also come through pick - up points, it's very difficult to have one identifier , a patient ID and then we have so many unique patients.

But what we are fi nding is that patient visits probably have been less this quarter. And it led us to ask many questions in our mind, one was we took a price increase, is that impacting it. We did this analysis for those tests where we had taken a price increas e, we didn't find any variation between though that portfolio which has been experiencing pric e increase versus the other one where we have not taken price increase. In fact, the growth rate for both sides is the same. So, that means we have ruled out that price increase has impacted it.

We also saw whether the competition is impacting it, we did this Tier wise analysis we again found that it's probably uniformly across this thing. We also found that we are n ot very sure it's a hypothesis whether because the bundle packages are going up, whether it's impacting the patient visits frequency, we still don't know that. I think the strong sort of reason that we can find out is that last year I think no n-COVID numbers are also impacted by COVID itself which is very difficult to identify for us. And the base of last year is much higher if you have that number with you, cl

ose to

6.

Bharath Uppiliappan : 25% of the patients are in Q1.

Dr. Om P. Manchanda : Yes, in Q1, so I think right now probably the only analysis that we have is the last year base is much higher that probably has impacted this volume growth. We will have to wait and see for this quarter and maybe two quarters later. We are very hopeful on an annualized basis we will see a smart recovery on the volume side, I think that's the only thing I can probably say right now.

Cyndrella Carvalho : Dr. Om I said what will drive this volume recovery for us, what are the efforts we are investing in which gives you the confidence that we will be able to report higher volumes?

Bharath Uppiliappan : So, like we talked about in the opening speech I think we are very focused on new customer acquisition programs, it can either be a physical hunting program or even a digital acquisition program. So, that is really what we are looking at as one stage. Second is around the whole infrastructure expansion program. So, we announced new labs, we are putting up collection centers in the regular space and we can accelerate that whole area. So, that is going to be a second lever of getting new customers in. And third is obviously improving our customer service levels as we move forward as well. So, that's taken as increases and visit frequency increases and recommendation by doctors also increase. And the Super Specialty portfolio also will continue to make more inroads in acquiring new patients propagating those tests. So, I think within these four levers which we have, we should be able to see a smart recovery like what Dr. Om mentioned in the full year term.

Cyndrella Carvalho : And one last question in terms of the cost if we look at the cost base, I think like all the participants are asking how we make the investments but somehow the cost base has remained steady, so how should we look at it?

Dr. Om P. Manchanda : I think structurally the P&L shape is changing in two to three aspects, one is if you plot data of the last sort of 10 to 12 quarters, our rental cost is directionally going

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down. And one of the reasons for that is a lot of business is shifting to collection center network. People are not coming to as walk-ins but rather they are going to nearby collection center, so that's one change that we are observing.

Second is we are seeing a very volatility in the mix change. I think one of the reasons probably last year our reagents cost was high because COVID testing was very high and that led to higher reagents cost, that receipts we are probably going back to the lower sort of the consumption cost. So, that second change that I am observing. Going forward you should see two heads going up #1 A&P and we are constantly going to increase this number, gradually I think as we get lot of confidence on margin recovery as we have seen in this quarter, we will try and plough back a lot of money back into the sort of A&P. The second area is IT cost, we have seen a sharp increase in the last three to four quarters and we will reassess the requirement, if required we will invest more in IT infra as well.

Moderator : Thank you. The next question is from the line of Aashita Jain from Nuvama. Please go ahead.

Aashita Jain : My question is more on the industry side, if I look at the price hikes that you have taken and several of your peers have also taken in semi-specialized and high-end kind of test. Will it be possible for us to take price hikes going forward in line with the inflation at least in these high-end tests, what is your take on that?

Dr. Om P. Manchanda : So, I think at a macro level I can only say that there is a bit of a rebalancing, this is going to happen on pricing. Traditionally my experience has been on high-end tests that were actually cross-subsidized by routine tests in the past. As the competitive intensity in

the routine is increasing, so I think some of these high-end prices will get automatically corrected because while sometimes the realization per patient high-end is higher, gross margins are not as high as they are in the routine tests.

I think first my sense is this industry is going to experience a lot of rebalancing of pricing. And another thing one must realize that the human component on high-end test is much higher, routine tests are highly automated, machine driven, that's why you see a lot of competitive intensity on routine, you don't see that much on high-end. How we can do it, yes, this is our first attempt in February this year; we will gradually sort of wait and watch and see how we go forward, but I think one should be able to correct it on a period basis.

Aashita Jain : My second was on the industry growth in general so recently in one of the CRISIL reports they have pegged the diagnostic industry growth at about 5% to 7% for the next five years. And this number was around 10% last year. So, just wanted to know your thoughts on this, how do you see industry growth panning out for say next three to four years? And where organized players like you stand in that?

Dr. Om P. Manchanda : I haven't seen this report, but I would be surprised if the industry growth rate falls to this level, because to my mind Tier-II and Tier-III markets are just opening up from a diagnostic perspective and penetration of diagnostics in this industry on a sort of qualitative basis I can say it's pretty low. So, I would be surprised if this number is true, but I will look up to this report and see what it is. This has come out recently.

Aashita Jain : So, in one of the annual reports of the healthcare major, I think they have reported this number 5% to 7% diagnostic drop .

Dr. Om P. Manchanda : And there is a reference to the CRISIL report, is it?

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Aashita Jain : Yes, so checked with the Company it was from CRISIL that is what I understand .

Dr. Om P. Manchanda : Well, my own personal view is I think it's quite a low ball, I don't think so. While it may not be still as high as it used to be in the past, I think this to me looks low.

Aashita Jain : And just lastly on the volume side I mean there are a lot of questions from that, if I look at your non -COVID sample growth it's somewhere around 6%, which is still lower than what you have been guiding around 8% to 9% and again lower than what the pre -COVID level. So, what is the trend that you are seeing on the volume side, are the volumes picking up gradually ? How should we see this going forward?

Dr. Om P. Manchanda : Actually, volume growth number that we have been talking about is, these are patient visits. And sometimes there is a spike also that happens in volumes, because as we enter into high sort of vector borne diseases like malaria, chikungunya or dengue etc. one patient actually visits multiple times during fever season. And certainly when you have a non -season , these visits tend to drop also very sharply.

So, I would wait and watch, I wouldn't get perturbed by these numbers, but I do believe that there should be gradual jump in these volumes. And one of the analysis that we have done is sequentially how it has done, I think that it has done better, right, from a Q4 to Q1 about there is a jump of 8% in volume and on an CAGR basis for the last three or four years, it's about 6.5%.

I think it's just that Q1 of this year versus Q1 these numbers look very low. So, I will probably hold on to this, but I think as I made an opening comment, I think it's lower than what we expected, so we will have to wait and watch for 2024.

Moderator : Thank you. The next question is from the line of Vignesh Iyer from Sequent Investments. Please go ahead.

Vignesh Iyer : Just want to understand on the EBITDA side of it, I mean is this number a more sustainable number and can we assume a more sustainable

number going forward
on the EBITDA side of it?

Ved Prakash Goel : So, as I mentioned earlier somebody else has also asked this question, so this is normally first half and we have higher margins, but full year I don't think this kind of margins we are guiding. What we are saying is at least we are confident that we can maintain our pre -COVID level margins for the full year.

Vignesh Iyer : On the tax rate side , what would be a blended tax rate for FY2024 , I mean are we shifting to something like 25% ?

Ved Prakash Goel : Yes, we don't have any advantage , I mean normal tax rate is 25% which is applicable to us.

Vignesh Iyer : No, like we have paid around 29% if I am not wrong for the quarter, right ?

Ved Prakash Goel : That was mainly because of the notional depreciation you have seen, but if you see the standalone, it is about 25%.

Moderator : Thank you. The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar : So, firstly my question is with respect to Swasthfit , over the long term what kind of proportion of the total revenue do we target for the 22%, how much of the total blended share could be contributed by Swasthfit?

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Bharath Uppiliappan : So, there can be multiple applications for Swasthfit from what we call a bundled test offering portfolio. We can create bundles for many other disease conditions , we can get faster diagnosis out. So, I think, one way to look at it it's a great thing we will grow this portfolio only.

Second is that say we will vary this and hence try to grow varied portfolio. So, yes, the future looks very bright, I don't want to put a number but between 20% to 25% in the near term should be reasonably possible as per our estimate.

Omkar Kamtekar : So, we are also looking to create other bundled tests so that those also contribute towards the growth, that would be the right thinking?

Dr. Om P. Manchanda : Yes, let's look at from the market perspective there is a need to create this bundling. I think bundling has also become much more affordable now. And doctors are also suggesting LFT/ KFT together which they were earlier prescribing separately, patients also want to do it together . In fact, I have seen request coming even for high -

end test bundling as well and see whether they can diagnose this early.

One which I keep saying the core problem our industry solves are two, first one is we must diagnose it accurately and second, we must diagnose it early. I think accuracy wise the industry has done great job; quality labs have come up. This early piece is being driven by multiple factors it could be genetic testing, it could be bundling, because you can do the entire testing in one go and diagnose it early. And I think patients in both the medical fraternity and patients are seeing great value in this because bundling is much cheaper than ordering individual tests.

Omkar Kamtekar : And lastly just a clarification, in the previous question when you were mentioning the Tier-II and Tier-III growth, I was not able to hear so what was the growth number what you were expecting from this Tier-II and Tier-III?

Dr. Om P. Manchanda : So, I was responding to someone mentioned about industry is going to grow only about 5% to 7% my thing was that I haven't seen the report myself, but my own gut says that it is too low, while one should not see quarterly numbers, but over a long period of time this whole Tier-II and Tier-III market is yet to open up.

Omkar Kamtekar : So, what would be the number if you could just have rough estimate?

Dr. Om P. Manchanda : Very early to put that thing, because the only way I can look at it is that you and I see the same numbers, 1.4 billion people and if you draw a pyramid I think most of this industry is still lying in top end of the market which is metros and mini-metros but it is still to go down, especially from organized industry point of view, while unorganized

sector may still be existing in Tier-II and Tier-III towns, but I think organized industry still has to make lot of influence in these markets. So, it is more a qualitative comment well I don't have any number to which I can.

Omkar Kamtekar : And lastly, so there is one problem you generally face in Tier-II and Tier-III or lower cities that generally patients would only try to go to a diagnostic center where the doctor has recommended. So, you may go to a specific lab or a specific technician to get their report. So, how will this be overcome?

Dr. Om P. Manchanda : No, I fully agree with you, there is a challenge. And I think 20 years back that was challenging even in cities as well. So, I think as gradually more brand awareness picks up, affordability goes up, and people would be willing to pay a brand premium for this as the overall cost component goes down, OPD prices go up. So, I think some of these practices would tend to go away, and then branded players will be

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able to succeed, that's probably I can, I think it will happen, but you are right there are challenges, this market is not there just for us to grab, just like that.

Moderator : Thank you. The next question is from the line of Rahul Agarwal from Incred Capital. Please go ahead.

Rahul Agarwal : One question Dr. Om was how we analyze retail businesses, we look at same store sales growth, I was just trying to apply that to diagnostics. So, let's say you had a franchisee network of 3,000 roughly at the end of March 2020 so will it be fair to say that that network is still growing on revenue or most of the topline growth which we are seeing is coming from the new networks which was added over the last three years, any analysis there please?

Dr. Om P. Manchanda : It's growing across, in fact you will be surprised our Delhi NCR has shown better growth while Delhi NCR has the same store, because there is hardly any scope to grow further outlets. So, technically if I look at this geography in the same store as a concept it is still showing growth. So, I think it is across, the profile of growth may be different in these markets. The same store growth may be coming more because people are shifting to bundling, people are shifting more preventively while it may be less in other markets, but there is growth happening in both the sectors.

Rahul Jain : I will move on to the second question, it was on cash flow. My sense is our cash profit for the quarter was about Rs. 120 crore as I understand the gross cash you have reported is Rs. 877 and the debt on the books is about Rs. 146 crore. If I do a math net cash is about Rs. 730 so it looks like there is hardly any CAPEX done in the quarter, is that understanding correct?

Ved Prakash Goel : Yes, you are right.

Rahul Jain : And lastly Delhi NCR you were saying that has improved and the growth is better, what's the revenue share here for the quarter?

Ved Prakash Goel : So, it's about 32% overall.

Rahul Jain : Including Suburban right?

Dr. Om P. Manchanda : Yes, all put together it is consolidated one figure, so we are not going to talk about non-COVID separately, one number all companies as one. All put together 32% is from Delhi NCR.

Moderator : Thank you. The next question is from the line of Karan Vora from Goldman Sachs. Please go ahead.

Karan Vora : My first question is on broadly, so you highlighted that from the competitive intensity point of view the market has kind of stabilized. But is the competitive intensity back to pre -COVID levels or it's still high. And from where is it coming from? Is it still the online players or you are also seeing it from the hospital players?

Also, in addition to that, in the Delhi NCR region you mentioned kind of a higher growth, but do we think that now that we have reached a sizable scale in this region, going back to pre -COVID volume growths would be difficult?

Id be difficult?

Dr. Om P. Manchanda : Let me first answer competition from before COVID and post -COVID whether competitive intensity is same or it has gone up, answer yes, it has gone up. So, but definitely it has come down from the recent past, but it has settled down at a much higher level. So, I would say it is a new normal that has settled down. Now where is Page 14 of 15:

it coming from, it came from three sides one was of course this new age players, private equity funded , second was hospitals and third was some of the pharma companies.

So, I think as the new normal is settling down I would put hospitals as number 1 sort of a competitive threat to industry or rather which can change the whole construct.

So, I think hospitals are getting more aggressive. Can they be Pan India, I have my sort of a doubt on that, but obviously in regional or city level, they have brand equity, and they are trying to leverage that which is what has changed.

The second question I think you had was on Delhi NCR growth -- so Delhi NCR growth though it has gone up, but I don't think it's sustainable what you we have seen in Q1 but Delhi NCR being a very sort of a high market share, market for us, so I don't see this growth would be in line with our overall blended growth for the portfolio. So, I think growth contribution from Delhi NCR will be lower than the rest of India as we go forward while a slightly better growth that we have seen in this quarter it will just be an exception.

Karan Vora : Another question on Suburban, so you mentioned in the call previously as to you are in the Phase II of the integration journey, so just if you can throw a bit more color on how many phases do you plan to completely integrate Suburban into and what is the timeline for that, like will it take a year or maybe more than that? And by when can you expect the margins to cross 20% levels?

Dr. Om P. Manchanda : When we acquired about a year and a half back that time our focus was to build Suburban as a separate company, as a separate team and separate unit. And we tried to do that. We have put out certain growth figures, unfortunately we didn't see those numbers coming in. So, essentially the idea was can we just drive topline and not worry about rest of the stuff, but then slowly we realized as the COVID receded we suddenly found the margins came down very sharply than what we had seen during COVID time.

And the growth rate was definitely not as high as what we expected, then we quickly have changed our gear saying that not only does this asset has to give us revenue synergy , but it also we have to look at cost synergies as well. And when the cost synergy started falling in place then we looked at two phases, Phase I was Can we look at our corporate cost of Dr. Lal PathLabs, can that structure provide some overriding support to this asset like Finance, HR, Marketing, IT etc. so that was Phase I.

And Phase II is when we went down to operations, we found that if we look at rationalizing our lab infra we can derive some synergies out of this , so currently we are in that phase. I think the last phase would be when you bring front-end synergies as well like the sales team becoming one and things like that which to my mind will happen as we go along, but right now Phase I and Phase II is the focus.

Karan Vora : And any guidance on the timeline for all of this to happen?

Dr. Om P. Manchanda : I would play it by the year and see how our integration happens, because the integration of lab ops is not that simple, because both the companies operate on different lab installation systems, different bar -coding system. So, I would not put any timeline right, but I would want to get it right the first time , slow and steady.

Moderator : Thank you. The next question is from the line of Dheeresh Pathak from White Oak Capital . Please go ahead.

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Dheeresh Pathak : I came in la

te so my apologies, but the way I am looking at the result is that non - COVID revenue grew 10% and the patient count is flat 7 million to 7 million. So, everything is coming from realization increase, now there is price increase and there is mix impact, so can you break this 10% non -COVID increase in realization into

price increase that you have taken on like -to-like and mix?

Dr. Om P. Manchanda : So, I think we have spoken enough on this maybe you were not there when we talked about this, but just to give a split of 10%, 3% is on account of price increase, balance is on account of mix. And if you take out COVID volume then the growth rate is about 2.5% on volume. Volume is if you take away COVID numbers out of that, but since you asked about non-COVID 10% break up right.

Dheeresh Pathak :: Yes.

Dr. Om P. Manchanda : So, non-COVID , whatever the number is to 3% is on account of price, balance is about 2.5% on volume and balance is the mix.

Moderator : Thank you. The next question is from the line of Omkar Kamtekar from Bonanza Portfolio. Please go ahead.

Omkar Kamtekar : So, my question is with respect to what will be the next geography for expansion that we are looking for, one? And secondly the cash that is in the books of Rs. 877 crore , will that be used for any inorganic expansion over the next one or two years?

Dr. Om P. Manchanda : I couldn't hear your first question, but I did hear your second one, the cash on the books yes , I think the primary objective is to utilize it for growth, both organic and inorganic. Since our business organically does not require too much capital, we will primarily use this for inorganic and we are still digesting our last acquisition that we did in Mumbai . So, as this stabilizes as the new opportunity comes up, we will always be open to look at inorganic opportunities. The first question I am sorry, I couldn't hear that, you were asking something about Tier II or Tier-III.

Omkar Kamtekar : Which are the regions are we looking to expand in the Western region further or we are going for South region , on the national front which region are we next focusing on for expansion?

Dr. Om P. Manchanda : South and West so there is no doubt, I think every call that we talk about South and West are gap areas for us, we will try and grow in these areas.

Moderator : Thank you. That was the last question. I would now like to hand the conference over to the management for closing comments.

Ved Prakash Goel : Thank you everyone for being with us on this call today. I hope we were able to address your questions.

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Call: Nov 2023

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National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q2 & H1 FY24

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q2 & H1 FY24 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above

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Dr. Lal PathLabs Limited
Q2 FY24 Earnings Conference Call Transcript
November 2, 2023

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Bharath Uppiliappan - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer
• Mr. Shankha Banerjee – CEO Suburban and other Group Companies
Participants who asked questions • Rahul Agarwal – InCred Capital
• Rishi Mody – Marcellus Investment
• Prakash Kapadia – Anived Portfolio
• Aneesh Deora - Nomura
• Nitin Agarwal – DAM Capital
• Lavanya Tottala – UBS Securities
• Jainil Shah – JM Financial
• Karan Vora – Goldman Sachs
• Punit Pujara - Helios
• Rahul Jeewani – IIFL Securities

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Moderator Ladies and gentlemen, good day and welcome to Dr. Lal PathLabs ' Q2 FY24 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference, please signal an operator by pressing (*)then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Nishid Solanki of CDR India. Thank you. And over to you, Nishid.

Nishid Solanki : Good evening , everyone and welcome to Dr. Lal PathLabs ' Q2 FY24 Earnings Conference Call.

Today we are joined by Senior Members of the Management Team, including (Hony) Brig. Dr. Arvind Lal - Executive Chairman; Dr. Om Prakash Manchanda - Managing Director; Mr. Bharath Uppiliappan - CEO; Mr. Ved Prakash Goel - Group CFO along with Mr. Shankha Banerjee - CEO of Suburban and other Group Companies.

I would like to share our standard disclaimer here. Some of the statements made on today's conference call could be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed disclaimer in this regard is available in the results presentation which has been circulated to you earlier and also available on the Stock Exchange website s.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives.

Dr. Arvind Lal : A very good evening and a warm welcome to everyone present on the call. We are here to discuss Dr. Lal PathLabs ' Q2 FY24 Earnings. I would like to take you through the progress that we have made, and initiatives outlined to sustain our performance trajectory.

Dr. Lal PathLabs has been meeting diagnostic requirements of the nation for several decades. And next year we shall be celebrating 75 years of our existence. Consequently, both patients and physicians regard our brand as a trusted healthcare partner. Toda y we are known for Quality and Accuracy, Accessibility and Affordability to meet the testing needs of all our patients and referring doctors throughout the country.

Our initiatives towards developing a model of Excellence and Diagnostics have not only earned us numerous accolades but have also helped us gain the confidence of the people of India as underlined in our new marketing campaign, "Bharat Ka Vishwas."

Our distinctive grasp of intricate market dynamics and omni -channel presence sets us apart as we aim to branch out in underserved Tier -III and Tier -IV markets. This along with sharp focus and leveraging a strong digital infrastructure will differentiate us as we achieve major milestones going ahead on a network rollout. We have implemented a new custom -built logistics solution and this product with enhancements will help us to serve our customers better.

The expected synergies between the two brands, Dr. Lal PathLabs and Suburban Diagnostics , are becoming stronger. And we now have a stake of significance in

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the western market. Suburban performance is improving day -by-day, and especially after the opening of the Reference Lab in Mumbai. The approach here is to combine our capabilities and serve as many patients as possible to offer them super -specialty, high -quality tests at affordable prices with quick turnaround. Our next phase of growth will be led by a combination of factors. The continuous shift from unorganized to organized is one of them. This will be supported by meticulous execution of strategy to further add to our network scale and by elevating our servic e standards through best -in-class operating processes and technological advancements.

The diagnostic sector in India exhibits substantial prospects for future development. And I promise that Dr. Lal PathLabs will be at the forefront of this opportunity as consumers move towards more reputable brands that offer

enhanced quality and best-in-class testing experience.

Thank you very much. That concludes my initial thoughts. I would now like to hand over the floor to Dr. Om. Thank you. Over to you, Om.

Dr. Om P. Manchanda: Welcome everyone to Dr. Lal PathLabs Q2 FY24 Earnings Call. I will talk to you about the Company 's strategic priorities and throw some light on the current business trends.

At the outset, I would like to mention that COVID and COVID allied contribution to the business has fallen now just 2%. Therefore, in our headline figures, we are not

reporting it separately. To further enhance the understanding of the overall P&L, we are also moving away from the term 'Normalized EBITDA' and will only be reporting one EBITDA figure as per IndAS that is net of ESOP and CSR expense.

Key Business Trends

For the last four quarters, LPL consolidated revenue growth trends both in value and volume terms are showing a healthy and steady rise. Last time we crossed the milestone of Rs. 600 crore in a quarter that was Q1 of FY22. But that was primarily driven by a 36% contribution coming from COVID and COVID allied tests. In the current quarter again, we have crossed this milestone, but this time COVID and COVID allied contribution is just under 2%. On total revenue basis, four years CAGR trend, that is from FY20 onwards for the current quarter, the growth rate is 13.2%.

Our strategic focus continues to be on similar lines, and I will just mention some of these areas that we are focusing on.

I) I think the biggest focus for us is to go deeper. As you all know that we have a strong brand franchise in Northern Eastern market. And there is a tremendous amount of focus on Tier -III and Tier -IV towns and go wider in Southern and Western markets.

II) The second priority for us is to enhance productivity and service levels to maintain a sustainable competitive advantage. And as you know, that as a team, we continue to look for running processes which are more efficient and that show up in our margins as well.

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III) We are driving participation of our partners like Collection Centers to drive growth. As you know, the contribution of Collection Center business over a period of time has now become 50%.

IV) We continue to have an enhanced focus on high -end tests in our portfolio.

V) Continue to build 'Suburban' in key markets like Mumbai, Pune and Goa.

VI) Keep focus on Preventive Healthcare Tests portfolio that is Swasthfit and LPL as well as Preventive Health Check Ups portfolio in Suburban.

VII) And lastly, leverage technology to drive process efficiencies and drive marketing programs.

That's it from my side. Thank you. And over to Bharath now to continue the discussion.

Bharath Uppiliappan : I warmly welcome you all on this call today. And wish you and your family members a joyous and healthy festive season ahead. I will now take you through the business and operating highlights.

Operating and Business Highlights of the Company

I am pleased to share with you that we delivered a robust quarter of revenue and profit growth while making good progress on our strategic growth agendas.

In Q2 FY24, we achieved a revenue of Rs. 601 crore, a growth of 12.6% in total revenue over Q2 last year. Net of RT -PCR test, our growth of revenue for Q2 FY24 is 14.4%.

In Q2 FY24, we served 7.5 million patients representing a growth of 5.2% over last year, and net of RT -PCR the patient growth for this quarter is 7.7%. You will notice that our patient growth is significantly higher than our Q1 FY24 numbers.

In this quarter, we tested 21.2 million samples representing a growth of 11% over last year. And net of RT -PCR, the sample growth for Q2 FY24 is 12%.

The RPP for Q2 FY24 is at Rs. 798, a growth of 1% over Q1 FY24 that is sequential, mainly due to mix.

It is also pertinent to point out that Suburban Diagnostics registered an encouraging

start

to its growth journey by registering a growth of 10.4% excluding RT -PCR in Q2 FY24.

The strong performance in Q2 FY24 is mainly on account of five key factors:

(1) Market activation and execution across all geographies including that of Suburban. Our D2C program has begun to gain traction and so has our key account management program

(2) Our investments in 35 hub labs across the country are bearing results now. And this has enabled us to process samples with better tech driven by automation.

This has enabled us to gain market share in these markets

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(3) Our expansion program in Tier -III Plus towns continues to show encouraging results. On the back of this response, we are planning to accelerate this journey

by opening 20 plus labs in Tier -III plus towns. This will obviously be backed by a strong collection center network expansion and market activation program
(4) As you all know, we had launched a marketing campaign "Bharat Ka Vishwas" with the aim of fostering trust and convenience for both doctors and patients. I am glad to share that we have started witnessing favorable outcomes, further solidifying the trust in our brand among healthcare professionals and patients
(5) Our product portfolio focus continues to do well. 'SwasthFit' together with ProSelf at Suburban Diagnostics delivered a robust growth of 25% during the quarter, and it currently constitutes 21% of our total revenue. The revenue from this portfolio bundle test during Q2 FY24 is at Rs. 125 crore enroute to Rs. 500 crore annual run-rate revenue

Our focus on growing super-specialty business across various therapies and investments in building digital infrastructure has played a key pivotal role in driving incremental volumes and attracting more patients. During this quarter, we have gone live with our new custom built, feature rich logistics app.
On the P&L side, our continuous efforts towards optimizing operational efficiency across all cost line items and reimagining the value chain has led to an improvement in profitability and gives us the headroom to invest in growth opportunities.
Overall, we are moving the right levers to optimally set a growth trajectory that will give us sustainable growth.

With that I would like to invite Ved to take you through all the financial performances. Over to you, Ved.

Ved Prakash Goel : Good evening, everyone and thank you for joining this call today. I am pleased to announce that we have reported robust performance this quarter, sharing some of the key highlights for the quarter's first half.

Key Financial Highlight for the Quarter

Revenue for Q2 FY24 came in at Rs. 601 crore against Rs. 534 crore last year same quarter, a growth of 12.6%. In the first half FY24, total revenue is Rs. 1,142 crore Vs. Rs. 1,037 crore last year, a growth of 10.2%. Revenue realization per patient for Q2 FY24 is Rs. 798 as against Rs. 746 last year in the same quarter, an increase of 7% led by price increase, test mix and higher contribution of SwasthFit. EBITDA for Q2 FY24 came in at Rs. 178 crore as compared to Rs. 144 crore in Q2 FY23. EBITDA margin for Q2 FY24 is 29.6% Vs. 26.9% in Q2 last year. In first-half FY24, EBITDA is Rs. 324 crore Vs. Rs. 261 crore same period last year with a margin of 28.4% Vs. 25.2% last year same period.

PBT for Q2 FY24 came in at Rs. 152 crore Vs. Rs. 103 crore in last year same quarter. PBT margin is 25.3% for Q2 FY24 against 19.3% for Q2 FY23. In first-half FY24, PBT is Rs. 270 crore Vs. Rs. 184 crore last year same period with a margin of 23.6% against 17.8% last year.

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PAT for Q2 FY24 came in at Rs. 111 crore Vs. Rs. 72 crore in Q2 FY23. PAT margin is at 18.4% for Q2 FY 24 against 13.6% for Q2 FY23. In first-half FY 24, PAT is Rs. 194 crore Vs. Rs. 131 crore last year with a margin of 17% against 12.6%.
EPS in Q2 FY24 is Rs. 13.2 Vs. Rs. 8.6 in Q2 FY23. With this, EPS

for first-half

FY24 is Rs. 23.1 against Rs. 15.6, last year.

As mentioned by Dr. Om, we are simplifying the reporting of profit numbers and accordingly all the above profit numbers are after CSR, RSU and Notional Depreciation because of consolidation of Suburban.

Net cash and cash equivalent as on September 23 is Rs. 780 crore.

Inventory holding is now at 31 days. And debtors outstanding is 14 days of total sales and 28 days of credit sales.

ROCE for the year is at 25.3%.

Total CAPEX for first half is Rs. 27 crore, primarily on account of new infra and investment in technology.

Once again, our continuous efforts toward improving operational efficiency has led to improved profit margins and thus provide greater opportunity to invest in underpenetrated markets, brand awareness and future technologies.

That brings me to the conclusion of my opening remarks. And I would now request the moderator to open the forum for Q&A.

Moderator : Thank you very much. We will now begin the question-and-answer session.

The first question is from the line of Rahul Agarwal from InCred Capital. Please go ahead.

Rahul Agarwal : I have three questions, two on business and one more structural. Firstly, the new slide which you put in the presentation on Tier-III markets. Just wanted to know,

how do you define Tier -III markets and below, in your mind, how do you classify that? And if you could elaborate a bit more on the accelerated program for these markets, because my sense is, as revenue share increases and centers take a bit of time to ramp up how does the incremental margins look like?

Bharath Uppiliappan : Mr. Rahul, I didn't get your second part of the first question, but the answer of Tier - I, Tier -II, Tier -III as per Government of India classification, as defined for I think Census and HRA and so on and so forth. So, we just follow the same classification.

Rahul Agarwal : The second part of the question essentially was, you mentioned about 20 plus labs getting opened into the Tier -III markets, just wanted to understand as the center when these labs ramp up, incremental margins how should we look at that, because I think if you do that and the revenue share further increases from here from 34%, how do the consolidated margins behave?

Dr. Om P. Manchanda: I actually would say that this is a typical business model which we have always followed. We open 15 - 20 labs every year and each lab that we open it further proliferates Collection Center and pick -up point in that area. So, I would say that it's not going to make any difference, this kind of model is already baked in into our

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margins. However, I do take your point that these centers would be in some of these Tier-III towns, but I really don't see materially margin getting impacted, because some of these places our cost structure is also equally very competitive. It's much lower than what we normally end up incurring in metros. So, if at all, there is something in the short term, but materially I don't think it will change the overall trajectory of the margins.

Rahul Agarwal : Secondly, on the revenue per patient I think obviously pricing mix -wise Swasthfit and all have contributed, but anything from the Delhi NCR market looks like your core must have done better this quarter. Is that true and how much was Delhi NCR overall business?

Dr. Om P. Manchanda: I think Q2 as everybody knows that it's a high sort of fever season. And fever is actually all across. We have seen the entire, this year, we have seen across India itself, but most of it is from Northern markets like UP, Bihar Delhi NCR included. So, it is from everywhere, especially September month. So, that jump has come all across.

Rahul Agarwal : And how much would be Delhi NCR now?

Dr. Om P. Manchanda: Delhi NCR contribution this quarter is 31%. Actually this, it used to hover around 34%. And now this year we have seen UP

and Uttarakhand markets have done far better. But we have seen growth everywhere.

Rahul Agarwal : And lastly this cash again is getting accumulated about Rs. 800 crore, I think will make much more money in the second half. Any thoughts, how do we deploy? I know you have been looking for assets into South India, you have been mentioning that in prior calls, but any thoughts?

Dr. Om P. Manchanda: Yes, I think our view on cash utilization has always been, is a three -pronged one of course we continue to invest in the business. Organically that requirement is not very high. We will continue to pay dividends as per our policy. And of course, we will keep scouting for acquisitions. But we probably would be a little more sort of value -driven and unless we find quality assets at the right price, we probably will be careful doing acquisitions, of course in order of priority South is definitely number one because the contribution from this market is the lowest. So, that's the way we will look at it. Right now , there is nothing that we can share with all of you.

Moderator : The next question is from the line of Rishi Mody from Marcellus Investment Managers. Please go ahead.

Rishi Mody : So, a quick bookkeeping question, first, could you just tell me what is your non - COVID revenue the way you all used to report excluding RT -PCR, COVID allied and D -dimer and all those tests?

Dr. Om P. Manchanda: It's Rs. 589 crore, out of Rs. 601 crore .

Rishi Mody : So, Rs. 12 crore is COVID revenue this quarter.

Dr. Om P. Manchanda: Non-COVID is Rs. 589 crore and COVID and allied is Rs. 12 crore as we were reporting it earlier.

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Rishi Mody : Dr. Om, in last quarter's call you mentioned that you are all expecting a smart recovery by the end of this year. Could you elaborate on what you mean t by smart recovery, and what growth are you all looking at?

Dr. Om P. Manchanda: See recovery in the sense that we were, I don't know what I mentioned last time, but recovery, essentially, I mean on volume growth as well as value both together.

And if you see the last four quarters' trends we were in a negative trajectory for volume. Now, we are actually coming into a very positive trajectory. And if I take out the RT-PCR volume, I think volume on non-COVID, pure non-COVID looks better. And we are hopeful that in times to come this will further go up. And in value terms also we have seen trajectory moving up. Of course, it's contributed by little bit due to price increase as well, but that is what I probably mean that in the growth rates are slowly inching upwards.

Rishi Mody : And like how much do you think we will be able to take this notch -up in terms of volume value?

Dr. Om P. Manchanda: See value as I mentioned in my opening comments, if I look at the last four years CAGR, we are now seeing about 13% growth for the quarter. There was a time when it had fallen below this number, I think in the early teens, we are now almost getting that number, I would say.

Rishi Mody : My third question is to Bharath, Bharath in the PPT comment that you have put in, you mentioned that Suburban has begun gaining traction. Could you shed more light here about what has happened over the last year? What are you seeing today which is making you comment that Suburban is gaining traction?

Bharath Uppiliappan : So, I think a lot of action has happened over the last one and a half years or so which includes stabilization of asset, customer base re-confidence gaining. In fact, last quarter, I think when we had met we kind of shared with you that we have put up a very aggressive marketing program in the city of Mumbai, led by ATL Digital and the full suite on that. And I think this quarter we followed up with a very strong below-the-line activation program as well. So, a lot of what I would call efforts are being taken on building the Suburban brand in the western market. And we are seeing t

he first green shoots of the growth trajectory for the business.

Rishi Mody : What do you think how big is it going to become over say one to three years as part of Lal's revenue, if you had to put a number there?

Dr. Om P. Manchanda: Maybe just to continue with the earlier question of yours. I think two – three fundamental shifts that have taken place for us in Suburban, number one Suburban as our business was more own asset driven, I think slowly, we have shifted towards a franchisee driven business. That is one change that has happened.

We also went through a bit of high employee turnover, which also has stabilized quite a bit. And that is what Bharath meant by a lot of effort has gone into stabilizing the ship as it changed hands.

I think the most important thing has been that we piloted few marketing programs and we are seeing some response to some of these things. So, we know fairly reasonably well know that what really works and what doesn't work.

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Now coming back to your second question, it's very clearly right now three-market approach Mumbai is number one, and second is Pune and then third is Goa. And you know that within Maharashtra, Mumbai is the largest market and there is no published data, but I think it's a fairly big market. And if we get our formula right, that's where we presume that we should actually be doing well. And for us, it's a very strategic market because our LPL presence in the Western region has been less. So, if it really works well, hopefully Suburban should become a meaningful sort of contributor to the total business.

Rishi Mody : And we are giving up the Madhya Pradesh market, if I remember Suburban had some operations there as well.

Dr. Om P. Manchanda: Yes, so I think there were one or two labs. Indore, Jabalpur and Rewa, I think that we are giving up, because it doesn't make sense for Suburban to put effort there because LPL is fairly strong in these markets. It is just a maintenance sort of thing, but really investment is primarily in these three cities and whatever spillover in rest of Maharashtra is.

Moderator : The next question is from the line of Prakash Kapadia from Anived Portfolio Managers. Please go ahead.

Prakash Kapadia : Dr. Om, if I were to take a mid-term view given that most of the competitive intensity seems behind us, would it be right to say there could be a 10% patient volume growth and with a bit of product mix and realization change, we should be able to grow revenues at 14% - 15% over the next three to four years, is that the right way to envisage the future?

Dr. Om P. Manchanda: Prakash, I wish I could give these numbers. I would hesitate to actually talk about some guidance. What all I can say is that as an organization we are fairly well spread out across the country. And as I mentioned in my opening comments that we want to go deeper and wider, I think this strategy of going deeper in to the

Northern Eastern Market is on its way. Today, there are 75 districts of UP, a very large population I think 22 - 23 crore of population we are number one and way ahead of the number two guy is very far behind us. So, as the country's economy grows, and especially UP, we are very well placed to actually gain a lot of market in these places, including Bihar, Orissa, the rest of North, you name I think our spread is probably one of the best. And I think we know what is really required to really go deep, that's one thing that we are doing.

Second is we continue to put effort to get wider and wider. I think as our footprint grows --. And the second thing is also we are trying to look at the business model which remains competitive so that we don't resort to, and our cost structure is competitive so that we can compete on price, etc. While some of this intensity may have slowed down a bit, but we still believe that intensity will continue to remain because this sector has always been

competitive from unorganized sector, smaller

labs it's just that now the organized competition is growing. So, the market will continue to grow because as evidence -based medicine grows, preventive health check -ups grow I think the market will continue to grow, we just have to look internally and see that our competitive advantage remains intact. And then hopefully, we should be able to achieve what we have set out for ourselves.

Prakash Kapadia : This time around you have given the contribution from Tier -III and beyond in terms of revenue contribution. So, is it fair to say North and East would be a large part of

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this Tier -III revenue. And could you take us through the journey in these Tier -III cities and beyond. Do you have basic packages? Is it largely B2B? Or it is Swasthfit and awareness seems to be increasing there also?

Bharath Uppiliappan : So, Prakash this Tier -III story is not new, we have always talked about this, I think for last more than two years in my recollection. There have always been constant efforts put up over here, on this front. In these Tier -III markets what we have is a classical suite, which we do is to first to start to put Collection Centers. As we build Collection Centers, we begin to realize what kind of tests sell etc., which are predominantly routine. But also, what we are now realizing is slightly higher in tests also come in, especially in Gynae and infectious diseases kind of portfolio. And, as we densify these clusters, we then identify the distance from the nearest other lab we have and start to populate those. And that is the reason why we arrived at this number which we just talked about, of putting 20 plus new labs in these towns, because we already have a Collection Center network, and this allows us opportunity to further densify and go down deeper as Dr. Om mentioned in his opening speech.

So, I guess this we have been doing for quite some time, it is just that it is coming to the fore from a sizable opportunity perspective now and we are talking more about it.

Prakash Kapadia : And would it be fair to say, B2B is still a larger portion, but now we are seeing traction on the consumer side.

Bharath Uppiliappan : So, Prakash, we have talked about this many times in the past, we continue to believe we are a n omni -channel player, we are not restricted to B2B, B2C, we will pull in all levers because there is an interplay between all the channels. A B2B patient today may walk in B2C tomorrow; B2C may go to a hospital tomorrow. So, it's important for us to kind o f do an omni -channel play and not just be focused on one or the other channel. So, given geography, we look at the healthcare ecosystem and the patient or the consumer ecosystem and try and build our offering around that.

Prakash Kapadia : So, it's a mix of both in this Tier -III and beyond.

Bharath Uppiliappan : Yes, of course there can never be either only B2B or only B2C .

Prakash Kapadia : My worry was, it should not be B2B driven, because that seems to be more price conscious .

Bharath Uppiliappan : No, so we obviously manage the mix judiciously.

Prakash Kapadia : And lastly, on Suburban is there an app launch planned in Suburban? Have we taken some pricing changes, if any in suburban to grow in Western and specifically Mumbai, because there seems to be a lot of consumer campaigns which we see in and around Suburban now as compared to earlier. So, some road ahead for Suburban, if you could share.

Bharath Uppiliappan : So, I think we have just begun the journey on building or let us say further strengthening the consumer franchise and the patient franchise along with the doctor network, which will support Suburban. So, it is going to be a long journey, but we are committed to making that happen. So, once again, in the case of the

focus market for Suburban, the full suite of omni-channel play will be put into say wh

at you are seeing is evidently what is apparent in the outside which is the B2C plan, but there is also a strong B2B program also being put in play.

Moderator : The next question is from the line of Aneesh Deora from Nomura. Please go ahead.

Aneesh Deora : Regarding the Dengue related cases, so the news article says that the Dengue outbreak in this particular season was much more pronounced and more so in the Delhi NCR and the North India region. So, would you be able to sort of quantify the Dengue-related revenues that let's say Dr. Lal would have seen in this quarter as against the same quarter in the previous years, any quantification there would help.

Bharath Uppiliappan : So, on Dengue, I think we saw September month having a larger kind of play. And we saw this not only in Delhi NCR, but across North and East geographies. This has been a past trend as well. However, this time what we did was, further, you know, we have this bundle portfolio on fever panel also. And that kind of really took off this season, because we had a panel, one for three fevers and one for five fevers. And this is really what caught people's, the patient's imagination. Because at one go, they could have got tested for the relevant fever without having to go through in a sequential format which they were doing in the past. So, bundling here again, helped us.

And I also talked about the hub lab program so most of our hub labs are equipped with ELISA testing, which is the gold standard for Dengue testing, Dengue . I think we leveraged that infrastructure, the Collection Center, the new Digital Logistics app to kind of significantly improve our service delivery in the marketplace.

Aneesh Deora : Could you may be as a percentage of revenues what were the Dengue related revenues in this quarter? Could you just maybe throw some light there?

Dr. Om P. Manchanda: We have not been separately giving these numbers. But for your better understanding, let me tell you every second quarter of every year has a fever season. So, whatever jump you see in this quarter, one would have already seen the last year also same quarter. But still for you to know, what is that jump, normally, I have seen that variation is maybe half a percent or 1% plus, minus on a total quarter figure. So, while the season is very high, at times this is not more than 1% extra or lower or higher that you see. Exact numbers, we normally don't share it because it creates more complexity about these figures, but otherwise, it is there last year also in the same quarter base as well.

Aneesh Deora : Secondly, I was looking at the gross margin levels. So, the gross margin for Q 2 has come to 79.6% which seems to be a historically high number, I mean, the highest. So, we just wanted your thoughts around what is driving this gross margin, are price hikes helping this or are there any softening of raw material prices also. And how should we sort of think about this number going forward? What would be the sustainable levels?

Ved Prakash Goel : So, this is both in fact, one is, of course, what you are saying is price impact and second operational efficiency has also led to this higher gross margin, because the consumption cost which was like suppose earlier was in the range of 22% now came down to 20% or so. So, this is because of certain tests mix also and operational efficiency like Bharath mentioned hub labs and where we are consolidating, and a lot of operational efficiency has come in consumption so both

price as well as operational efficiency. And I am hopeful that this consumption cost is stabilizing here.

Aneesh Deora : So, would these 79% - 80% odd gross margins be the sustainable margins to work with, going forward?

Ved Prakash Goel : So, let's suppose, if we have to remove the price impact then obviously this will be higher than earlier.

Dr. Om P. Manchanda: I just want to add one more thing to this questi

on, I think one is probably what Ved is saying there maybe because volume growth is not that high as a value growth so that may be helping this margin as well. Second, mix plays a very important role, we have usually seen in Q2, our mix is very drifted toward routine test where the gross margin is generally very high. So, as you move into Q3 and Q4 routine tests actually will drop, and high-end tests actually go up where the gross margins are not that high. So, don't take Q2 gross margin as a representative number for the year. That's the only request we have. I think what you should look at is what the

annual figure would be , that maybe a representative figure.

Moderator : The next question is from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal : On the gross margin level just sort of going back to the question. Now, how much further scope do we have with respect to whatever, there is obviously a meaningful reduction we are getting in gross margins this year given the way trends are. I mean, do we have a meaningful scope still left to further squeeze out efficiencies in the gross margin level?

Ved Prakash Goel : No Nitin, so as I mentioned that we are looking that this is stabilizing, I don't think you should factor any further improvement on this. So, whatever we are showing now it is not even representative for the year, as Dr. Om mentioned, because there is some mix change in this quarter which is impacting this gross margin. So, on a sustainable basis I think we should say about 74% to 75%, kind of margins.

Nitin Agarwal : And secondly, on the overhead costs given the fact that we made a lot of references , we are focusing on Tier -III markets on a growth basis going forward. I mean, what implications does it have for our operating costs? I mean, obviously, lot of the growth is going to come from the franchise route, the fee -for-collection as a percentage, probably at around 14% continues there. But how should we think about the other overhead costs when we try to go out in the smaller areas?

Ved Prakash Goel : I think as Bharath mentioned earlier see one is this channel obviously, we have higher contribution. Now 45% -46% of business is coming from the CC. And obviously as a percentage it is inching up and so as a percentage of this revenue share is going up. But this is not because we are increasing the margins of the CC. This is because of mixed change I mean the contribution of these channels.

Dr. Om P. Manchanda: I will probably take a little time on this question, because it's important to answer this. See if you open a lab in a Tier -III town broad cost of running that lab would not be my sense is Rs. 50 to Rs. 70 lakh in a year. If we are able to generate Rs. 2.5 crore of turnover and 30% of that is roughly Rs. 75 lakh is what normally we will end up paying to a Collection Center if we open there, that's our normal way of

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growing it. So, the moment we open a one lab in a Tier -III town, the moment you reach 2.5 you are virtually breaking even in terms of Collection Center Vs. the lab. So, to my mind, as I was answering Rahul's question earlier in the short term you might see a bit of a change up and down, but over a period of time, it is a way of growing our business. But what this lab would do for us is that it will help us to open further, more Collection Center as we go deeper in Tier -IV towns. So, for us, it's a way of life, I don't think one should look at it as a dilution of the margins.

Nitin Agarwal : The other point essentially is that in terms of when we look at the overall cost structure, we have done a phenomenal job over the years including this year, the way trends are in terms of managing our costs. While I presume will be a continuous exercise in the firm to keep out efficiencies I mean, I am just wondering, how much further scope do we have on an overall level to take out further cos

t efficiencies?

And the reason why I talk about that is because in the past we have talked about EBITDA margins essentially stabilizing at about 25% - 26% levels. We seem to be now again trending reasonably higher from beyond those levels given where H1 has been.

Dr. Om P. Manchanda: That's mainly because of operating leverage, the moment you touch a number of Rs. 600 crore your EBITDA margin will improve . So, I think as a team you have to find out ways and means of increasing top -line, rest everything will fall in place.

Ved Prakash Goel : Just to add Nitin as I mentioned in my opening speech maybe this is the opportunity where we can invest in the business. And even if we are improving efficiency, operating leverage is coming so, it is giving opportunity where we will do the investment in th e business, whether it is technology, whether it is newer geography and so on so forth. So, I don't think this 29% or 28% EBITDA margin is a representative margin for the year.

Moderator : The next question is from the line of Lavanya Tottala from UBS Securities . Please go ahead.

Lavanya Tottala : Just one thing on the margins which you were discussing. So, now that with increasing share of bundled services like with improving sales per patient, shouldn't our margins be better than what it was before with operating leverage, it should sustain, right .

Ved Prakash Goel : As I mentioned this, maybe because operating leverage is very high in our business as Dr. Om mentioning Rs. 600 crore revenue will always give very high margins. But this is again an opportunity for us whenever we are getting improved margins

we will invest back into the business. So, I don't think we should factor in these kinds of margins for a longer period.

Lavanya Tottala : So, the sustainable levels should be at what level?

Ved Prakash Goel : I think around 26% we should see that these are more sustainable margins.

Lavanya Tottala : I was just asking because with increasing bundled services even in this quarter with a fever and all the bundled services will give us better operating leverage. So, that's why I was checking if the sustainable levels will be higher?

Dr. Om P. Manchanda: Actually, bundled services generally now is stabilizing at 21% - 22% of the total revenue. So, I think it will probably stay here for some time is what my sense is.

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Lavanya Tottala : Just can you help me with the revenue and margin numbers for Suburban this quarter, I don't know if it was earlier covered.

Ved Prakash Goel : Suburban revenue for the quarter is Rs. 42.7 crore , total revenue for the quarter.

Lavanya Tottala : This was last year, how much?

Ved Prakash Goel : It was Rs. 40 crore total, but if we out COVID out of that, it was Rs. 37.5 crore.

Lavanya Tottala : And margins?

Ved Prakash Goel : Margins is around 13% but again, as we always mention that right now the focus is to improve top -line and invest in the business. Obviously, operational efficiency or some of the backend synergies are coming so gradually that margins will improve.

But right now, the focus is on the top line.

Moderator : The next question is from the line of Jainil Shah from JM Financial. Please go ahead.

Jainil Shah : My first question is about the competitive intensity. So, is there some more room for improvement in the current scenario? And can you throw some more light on what trends we are seeing? And especially how is the competitive scenario from hospitals in the Northern region? Also, if you can talk about the scope for further price hikes over the next two to three years?

Bharath Uppiliappan : I think the competitive intensity, you know, if you cut it in multiple segments there are varying kind of degrees. So, if you look at online players obviously the extra burn, it seems, has come down significantly and they have also taken up price increases. So, both A & P for them has come down in my assessment.

nt. But they also

moved up pricing and some of them have publicly talked about the same.

If you look at our regular competitors, I think we all continue to be as competitive as in the past, if not more. And I think that is a way of life for all of us. So, I think what you must do is to address competitive intensity segment by segment and not just take an overall macro view, but yes, the online intensity has come down a tad and we are also improving our mix and our offering. Our D2C program has really done well. And we continue to gain traction on our Direct -to-Consumer program as well.

As far as price increases are concerned, at this stage we don't want to make any comments on that. We are comfortable where we are, when we are focused on driving the volume growths up. And we will figure this out if and when we have to, but our current focus is on driving volumes are very clear .

Jainil Shah : Just coming back on the competitive intensity side. So, which particular segment is hurting us the most? And what would be the current share of revenue from aggregators to our total top -line?

Bharath Uppiliappan : Our share of revenue from aggregators is very minuscule , single digits if at all or lower single digit at the very best not significant at all.

Jainil Shah : Which particular competitive segment would be hurting us the most or has impacted us over the last one or two years?

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Bharath Uppiliappan : In the last two years back I think if you look at a two year history in the past then online was very clearly the one which was really kind of what I would call giving us a bit of a run, because we were also not prepared for that kind of scenario. But if I look at today, we are very well equipped internally as a team to be able to handle some of these challenges. So, one part is about competitive intensity but the second is about organization's learning ability to be able to implement stuff, which will protect us in the future as well. On that metric, we have moved up significantly.

Dr. Om P. Manchanda: I would also add one more thing to what Bharath just said, I think more than hurting what has happened is that this new age player actually were bringing sort of mega trend the way customers were availing diagnostic services. While we had started home collection, but they started using tech to really order booking, paying online, etc. I think that 's something in the last three years, our teams have done a

tremendous work to really catch up. So, we are now fairly well tech company like any other company. So, I think that is one change that I have seen in the last three to five years. But otherwise, yes, the cash burn and competitive intensity in terms of advertising that has always been there, but now to some extent is moderating.

Jainil Shah : My second question is on the EBITDA margin, you are guiding for around 26% sustainable EBITDA margins. With operating leverage playing out in Suburban as well as Dr. Lal isn't it pretty conservative, because for the last probably two years we have been having digital spends as well. How much can the new lab investments probably offset the operating leverage? So, basically, how much reinvestment would there be in the business?

Dr. Om P. Manchanda: There are inflationary pressures, there is a cost of hiring, growing up, a lot of tech talent is required in the system, so all other costs are also going up. And I think many of you have asked this question as we go to Tier -III, Tier -IV towns, what happens to the cost structure, of course, that will have an impact and, in any case, we have not resorted to price increase. So, I rather would maintain the competitive advantage in terms of pricing and operate within range bound margins so that we are able to actually address a larger sort of a market segment. And not always resort to price increases.

So, our attempt has always been to stay very competitive, run the busi-

ness

efficiently and see that we are able to address all market segments. So, that's also driven, it's not that we can't increase margins, but I think it's important to see that can this Rs. 600 crore number move up very sharply when we are well positioned to gain much higher market share, because this industry is so fragmented while we may be the India's largest diagnostic company, but our market share still would be single digit so, how we go up is what probably our focus is.

And by the way, having higher margin you are also attracting more competition. So, you might as well be a range bound and then increase our top -line.

Moderator : The next question is from line of Karan Vora from Goldman Sachs. Please go ahead.

Karan Vora : Basically on volume growth can you highlight what is the patient volume growth for Suburban and Dr. Lal core business. So, just trying to get some sense as to whether most of the growth for Suburban is coming from volumes, or it is also price that is a meaningful factor, that is my first question.

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Bharath Uppiliappan : Our total volume growth for the quarter is 5.2%. However, since COVID is going away RT -PCR test has gone to a large extent. So, if we remove only RT -PCR, our growth for the quarter is 7.7% including Suburban.

Karan Vora : What would be Suburban's volume growth, Suburban base last year to Suburban this year and ex -Suburban the core business volume growth?

Dr. Om P. Manchanda: The numbers are not readily available, so we probably can offline share .

Karan Vora : My second question would be so on margins you previously highlighted to 26% as a sustainable range. So, with Suburban kind of ramping up over the medium term, so basically, should we assume that incrementally over and above 26% you would be investing back in the business or that flow through could still come in and we can probably see year -on-year margin progression to some extent.

Dr. Om P. Manchanda: I think directionally we will invest back into the business. See these fluctuations in the margins actually happen on a quarter -on-quarter basis. And I think, when we see them in Quarter 3 call, you will actually find a different picture. So, that's why, in the first half our margin is generally higher than the second half. So, you will see that trajectory going down. So, I think over the period, on an annual basis, we will try and manage this margin. And then if there is a tendency for it to work, we will invest back into the top -line, because for us growth is most important.

Moderator : The next question is from the line of Punit Pujara from Helios. Please go ahead.

Punit Pujara : Bharath, in your comments in the PPT you mentioned that the Key Account Management program has been showing some traction. So, could you elaborate a bit on that? And while you do that, could you just spell out the B2B revenue contribution of the consolidated topline in H 1 FY24?

Bharath Uppiliappan : The Key Account Management Program is aimed at our pickup point business, which is a B2B business, where we are trying to create segments and then give a solution for each of the segments in a very customized form. And even within those segments Tier -III customers there is an A, B, C kind of classification and give differential service. This has met with a lot of success, because we have been able to solve that particular sub -segment's problem in this very specific manner, rather than try and build an omnibus kind of solution. So, segmentation of B2B clients is

really what the Key Account Management program tries to do. Obviously, this is at a very rudimentary stage. We can go a lot higher on this count and we will polish up this program as we move forward. On the B2B volume, I think the number of contributions is around 25% to 30% give or take.

Punit Pujara : 25% to 30% as a percentage of revenue in first half of this Fiscal Year , correct?

Bharath Uppiliap

pan : Yes, give or take.

Punit Pujara : My second question is for Dr. Om, so it's almost two years now since we consolidated Suburban. So, what would have been the key learnings and some of the mistakes that you would want to avoid going forward while you prioritize inorganic acquisitions especially for the South region.

Dr. Om P. Manchanda: One learning is in one of event one must be careful, don't get carried away, we didn't realize the COVID will suddenly fall so sharply, so I think that's one. I think the second learning is that we must really know what is very strategic, for us

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Bombay, Maharashtra was very strategic. There are times when we get leads from other markets where we are already very strong so we can't afford to pay that kind of valuation that we have done in Western markets, because we are already very strong in the Northern and Eastern markets. Sometimes people tend to benchmark the same value because they expect the same valuation , so one then needs to be careful on that. And I think the third is some of the integration challenges that we faced post the acquisition that we would be really more sort of aware about this and try and prepare it before we do the transaction.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities. Please go ahead.

Rahul Jeewani : Now on this four -year value figures which you called out as 13%, if I dissect these numbers on an ex -Suburban basis then our ex -Suburban value figure is somewhere around 10.5% and if I also back calculate the volume growth for us on an organic basis that is still trending around 7%. So, both in value as well as volume figure our growth is still below pre -COVID level. So, pre -COVID FY17 to FY20 our volume growth was trending around 12% - 12.5% kind of a number which was driving a mid-teens kind of growth for us. So, we are still very far off as far as that mid-teens growth on an organic basis is concerned. So, can you call out in terms of why we are still not back to pre -COVID level growth ?

Dr. Om P. Manchanda: Yes, I think you are absolutely right, it's not only for us I think overall industry if you look at, I think the numbers that are in the public domain, so everyone's growth is below what they were trending before. But all I am saying directionally it is improving, but otherwise you are right, we are definitely not in line with what we used to grow earlier, which is true for the entire industry. I think the numbers of other companies that are in public domain same trajectory is there.

Rahul Jeewani : So, any reason in terms of whether organized players, because you have been pointing out that the competitive intensity in the space is coming down, we have taken price increases as well. So, with these two factors working in our favor, still the growth is not back to what we were seeing earlier. So, anything in terms of structurally which could have change or which is changing due to which the growth at an industry level seems to have slowed down?

Dr. Om P. Manchanda: I can give a slightly my own answer based on the experience that I have had. I think this industry has seen a lot of growth for organized players, because there was a shift from unorganized to organized in Metro, right in the last let's say 15 - 20 cities. And a lot of players have come in this area, the market has saturated. Now the next phase of growth is actually going to come from the next layers of town which is Tier - II, Tier -III, Tier -IV. Those towns actually we will see the same sort of thing, but they are in transition because doctor influence is still very high, direct -to-consumer is just taking shape. So, in my view it's maybe a market in transition but overall potential for the country is huge, because we are a 1.4 billion population country. So, my sense is that a lot of us grew up in large metros to start with and the base has grown bigger for many o

f us. So, that's why you will see a slowdown in growth.

And also, inability to take price increase due to competition etc., all that has contributed to a bit of a slowdown. But as we go deeper into Tier -II, Tier -III, Tier -IV towns I am hopeful that this market will again crack open in favor of organized players.

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Rahul Jeewani : Just one related question to that aspect, so you are talking about growth in some of these smaller markets. So, how do you see the healthcare infra in the smaller markets because what we hear from some of the other healthcare players is that the availability of doctors in Tier -III, Tier -IV towns is very limited, due to which if you look at the pharma companies many of these pharma companies approach the smaller markets towards say generic or a retail led distribution business rather than a doctor led approach. So, can the healthcare infrastructure in these smaller markets be a hindrance to your growth in these smaller towns?

Dr. Om P. Manchanda: So, let's start step by step first of all is there a need for healthcare . The answer is a big yes in these markets. Now earlier affordability could have been a challenge, but I think in relative terms as the country's economy grows, affordability is also growing up. I think the challenge would be basically access, I think ecosystem will find its own solution to cater to this market. I have a very strong sense that distribution of health infra is going to go down to pop -strata. I think Government is also giving a big push. I am very hopeful that investment also will go in that area because the market is probably getting ripe for it to be topped.

My other thing is access also will get improved through tech solutions. I see a lot of things that are happening in telepathology, teleradiology, tele -consultation. So, I think a lot of these solutions will come which will provide access to these Tier -III, Tier-IV towns. So, overall, I think everybody is eyeing these markets. So, even in hospitals also I think they may be done with the metros now they are going to look into Tier -III, Tier -IV towns.

Overall, I think healthcare will attract a lot of investment in these. Yes, I agree with the shortage of manpower is probably getting resolved through tech solutions, let's see how it goes but I am pretty hopeful that health infra will grow in these time s pan.

Moderator : Thank you. Ladies and gentlemen, we will take this as the last question for today. I now hand the conference back to the management for their closing comments.

Thank you. And over to you.

Ved Prakash Goel : Thank you everyone for being with us on this call today. I hope we have satisfactorily addressed all your queries. If you have any more questions, please feel free to reach out to us. Thank you once again and a very Happy Diwali and upcoming festivals too. I would now request the moderator to close the call. Thank you.

Moderator : Thank you very much. Ladies and gentlemen, on behalf of Dr. Lal PathLabs that concludes this conference.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure high level of accuracy.

Call: Feb 2024

February 07 , 2024

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Conference Call on Financial Results for Q3 & 9M FY24

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Conference Call on Financial Results for Q3 & 9M FY24 .

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
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Dr. Lal PathLabs
Q3 FY24 Earnings Conference Call
February 01, 2024

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Bharath Uppiliappan - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer
• Mr. Shankha Banerjee – CEO Suburban and other Group Companies
Participants who asked questions • Prakash Kapadia – Anived Portfolio
• Rahul Agarwal – InCred Capital
• Rahul Jeewani – IIFL Securities
• Omkar Kamtekar – Bonanza
• Punit Pujara - Helios
• Prashant Nair – Ambit Capital

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Moderator: Ladies and gentlemen , good day and welcome to the Dr. Lal PathLabs' Q3 FY24 Earnings Conference Call .

As a reminder , all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference , please signal an operator by pressing '*' then '0' on your touchtone phone. Please note that this conference is being recorded. I now hand the conference over to Mr. Siddharth Rangnekar of CDR India. Thank you and over to you sir.

Siddharth Rangnekar : Good evening, everyone and welcome to Dr. Lal PathLabs ' Q3 FY24 Earnings Conference Call.

Today we are joined by the Senior Members of the Management Team including (Hony) Brig Dr. Arvind Lal – who is the Executive Chairman , Dr. Om Prakash Manchanda – Managing Director , Mr. Bharat h Uppiliappan – CEO , Mr. V ed Prakash Goel – Group CFO along with Mr. Shank ha Banerjee – CEO of Suburban and the other Group Companies.

I would like to commence by stating the standard disclaimer here. Some of the statements made on today 's call could be forward -looking in nature and the actual results could vary from these forward -looking statements. A detailed disclaimer is also available in the results presentation that has been circulated to you earlier and is also available on the stock exchange website.

With that , I would like to invite (Hony) Br ig Dr. Arvind Lal to share his perspectives.

Dr. Arvind Lal: Good evening , ladies and gentlemen and a warm welcome to all participants on the call. We are gathered here today to delve into the Q 3 FY24 earnings of Dr. Lal PathLabs. I wish to begin by sharing my thoughts on the evolving market dynamics and the progress that we have made.

India 's diagnostic sector is undergoing a rapid transformation . Increasing burden of non-communicable diseases as well as new emerging and existing communicable diseases , increased adoption of evidence -based treatment and a growing emphasis on preventive healthcare diagnostics are expected to sustain the growth momentum . While the competition in this space has increased , the established pan-India chains like Dr. Lal Pat hLabs are strategically positioned to exceed industry growth , progressively expanding their market share compared to both organized and unorganized players. This is possible due to the continuous improvement in scale achieved through our deep penetration in Tier-2 and Tier-3 towns and offering the largest portfolio of tests and services at competitive prices.

Dr. Lal PathLabs has continued to be the preferred choice for customers seeking latest high-end testing and accurate diagnosis , over the last 75 years. Yes , this is our platinum year. For example , we have achieved a milestone in Histocompatibility and Transplant Immunology testing with Next Generation Sequencing or NGS by carrying out 600 runs of HLA-NGS with 27 ,000 samples analyzed for Bone Marrow and Solid organ Transplantation across India , Africa , Bangladesh and Nepal. We have also taken the lead in establishing 'donor derived cells free DNA ' as a non -invasive monitoring test for solid organ transplantation and have recruited 475 patients for post-transplant monitoring of which 222 patients are for kidney , 70 patients for heart and 18 3 patients for lung transplantation.

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To maintain affordability, we have predominantly retained our pricing mix , barring minor adjustments in prices to account for cost inflation in special ity and super special ity tests. Through this , we aim to cater to a larger patient base , solidifying our position as the leading diagnostic brand in the country.

Our performance going ahead will be guided by expanding our presence in the underpenetrated Tier-3 and Tier-4 markets , coupled with the targeted focus on high -potential markets in the west and south region

s. This is in addition to extensively

utilizing our digital infrastructure where we have made investments across AI , data analytics , machine learning etc. Not only will this enhance the overall patient experience and provide tailored solutions but will also accelerate the volume trajectory .

In the western markets, through Suburban diagnostics and especially its Reference Lab we are emerging as a formidable player , aiming to achieve rapid scale by outpacing industry growth. Our synergies are beginning to unfold, and we are on course to fortify our position in this market.

That concludes my initial remarks. Thank you and I would now like to hand it over to Dr. Om . Over to you , Om.

Dr. Om P. Manchanda: Welcome to our call today . I shall walk you through the evolving industry scenario and progress on strategy .

In the current financial year , at an industry level , whatever the impact of Covid -19 testing was on the revenue side has all gone away . We have also experienced the

same, therefore we stopped reporting the split since last few quarters. It is now only one figure that we report, thereby making the analysis easy and simple. Covid -19 also created an unevenness on growth pattern both on revenue and volume. That pattern is now getting smoothed.

Our business has delivered double-digit top-line growth rates on Y-o-Y basis both in the current quarter as well as on YTD basis. We have significantly improved our operating and net margins profile driven by efficiencies of scale and productivity initiatives. Our efforts continue to look for expansion in Tier-3 and Tier-4 towns through organic means of growth. Our teams have carefully identified pockets and the markets that will drive future growth for us.

Industry experienced bundling of routine tests as a new consumer trend. We led this trend and successfully built a brand – Swasthfit. The revenue contribution from Swasthfit has now stabilized, ranging between 19% to 21% of our total revenue. The next wave of bundling, we believe, would be in the specialized test as that could be medically more efficient for clinicians.

Our investments in technology and digital at the infrastructure level, at the front end and in the interactions with patients are also translating into salience. We are pushing ahead with some of the brand and chosen geographies, while meticulously executing a network and test menu expansion agenda. With that, I would like to hand over to Bharath to continue the conversation. Over to you, Bharat.

Bharath Uppiliappan: I warmly welcome you all on this call today. I will now take you through the business and operating highlights.

I am pleased to share with you that we have delivered yet another quarter of double-digit total revenue growth and a healthy margin profile. In Q3 FY24, we clocked a revenue of Rs. 539 crore representing a growth of 10.1% by serving 6.7 million

patients and testing 18.6 million samples, representing a sample growth of 7.1% over last year.

The RPP for Q3 FY24 is at Rs. 807, a growth of 1% over Q2 FY24 mainly due to mix.

Further, to accelerate the patient volume and revenue stream as a team we are sharply focused on putting into play a white space expansion plan. Last quarter, we shared that we have put in place the program for 20+ new labs expansion. This plan is progressing on track and parallel to the creation of collection centers and pickup point ecosystem is also tracking well.

During COVID, a trend of acquiring and serving patients digitally emerged, popularly known as D2C. I am pleased to share that we have in a focused manner developed end-to-end D2C capabilities starting from digital marketing to lead generation to servicing the patients at home. We are now expanding this D2C program to Tier-2 and Tier-3 cities as well.

Our key product programs like Swasthfit contributing to about 20% of revenue. Med

ical centers of excellence like LACE and LCoRD continue to fortify their position amongst the medical community and patients.

In addition, digitally enabled initiatives like CHIPS and recommendation engine continue to make meaningful contribution in enhancing patient loyalty and stickiness. Overall, we are moving the right levers to optimally set a growth trajectory that will give us sustainable growth.

With that, I would like to invite Ved to take you all through the financial performance. Over to you, Ved.

Ved Prakash Goel: Hello everyone and thank you for joining this call.

I am sharing the key Financial Highlights for Q3 and 9 months:

- Revenue for Q3 FY24 came in at Rs. 539 crore against Rs. 489 crore last year same quarter, a growth of 10.1%. YTD revenue for 9 months is Rs. 1,681 crore Vs Rs. 1,526 crore in last year same period, a growth of 10.2%.
- EBITDA for Q3 FY24 came in at Rs. 141 crore as compared to Rs. 113 crore in Q3 last year, registered a growth of 24.6% with EBITDA margin of 26.1%. 9 months EBITDA is Rs. 465 crore Vs Rs. 374 crore, last year same period with a margin of 27.6%.
- PBT for Q3 FY24 came in at Rs. 116 crore, registered a growth of 51.6% with PBT margin of 21.5%. 9 months PBT is Rs. 386 crore with a margin of 22.9%.
- PAT for Q3 FY24 came in at Rs. 82 crore Vs 54 crore last year same quarter, registered a growth of 53.3% with PAT margin of 15.3%. 9 months PAT is Rs. 277 crore with a margin of 16.4%.
- EPS in Q3 FY24 is Rs. 9.8 with a growth of 53.2%. 9 months EPS stood at

Rs. 32.9.

- Cash and cash equivalent as on December '23 is Rs. 957 crore. Net of borrowing it is Rs. 853 crore.

At last, we are pleased to announce that the board of director has approved a second interim dividend of Rs. 12 per share. With this, total dividend for the year 2023 -24 stood at Rs. 18 per share.

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That brings me to the conclusion of my opening remarks, and I would now request the moderator to open the quorum for Q&A.

Moderator: The first question is from the line of Prakash Kapadia from Ani ved Portfolio Managers Private Limited.

Prakash Kapadia: A couple of questions from my end, diagnostic industry growth has definitely slowed down from earlier pre-Covid days. So, what is our sense, is it customer fatigue? Is it something else which is affecting volume growth for the sector? We have obviously grown at 10%. But this is on a 9% base of last year. So, any sense you could give us for the industry?

Secondly, if I were to envisage higher growth from hereon for FY25 and beyond, so for us what would drive that? Is it Suburban scale? Is it increased contribution? Is it newer geographies where we have been pretty aggressive in Tier-3, Tier-4 cities where we have an early mover advantage? Can specialized test increase? As you know the price rise base is also now anniversary is happening in February so, what will drive growth from here on? And lastly, on Suburban what is the game plan? Because now centers are added, labs have opened, Reference Lab is done, the integration of infrastructure is done. So, can we expect much higher growth in Suburban from here on or not? Those are my questions.

Dr. Om P. Manchanda: You have asked 2-3 questions. So maybe I will first pick the industry part then probably give the other growth and Suburban questions to my colleagues later. I think on the industry front, you are absolutely right that the growth rates of all the companies which are the figures are in public domain. If I add it up, industry growth rates don't seem to be adding up to what we were having pre-Covid levels. I think what you just said is very right. In fact, we are industry-leading growth because double digits many of the other players we see are in single digits.

Prakash Kapadia: Very low single digits.

Dr. Om P. Manchanda: Very low single digit. Yes, and I think in fa

ct our number is actually all inclusive. If

you take their numbers all-inclusive then it is even contrasting now. I think earlier all of us were attributing this to new-age competition. We were thinking that probably it's losing out to that even. They also seem to be in some difficult situation though I don't have any data to support that. But I think as we travel in the market and overall, in general, that intensity which was there 2-3 years back is not there anymore.

The question arises what is going on? One hypothesis that we have probably been that fatigue factor that you talked about is during COVID times there's a tremendous amount of testing and a lot of people went for health checkups etc.

There is a bit of a pause because this is not a want that you want to get tested every now and then. It's a bit of a need base and then suddenly you say listen I just got myself tested last year. Then can I give it a pause because there's too much testing and healthcare and all being talked about. So, there could be one qualitative reason but there is nothing that supports this argument. Second also could be a lot of these bundling things that we have done, like say for example we were discussing earlier within us is that this fever panel is one test and one visit. Now in the earlier years, this fever panel may have been multiple visits because you test for dengue and then you test for malaria you test for something else. Now you want to get everything tested in one sample itself. So that probably could be there could be a correlation.

While one has seen in the industry revenue per patient going up, but volume growth is not that high. I think revenue per patient for most organizations has been significantly higher, but volume growth has not been. So, could the bundling be causing this less frequency of visit? The third hypothesis could be that many of the

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hospitals have woken up to this idea of how they keep their captive business intact and not let this leak out to private players. At least this is someone's phenomena that I've seen in larger cities where probably some of the hospital chains have become a little more aggressive in retaining their OPD business for diagnostics as well. So, I think on the whole this is what probably I can think of. Now I think the million-dollar question is that going forward do we see the softness, or do we see this reviving, my personal view and I think I've always maintained that is that diagnostic is the important part of any medical practice. Without this, actually to my mind there is no

medicine. And these days wherever I go without prescribing a test, doctor s really don't shift to medicine. I think the next phase of growth actually lies as we go down the pop setup. A lot of action has been in the diagnosis in the larger cities. I think we just need to go down and probably trigger more shift from unorganized player , trigger more sort of prescription habits of higher order test. So , I think that is where the industry growth lies and we have been talking about this Tier-3, Tier-4 towns now for last three -four quarters and we have separately started tracking this data. So , we are putting a tremendous amount of focus in these towns , technically speaking going deeper into our core markets. The idea is to really increase market access , get more patients by getting deeper and wider is where we are . Now other questions related to Suburban, and I think you asked something else as well. I 'll hand it over to Bharat h to respond.

Shankha Banerjee : On the Suburban question, I think like you rightly said that the reference lab has been set up. It 's almost a year. The infra expansion of CC is a ctually not only expansion we are also looking at strengthening the way that channel is being run. I think quite a lot of work on that has happened and to cut the long story short yes , growth rates will

improve in the future. That is really what we are working towards. There was something on Dr. Lal PathLabs . I think Bharat h will take that question.

Bharath Uppiliappan : That Dr. Om address ed on Tier-3 and Tier-4.

Prakash Kapadia: So, in terms of the growth drivers if any I was just trying to understand , will Tier-3, Tier-4 be the biggest driver or will specialized test drive this , increased worse contribution looks unlikely to majorly drive growth from here on or it 's going to be Suburban scale which will grow now at a much faster pace. So , the overall revenues could be significantly higher or much better than what we are currently trending is what I was trying to understand .

Bharath Uppiliappan: So, I think what we have said so far is that we will significantly scale up a Tier-1, Tier-2 presence led by Suburban obviously in Mumbai and some parts of Maharashtra.

It's a w ide space for us. We will build an aggressive presence in Tier-1, Tier-2 cities in that fashion. The second thing we have said so far is that Tier-3-4 is where Dr. Lal PathLabs in the core markets will focus to drive its growth from. From a product point of view , Dr. Om mentioned in his opening speech that apart from doing bundling on Swasthfit we are now looking at bundles on the higher end specialty tests also. So , you must see as product innovation continuous pipeline as other source of growth.

In my opening remark , I mentioned D2C as a capability we have built in. So , we are building multiple other capabilities in the company to drive the channel growth including PUP, key account management etc. So , there are three verticals , the Geographic Expansion is one vertical , Channel is one and Product is the other one.

Prakash Kapadia: You are saying all of this should at least directionally looks like we should head for higher growth.

Bharath Uppiliappan: Yes, we are trying the best we can to get the numbers moved up from where it is.

Prakash Kapadia: Yes, despite a low base.

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Dr. Om P. Manchanda: No, low base for Q3 always remains low base. So , I don 't think that is because Y-o-Y is the best way to look at. In fact , this quarter number by the way is very similar to what we did in Q 1 and if you look at historical trend Q1 and Q3 being very equal. So , to that extent it 's not below.

Prakash Kapadia: I'm saying industry , obviously doctor has slowed down. What I was referring to the low base was last year in the base of Q3 there was a 9% growth in non -Covid revenues. That is what I meant. From that perspective, despite being 9% non -Covid revenues , our overall revenues are up 10%. I 'm just trying to figure out what is that missing link because everything seems to be there but it 's not translating one for the industry. Obviously , we are doing better than the industry but looks like there is a large potential and the scientific based or evidence -based testing because anywhere you talk , go talk to friends , fraternity without a report no doctor is willing to give a medicine. So somewhere I was trying to understand what I am missing .

Moderator: The next question is from the line of Rahul Agarwal from InCred Equities.

Rahul Agarwal: I have four questions. I 'll try to be very quick. Firstly , on this Tier-3, Tier-4 cities. Dr.

Om basically the management commentary around this has obviously increased as you also alluded that last 3-4 quarters , we have been highlighting this. I wanted to understand what does management really expect in terms of the change in business when we started tracking this ? There are two things here , one is purely in terms of operations , how does it change your daily routine in terms of the entire effort of capital allocation and sales force being focused on Tier-2, Tier-4?

Secondly in terms of financials as analysts and shareholders what should we focus on when this business actually sees higher growth in Tier-3, Tie

r-4? It could be

revenue mix, it could be more B2B, it could be higher working capital, it could be lower margins, anything. And related question to that also was that the doctor influence in these cities according to me should be even tougher to break in Vs. what we see in metros. So, your thoughts please.

Dr. Om P. Manchanda: So, I think one of the hypotheses that we have is that our strong areas are UP, Bihar, Orissa, the rest of north like Punjab, Haryana etc. We have a great sort of franchise in larger cities. And if you look at the population of this block, I'm told UP alone is nearly 20+ crore. It's a huge market and there are 75 districts and 335 tehsils. Even in all the districts we still don't have labs. And these districts actually they don't fall in big sort of population numbers. So, the idea is how do you go deeper in these markets and there's so many businesses lying there. So, this Tier-3, Tier-4 idea comes that if we take larger test menu to district headquarters, so that will give us the ability to go deeper.

If you look at it, Government medical infrastructure also operates something like that. You have a district hospital which have very large patient flow coming in and if you analyze that flow, actually it's all building it up from ground up which is like Tier-3, Tier-4 villages etc. So, we just want to actually follow that pattern and go deeper and leverage our strength in these places because our brand is very strong. Now obviously this Tier-3, Tier-4 strategy will not work in the South and West market because there even in metro and Tier-1 town itself our presence is weak. So that clearly falls in this bucket of looking at inorganic opportunities. That's the whole idea of Tier-3, Tier-4. Now I think your next question was how do you look at from working capital perspective, operations perspective and infrastructure? I think from a financial perspective, I don't see any difference between what we do otherwise and higher folks on Tier-2, Tier-3. I know the realization would be lower as we go down the pop setup, so would also be the cost as well because many of these tests that will come from these places will be routine in nature. And routine testing is not in the range of 20%-21% of the revenue but it's more in the range of 15% odd. So, to my mind the

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financial profile of the P & L and the balance sheet may not change. Operationally yes, it's a bit of a challenge for us because you are technically managing a network of business which is much more spread out than what it is today. But that I think has been our differentiation as well because that is why we have grown the way we've grown in the past mainly because our management capability of managing remote infrastructure and do it in a quality manner. So, I think operationally there is a bit of a change for us because we will have to really get the management more decentralized than running it from the corporate office is what I can think of.

Rahul Agarwal: Anything on the doctor influence?

Dr. Om P. Manchanda: Doctor influence, I think that piece is where I would say we will have to use the whole D2C approach and brand building approach. My personal experience has been as we expanded out of Delhi NCR, went into other markets, it's definitely a long-drawn process but over a period of time if we have a reason to communicate with the consumers which is more in the form of service, more in the form of health checkups, I have a sense that once brand crosses a particular critical mass, it becomes difficult for doctors to de-sell you because if everybody is using Dr. Lal PathLabs if some doctor says don't go there then doctor runs the risk of losing his or her own credibility. So, I think we know the entire brand-building process. It takes a bit of time but that is what we will try and do as we go forward. But obviously we will have to

offset the

doctor influence and build a Direct-to-Consumer brand.

Rahul Agarwal: One more thing was this 32% to 34% of total top line which is coming from these markets. I see that number in a very narrow band over the last 4 years. I don't have the 9-month number but when I look at 19-20 what you have disclosed, it's not really moved ahead. So, anything different you are doing this time around that this number, will that actually track higher going forward?

Dr. Om P. Manchanda: I think maybe this is the way our internal number is right now. We are giving more focus in the system. That is why it's coming. I think the first stage is for us to know where we are. The second is if the growth rate in Tier-3, Tier-4 is higher than other places, the answer is definitely, yes. Now, I think our teams are aiming that how do we accelerate their growth. So that's the whole idea. I think it just tells us that we have a long way to go. Yes, you are absolutely right that it may not have moved much, it's still moving within the narrow range, but it needs to move up. That is what the whole idea is.

Rahul Agarwal: Secondly, I wanted to discuss one new topic with you and the team because this business is very largely dependent on franchisee. I don't know if the metric called Revenue per Franchisee really matters and I will give you the context. What I'm

thinking is because the growth is really not picking up as what we all want to mid-teens. And my understanding is Dr. Lal has a very decentralized franchisee network which basically means that each franchisee is running at max three centers maybe nationally. Would it be a good idea to track revenue for franchisees? That is one question. And secondly would it be a good idea to basically aggregate and have larger franchisee partners so keep them motivated, compete better, commit more capital and effort to basically get the growth higher?

Bharath Uppiliappan: We have a metric called Revenue per Franchisee and we track it internally and our regional teams do that because underlying is the profitability of the franchisee. We are very conscious of that. Second thing I want to just say that is that we have got about 300 to 400 odd multi-franchise operators with us. So typically, it works well in larger cities where we can put up same person and put up three four shops and then kind of supervise them. Beyond that, since we have a lot of other smaller town operations, one person doing multiple centers from a quality of talent management bandwidth etc. that becomes difficult for him. And that is our observation. We

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encourage multi-year franchising which is we haven't stopped people from taking multiple centers. We got people who have gone up to 6-7. Our view is that beyond 7 it becomes difficult to manage, the geographic dispersion, people rotation etc. But yes, it is an important angle of how we build franchising in the future.

Rahul Agarwal: So, it does not really make any difference and there is no thought internally that we should do this kind of things to basically get growth up. It doesn't really help, right?

Bharath Uppiliappan: We track profitability which is a key metric and as long as we manage that then we are okay with that.

Rahul Agarwal: And the revenue per franchisee trend over the last post-Covid last 3 years, how is that behaved, according to me it should have been declining but your thoughts?

Bharath Uppiliappan: No, it doesn't decline because of the bundling effect, the bundle test they make more money on that. So, on a retained revenue basis they are making more money.

Rahul Agarwal: Seasonally Third Quarter has never seen such high gross margins. I think when I look at the past 4-5 years history, Q3 generally you see a drop in gross margins Q-o-Q, what was different this time?

Ved Prakash Goel: No, as Dr. Om mentioned in the opening speech, the operatin

g efficiency and

obviously the mix is also which sometimes create that. You are right in the past, there may not be that kind of margins. But yes, what we are doing is, right now probably you have seen which has improved some gross margin this time.

Dr. Om P. Manchanda: No. I think just to add to what Ved said is that this year definitely a lot of initiatives we took to improve efficiencies on both purchase, consumption, consumables and all that has come and baked in these numbers that is also showing up.

Ved Prakash Goel: But sequentially if you see Rahul, it is not very different from Q2-Q3.

Dr. Om P. Manchanda: Revenue was down, so he was expecting lower gross margins, right?

Rahul Agarwal: Yes, I was expecting a lower number.

Dr. Om P. Manchanda: You should be happy.

Moderator: The next question is from the line of Rahul Jeewani from IIFL Securities.

Rahul Jeewani: Now if we look at our value growth for both the Third Quarter and 9M, our value growth has been 10%. But if we disaggregate that between volumes and realization, then the volume growth both for the Third Quarter and 9M seems to be between 2.5% to 3% kind of a number. The volume growth in our portfolio both for the Third Quarter and the 9M has been lower only. So, can you comment in terms of slim growth settling over the next four quarters and are you planning to take another round of price increase?

Bharath Uppiliappan: Let me take the second question. We are not planning a price increase as of now, but we keep the options open. On the volume front yes, it has been a bit softer than what we had anticipated. What we are trying to do is to revise the volume growth by going deeper in the geographies of Tier-3, Tier-4. We're trying to strengthen the Suburban business so that Mumbai, which is a Tier-1 city we start to get a better presence and get a bit of medical equity also going by the new centers of excellence. So, there are a host of initiatives we are taking as a team to drive the volume growth up and we are hopeful that this will start to turn around in some time.

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Rahul Jeewani: If you are not planning to take price increase this year and if we don't see much of an acceleration in volume growth next year, do you think that we can sustain this current growth going into next year?

Dr. Om P. Manchanda: No, I think that's mathematics. So, the idea is our pricing is the last resort. We don't want to use pricing as a lever to show higher revenue growth. We definitely want to make sure that the volume is the lever to show higher revenue growth. There's no

doubt about that and we don't want to also get into a situation where we keep increasing prices and start losing our competitive advantage. We just took price increase last year. As Bharat h mentioned , that as of now there's no plan . If the situation demands if we are not able to cover up our cost through higher gross margin, then we'll consider this . Because last year price increase was not taken on the entire portfolio. It was only taken on the part of the portfolio. We still have a possibility of taking other part of the portfolio, but we will use that as a last resort because we want to stimulate volume growth. That is very clear in our mind.

Rahul Jeewani: Since you pointed out that you are trying to accelerate your growth by ramping up Suburban , so can you call out the growth for Suburban this quarter both in terms of value and volume? I'm assuming that the value growth in Suburban would be higher than the 10% number which we have delivered at an overall company level.

Dr. Om P. Manchanda: I think Suburban though Shank ha mentioned. But last time also I think I talked about this. We are actually doing a very strategic shift in Suburban . The shift is basically driving growth through channel collection centers. And we are also doing a strategic shift from our B2B doctor driven b

usiness to Direct to Consumer . So, there are early green shoots where we are seeing this strategy is working. I think you will have to give us a little bit of few more quarters to actually start sharing these numbers in much more detail. Right now , I think numbers are still subdued but we are very confident that this whole franchisee network driven Direct to Consumer business should work for us and will give us long term sustainable advantage. Because I think Rahul or somebody asked about this doctor influence , we want to really come out of that mode and drive direct to consumer growth. So , give us few more quarters to talk about Suburban as we go along.

Moderator: The next question is from the line of Omkar Kamtekar from Bonanza.

Omkar Kamtekar: First question that I wanted to ask was what was the Free Cash Flow generation for the quarter and for the Nine months ?

Ved Prakash Goel : We have Rs. 853 crore net cash in our books and for 9M we have added about Rs. 250 crore roughly.

Omkar Kamtekar : What is the FCF generated for the quarter.

Ved Prakash Goel : For the quarter it is roughly Rs. 116 crore.

Omkar Kamtekar : If we calculate the Revenue per Patient , the revenue per patient for the year YTD 9 months is Rs. 796, so let's round it up to Rs. 800. Other than the change in the mix , what are the drivers that we can see that this revenue per patient can increase?

Because it has been very steadily increasing. It's now close to Rs. 800. What do you think it will take to reach maybe Rs. 850 and higher levels?

Dr. Om P. Manchanda: I think there are some things that have helped as you rightly mentioned , one is Swasthit package , price increase , mix change , all that has contributed to this. Now going forward, we clearly foresee that this bundling of high -end tests is a low-hanging opportunity for us, and I think should drive Revenue per Patient. But I won't change
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this number as much as the volume growth because I know if I go down the pop strata, go to smaller towns that may pull this number down as well. So, as a metric I think to me this could be a side effect rather than the main effect that we will probably drive. Our focus essentially would continue to remain on two things is that how do we get more number of tests per patient and how do we get more patients? That's it. Rest I think this P & L will take care of itself.

Omkar Kamtekar : On that point, a nswering in the previous question you said that we are focusing more on the B2C segment and trying to get the customer more aware about our offerings .

Dr. Om P. Manchanda: Let me correct this. I think the whole B2C or D2C idea is how do we keep the brand as top of mind. I would actually say that we want to keep the top -of-mind awareness of Dr. Lal because as and when the need arises the person should think of Dr. Lal PathLabs rather than any other brand. And that can be served through many other channels. It can be served through even hospital also. No problem. It can be served through a smaller lab also as long as the person is very keen to have a report from Dr. Lal Path Labs in his or her access.

Omkar Kamtekar : So, it's like we are trying to position ourselves as the product leader , service leader . So, with that in mind are we looking at , for example with respect to the other players that are there in the industry what they are doing is they are entering into contracts or maybe MoUs with these smaller town Tier-2, Tier-3 hospitals , district hospitals , etc., and they can get a captive audience right from Day1 . The volumes are there for them because the people are there right from Day1 everybody is going to these specific hospitals in their regions. So, is that something of a route that we may go down? Maybe we can even approach Government hospitals for that matter so that the volume that we have been missing could be brought on board. Is that something

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at could be taken up?

Bharath Uppiliappan : Let's put it this way that we will look at all channels as long as we are assured of let us say the volume and also the payments coming in through . Government contracts we evaluate very carefully because there are multiple angles to be taken care of. So, we have been kind of present in all the areas where the Government hospitals are. We do have contracts with the Government on many fronts. So as long as we look at the total commercial package and it makes sense for us, we will definitely do those agreements and sign-ups. But if they don't , then we will rather walk away from those opportunities.

Dr. Om P. Manchanda: I think we need to be careful about , are we managing the optics of volume growth or are we exploring long term sustainable advantage? If we are to manage optics of volume growth there are multiple ways to get this volume so we can actually tomorrow run a huge sugar camp and you will have la khs and lakhs of customer coming in. So, I don't think that is what we want to do. We want to make sure that whatever we do is sustainable and profitable.

Omkar Kamtekar : On the cash flow question , we have approximately Rs. 800 odd crore worth of cash in the books. Any color with respect to any acquisition in the pipeline or maybe something that is on the block. Not that I ask you to disclose it but are we looking for any specific acquisition or something so that we can utilize this cash?

Dr. Om P. Manchanda: I agree with you actually our strategy is fairly clear which is that we want to have market access and that can come through both organic which is what Tier-3, Tier -4 we keep talking about , will come through organic means in our strong markets and other markets it's all M &A. I think the last M &A that we did was Suburban 2.5 years back and our thesis is that okay if you do 10 M&As, 3 -4 may not work but the other 4-5-6 which will work will take care of the failures. I think the last 3 years' valuations had gone through the roof and expectations of even private players was in line with Page 12 of 14

the sort of public market multiples which we can not give. And I think now with a smoothening of curve, and I can see the numbers of other companies as they are getting declared. My sense is that the P &L of many of these companies will come under a bit of stress. And hopefully this whole M &A piece should open up and we are open to doing M &A in those markets where we need it.

Omkar Kamtekar : Have we scouted any candidates ?

Dr. Om P. Manchanda: Right now , I don't think we have anything that can be shared with anyone, but efforts are always on.

Moderator: The next question is from the line of Punit Pujara from Helios Capital .

Punit Pujara: If I look at margins on a 9M basis be it gross margins or EBITDA margins so gross margins are expanded by 2% points and EBITDA margins 3% points. And Dr. Om, I remember in the last call you mentioned that having higher margins you also attract more competition. So, the question I'm coming to is, is there room for you to reinvest these margins to accelerate volume growth which I believe comes out from the call that's a key metric you want to improve.

Dr. Om P. Manchanda: Sorry I missed the last line.

Bharath Uppiliappan : Is there a way to reinvest the extra margin?

Dr. Om P. Manchanda: I think we have done that piece quite a bit. While I don't know whether line-wise items are visible to you or not . We have invested in 2-line items aggressively. One is our A&P which has significantly gone up what we have done in the last couple of years.

Ved Prakash Goel : And IT expense.

Dr. Om P. Manchanda: There was one more thing I think on the technology front we are actually a tech company. A lot of investment has gone in these areas. Maybe one area that probably we would like to invest is go deeper and spend on bit of a Lab infra in Tier

-3, Tier -4

that we have repeatedly talked about on this call because sometimes it's very difficult to actually know what all these efficiency efforts will lead to since we believe we have a buffer and so we will start investing. And I think some of those efforts have already started in the second half of this financial year. But to reassure you that we want to invest for growth. We are not here to just maximize the margins and slow down. The idea is to really reinvest some of this profitability back into the top-line growth and that too more on volumes.

Punit Pujara: Dr. Om, you also mentioned about driving a durable growth which in my opinion is through number of patients increasing but on 9M basis , patients have increased by 2.5% odd and your number of tests have grown by 8% which implies that this bundling of test is working really well. But organic patient volume growth in your mind will that be a right metric for that durable growth that we just referred to?

Dr. Om P. Manchanda: Actually, I know we have always talked about patient growth , volume growth. These are actually not patients. They are actually patient visits. And what we are little unsure about that whether all the efforts of bundling , etc., or fever panel , etc., are leading to reduction in patient visits. You understand what I mean?

Punit Pujara: Yes.

Dr. Om P. Manchanda: So that piece actually we are not able to establish through data because it is not a regular purchase. Patient comes to us only when that person needs us. So sometime

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that patient can drop off. It's not that the patient has dropped off because he or she has gone to competition. Patient has dropped off because that patient now is cured and does not need us and will come back maybe after 2-3 years whenever the need arises. So, these are all patient visits. So, I think we are also parallel ly tracking. We are not in a position to share that data because we do not have the capability to get the exact number. But from a trend perspective we are constantly looking at what's happening to unique patients rather than looking at just patient visits. And also, please remember these patient visits as we are talking about , I think 3 quarters back were also in a trajectory where it was negative growth by the way . We may have forgotten this. So, to that extent actually we are seeing , I have the data in front of me. Q2 FY23 -2.3% , Q4 FY23 -4.8% , Q1 FY24 -0.9%. I think on the back of that negative growth having these 3 quarters positive also is a positive sign I would say. So, we are improving. We are still not there but I think there is a slight improvement on these numbers.

Dr. Om P. Manchanda: And things have done exceedingly well on sample growth. So, this growth of revenue is not only driven by pricing . It is driven by samples growth which is more testing also.

Punit Pujara: This Tech investments that you referred to will enable you to track this unique patient's visit in a better way I believe .

Dr. Om P. Manchanda: Only in one segment because some segments like which comes through hospitals , which comes through even collection center s also a limited way, but which is coming directly to us that we can track definitely.

Moderator: The next question is from the line of Prashant Nair from Ambit Capital.

Prashant Nair: Dr. Om, I think you partially addressed this but if you could elaborate a bit more .

When you talk about maybe increasing lab infra to target Tier-2, Tier-3 cities , is there any way you can quantify either number of labs , or if you could even give qualitative color on will these labs be similar to what you currently run in the bigger cities , or will they kind of have slightly more limited test menu , will the number of spokes they can support or number of connection centers they can support be higher , lower than what you have currently? I mean is the nature of the network likely t

o change or is it just

adding the way you have been adding so far?

Dr. Om P. Manchanda: If you study our infra , we went through a stage where there used to be Satellite Labs and there was a Central Lab, if you go back 10 years . And now we are sitting on a format where there's an in between layer called Hub labs. These Hub labs are more similar to Central Lab but they are still not Central Lab. So, I think it's more a colonization of that cluster where you have put already a Hub lab, let's say , in a city like Varanasi or city like Kanpur. Now you want to go deeper in that cluster with satellite labs. Obviously , the answer to your question is that this will have a limited or may not be that wide a menu but as the volume picks up the menu will also expand. These labs will act more as a feeder to the big lab that we have in the region. So, it will have a limited menu.

Prashant Nair: Is there any target in terms of setting up XX number of labs .

Dr. Om P. Manchanda: We are in the process of identifying those markets .

Bharath Uppiliappan : We have done 20 , announced 20 now .

Dr. Om P. Manchanda: We have already said that we will do 20 labs , by when will that be ready?

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Bharath Uppiliappan : March -April.

Dr. Om P. Manchanda: By March -April, 20 more will be ready.

Bharath Uppiliappan : We are actually in the process.

Dr. Om P. Manchanda: 20 labs are right now in the works and by May when we come for the next results , those labs will be up and running. While having said that we are also doing planning for next year because we want to identify the potential towns for us and the support system for these labs and then maybe at the next meeting we'll be in a better position to highlight those. But I think directionally , you should get an idea that we are expanding our market access going deeper.

Moderator: Ladies and gentlemen , we will take that as our last question. I would now like to hand the conference over to the management for closing comments. Over to you sir.

Ved Prakash Goel : Thank you , everyone , for being with us on this call today. I hope we are able to answer all your points satisfactorily. Please feel free to reach out to us in case you have any further questions. Thank you once again.

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Call: May 2024

Classification: Internal
May 15, 2024

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q4 & FY24 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q4 & FY24 Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 16
Classification: Confidential

Dr. Lal PathLabs Limited
Q4 FY24 Earnings Conference Call
May 10, 2024

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Shankha Banerjee - Chief Executive Officer (Designate)
• Mr. Ved Prakash Goel – Group Chief Financial Officer
Participants who asked questions • Binay Singh - Morgan Stanley
• Amey Singh – JM Financial
• Kunal Randeria – Axis Capital
• Nitin Agarwal – DAM Capital
• Pranav Chawla – Antique Stock Broking
• Prakash Kapadia – Spark PWM
• Anshul Agarwal – Emkay Global
• Jainil Shah – JM Financial
• Shaleen Kumar – UBS Securities India
• Dheeresh Pathak – Whiteoak Capital
• Sumit Gupta – Centrum India

Classification: Confidential Moderator: Ladies and gentlemen, good day, and welcome to Dr. Lal PathLabs Q4 FY24 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing star, then zero on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Siddharth Rangnekar from CDR India. Thank you, and over to you, sir.

Siddharth Rangnekar: Thank you, Michelle. Good afternoon, everyone, and welcome to Dr. Lal PathLabs Q4 FY24 Earnings Conference Call. Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal, Executive Chairman; Dr. Om Prakash Manchanda, Managing Director; Mr. Shankha Banerjee, CEO (Designate) and Mr. Ved Prakash Goel, Group CFO.

I would like to share that some of the statements made on today's call could be forward-looking in nature and actual results could vary from these results and forward-looking statements made. A detailed statement in this regard is available in the results presentation, which has been circulated to you and is also available on the stock exchange website.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspectives. Thank you, and over to you, Dr. Lal.

Dr. Arvind Lal: Thank you, Siddharth. Good afternoon, and a warm welcome to all participants on the call. We are here to discuss the Q4 FY24 earnings performance of Dr. Lal PathLabs. I would like to commence by discussing my insights into the changing market dynamics and the advancement achieved at Dr. Lal PathLabs.

As we approach our 75th/Platinum anniversary this year, Dr. Lal PathLabs continues to be a trusted healthcare partner, renowned for its commitment to quality, accuracy, accessibility and affordability. Our marketing campaign, "Bharat Ka Vishwas", exemplifies this dedication and reinforces our position as a cornerstone of nation's diagnostic landscape.

Our relentless pursuit of excellence in diagnostics has earned us numerous complements and the confidence of patients and physicians alike. We remain focused on deepening our presence in underserved Tier -3 and Tier -4 markets, while leveraging our robust digital infrastructure to differentiate ourselves and achieve significant milestones in network rollout. The recent introduction of a custom-built logistics solution further enhances our ability to serve our customers better.

The integration between Dr. Lal PathLabs and Suburban Diagnostics continues to yield promising results in the Western market, where we have established a growing presence. We are making targeted investments in both marketing and process efficiencies to strengthen the brand while elevating the patient experience. Our reference lab in Mumbai has begun to receive increased volumes of various clusters across Maharashtra for both routine and semi specialized tests. Our collaborative approach is enabling us to combine inherent strengths and provide patients with a wide array of tests across multiple categories with speed and precision.

Looking ahead, our growth trajectory will be fueled by the ongoing shift from unorganized to organized, more awareness on accurate diagnosis as well as focus on wellness portfolio, among others. We remain committed to expanding our network and elevating our service standards through operational excellence and technological advancements. Despite increased competition, we are strategically

Classification: Confidential positioned to deliver sustained growth and further consolidate our market share, driven by our comprehensive test portfolio and competitive pricing strategy.

That concludes my initial remarks. Before I hand over, I would like

to extend a note

of appreciation for Bharat and wish him well in his future endeavors. As you know, we will be having Shankha Banerjee, leading the business as CEO, going forward, and he is here today. I welcome him into this role.

Thank you. I would now like to hand over the floor to Dr. Om Manchanda. Over to you, Om.

Dr. Om P. Manchanda: Thank you, Dr. Lal, and welcome all of you to our call today. I shall walk you through the evolving industry scenario and progress in our strategy.

I'm happy to announce that we have achieved double-digit top line growth on a year-on-year basis. This growth has been accompanied by a significant jump in our operating and net margins, primarily driven by efficiencies of scale and productivity initiatives.

Our strategic focus continues to remain widening and deepening the brand's presence geographically, thereby driving sustainable volume and value growth.

There is also a sharp focus on expanding into Tier -3 and Tier-4 towns organically in LPL core markets. And in the rest of North India, we have made good headway with this strategy. Additionally, we are also establishing 20 new hub labs under the DLPL brand name to broaden our reach. As the year progresses, we shall undertake to add more such hub labs.

We have identified key growth areas, aiming to strengthen our presence in Mumbai and in select parts of Maharashtra via Suburban. In order to boost momentum in Suburban Diagnostics itself, we are promoting higher sampling and enhanced branding.

Our 'Swasthfit' brand, now a consistent revenue driver, comprising 22% to 24% of the total revenue is being extended to many other markets. We are moving towards bundling specialized tests as well. The medical centers of excellence that we have developed are scaling well, rendering better patient engagement and outcomes. Being guided by a strategic focus on sustainable volume growth, these initiatives would attract patients from unorganized labs, encouraging high and specialized test prescriptions.

Our investments in technology and digital infrastructure are yielding tangible results, enhancing our brand visibility and patient interactions. We continue to push ahead with brand initiatives in select markets, while also executing a strategic agenda for network and test menu expansion.

We remain vigilant of the market dynamics and evolving the regulatory frameworks, adapting our approach to ensure compliance and sustainability.

As we press forward, maintaining a robust financial footing is paramount. With a focused approach towards cost optimization and strategic investments, we aim to uphold our financial metrics and cost structure.

I Now hand over to Shankha to delve deeper into our strategic initiatives and operational highlights. Over to you, Shankha.

Shankha Banerjee: Thank you, Om. I extend a warm welcome to all participants joining this call today. I will proceed to share the business and operating highlights with you.

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Classification: Confidential I am happy to share that we have closed the year on a strong performance at both top line and profitability. In Q4 FY24, we achieved a revenue of Rs. 545 Crore, marking an 11.1% growth. This was made possible by serving 6.5 million patients and testing 19.1 million samples, reflecting a sample growth of 8.1% compared to the same period last year.

Our Q4 FY24 revenue per patient stands at Rs. 833, a 3.2% growth over Q3 FY24, primarily driven by favorable mix management. In order to intensify our efforts in key clusters of Western and Southern India, we are expanding our lab infrastructure. This is being executed in a time-bound manner and alongside, we are building a robust ecosystem of collection centers and pickup points.

Our flagship bundle program – 'Swasthfit', continues to gain healthy traction, contributing to approximately 24% of total revenues in Q4. Building on this success, we would

continue to expand its reach and develop programs to sustain the growth trajectory. Furthermore, medical centers of excellence, such as LACE and LC oRD, are solidifying their positions within the medical community and among patients. Leveraging our digital infrastructure, which includes advanced AI and data analytics, continues to drive efficiency and elevate the overall patient experience. Technology lies at the heart of our operations, and we are exerting every effort to maximize the use of our assets to achieve sustainable and profitable growth.

To conclude, I would like to say that we are moving in the right direction. Our performance going ahead will be fueled by expanding our reach, enhancing our digital capabilities and focusing on product innovation and value-driven offerings. Thank you for your continued support and trust in our journey.

That concludes my opening thoughts. I would now request Ved to take you all through the financial performance. Over to you, Ved.

Ved Prakash Goel: Thank you, Shankha. Good afternoon, everyone, and warm welcome. I'm sharing some of the key financial highlights for Q4 and full year FY24. Revenue for Q4 FY24 came in at Rs. 545 crore against Rs. 491 crore last year same quarter, a growth of 11.1%. Revenue for the full year came in at Rs. 2,227 crore v/s Rs. 2,017 crore in FY23, a growth of 10.4%. Revenue per patient for Q4 is Rs. 833 and full year is Rs. 806 v/s Rs. 750 in the previous year, a growth of 7.5%.

Sample per patient for Q4 is 2.91 and 2.83 for the year, a growth of 4.7% and 5.3%, respectively. EBITDA for Q4 FY24 came at Rs. 145 crore v/s Rs. 116 crore in Q4 FY23, registered a growth of 25.1% with an EBITDA margin of 26.5%. For full year, EBITDA came in at least Rs. 609 crore v/s Rs. 490 crore in FY23, registered a growth

of 24.4% with an EBITDA margin of 27.4%. PBT for Q4 FY24 came in at Rs. 120 crore , registered a growth of 44.1% with a margin of 22%. FY24 PBT is Rs. 505 crore , registered a growth of 47% with a margin of 22.7%.

PAT for Q4 FY24 came in at Rs. 86 crore v/s Rs. 57 crore last year same quarter, registered a growth of 50.8% with a margin of 15.7%. Full year FY24 PAT is Rs. 360 crore -- sorry, Rs. 362 crore , registered a growth of 50.3% with a margin of 16.3%. EPS in Q4 FY24 is Rs. 10.1 and Rs. 43 for the year with a growth of 49%. Cash and cash equivalent as on March'24 is Rs. 960 Crore . Net of borrowing, it is Rs. 877 crore . Our receivables and inventory have further improved. Now , the DSO as on March 31, 2024, is only 25 days of credit sales and inventory is at 30 days. We still have a negative working capital of 26 days.

At last, we are pleased to announce that the Board of Directors has approved a dividend of Rs. 6 per share. With this, total dividend for the year stood at Rs. 24 per share.

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Classification: Confidential As mentioned by Dr. Om, we are committed to continue our efforts to optimize cost, increase productivity and enhance efficiency through tech -driven initiatives. With this, I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A.

Moderator: The first question is from the line of Binay Singh from Morgan Stanley. Please go ahead.

Binay Singh: I had 3 questions. Firstly, on the industry growth and competitive intensity. Any sort of update from what you shared last call, any changes that you see there in? The second question I have is on the Suburban side. If you could talk a little bit about the growth you are seeing, and fair to believe that the initiatives you talked about sampling promotion, brand building, these are more recent. What kind of traction you expect going ahead?

And lastly, the number of tests that you report in the presentation. We see a sharp drop in the tests under radiology from 1,947 to around 1,400 in this quarter. So any read-through from that?

Dr. Om P. Mancha

Dr. Om P. Mancha: Okay. This is Om here. Let me take the first question. And maybe the other 2 questions, I'll ask Shankha and Ved to address. So , coming to industry growth. Based on the sort of trend analysis that I have done in the last 3 years, I'm beginning to feel that probably COVID was impacting base not just due to COVID alone, I think some of the non -COVID sales in the previous years was also influenced by COVID. As the COVID has gone away, I think that FY24 base for the entire industry will be completely cleaned up, and you won't have any impact of either COVID or non -COVID sales influence due to COVID.

My personal view is our industry would see better growth in FY25 than what we have seen in FY23 or even FY24 . Second part of your question was on this competitive intensity. In terms of number of players, I don't see anybody going down or getting out of the business. Whatever had to happen has already happened during COVID time.

So now, whosoever has been in the business, my sense is they'll continue. But I think the level playing field is becoming a little more , a little better compared to, I think, some of the players have already taken price increases. So the deep discounting or if I were to call, predatory pricing, I think that is now not there in the market. So overall, I would say it's a positive news as we go forward into FY25.

Shankha Banerjee: So right, there was a question on Suburban. So this is Shankha here. So yes, Suburban growth traction on revenue side definitely looking up. I think our Q4 revenue growth is inching towards double digits. We are very close to double -digit growth now on Suburban. And a lot of these patient acquisition activities, I think which Om mentioned, as well as some other future improvements that we have done, should ensure that we are able to not only sustain this growth trajectory, but maybe also build upon it going forward. You want to take the point on tests?

Binay Singh: Sorry, this is double digit as in early double -digit or high double -digit growth?

Shankha Banerjee: No, no. So this is just reaching double digits. It's just sub double -digit level, yes.

Binay Singh: Anything on market share of Suburban in Mumbai?

Shankha Banerjee: So market share is obviously a very difficult number. I think in terms of assessing what the overall market potential itself is, we don't have any official data on that. But one thing which we know is that Suburban as a brand , will be the third largest brand in Mumbai as a city.

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Classification: Confidential Ved Prakash Goel: Right. Binay on the number of tests, we have rationalized some of the tests, which

were not in use. So that's why we have taken away from the list duplication.

Dr. Om P. Manchanda : The same thing that what we discussed the other day. Yes. I think this rationalization is primarily done because there are a lot of medical terms, which are actually very overlapping in nature. So some of these cleanup we've done. It's not that our portfolio has shrunk, but it's just that cleanup in our nomenclature.

Binay Singh: Great and also congratulations on almost completing 75 years, big milestone. Congrats on that.

Moderator: The next question is from the line of Amey Chalke from JM Financial.

Amey Chalke : Congrats on a good set of numbers to the management. The first question I have is, is it possible for us to gain volume growth for Delhi and rest of the North India region. And second, in Delhi we are seeing a lot of hospital chains coming back to their own lab business. There is a captive business as well, but they're also venturing into the outside market. Are you seeing the impact of this activity on your business?

Dr. Om P. Manchanda: Okay. I think on overall Delhi NCR business, we have seen a lot of positive traction this year. In fact, if at all, there is one part of the business, which has done well for us, is Delhi NCR in FY24 .

I think your point is valid. Some hospital lab chains have come into this business . My reading is that whatever gains they had to make, they probably have already made because of their own customer base while they'll continue to remain in the business.

But to my mind we are seeing a much higher growth than what we have experienced in the previous 3 -4-5 years. So overall, I would say that we are well positioned in Delhi NCR especially given that Swasthfit business is doing pretty very well in this space.

Amey Chalke : Sure. And the second question I have is on Tier-3 contribution which is around 34%.

Is it possible for us to give example of the cities what you consider in the Tier-3? And the second thing is that we see in the labs as a proportion of total labs around 40%, but the PUP number seems to be on the higher side, the collections number seems to be on a higher side. So . is the business model a bit different, you need to invest much more on the collection center side when you go for the Tier-3?

Shankha Banerjee: There is a list of cities which is their Tier-3 plus. If you remove the top 108 -odd cities of the country rest of them are then classified as Tier-3 and Tier-4. So that classification is something which we have taken from whatever the government of India does the classification on. It's publicly available data.

The second question which is on the model. So the collection network primarily which is linked to all the lab infrastructure is franchisee -driven. So to that extent there isn't a heavy company investment in terms of building the network, but what it definitely does is that in the Tier-3 cities and below we have a much larger geographic footprint from which the lab actually collect samples. And therefore, we see a larger a number of collection or larger collection network associated with the labs which are operating in Tier-3 or below.

Amey Chalke : Is it because of the scale of the margins of this business will be similar to our company average or how that will be?

Shankha Banerjee: I think we have seen that and you would have seen that over the last 3 years to 4 years the contribution of the Tier-3, Tier-4 markets have increased or improved in our overall mix. However, we've been able to sustain our EBITDA profile. So we have

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Classification: Confidential obviously worked out modalities in which the overall company margins is not going to be impacted adversely.

Dr. Om P. Manchanda : So just to add to what Shankha is saying I think the trick lies in the way you model your business and manage the logistics of it. So idea is that how you really align these Tier-3, Tier 4 cities to the nearest lab is how we are confident of managing the margins.

Amey Chalke : Thank you so much. I will join back the queue.

Moderator: The next question is from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: Sir just going through some of the operating parameters like the number of labs, the pickup points and so on. So , at a consol level you have added let's say three labs in the last 2 years and around 1,000 -odd pickup points. But if I were to look at Tier-3 centers there you have added maybe 15 labs and maybe 1,600 or 1,500 pickup points. So , it basically means that you're scaling back in your Tier-1, Tier-2 cities. So while I understand you're doubling down on Tier-3, but scaling back is something which I don't completely understand this, is this competition or is it maybe some inefficiencies which have crept in and you are just trying to sort of rationalize it going forward?

Dr. Om P. Manchanda: No, I think the way you should look at it is the following ; because having more labs

technically is not a demand generation activity. It's the ability to service the demand. So most Tier-1 cities we already have labs. In some places, we will have multiple labs also. In some places, we'll have HLMs as well. It's

only Tier-3 and below where there's a huge gap, we don't even have labs at all. That's why in terms of expansion of our lab infra you will always see that more labs are coming in Tier-3 by default itself because as per the government definition I think Tier-1 has about 8-9 cities and then Tier-2 have around 98 cities. So all other towns are falling in Tier-3 and below. So those are the places where we don't have too many labs. So that's why you will see expansion coming only in Tier-3. So it's not about scaling back in Tier-1, but it's more about the gap that exists is only Tier-3 and below.

Shankha Banerjee: And I think specifically on this point of net add of 3 whereas you see an add of 15 in Tier-3. Yes, there has been a bit of an overlap rationalization that has been done, but it's kind of onetime. It's not any scaling back. It's more of a onetime rationalization to see that the overlaps are reduced.

Kunal Randeria: Sure. And the same kind of logic applies on the pickup points also because there also seen a number of pickup points seems to have gone down in Tier-1 and 2's?

Shankha Banerjee: Pickup points, again, we've got a dynamic way of looking at it because sometimes if a pickup point is not giving us regular samples for a period of time, we kind of deactivate them in our system. So, there is no scaling back so to say. I think these numbers of our pickup point acquisition activities in Tier-1 and Tier-2 continue to be operational.

Dr. Om P. Manchanda: I think broadly this chart of Tier-3 plus 1 should read as if like we are deepening our presence in the market. That's the way one should look at. Now the play between CC pickup points and labs, it's a very dynamic process mainly because sometimes our intention is always to maximize throughput per lab. And once the throughput per lab is maximized then it's a huge positive on all sides. You will expand your test menu. You can operate 24/7. You can provide the courier pickup 24/7. You connect many roads. So it's like if you get higher throughput per lab, a lot of other parameters just fall in place. So I think that number is more always constantly optimizing operations. But the reason why we probably are giving the chance of Tier-3 plus is

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Classification: Confidential just to say that we are not just a few city dependent company, we are widening and deepening our presence across the country.

Kunal Randeria: Sure, sir. And just carrying forward on that. Any particular region, not Tier-3, any particular region you would like to call out which has surpassed expectations and where would you look to expand further?

Dr. Om P. Manchanda: It's mainly Northern part of India. I would say UP, Bihar. Because most of the cities in UP, if you look at the population is just ballooning beyond over 22-23 crore. Now such a large population base is I think if you pick cities, most of these cities are in the category of Tier-3 and below. Maybe just say Lucknow, Kanpur, etc. And that is where our presence is just growing. And this State is a huge state for us, and I think it shows good sign of growth. I would call out this particular State.

Kunal Randeria: Got it, sir. And just one more, if I can. Swasthfit contribution is 24% of revenue is probably the highest, I think, what is the kind of headroom you see over here?

Dr. Om P. Manchanda: It's a very interesting, we are currently in the process of identifying because virtually, every quarter we are seeing this contribution inching upwards. We're trying to figure it out as to what is really happening. My hypothesis there could be 2-3 factors that are leading to this. One is patients have seen greater value for money because obviously, it makes sense to go for a bundled package, we can do more tests at a much lower price. Maybe medical doctors are seeing greater value because then they don't need to ask for a follow-up test in one go, they get multiple te

sts and their

ability to diagnose what the underlying cause for any illness is there. Third could also be the way consumer behavior is changing. Because most of these now, even if they come to the lab or they book online, I have a feeling it's easy to book a bundled test. You just press a button, and it all gets done or repeat the order. All that is probably happening. So there is a sense that I get is a combined factor of all these 3 leading to higher percentage of contribution from Swasthfit.

We also believe, I think I've spoken about this in the earlier calls, is that as the bundling goes up, it is having ramification of two other parameters. One is the number of tests per patients are going up because bundled packages have more tests. I'm also getting a sense that footfall is also falling. Because once you get these tests done in one go, then you don't need to come back again because all the tests you've done in one visit itself which to some extent commercially is not a bad idea

because our overhead consumption is servicing the patient is highest.

It's not in the testing that is more, but it's more the servicing the patient visit. I think we should look at a volume component both together, number of tests per patient as well as number of patient footfall. I think combined together is where the volume should be looked at. And it's an interesting sort of the thing that is emerging. Let's see how it goes, but we are very confident that Swasthfit is going to grow further.

Moderator: We'll take the next question from the line of Nitin Agarwal from DAM Capital. Please go ahead.

Nitin Agarwal: Just taking forward your previous comments. This bundled tests, obviously, moving up the overall realization per patient. I mean, where do you think this number really ends up? At what stage it will start to, in your assessment stops growing? Because we've had a consistent Q-o-Q growth on this number for the last several quarters now.

Dr. Om P. Manchanda: There are three distinct segments in the market. One is pure screening -screening, where there's no illness and people are just getting themselves checked to know whether there is a problem or not. If that factor is dominating and driving this number up, that means the market is expanding. The other is a lot of people who are suffering

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Classification: Confidential with NCD which are chronic diseases like diabetes, high cholesterol or some other illnesses, where they need to go for regular checkups. Those are basically upgrading from an individual test. Even there is also no problem. Let these bundled tests grow because many of these NCDs are interconnected also.

Third area is which is an illness driven or communicable diseases where things like typhoid, malaria, dengue, these are all seasonal flu, etc. To my mind, these are three distinct segments. If screening and NCDs are there, then this number, actually, my sense is can even go 30 %-40% as well. So I think it will continue to grow as we go forward. And also, I think as the bundling goes up, it's a greater value for money. I think that we are also hovering around price points of between Rs. 1,000 - 2,000 which is just a ticket size which is easy to take out from the pocket. This may just become a new norm in the industry.

People will just get to know, I think I mentioned to you earlier also, if you trace the history over the last 20 years, it started with a single test, then it became like a panel test, like LFT, KFT, they're also technically bundled tests for a particular organ. What the next stage has been the bundling of these panels only, LFT, KFT clubbed together. So this is, to my mind, is a new way of life, and I think this will become a norm in the industry. I think your specific question is to what extent it can go? Anybody's imagination. My sense is it can be 30 %-40% of the portfolio as well.

Nitin Agarwal: And sir, I guess, based upon

your comment, probably in the past, you were tracking the patient volume growth number more closely, given the way the dynamics in the market are playing, probably you think sample growth, probably the more apt number to look at as things are.

Dr. Om P. Manchanda: Sorry to interrupt you, I would say both.

Nitin Agarwal: Okay. And secondly, on the cost part of it. We've had a pretty sharp jump up in our gross margin this year. I mean how much scope do we have for that to squeeze out more, especially at the gross level, contribution level?

Shankha Banerjee: I keep asking this question to Ved as well. So Ved should answer.

Ved Prakash Goel: So I think it's pretty high. I mean 80% roughly is gross margin. I think what we can do is, in my view, we can sustain here. But beyond this, I don't think there is a room to improve further, especially on the gross margin. But I think we can sustain between this 78% to 80% kind of margin.

Moderator: The next question is from the line of Pranav Chawla from Antique Stock Broking Limited. Please go ahead.

Pranav Chawla: I just wanted to ask what is our regional mix for the fiscal. We used to highlight the split between Delhi NCR, South, West, International, East and North. Can you provide this?

Ved Prakash Goel: Delhi NCR contribution is about 31% for the quarter.

Pranav Chawla: Sir, for the fiscal?

Ved Prakash Goel: 31%, for the full year as well.

Dr. Om P. Manchanda: Yes, I'll tell you, 31% Delhi NCR, 31% Rest of North, 15% is East, 15% is West, South is 6%, and other 3%. Others will constitute international and CTL.

Pranav Chawla: International as well. Sir, and what is our B2B, B2C mix for the fiscal?

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Classification: Confidential Dr. Om P. Manchanda: It is always estimate, right?

Ved Prakash Goel: Retail, including home collection, is about 32% for the year.

Dr. Om P. Manchanda: But that is our own infra. There is also a retail component in collection center.

Ved Prakash Goel: For franchise it's about 37%, which is collections center.

Dr. Om P. Manchanda : For full year?

Ved Prakash Goel: Yes.

Dr. Om P. Manchanda : I can come back on that, 1 second . The retail component of direct -to-consumer, our own infra is very clear. But the retail component of our collection center is generally we take an assumption that 50% of their business would be direct -to-consumer and balance 50% would be a pickup business.

Ved Prakash Goel: That exact split, we don't have.

Dr. Om P. Manchanda: We don't have that, but we normally tend to assume that number and add it up.

Pranav Chawla: Okay. Perfect. Sir, just can you provide us a color on how the competition has been in a core market of Delhi NCR? And how the region of Mumbai has been doing? How Suburban is growing? Even if you can share the quarterly revenue number of Suburban, that would be great.

Shankha Banerjee: I think, firstly, to answer Suburban. I think there are quite a lot of revenue and patient growth activities that one is doing. I think , we talked about a lot of sampling activities, patient acquisition, offline, online activities that are happening. Also, we are expanding geographic testing, footprint for Suburban to get into newer pockets and tap into those opportunities. Also, running a lot of clinician connect programs to get better prescriptions and conversion programs there as well. Also beefing up the sales team, creating some specialized sales team structures to get better extraction from B2B clients as well. So there are multipronged activities currently on for driving revenue growth, and those things are going to be sustained over the next few quarters.

Dr. Om P. Manchanda: Just to add to what Shankha said, now it's been nearly about more than 2 years. We have more than 2 years. We mentioned about Suburban , it went through 2 phases.

Phase 1 was basically to stabilize the ship because it is just changing hands from

promoter to us. And then that piece was done. I think now the second piece was actually getting the strategic building block in place. If you recall, a major shift that we've had is the franchising of collection centers. That's the model which LPL follows. And we find that entrepreneurial energy in franchising is extremely high. And we've used that to drive a lot of sampling, a lot of camp activities in various colonies of Mumbai.

And I don't know if you guys must have seen in Pune as well. And I think now with Shankha moving as CEO in LPL, we are now going to integrate this company much more with the parent as well where a lot of back -end support operation will be driven from here. So, we are very confident that all the building blocks of future growth for Suburban are in place and we are going to be driving this business from the center. Now early days, but good sort of numbers are there last quarter. It is definitely driving a lot of confidence in this. And I'm very, very confident because as a brand, as a single largest brand in one city, there are not too many of them in this country. And the suburban, 80% of its business comes from just one city of Mumbai. And that's a huge, huge sort of advantage platform for us to expand from here on. And it also has a brand which lends very well to its sort of a direct -to-consumer. It's brand awareness

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Classification: Confidential is fairly high compared to many other small time pathology labs. So I'm fairly confident that going forward, Suburban would turn out to be a good asset.

Pranav Chawla: Okay. Sir, then can you just give us some broad color on Delhi NCR region , like how is the competitive intensity, pricing, realization, etcetera, just for Delhi NCR?

Shankha Banerjee : Competitive intensity in Delhi NCR hasn't really changed too much v/s what I think we spoke about in the last quarter. So I think a base level of competitive intensity is always there. We haven't seen any sharp up move or down move in terms of competitive intensity when it comes to Delhi NCR, at least not in this last quarter.

Moderator: The next question is from the line of Prakash Kapadia from Spark PMS.

Prakash Kapadia: I have two questions. If I look at the Swasthfit contribution. It has been increasing. It has been doing well for us. So is it currently just a few top cities or metro specific, is there a scope to take this beyond some of the top cities? That's my first question. And secondly, if I look at where the industry is, given that some of the price increase has happened, competitive intensity has stabilized or decreased.

So from year on, if I were to build a scenario of doubling our patient count of 27.5, 28 million over the next few years, what are the key variables which will get us there?

Is it convenience? Is it accuracy? Is it increased genomics contribution? Is it doctor advocacy, if you could give some direction on those parts will be helpful?

Shankha Banerjee: Right. Okay. Thanks for the question. So on Swasthfit, it is not a metro phenomenon for us. I think over the last few years, we have been consciously expanding and driving Swasthfit into multiple geographies. And that maybe one of the underlying reasons why we are seeing consistent growth, and we will continue to do that. So

that expanding the footprint for Swasth is something which we are actively working on and it's a continuing process. So that point is there. On the other question on where and how will the future growth come from? So yes, there will be, like we said, the patient and samples, there are two measures of volume that we see. I think both of them are what we are really eagerly looking forward to be driving, although there is a price increase taken by competition and there is this sense or feeling that intensity is low. We believe that competitive intensity underlying intensity is still what this industry has been used to or has been

seeing in the past continues.

So we are not really looking to a price increase driven growth. We continue to believe in driving our growth through patient and sample volume addition and mix improvements.

Prakash Kapadia: I get to the point of price increases, not the route. But in terms of the patient volume growth, I'm just trying to envisage a scenario what will that one or two variables be? Because in terms of branding, we are one of the oldest companies. In terms of cost per sample, we are amongst one of the best. So why is that volume growth not really coming through for us? So, is it going to be some of the newer tests, which will contribute more or some specific convenience factor from a consumer standpoint? I was more trying to understand that.

Dr. Om P. Manchanda: Okay. This is Om here. I don't think we should really get hung up on just the volume number, okay. As I mentioned to you, I think there's a lot of interplay between test per patient and the footprint. The volume number that we are talking about technically is a footprint. It's not a unique patient. I think the question that you asked is a very great question in terms of what is the variable which we probably need to address to look at the entire market. My view is that any category, if you pick up, they're always bucketed into three price points: One is the premium end, mass market and the discount end.

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Classification: Confidential Right now, I think Dr. Lal PathLabs fuddles in all these three segments. As we go forward, we'll have to carefully segment the market and design our proposition in such a way that premium customer gets premium service, but at a premium price as well. Discount customer gets discounted service at a discount price. Now getting it offered under same names sometimes is very, very difficult to achieve that in the same market.

To me, I think we'll have to probably look at that. That is where, somebody used to ask me that Suburban, are you going to keep it separately, the answer is yes. It has multi-brand strategy and also different services and at different price points. I think we are reaching at that stage where market will have to address these things as we go forward.

Moderator: We'll take the next question from the line of Anshul Agrawal from Emkay Global.

Anshul Agrawal: So, considering in the last quarter, we have seen some upward pricing action from online players as well as other national chains. Do you believe that our volume growth is a result of this pricing action since you have not taken any hikes and follow-up on that would be then could these be our sustainable volumes considering that we are not looking to take a price hike in the next few quarters?

Dr. Om P. Manchanda: It's early days, and some of these competition that you are talking about, our friction with these guys is not really head on, except in few cities. The markets where we are present, actually, we don't really compete too much with them. The early sort of a sign of increase in volume that we are seeing maybe 2. A). of course, base effect because last year, volume growth has not been that much. Second could also be that, which I'm not fully sure, but I still will make that point. I get a sense that last two years, non-COVID volume also was being influenced by COVID. As that has gone away now, we are actually in a clean base, and we are seeing now original non-COVID volume, so to call it. I think from here on, we'll have to probably wait it out for another quarter before we start making any sort of view on this. But definitely, directionally, I'm seeing a better volume growth than what we have seen on a year basis.

Anshul Agrawal: Got it. And another question on competitive intensity, Dr. Om, if I may. So, considering that one of the other listed regional chain is getting very aggressive in Pune. How do we aim to protect or expand our market share or suburban's market

share in the Pune region?

Are there any thoughts around this? Any strategy?

Dr. Om P. Manchanda: First of all, I think market is very large. So, it's that organized players, put together is not even 15% of the total market. So, there is a lot to gain for all of us from unorganized space. And if I were to guess the name of this company, which is

probably they are more an integrated business of radiology and pathology compared to us, which is just pathology. So, right now, our business model is to actually play more in pathology, which is more hub-and-spoke model, low capex, high ROCE v/s, let's say, take integrated model. While both models are well proven, there's nothing wrong with the other side as well. But I would say the market is large enough for all of us to stay.

Anshul Agrawal: Sure. Just one last question from my end. Any sense on what would our SSG volume growths would be for the current year and the contribution from the new centers?

Ved Prakash Goel: I think, Anshul, we don't track like this because like Dr. Om was mentioning hub and spoke, so we have geography or a cluster where we have lab is one. But we have collection center, and we have pick points around that. So as a cluster or geography, we track but as a same-store, we don't look at that way. But separately if you see Delhi as a political state, probably we are not expanding that much, which is growing,
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Classification: Confidential let's suppose high single digit. That is where you can look at, and rest is coming from the new infra or expansion of higher growth in the newer markets.

Anshul Agrawal: Got it. Very clear. Just one last question from my end for Ved -ji probably. Just as you mentioned that gross margins would remain range-bound. Any thoughts around EBITDA margins, given our expansion plan?

Ved Prakash Goel: I think we are saying pre-COVID level margins, which is around 26% EBITDA margin, where we are confident that we can maintain.

Moderator: The next question is from the line of Jainil Shah from JM Financial.

Jainil Shah: My first question is on our guidance for next year. So, we alluded to our growth being higher in FY25. So just wanted to understand how much will be volume-driven and how much will be price-driven and how much growth are we budgeting for suburban here?

Dr. Om P. Manchanda: I think the way we look at our numbers is that internally, the view is that we should probably not take a price increase as things stand today. If we don't do that, I would say that we will probably better our growth slightly than what we have delivered in FY24 without the price increase. The next question is, where is it going to come from? It's going to be a combination of both volume as well as number of tests per patient, and maybe a mix also because we are seeing higher end tests, also being prescribed more. So, our internal sort of desire is to beat the last year growth without the price increase.

Jainil Shah: Sure. And would you say that FY24, at least 4Q, we would not be having any COVID revenues? So essentially, we've grown at around 14% on a non-COVID base?

Dr. Om P. Manchanda: So, right now COVID was not there, that is fine. But what is the other thing you said 14%.

Ved Prakash Goel: So full year, growth is 10.4%.

Dr. Om P. Manchanda: So I think the best way to look at the following. We are exiting the year, which is Q4 at 11.1% revenue, okay, which is primarily without any COVID impact, either in the base or in current year, right. More or less. Now this 11.1% has an advantage of one month where there was no price increase, has a disadvantage of base of two months, which is Feb and March. If we continue without the price increase, we are confident that we should be on this number or slightly better.

Jainil Shah: And just wanted full year revenue and EBITDA for suburban?

Dr. Om P. Manchanda: This year?

Jainil Shah: FY

24, yes.

Ved Prakash Goel: This year, we did Rs. 164 Crore revenue for suburban.

Jainil Shah: And EBITDA would be? Or we are incurring losses there because of the selling lab?

Ved Prakash Goel: 17% EBITDA margins.

Dr. Om P. Manchanda: No, that is for the quarter. Full year?

Ved Prakash Goel: Full year is about 12%-13%.

Dr. Om P. Manchanda: Full year is 13%, Q4 is 17%.

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Classification: Confidential Moderator: The next question is from the line of Shaleen Kumar from UBS Securities India.

Shaleen Kumar: Sir, just a clarification, right? Most of my questions have been answered. So, Ved sir has been saying that our margins should be pre-COVID level 26%, right? But you have already delivered more than 27%. And I understand, let's say, gross margin remains where they are, but there is definitely going to be operating leverage. Now two things can happen from here, either your margins will expand or probably you said you want to invest in growth, which means that growth expansion should happen, right? So, is my understanding right that you have decided to invest in growth and then the growth uptick will happen while margin may remain where they are, with a lag, that's okay, but that's the idea, right?

Dr. Om P. Manchanda: I think Ved was explaining gross margin, there's hardly any scope left now for gross margins to improve. And your point of operating leverage is definitely valid that if we grow from here organically, you may see expansion of EBITDA margins. But we want to really plough it back into the market going to Tier-3, Tier -4 towns, where maybe pricing could be , may not be exactly the way Tier-1 pricing is, where our cost structure may be very different. But we will try and invest more behind growth rather than expanding our margins .

Shaleen Kumar: Which is fine. Sir, my question was that, that will reflect in our growth, maybe with a lag over a medium term, right? So that's the way we should look at it, right? And the main part of that will come in .

Dr. Om P. Manchanda: Yes, ultimately, because there are so many white spaces in the market, our company has to focus on growth. There's no doubt about that.

Shaleen Kumar: Which is fine with us, sir. I just wanted to have a clarification on that.

Dr. Om P. Manchanda: No, no. It's very clear. And that would also include M&A as well. So , it's not that , while it may be a bit of a time since we did last M&A, but that has not been our eyes are not on inorganic. We will also search for inorganic.

Shaleen Kumar: Would you actually answer my second question, because you're already close to Rs. 1,000 Crore of cash. And I mean, you will be generating roughly Rs. 400 Crore of cash on average. So is this something, which we can expect in the near term in terms of the organic?.

Dr. Om P. Manchanda: I don't know, right? Let's just see how it goes. Right now, there's nothing that we can share with you. But definitely, it's part of our stated strategy.

Moderator: The next question is from the line of Pranav Chawla from Antique Stock Broking.

Pranav Chawla: I think we highlighted we plan to add around 20 hubs going forward. So , can you allude us, what time line and what CAPEX kind are we looking at? And what regions this will be in?

Dr. Om P. Manchanda: See, our average capex for this normally is about Rs. 50-60 Crore , which is a combination of investments in IT, investments in instruments, investments in labs.

So I would say that will continue. It's about Rs, 60-odd Crore of normal CAPEX . Most of these expansion on opening labs primarily been in Northern part of India and rest of north, essentially where our brand is strong. That is where we are seeing better traction for the business for our lines. I think that's where we will go.

Pranav Chawla: Perfect. And sir, what markets we'll be looking for inorganic expansion? Because I think Calcutta, East, we have been t

rying to grow organically and still the size of the business is pretty small. Same is the case with I think West is also now at 15%. So what target markets we'll be looking at?

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Classification: Confidential Dr. Om P. Manchanda: I think the answer is in these numbers only, so just 31 Delhi NCR, 31 rest of North,

15 East, 15 West, South is only 6. So these numbers are telling me where I should go. So obviously, South is one, definitely number one priority. It's not that we can't do M&A in other markets as well, but sometimes it becomes very challenging because with same surveys, a lot of friction that happens on the ground in terms of teams, in terms of brands . It's always better to go in those markets where brand presence for Dr. Lal PathLabs is weaker. And in any case, we are not doing these M&As to ramp up our top line, we are doing it more as a market entry strategy. So it is natural for us to look at in those places where our business is weak, which, to my mind right now it looks like South is the place.

Pranav Chawla: Sir, any large players or large competition in these markets like that we may explore, like number two, number three players .

Dr. Om P. Manchanda: No. I think we are indifferent size. My sense is we look at even not just region, we also look at just city level entry as well. So we are open to even smaller size acquisitions as well. So there's nothing that holds us back. In fact, our PUPL, we haven't talked about it much. But places like Indore, places like Baroda, we've actually bought individual labs, which was just about Rs. 8-10 crore as well. Now since the number is small, they don't come up on the radar for us for discussion. But we are open to doing those tuck -in acquisitions as well.

Moderator: The next question is from the line of Dheeresh Pathak from White Oak Capital.

Dheeresh Pathak : I'm sorry if you already answered this sir, but for the full year and for the quarter, if you can just give how much growth came from pure realization increase, like -to-like realization increase, not the effect of mix?

Dr. Om P. Manchanda: Taking out the mix impact. You mean due to price?

Dheeresh Pathak : Just from price increase, not coming from any mix effect or anything. Just your price increase, how much growth came for the full year and for the quarter?

Shankha Banerjee : Just pure price, we got realization benefit of 3.5% for full year.

Dr. Om P. Manchanda : On a year basis, but it's lesser in Q4.

Shankha Banerjee : In Q4, it is lesser. But on a full year basis, 3.5% is pure increase driven, which is for full year.

Dheeresh Pathak : And this is annualized in this year? This got annualized in Q4 now, right?

Shankha Banerjee : Yes. So the Q4 contribution of price increase is only about 1%, 1.1% or something.

Dheeresh Pathak : 1%, okay. And provided if you don't take any price increase further, then this will be almost negligible in the coming quarters, right?

Dr. Om P. Manchanda: Impact of price, that's what we are saying that we want to beat the last year growth without the price increase this year.

Moderator: This will be the last question for today, which is from the line of Sumit Gupta from Centrum India .

Sumit Gupta: So just want to understand about like pricing scenario for overall test menu. So just can you highlight like how much is the specialized test menu? Like what kind of price increase are you taking in the specialized menu v/s obviously the routine test?

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Classification: Confidential Dr. Om P. Manchanda: No. as of now, we don't plan to take any price increase neither on specialized nor on routine.

Sumit Gupta: Okay. And how much is the specialized test menu as the overall test menu , specialized and overall test menu?

Shankha Banerjee : So specialized portfolio today for Q4 has contributed to about 20% of our overall revenue.

Dr. Om P. Manchanda : And by the way, they a

re not comparable because each company may have its own definition of specialized, so just keep that in mind as well. So it's not that somebody tells you that they're specialized menu is 30%, you start assuming that we're lower.

So ev erybody has his own way for defining these tests.

Moderator: Ladies and gentlemen, we'll take one more question, which is from the line of Shaleen Kumar from UBS Securities India .

Shaleen Kumar: So when you said that you will be delivering better performance compared to FY24 and there will not be any price increase there, right? So assuming that our sample per patient growth would be similar of what we have in FY24 , so we are looking at roughly high single digit of volume patient growth. Is that the right thing? At least, if not high, that means it is something like 7% odd kind of a growth quarterly?

Dr. Om P. Manchanda: No, no. How it is 10%?

Shaleen Kumar: No, no 6 %-7%, not 10%.

Dr. Om P. Manchanda: Okay. It's a bit early. Yes, I think that's the sort of target we are keeping in our mind that it must definitely move beyond 5 %-6%. Because I think for year basis , right now, this year is about 3%?

Ved Prakash Goel: 3.2%.

Dr. Om P. Manchanda: 3.2%. We want to see definitely this number moving up. And should be about 6% quarterly was the better number, yes, you're right.

Shaleen Kumar: So basically, even if 12% revenue growth we're looking at, 5% coming from higher tests .

Dr. Om P. Manchanda: So Shaleen, I think that depends on how ramp up on Swasthfit happens because that will give us a higher number of tests per patient. But you are right, we are looking at definitely, if you look at this quarter exit, which is 11.1%, has some advantage of base in pricing due to pricing. But we want a really better base and with a combination of volume as well as number of test per patient. But tests per patient, in any case, is baked in. So most of the growth will come for higher volume g rowth.

Moderator: Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments. Over to you, sir.

Ved Prakash Goel: Thank you, everyone, for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries satisfactorily. Please feel free to reach out to us in case you have any further quest ions or queries. Thank you once again.

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Call: Aug 2024

Classification: Internal August 13, 2024
National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street

Mumbai – 400 001
Scrip Code: 539524

Sub: Transcript of Q1 FY25 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q1 FY25 Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral

Company Secretary & Compliance Officer

Encl.: As above

Page 1 of 15

Classification: Restricted Dr. Lal PathLabs (DLPL)

Q1 FY25 Earnings Conference Call Transcript

August 07, 2024

Call Duration •1 hour

Management Speakers •(Hony) Brig. Dr. Arvind Lal – Executive Chairman

•Dr. Om Prakash Manchanda - Managing Director

•Mr. Shankha Banerjee - Chief Executive Officer

•Mr. Ved Prakash Goel – Group Chief Financial Officer

Participants who asked

questions •Rahul Agarwal - IKIGAI Asset Management

•Karthik Chellappa – Indus Capital Advisors Hong Kong Limited

•Binay Singh – Morgan Stanley

•Prakash Kapadia – Spark PMS

•Sumit Gupta – Centrum India

•Adrit Chaturvedi - Nomura Financial Advisory and Securities India

•Kunal Randeria – Axis Capital

•Shaleen Kumar – UBS Securities India

•Deven Kulkarni – Marcellus Investment Managers

•Abdulkader Puranwala – ICICI Securities

•Pranav Chawla - Antique Stock Broking

•Ashutosh Parashar - Mirabilis Investment Trust

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Classification: Restricted Moderator : Ladies and gentlemen, good day and welcome to Dr. Lal PathLabs ' Q1 FY25 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Siddharth Rangnekar of CDR India. Thank you and over to you.

Siddharth Rangnekar: Good evening, everyone, and welcome to Dr. Lal PathLabs ' Q1 FY25 Earnings Conference Call. Today, we are joined by senior members of the Management , including (Hony) Brig. Dr. Arvind Lal , Executive Chairman, Dr. Om Prakash Manchanda, Managing Director, Mr. Shankha Banerjee, Chief Executive Officer and Mr. Ved Prakash Goel, the Group Chief Financial officer.

I would like to share that some of the statements made on today's call could be forward -looking in nature and actual results could vary from these forward -looking statements. A detailed statement in this regard is also available in the results presentation that is available on the stock exchange website s and has been

separately circulated to all of you.

I would now like to invite Dr. Arvind Lal to share his perspectives. Thank you and over to you, Dr. Lal.

Dr. Arvind Lal: Thank you, Siddharth . Good afternoon and a warm welcome to all participants on this call. We are here to discuss the Q1 FY25 Earnings Performance of Dr. Lal PathLabs. I would like to commence by sharing my insights into the changing market dynamics and the advancements we have achieved.

This year also marks our 75th anniversary. Dr. Lal PathLabs continues to be a trusted healthcare partner, renowned for its commitment to quality, accuracy, accessibility and affordability. The diagnostics market in India is expected to grow as people live longer, more tests get prescribed, and the shift from unorganized to organized sectors builds momentum.

Our unyielding attention to excellence in diagnostics has earned us the trust of both patients and physicians over the years. Utilizing our digitally enabled network infrastructure, we remain agile to service growing demand for testing. Our custom - built logistics solutions further enhance our capability to serve our patients more efficiently.

The integration between Dr. Lal PathLabs and Suburban diagnostics is progressing as per our expectations. With the Suburban brand, we are making volume expansion a priority. We are making strategic investments in both marketing and process efficiencies to fortify the brand while enhancing the patient experience. We are creating opportunities to grow sample volumes with the help of hub labs in important clusters around the rest of the country. Our synergistic approach enables us to provide patients with a comprehensive array of tests across multiple categories with speed and precision.

Looking ahead, our growth trajectory will be driven by the ongoing transition from unorganized to organized sectors, heightened awareness of accurate diagnosis and emphasis on wellness among others.

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Classification: Restricted We remain dedicated to expanding our network and enhancing our service standards through operational excellence and innovative technological advancements. I am confident that the capabilities that we have created to achieve sustained growth will further consolidate our market share. Thank you and I would now like to hand over to Dr. Om Manchanda. Over to you.

Dr. Om P. Manchanda: Thank you, Dr. Lal. Welcome to our call today. I shall walk you through the evolving industry scenario and progress on our strategy.

I am pleased to report that we have achieved 11.3% growth rates on a YoY basis.

This is a result of the consistent

execution of strategy of expanding presence in identified towns and cities where our brand enjoys salience, particularly in West, North, Central, East and across the rest of the country.

The contribution from suburban diagnostics to the top line is scaling up in line with the initiatives that we are driving on franchising and brand communication. We continue to prioritize broadening and deepening the brands , geographical presence in order to derive sustainable volume and value growth. Our focus, therefore, is on scaling the next tier, expanding into Tier-3 and Tier-4 towns with lab infrastructure. We are establishing 20 new labs to broaden our reach with plans to add more labs as the year progresses.

DLPL is present in most prominent cities where we continue to densify our PSC network. Suburban is the fulcrum for the thrust into Mumbai, parts of western India including Goa and Pune. We are actively exploring opportunities to expand our footprint there, focusing on assets that align with our core values and strategic goals. Our Swasthfit brand, a steady revenue contributor has delivered around 25% of the total revenue and continues to expand. The medical centers of excellence which we have established are scaling effectively and enhancing patient engagement and outcomes. Driven by strategic emphasis on sustainable volume growth, these efforts aim to draw patients from unorganized labs, promoting the adoption of a quality and accuracy -backed approach to testing.

Our investment in technology and into infrastructure are delivering concrete outcomes, strengthening brand visibility and facilitating smoother interactions with patients. Patients today access our services across channels and our infrastructure naturally has kept pace with these requirements. As its matrix evolves, brands like ours are in a strong position to access growth. We remain attentive to market dynamics, adapting our approach to ensure sustainability of outcomes.

Moving forward, maintaining a solid financial foundation is crucial. To focus efforts, we aim to maintain our robust operating metrics and drive a strong cost structure that

will form the basis of the growth initiatives.

I now hand over to Shankha to delve deeper into business initiatives and operational highlights. Over to you Shankha.

Shankha Banerjee: Thank you. I welcome all participants joining this call today. I will proceed to share the business and operating highlights with you.

We are pleased to announce a strong start to the year, achieving robust performance in both revenue growth and profitability. In Q1 FY25, we achieved a revenue of Rs. 602 crore, representing a 11.3% growth over Q1 last year. This growth in revenue is driven by patient volumes, which is at 7.2 million and 21.1 million samples. Patient volume growth for Q1 FY25 is 5.5% and sample volume growth is 9.6% over Q1

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Classification: Restricted FY24. Notably, we saw a 10.4% increase in patient numbers in Q1 FY25 as compared to Q4 FY24. Our Q1 FY25 revenue per patient stands at Rs. 833, which is same as Q4 FY24.

We are strengthening our digital infrastructure and capabilities encompassing cutting edge data analytics in order to enhance testing and service outcomes for patients.

This is going to be a focus area for us. Dr. Lal PathLabs and Suburban are driving the omni -channel experience forward with the help of technology.

In the period to come, we expect a combination of volume gains and realization mix to drive the revenue growth. This shall include favorable tests and geographic mix, the traction in bundle testing and build up in volumes in specialized tests shall also contribute to this.

In conclusion, I would like to emphasize that we are progressing as per plan on our growth objectives. Our future performance will be guided by expanding our footprint in identified clusters, enhancing our digital capabilities and enhancing patient servi

ce

outcomes. Thank you for your ongoing support and trust in our journey.

This concludes my opening thoughts. I would now request Ved to take you all through the Financial Performance. Over to you, Ved.

Ved P Goel: Thank you, Shankha. Good afternoon, everyone and a warm welcome. I am sharing some of the key Financial Highlights for Q1 FY25 . Revenue for Q1 FY25 came in at Rs. 602 crore against Rs. 541 crore last year same quarter, a growth of 11.3%.

Revenue per patient for Q1 FY25 came in at Rs. 833 v/s Rs. 789 in Q1 FY24.

Sample per patient for Q1 FY25 is 2.92 with a growth of 3.9%. EBITDA for Q1 FY25 came in at Rs. 170 crore v/s Rs. 146 crore in Q1 FY24, registered a growth of 16.2% with the EBITDA margin of 28.2%. PBT for Q1 FY25 came in at Rs. 150 crore, registered a growth of 27.6% with PBT margin of 24.9%. PAT for Q1 FY25 came in at Rs. 108 crore v/s Rs. 84 crore last year same quarter, registered a growth of 29.1% with PAT margin of 17.9%. EPS in Q1 FY25 is Rs. 12.8 v/s Rs. 9.9 in Q1 FY24. Net cash and equivalents as on June 30th, 2024 is Rs. 1,044 crore.

We are pleased to announce that the board of directors has approved an interim dividend of Rs. 6 per share for FY25.

With this , I conclude my opening remarks , and I would now request the moderator to open the forum for Q&A.

Moderator: We will now begin the question -and-answer session. We will take our first question from the line of Rahul Agarwal from IKIGAI Asset Management. Please go ahead.

Rahul Agarwal: I have three questions. Firstly, Dr. Om, on the Tier-3, Tier-4 markets, as you are expanding infra, I just wanted to know any learning from patient behavior here like, how are these patients different from metros and Tier-1, what are they seeking from a brand like yours?

Shankha Banerjee: In terms of expansion into Tier-3, Tier-4 primarily it's a lot about looking at local unorganized players and we are going to compete with them and our offering there is quite similar to what we offer in Tier-2 and Tier-1 cities as well. We lead with quality and service and obviously we are finding quite a lot of patients and clinicians who are embracing us in these cities. The offer is really not significantly different from what we are doing in our other cities.

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Classification: Restricted Rahul Agarwal: Is the pricing very different or even that remains the same?

Dr. Om P. Manchanda: Rahul, I will probably slightly give you a higher -level perspective. What we have seen is that most of these smaller towns, their behavior is influenced by nature of the prescription given by doctors. What we are seeing is that the healthcare is now travelling to Tier-3, Tier-4. And I think it's a bit of a qualitative comment. I think overall quality of medical manpower is also going up as more qualified doctors settle down in smaller towns, they tend to prescribe tests which are also higher order. I think this is more a market evolution where we will see level of testing going up with time and directionally it's quite visible in our numbers as well. I think that's where I would want

to make comment.

Rahul Agarwal: Secondly, on this Medical Center of Excellence, just wanted to know, could you elaborate what is this exactly, are we putting up a different infrastructure here or is this the current lab is getting transformed into something?

Dr. Om P. Manchanda: I think it's a good question. I will explain to you. I think you guys cover lot of pharma businesses. If you look at the journey of this entire pathology, most of the differentiation in the past has been service, branding, brand awareness, top of the is stuff and all of that. But as we have progressed in the last 20 years, the market has grown, more competition has come, so everybody's offering the same thing. Now the only way you differentiate is that you start looking at different segments. Like in pharma, you have a Gastro division,

you have an Oncology division, you have that and this. Answer to this question is we are trying to segment the market and look at broad segments and especially on high end, the segmentation is done based on busy state and that's how we have identified certain segments like Oncology, Autoimmunity, Reproductive diagnostics and trying to get more focus from the corporate side, so that whether it's introduction of newer tests, whether it's introduction of new communication with the medical fraternity, organizing seminars or creating patient cohorts. A lot of those activities in a focused manner, we are trying to create these categories and that's the whole idea. Operationally, though we have some people on ground, but we are not trying to create different teams for these segments.

Rahul Agarwal: These are like existing people working at different labs which are identified as where you want to increase communication and you want to focus on specific specialties, is that correct?

Dr. Om P. Manchanda: I would say higher focus on marketing efforts, everything else remains the same, that's it.

Rahul Agarwal: Lastly, on this Swasthfit specialized portfolio, just wanted to know, I know that it's small right now, but what kind of scale or total mix into overall Swasthfit, do you visualize over the next 3 years?

Shankha Banerjee: The Swasthfit specialized portfolio is primarily aimed toward clinicians, and we are still kind of working on it. It's not something which has reached the level of scale for us to differentiate and start reporting those numbers separately.

Rahul Agarwal: Shankha, I understand. What I was asking was is more like a 3-year view, would you visualize like what kind of a specialized mix would you have within Swasthfit, would you have that?

Dr. Arvind Lal: The people around India especially in these Tier-3 and Tier-4 towns they have not realized the importance of these so-called non-communicable diseases and lifestyle diseases which we are trying to make an awareness and that is also a part of why the Swasthfit is doing well.

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Classification: Restricted Moderator: We have our next question from the line of Karthik Chellappa from Indus Capital Advisors, Hong Kong Limited. Please go ahead.

K Chellappa: Sir, I have three sets of questions. The first one is, if we were to look at our volume growth, let's say of about 9% or so, could you give us some sort of perspective on geography wise, how that is faring, I mean, which are like the geographies driving double digit and which are the ones which are still lagging?

Ved P. Goel: Karthik, I think we are not giving geography wise split quarterly, but once in a year we are giving it. Having said that, the geography is like rest of north and east which obviously we are performing well, and they are doing better than our company average. Where we are going much deeper, which is, let's suppose rest of north and especially UP where we are going Tier-3 and plus, that is obviously the higher growth as compared to company growth.

K Chellappa: My second question, sir, is what would be the like-for-like price increase that we have taken on a YoY basis across tests?

Dr. Om P. Manchanda: We took last price increase, that was in February 2023, and we completed one full year of cycle of that in Feb '24 and we have not taken any price increase and neither we have intent to take as of now.

K Chellappa: Which means if I were to look at, let's say the realization per test, which is probably up 2% YoY, that's probably driven more by mix, right, rather than anything else on a like-for-like?

Dr. Om P. Manchanda: Absolutely. This is the mix which is driving this revenue per patient up.

K Chellappa: Sir, would you be able to share the revenue and EBITDA margin for suburban this quarter?

Ved P. Goel: Suburban revenue growth for this quarter is about 8% and EBITDA is about 14%.

Moderator: We will

take our next question from the line of Binay Singh from Morgan Stanley.

Please go ahead.

Binay Singh : The 8% growth in suburban you commented so basically , we would have done around Rs. 40 crore or so of revenue. Is that YoY that you gave?

Dr. Om P. Manchanda: Yes, it's YoY, same quarter last year.

Binay Singh : Any reason for suburban growth to be lower than your overall growth, any comments on that?

Dr. Om P. Manchanda: I know this is the question which is in everyone's mind. I think the number doesn't show, but we are not unduly worried about this. I think the fundamental thing what has happened in Suburban is there is a very strong strategic shift that is taking place in that company. This company used to be more driven through Doctors network in the past, own infrastructure in the past, we are strictly going by franchising, infrastructure direct -to-consumer, there was a lot of the business of CRO al so which actually was not a business which was very B2B, not a sustainable thing. So, we are trying to build this company more direct -to-consumer, make sure that health checkups and things like that. I think overall directionally 8.3% is definitely an improvement from what we have experienced in the past. Directionally, I think we're getting more confidence about suburban. I am not really too much worried about this number. Ved is telling me core business growth is 13%. I don't want to get into this.

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Classification: Restricted To me, one number is where we stick to, we don't really slice this number into core and non -core. I stay very confident on suburban. That's the way I would say. See, it's actually not run QoQ. It's a need -based business. You just keep doing your work slowly, slowly, one gets the results. I am very, very sure about this. Look at margins of 14.1%. Even before our transition , pre-COVID had single digit margin. So, there is an improvement which is visible. There are green shoots here.

Binay Singh : And sir, secondly, on your own margins, because in the past we have always seen that Q2 is usually the strongest quarter for the company and given revenue leverage that falls in, so margins also tend to be the highest in Q 2. Would you sort of expect a similar trajectory into this year also?

Dr. Om P. Manchanda: Yes, Q2 is always higher. You are right. Most of the spike actually happens around September. But there is no reason why it should not happen because the season, rains, etc., all that happens every year. But as the revenue tends to be in absolute terms also higher in Q2, everything just flows into the margin. I presume that Q2 will always remain higher margin compared to a new quarter.

Binay Singh : In the sense, your margins could at least hold up or inch up on an annualized basis from these levels as you are getting leverage gain like it's visible in your Tier-3?

Dr. Om P. Manchanda: But it gets knocked down in Q3. I just don't look at quarter wise. My eyes always are on the annualized margin where we should really focus.

Binay Singh : And lastly, like after many calls, you've not made much comment on competition this time around in the call. Is that because you see it easing or there was not much to add, that's why it was not there in the opening remarks?

Dr. Om P. Manchanda: I think there are two directions one can talk about competition. One is the intensity of competition which to my mind will continue to remain, it was always there. I think competition became visible because a lot of private equity money and I think there were different format s of cash burn, etc., they were spending a lot of money on promotions which to me has eased off. That's why they are not that visible now. So, competitive intensity from visibility perspective has come down, but it' s not that they are out of the business, they are still there. Those companies that one used to talk about , it continue s to operate. But I think th

e spend to acquire customers, spend to

actually just be visible anywhere and everywhere, that too has come down. I think the other intensity in terms of predatory pricing, this kind of promotion, this Rs. 199, Rs. 99 tests and all, all that actually also has gone away. But in terms of number of players, I think that intensity still continues. And I think as the overall market is growing and you know a healthy sort of margin profile in the industry will always attract competition. And I've always maintained competition is good for the industry because it overall brings up the table in terms of quality level -playing field, and at the end of the day, organized market stil l has to grow because it has to grow from unorganized and which is a very large segment. So, there is a space for everyone to be there as long as there is healthy competition. I think to my mind that extreme, unhealthy competition has now become healthy co mpetition, but competition is still there.

Moderator : We will take our next question from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: Could you give us some sense of specialized test as a percentage of overall revenues maybe last year and do we see that as a differentiator going forward

because we seem to be investing in R&D, we seem to be doing a lot of work on newer tests, so where are we in our journey? Secondly is on Suburban, what will it take to get that inflection point because it's almost now 3 years and we have been

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Classification: Restricted investing in process, we have built the infra we have transitioned from being a doctor advocacy to a consumer brand and I think we are still #3 in Mumbai, what will it take for us to grow Suburban faster than the company average?

Shankha Banerjee: I think on the first question regarding specialized, the specialized portfolio is again the way we define it, because specialized portfolio definition can vary from organization to organization, the way we define it, it currently contributes about 23% of our revenue v/s let's say similar period last year, the contribution would have grown by about 1 -1.5% points. So, yes, that contribution is slowly increasing. It is a differentiator in terms of our ability to really get into larger institutional businesses and also get key opinion leaders to kind of endorse the brand. Is it a huge volume builder? As of now like I said, the contribution is more or less stable, maybe 1% up over a similar period last year. Coming to the question on Suburban, I think the way to look at Suburban is that there is a core geography focus that we talk about, and that geography is slowly and steadily responding for us. But, yes, as a number one when we look at what is happening to suburban, there is quite a lot of non-core business which is still a drag on the overall number, and one would imagine that it still would take maybe few more quarters for us to start seeing the overall suburban number growth rates to maybe move to double digit and early teen level. And after we reach there, we will see how we can get it going faster than DLPL.

Prakash Kapadia: That would be Shankha, from Mumbai only or this Pune or Goa initiative will get us there?

Shankha Banerjee: No. So, we have defined Mumbai, Pune and Goa as the three core. I think those are the three geographies that we are spending all our efforts on right now and those markets are responding quite positively as well.

Moderator: We have our next question from the line of Sumit Gupta from Centrum India. Please go ahead.

Sumit Gupta: I have some questions on Swasthfit. I just want to understand what is driving Swasthfit. So, if we see like overall contribution has been increasing on a sequential basis also? And can you explain how much is coming from rural and urban?

Shankha Banerjee: Let me try and take the first part of the question as to what is driving Swasth

fit. So,

there are three screens that we see. First and foremost, there is this whole awareness amongst the public regarding preventive testing. The whole area in the metro cities as well as kind of approach that we put on online, etc., so there is a slice of that preventive wellness market that we see is helping Swasthfit growth. Secondly, there is also something which we see with channel expansion. We also notice that there is some upselling which happens in the channel. And lastly, we are also seeing in certain markets that clinicians are moving toward starting to prescribe packages. So, those are really drivers. And as we expand our reach further in metro as well as Tier-2, Tier-3 kind of towns, we see that Swasthfit growth is likely to continue. That's really maybe the first part of the question. In terms of split between urban and rural, I think we haven't got those numbers ready. So, for us it is primarily from Tier-1, Tier-2, Tier-3 towns, we will have to see what is the exact split and we can maybe share that number subsequently sometime.

Sumit Gupta: To take a broader view over the next 3-4 years, what is the peak contribution that you expect the Swasthfit to contribute to overall revenue?

Shankha Banerjee: That's a quite a pie in the sky kind of a question. At least one thing we can definitely foresee is that if you see next 3-4 years, it should be higher than where we are today. But how much higher and where is the limit? I think that's something which we are still trying to work out.

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Classification: Restricted Dr. Om P. Manchanda: We did a math and we found out that for the last 5 years, directionally every year we

are increasing by 1.5%. Now, obviously that was on a much smaller base. So, let's hope that continues for some time for next 2-3 years.

Sumit Gupta: How do you see the margins inching up, faring well due to Swasthfit or overall business?

Ved P Goel: As Dr. Om mentioned, we don't see margins QoQ, but on annualized margins.

Sumit Gupta: Overall like over the next 3-4 years?

Ved P Goel: On a longer term, we said in the last call around 27% kind of margins because we also are doing lot of investments, we are going deeper, we are spending more and lot of new investments are required which will give obviously top line in a longer

period. But in the longer term, I think you don't think these kind of margins, much sustainable is around 27% kind of mark.

Sumit Gupta: What was the revenue contribution from North and West, the overall geography mix for this quarter?

Ved P Goel: I think again quarter wise it will not be representative, but I am saying we had like Delhi and NCR which is 31 %-32% kind of contribution and rest of India is balance.

Moderator: We will take our next question from the line of Adrit Chaturvedi from Nomura Financial Advisory and Securities India. Please go ahead.

Adrit Chaturvedi: I would just like to follow up on the awareness campaign bit that you had mentioned earlier. So, in these awareness campaigns, are there also any discounts that you are offering to drive up a lot of these bundle testing volumes?

Shankha Banerjee: So, I think these are two different things in itself. I think bundle testing by definition is discounted. There are awareness campaigns which are built on promoting the bundle test. Like I said, if you look at a package, the package is inherently itself discounted. That's one part of it. But I think some of the maybe visible campaigns that you observe, those are more to do with building awareness of the brand and the service, which is slightly independent from specific package promotion.

Adrit Chaturvedi: Would you have the number handy and if you could give it like at MRP if there are any kind of increases in these bundle testing, right now they're discounted, so there's no true sense of volume that we could gauge, if there's something like a GMV number on that?

Ved P Goel: Generally, I mean these packages are discounted at a 20% -30% kind of discount on MRP and depending on the packages.

Adrit Chaturvedi: Also, I think a lot of these bundle testing are also driving up your test for patients, right? So, right now it's at 2.9. As the share of this increases, do you have like a sense of how up it could go like it will go across 3 or like a 3.1 or is there a vision there that you would like to have more tests per patient or an average number that you track?

Shankha Banerjee: We don't have any operational target on saying that I want to drive test per patient. I think our operational targets are more around increasing our reach and also making these bundle tests available and promoting them. Naturally we see by doing all of these test for patient grows as well as revenue per patient grows. Now, we don't really work on a target saying that I got to grow so much basis. Because it's also a

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Classification: Restricted lot of other factors which drive including geography mix, specialized test portfolio and also the channel through which we sell. We really don't work on a specific target on that. But as the contribution increases of Swasthfit, the test per patient is likely to go up.

Adrit Chaturvedi: This specialized portfolio, so now that it's like 23%, what kind of gross margins are you seeing on that? And I presume that there will be on the lower end of the whole portfolio?

Ved P Goel: Gross margin is different on this specialized v/s routine, but net -net on EBITDA level, I don't think there is much difference because these are high end mostly, we are getting outsourced from hospitals or some 3rd parties where we just spend on logistics. But on B2C there is an infra cost, servicing cost which is much higher. Gross margin is different but on EBITDA level there is not much difference.

Adrit Chaturvedi: Could you qualitatively or like tell me how different could they be like an average specialized versus a routine like what kind of gross margins differentials are you observing?

Ved P Goel: Those are very difficult to measure, but we are measuring on bottom line which is net-net on EBITDA level.

Adrit Chaturvedi: And do you have like a mix of the institutional business through Swasthfit that you are doing versus your retail?

Shankha Banerjee: Swasthfit is predominantly a retail product. It isn't so much into institutional product.

Moderator: We will take our next question from the line of Kunal Randeria from Axis Capital. Please go ahead.

Kunal Randeria: You've been expanding aggressively in this year. From Tier-3 and Tier-4, on company level average for Swasthfit is around 25%. So, even from Tier-3 and Tier-4, is it similar or would it be substantially lower?

Shankha Banerjee: I think this question was asked earlier, urban -rural split, etc., I don't think we have the numbers handy. I think once we have those numbers, we will look at it and see whether it's different or not. However, that may not be the right metric because the competitive set and the growth strategies in these markets could be slightly different, Tier-3, Tier-4 v/s Metro or a Tier-2 town.

Kunal Randeria: Second question is actually for Ved. Ved, you mentioned a 27% is the kind of

EBITDA margin you are comfortable guiding with. You are already at 27% and I am assuming that the Suburban should improve, which means the core margin, the core ex-per cent margins should turn around in the coming year. Is that because you'll be making higher investments if you can just run it through your cost?

Ved P Goel: Yes, Kunal, it will compensate through higher investment in newer markets and spending on awareness, some bit of digital and automation, all these things.

Moderator: We will take our next question from the line of Shal een Kumar from UBS Securities India . Please go ahead.

Shal een Kumar: Two parts of the portio

n. First, we can see that our margins are improving , and they've been consistently improving , and I believe that we have been investing in growth as well. Can you talk a little bit about how we are investing for future growth in terms of maybe more accelerating our patient service centers or entering into new
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Classification: Restricted segment? And second part, when will we see the impact of that coming in? It could be any positive momentum on the growth maybe 3-6 months down the line you see because of investments happening?

Shankha Banerjee: Shaleen, let me try and answer that. In terms of where we are investing, it is in quite a few dimensions. I think Dr. Om mentioned about these 20 labs that we are in the process of setting up. First and foremost, it's in new lab infrastructure and accompanying that obviously you have other overhead costs that get built into the system. Over and above that, we are also improving our spending, whether it's on the digital channel as well as in offli ne to build awareness and also see how we can drive more patient acquisition activities. And as we expand more into non -core geographies in West and South, obviously, the spending needs are substantially higher than in brand strong markets. Primarily those would be where these spends are going to keep going. And lastly, some of it will also go into our frontline, manpower , expansion in sales and marketing kind of areas. Your second question was where we can see the impact. Now actually this is an ongoing activity. It isn't as if that we started this action a few quarters back. This is an action we continue to do over a period of time. So, what we see benefits today are result of acti ons which have been taken maybe a few quarters or a year back. So, it's a rolling action. In case you are expecting a certain spike because of these investments we are making, it may not be so, but yes, steady growth and also ability for us to do better in the newer geographies that we are entering is what we see as the likely outcome.

Shal een Kumar: Is it right to assume that, again, the intensity of our efforts has gone up because of our healthy profitability , strong position in terms of the character and all possibility.

The intensity vis -à-vis later 2 -3 quarters v/s where we are right now, it should be better, right because of where we are and hence , I am not expecting step jump, but a gradual improvement should be followed if the intensity is high , or intensity is increasing?

Shankha Banerjee: Intensity I would say is because of two reasons , one can look at. In our core geographies, this whole expansion into Tier-3, Tier-4 is happening which needs quite a bit of investment from our side, and also in some of our weak brand markets in west and south, there is investment required and the new channel that we have started spending on which is in digital. These is where the whole promotional spend is going. And yes, the outcome expected is a gradual improvement in our growth rates.

Shal een Kumar: Any new segment you are trying to tap in , kind of segments which you are kind of getting excited about?

Shankha Banerjee: I think in terms of segmenting in the market, Dr. Om mentioned that there are various categories that we are trying to create and work on. Obviously, the exciting area is around non -communicable diseases. That's the whole area where Swasthfit and some of our other approaches are really focusing on, variou s things around that, and also some of these specialized areas in and around genetics and testing in that area.

Moderator: We have our next question from the line of Deven Kulkarni from Marcellus Investment Managers. Please go ahead.

Deven Kulkarni : Hi, Ved. For this quarter effective tax rate is around 28%, which seems to be on the higher side. So, just wanted to understand why is that?

Ved P Goel: There is the deferred tax and Suburban depreciation, w

hich obviously has impacted.

There is some adjustment on account of deferred tax. But on an average, this tax rate is not changing, if you will see annualized basis, I don't think there is a change in tax rate. Don't see this quarter as a representative quarter for tax rate.

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Classification: Restricted Deven Kulkarni : And secondly, currently what's the net cash position as on 30th June?
Ved P Goel: Rs. 1,044 crore.

Moderator: We have next question from the line of Abdulkader Puranwala from ICICI Securities.
Please go ahead.

A Puranwala: In the past, we have highlighted some bit of competition coming directly from hospitals where they are trying to forward integrate. I mean any thoughts on how we can tackle this or any measures we have already taken in those areas, if you could highlight?

Dr. Om P. Manchanda: Our experience is the hospital competition is very limited to where the brand salience is there and that's mainly in some catchment area of 4 -5 kilometers. Building a large network for hospital business is very, very challenging, We are not unduly worried about that. As I keep mentioning that it's better that good quality name come into the market, so the overall table will go up. But there have been 4-5 such players who come in, but they have done well only in those markets where the hospital brand is strong and most of them are also listed players, I don't think there's any other listed player who's trying to do this. It's okay, there's a lot of space in the market for competition to be absorbed.

A Puranwala: My next question is with regards to the underlying growth in Q1. So, if I exclude the 8% growth what you mentioned about suburban and Swasthfit, the balance business also seems to have grown at 7 %-8%. Any thoughts over there, I mean is there some bit of lumpiness in a few tests due to which the growth was in single digit?

Shankha Banerjee: Looking at growth excluding Swasthfit may not be appropriate because like I said in terms of the way Swasthfit business is coming, there are places where upselling is happening, also now some doctors ' prescriptions are moving towards Swasthfit, so something which was a non -Swasthfit earlier may also be turning into Swasthfit. I think we need to look at the totality of the business . You can look at maybe suburban separate and DLPL if you want to look at in organic, maybe that will be a more appropriate way to look at it.

A Puranwala: In terms of the margins for Suburban, I mean I understand we have put considerable efforts to improve the margins here. But on a sequential basis also, we did I think close to 17% last quarter and now it's close to 14%. Just wanted to understand is there any lumpiness into this business and on an annualized basis what is the kind of a sustainable margin we should look forward for next 2-3 years perspective?

Dr. Om P. Manchanda: I really won't go by a few percentages here and there because base is too small, Rs. 40-50 lakhs here and there can swing that margin. Our focus clearly in Suburban is to see higher top line growth. I won't be worried about too much about this margin fluctuations. For the next 2 years, we just want to make sure that we get our growth trajectory right and then everything else will fall in place if you have good top line growth.

Ved P Goel: Plus, if you see QoQ , it's improving, last year same quarter it was much lower. So, from that sense because there is a quarterly impact, increment also. On an annualized basis we are very confident that we should be improving.

Moderator: Our next question is from the line of Pranav Chawla from Antique Stock Broking.
Please go ahead.

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Classification: Restricted Pranav Chawla: Sir, I have just one question majorly on your Delhi NCR market. Have we seen the growth rate of NCR slowing down over the past years?

Shankha Banerjee: I think the good news is that we aren't really see

ing a slowdown in the growth rate

rather we see it being sustained at a level which is slightly higher than what we had maybe pre -COVID.

Pranav Chawla: If we are seeing NCR, that is our biggest geography, if that continues to maintain its growth momentum, which would be the market that we are seeing a slowdown as of now, because NCR is among the biggest markets that we have?

Dr. Om P. Manchanda: My sense is that Delhi NCR market has gone through 2-3 phases where there was a phase, there was heightened competition, I presume some bit of a shift may have happened to them in the last couple of years, many of our old customers whom we lost may have come back to us. In between , we had a slightly slower growth in Delhi NCR, but as Shankha mentioned that we are inching towards slightly higher number. Can it go up further? I still doubt because in our business planning, we still don't want to project higher Delhi NCR growth because the base is too high and the market, size is still 3 -4-5 crore of this cluster, while all other clusters like UP, Bihar, Uttarakhand, they are firing for us, where the market size is nearly about 20 -25 crore population. So, that's the way I would put it. Basically, we are very strong in Delhi NCR. Some of our customers who may have gone to the competition may have come back in the last 8 -12 months. This is what we are seeing, and I think Swasthfit

contribution also has pushed up the revenue growth. But I will still stay conservative on Delhi NCR and push the other markets.

Pranav Chawla: Have we seen any change in patient behavior because our patient volumes have been pretty muted for the past couple of quarters, have we seen any improvement on this front?

Dr. Om P. Manchanda: There is an interplay that we are seeing between as the contribution for Swasthfit goes up, the number of patient visits are slightly muted. That may have impacted this growth. Because what we are reporting as a patient volume growth is technically, not unique patients, they are patient visits. But in Swasthfit per visit you end up doing a lot of tests in one -go, the number of visits may actually get reduced. So, I think we are seeing some kind of interplay as the contribution of Swasthfit is going up. I have a sense that patient visits may be inversely related to that. I think there may be some impact that we are seeing. But I think overall one should see the sample growth, which is nearly about 9.6% which is a combination of number of tests per patient into patient footfall.

Pranav Chawla: Can you give us some color on FY25 as well as FY26 growth that you are internally projecting?

Dr. Om P. Manchanda: I think FY25, I told you last time we will try and beat the FY24 number. This quarter is 11.3, its better than 10.4%. I have a feeling we will continue to do better and then definitely beat 10.4% of last year. As I mentioned that September is a crucial month for the entire industry, not only for us, I don't know how it turns out to be, but I think I can confidently say that we will beat the last year's number.

Shankha Banerjee: Adding to that, we beat last year growth number without having to take a price increase. That is what we are trying to drive the sales increase. And once we see 2-3 quarters of this year, maybe we will be in a position to then start looking at what the next year outlook.

Dr. Om P. Manchanda: I think October might be just a good month to talk about FY26.

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Classification: Restricted Pranav Chawla: What is the strategy of cash utilization given that we have cash building on the balance sheet?

Dr. Om P. Manchanda: Time and again we have said, our priority #1 is to look for strategic assets, inorganic which are in South and West Region, in West we have done Suburban. If you look at contribution wise, region wise obviously South is still very weak. We would want to fill that gap. There is no doubt about that. But

we also need to have a quality asset

at the right price, much high on governance, etc., so that's given. In absence of that happening because you don't know that doesn't happen every month or every quarter, it may happen, may not happen. But in the meantime, we have increased our dividend payout, we are trying to balance between the two and also investment in our new labs and technology. I think combination of all these 3s where we want to utilize cash. Cash and the balance sheet is also good because our ability to do certain transactions can actually go up as well. #1 priority is to see deploy this money for growth primarily in acquisition because it's a global experience that in new markets it's very challenging to establish organically. So, one has to look at that.

Moderator: We will take our next question from the line of Ashutosh Parashar from Mirabilis Investment Trust. Please go ahead.

A Parashar: So, just a couple of questions. On the 20 labs that we are planning to open this year, can you give us some color on the geographic spread of these labs. Are these likely to come up in the markets of Bihar and UP? And how many of these are planned for Suburban?

Shankha Banerjee: The primary focus is going to be, we have been doing in Tier-3, Tier-4 geographies of North and East. That's where the primary focus is going to be. Suburban, there isn't really a plan to add too many labs, there are 1 or 2 maybe gaps which we will fill, we are still evaluating that, but this is more in our core geographies of North and East.

A Parashar: On the Suburban trend, it's been some time since we have integrated the acquisition. Are there still some gaps on testing mix and all that we have to address or we have largely addressed that?

Shankha Banerjee: There are various layers of integration that are currently working. From a test perspective, now the whole test menu, which Dr. Lal PathLabs operates is today available through Suburban as well. But having said that, there are certain background integration, IT integration work, which is still happening to make it even more seamless, that work is going to take some more time. But as of now, clients in the geography, in which Suburban is operating have access to all test menu from Dr. Lal PathLabs.

A Parashar: What would be the average realization for our Swasthfit test?

Dr. Om P. Manchanda: You see, my reading is, you can do a reverse math in any case, right? 25% contribution, you know the portfolio revenue contribution earlier and now I think you can figure that out, but exact figure we have not really shared so far.

Moderator: Ladies and gentlemen, that was the last question for today. I now hand the conference over to management for closing comments. Over to you sir.

Ved P Goel: Thank you everyone for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries satisfactorily. Please feel free to reach out to us in case you have any further questions. Thank you once again.

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Call: Oct 2024

Classification: Internal
October 28, 2024

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q2 & H1 FY25 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q2 & H1 FY25 Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 15

Dr. Lal PathLabs
Q2 & H1 FY25 Earnings Conference Call Transcript
October 23, 2024

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group Chief Financial Officer & CEO – International Business
Participants who asked questions • Amey Chakraborty – JM Financial
• Tausif Shaikh - BNP Paribas Securities
• Karthik Chellappa – Indus Capital Advisors Hong Kong Limited
• Prakash Kapadia – Spark PMS
• Anshul Agrawal - Emkay Global
• Nancy Yadav - Allegro Capital Advisors
• Saion Mukherjee - Nomura Securities
• Prashant Nair - Ambit Capital
• Bino Pathiparampil - Elara Capital
• Sumit Gupta – Centrum India
• Siddhesh Raje - ICICI Prudential AMC Ltd.

- Bhavya Sanghvi – Phillip Capital
- Shyam Srinivasan - Goldman Sachs
- Vaibhav Saboo - Nippon AIF

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Moderator: Ladies and gentlemen, good day and welcome to Dr Lal PathLabs Q2 FY25 earnings conference call. As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference to Mr. Siddharth Rangnekar from CDR India. Thank you and over to you, sir.

Siddharth Rangnekar: Thank you, sir. Good evening, everyone, and welcome to Dr Lal PathLabs Q2 & H1 FY25 Earnings Conference Call. Today, we are joined by senior members of the management, including Hony. Brig. Dr. Arvind Lal , Executive Chairman, Dr. Om Prakash Manchanda , Managing Director , Mr. Shankha Banerjee , Chief Executive Officer and Mr. Ved Prakash Goel , Group Chief Financial Officer and CEO of International Operations.

I would like to share that some of the statements made on today's call could be forward -looking in nature and actual results could vary from these forward -looking statements. A detailed description in this regard is available in the results presentation , which is available on the stock exchange websites and has been separately circulated to all of you.

I would now like to invite Hony. Brig. Dr. Arvind Lal, to share his perspective. Thank you. And over to you, Dr Lal.

Dr. Arvind Lal: Thank you, Siddharth and good evening, ladies and gentlemen and a very warm welcome to all the participants on today's call to discuss our Q2 FY25 performance. I will commence by discussing the evolving market dynamics and our key achievements.

Our ability to maintain our market position in the highly fragmented and competitive Indian diagnostics industry is a testament to our strong brand, exceptional service and extensive network. We are leveraging these strengths to penetrate deeper into under served Tier-3 and Tier-4 regions by offering affordable high-quality diagnostics to the patients.

Dr Lal PathLabs have scaled up nationally into a well -run operation with an extensive franchise base, consistent delivery of high quality and affordable diagnostics reports to our patients has earned us this position. We believe this to be a core factor why patients have faith in our brand.

In today's evolving healthcare landscape, digital integration has become imperative, and we are actively leveraging technology to enhance patient experience and operational efficiency. Currently, we are also expanding our home diagnostic services to meet the growing demand from patients who value convenience. We are committed to achieving these objectives without taking any price hike and focusing on driving patient volume, growth and efficiency gains.

India has a vast unserved and underserved population in terms of healthcare and diagnostics. The headroom of growth therefore is very much there. Together, with policy impetus, tech innovation and growing propensity to avail of quality and accurate diagnostics, the share of national brands like ours is rising. We continue to benefit from our scale of operations and brand recognition, both of which are crucial factors to seed consolidation at the industry level. In the interim, our focus remains on building out organically while evaluating inorganic options that fit our objectives and values.

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As we celebrate 75 years of excellence, it is an opportunity to reflect on the past, celebrate achievements and look forward to the future. We remain committed to serving the patient community through consistent service and quality of diagnosis. And at this point of time, I would like to apprise you of a development. As you are all aware that Dr. Om Manchanda ends his term on March 31st, 2025, he will be stepping down from his position as managing

director into an advisory role. We

personally thank him for all the contribution he has made in building Dr Lal PathLabs into the leading and best diagnostic company in India. Nearly, 20 years of his operational and management excellence has helped the company in achieving this goal. We wish him the very best in his new role and look forward to his continued guidance and direction.

Thank you and I would now like to hand over to Dr . Om. Over to you, Om.

Dr. Om P Manchanda: Thank you, Dr. Lal and very warm welcome to all the participants on this call. First things first , as Dr. Lal mentioned that my term ends on 31st March 2025, post that I shall transition to my new role as an advisor to the company. Incidentally, I also turn 60 next year and this milestone coincides with end of my term. I thought this is the best time for me to transition into a new phase of my career from an operating role to an advisory role. A large part of my executive role is already transitioned to our CEO , Mr. Shankha Banerjee. I shall further transition to him during the remaining period of my term. Now, let me shift to the business and industry insight .

I think the very important insight that I have picked up and which we have been talking about the last few quarters, that is the concept of bundled test packages, has become a very, very firm trend in industry . In my view, it is having the following impact :

Number one, it is leading to higher realization per patient resulting due to higher number of tests per patient and this is further leading to slight improvement in the industry margins is the trend that I notice.

The second important impact of trend is that the business processes are getting very simplified in terms of whether it's ordering by the patient or collection of blood samples and even in the lab operations.

Our Bundle Test Program, which is "Swasthfit " continues to do well and is now almost 24% of the company's overall revenue. As we reach out to a wider patient base, the growing contribution from such bundled offering both in routine, semi -specialized and also specialized testing will further add to our performance going forward.

The second point that I want to make is that our efforts in building West region platform has started yielding results. West business is now nearly 15% of the total revenue, that is nearly Rs. 100 crore in a quarter. It is a very unique business in nature as 60% of the turnover is inorganic. This quarter the West region has delivered better growth than overall company growth. It will be a great learning platform for us as we build out our hybrid strategy in non -core markets, that is West and South.

We are continuously investing in our capabilities not only in technologies that drive growth and efficiencies, but also on the cyber security aspect, further solidifying defense around patient data. Going forward, we are also investing towards organizing a nd analyzing data better so that it helps in raising the quality of our offering.

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With that, now I'd like to hand it over to Shankha for his thoughts. Thank you.

Shankha Banerjee: Thank you, Dr. Om. A warm welcome to all participants on this call today. Let me share the business and operating highlights with you .

The positive momentum in performance has continued with revenue and profit after tax growing by 9.8% and 18.1% respectively in the 2nd Quarter of FY25 . This performance is a testament to our strategic focus on both expanding our service network and meeting evolving patient needs.

The growth in revenue is driven by sample volumes, which stand at 23 million. Our patient volumes came at 7.8 million. Sample volume growth for Q2 FY25 is 8.6% and patient volume growth is 3.9% over Q2 FY 24. Our Q2 FY25 revenue per patient stands at Rs. 844, an increase of 5.7%. The realization improvement does not have any price increase impact, rather it is due to product a

nd geography mix.

We are happy to share that in Q2 FY25 Suburban has delivered revenue growth in double digits in line with our plans. It has started contributing positively to our overall and West region growth.

Our ongoing expansion initiatives are taking concrete steps. As indicated earlier, we will be opening 15 to 20 additional labs in this financial year and are accelerating the addition of collection centers to support these labs and meet incremental demand.

These developments are crucial in widening our footprint across Tier-3 and Tier-4 markets and deepening our presence in core regions.

To align with our long -term vision, we are expanding our services to include bundled testing for non -communicable diseases beyond the Tier -2 market. This addresses the increasing demand for healthcare services, particularly in managing chronic illnesses.

Our commitment to reaching underserved markets remain steadfast and we are also strategically strengthening our footprint in creek clusters, particularly in West and South India We are undertaking targeted brand campaigns in selected cities to increase visibility and stay connected with local communities. In addition, we are driving several initiatives to foster loyalty to encourage patient growth within our core

audience.

We remain confident in realizing growth organically backed by network outreach and investments in communication and technology. As we have the benefit of scale, we are in an advantageous position to benefit from positive trends shaping the diagnostic landscape.

I would now like to hand over the call to Ved who will walk you through the financial performance. Over to you, Ved.

Ved P Goel: Thank you, Shankha. Good evening, everyone, and a warm welcome.

I will be sharing the key financial highlights for Q2 and First Half of FY25

- Revenue for Q2 FY25 came in at Rs. 660 crore compared to Rs. 601 crore in the same quarter last year, reflecting a growth of 9.8%. The first half of FY25 revenue stands at Rs. 1,262 crore, a growth of 10.5%.
- Revenue per patient for Q2 FY25 is Rs. 844, 5.7% higher compared to Rs. 798 in Q2 FY 24.

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- Test per patient for Q2 FY25 is 2.94 v/s 2.81 in Q2 last year, registering a growth of 4.5%.
- EBITDA for Q2 FY25 came in at Rs. 202 crore compared to Rs. 178 crore in Q2 FY 24, registering a growth of 13.9% with an EBITDA margin of 30.7%.
- EBITDA for first half of FY25 stand at Rs. 372 crore v/s Rs. 324 crore in FY24 , registering a growth of 15% with EBITDA margins at 29.5%.
- PBT for Q2 FY25 came in at Rs. 183 crore, registering a growth of 20.3% with the PBT margin of 27.7% PBT for first half of FY25 stands at Rs. 333 crore with the margin at 26.4%.
- PAT for Q2 FY25 came in at Rs. 131 crore compared to Rs. 111 crore in the same quarter last year, registering a growth of 18.1% with the PAT margin of 19.8%.
- PAT for the first half of FY25 stands at Rs. 239 crore versus Rs. 194 crore in FY24 , registering a growth of 22.8% with the margin of 18.9%. Improved profitability is on account of operating leverage and strategic initiatives to optimize cost by using technology.
- Earnings per share for Q2 FY25 is Rs. 15.5 compared to Rs. 13.2 in Q2 FY24 . First half FY25 EPS stands at Rs. 28.3 as compared to Rs. 23.1 in FY24 .
- Net cash as on September 30th , 2024, is Rs. 1,095 crore.
- The DSO as on Q2 FY25 is 25 days and we are still enjoying negative working capital of 22 days.
- Further, I am pleased to share that the board of directors of the company have approved a second interim dividend of Rs. 6 per share for FY25 .

With this I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A. Thank you.

Moderator: We will now begin the question -and-answer session. First question is from the line of Amey Chalke from JM Financial. Please go ahead.

Amey Chalke: The first

st question I have is on the Suburban for the improvement in the operational performance for QoQ . Is it possible for the management to clarify? Slight margins have improved to 20%, last quarter it was around 14%. What were the drivers during this quarter , is it only the operating leverage which has played out or something else which we should watch?

Ved P Goel: Yes, Amey, you are right that operating leverage is playing and there are certain strategies and initiative which we are taking, with use of technology, with the back - end efficiency, productivity enhancement and so on and so forth. I think that is the reason for Suburban margins is inching up.

Amey Chalke: Going ahead, how should we think the trend for the following quarters for 3Q and 4Q because 3Q could be stronger, but again, there could be a weaker quarter, so like how should we think about for a whole year?

Dr. Om P Manchanda: I still will continue to maintain that we should not focus too much on margin in Suburban. Our intent is to drive growth rate even further and in any case, our overall company margins are pretty healthy. I would rather invest in driving growth at the moment than worry about improving the margins. I will sacrifice some margins to drive growth in Suburban , that's the way I look at it. If we get leverage benefit, no problem, but I think our eyes are focused more on top line right now.

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Amey Chalke: Is it possible for the management to give a growth region wise for the quarter or at least how much the growth was in north and ex of north?

Dr. Om P Manchanda: I do not think we actually share region wise growth on quarterly basis, but one thing

I can share with you is that interestingly it's uniformly spread across the regions. Maybe couple of percentage higher and lower here and there. As I mentioned in my comments, We st is slightly better. I also get a sense that the West region had higher incidence of fever this time than maybe rest of the region. But I think overall our growth rates across all regions is uniformly spread.

Moderator: Next question is from the line of Tausif Shaikh from BNP Paribas Securities . Please go ahead.

Tausif Shaikh : I have two questions mainly on your expansion plan in Tier-2 and Tier-3 cities. Can you throw some light how the realization is in these cities and how is the volume ramp up , are they slower compared to the metro cities when you open a new lab?

Shankha Banerjee: Our Tier-3 and Tier-4 city expansion is predominantly in our core geographies of North and East and in these geographies , we are going deeper. So, pricing in these geographies is quite similar. In the contiguous market, the pricing is not lower. So, realizations are similar in terms of the contiguous geographies that we are going into. And in terms of ramp up, it takes the normal usual time that one would take to ramp up. But the throughput of these labs is managed in a way that it gathers slightly bigger radius to ensure that the volume throughput or lab is taken care of.

Tausif Shaikh : And also, what kind of challenges do you see in these kind of cities ? Is it fair to assume the unorganized player offer s a path and radio mix business, so that's some kind of challenge which these kind of players like Dr . Lal face a challenge instead of a pure path player?

Shankha Banerjee: In all of these markets, the predominant task is converting the unorganized to organized and I think challenges are more or less similar whatever we have seen in our previous years as we have done it in other geographies. There is always a mix of players of fering path and non -path as well as pure path. Those are not really challenges which are different than what we have done in the past. I think differential challenge to that extent is not something which is very different. It is more or less similar. It's just that it's a smaller ground that we're reaching out today.

Dr. Om

P Manchanda: Yes, I think I may just add what Shankha just mentioned. I think it's true not only for diagnostics, but for all healthcare sector, even hospitals also I am sure must be facing a challenge. As we go in smaller towns, there are very strong relationships between patient and doctor and between the doctors also. For any brand which is slightly global or acting in nature to penetrate to in that relation becomes very, very challenging. It's a very slow process , gradual process , you have to really communicate your proposition very well so that you can overcome the power of those relationships with the kind of service that you offer. But once you reach that tipping point, then I think the whole shift happens from unorganized to org anized. And it takes a little bit of time, but as we build scale my sense is the time it takes now is much lesser than what it was taking before.

Moderator: Next question is from the line of Karthik Chellappa from Indus Capital Advisors Hong Kong Limited. Please go ahead.

K Chellappa: My first question is, if we were to look at our volume growth at least on samples, for the last 4 quarters, it has been somewhere in the range of about 8% -9% and it is backed by very healthy margin expansion. I am just curious to see whether there is scope for us to reinvest some of these margins to drive a higher volume growth? And

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in your opinion, what needs to happen for volume to kind of move up to a more sustainable double -digit level?

Shankha Banerjee: Firstly, let me try and answer the question that the sample growth is driven by also improved contribution which is coming through Swasthfit and we are seeing even in the non -Swasthfit portfolio the test per patient is slightly up compared to what we were seeing maybe a year before , that is what is driving sample per patient and therefore our overall sample growth. Now, I think investing back into growth is something that we are continuously doing. Like I mentioned in my opening remarks, this year we are planning to open a higher number of labs between 15 to 20 labs, we will be adding this financial year. Also, we are doing investment in terms of communication in key cities very strategically wherever we feel that adds value to our brand and our position.

We are also strengthening our market connect and our sales teams and those kind of investments are continuously being made. It is not as if whatever is there investments to be done that are being held back, because if you see our A&P spends are actually inching up every quarter when you compare with the same period last quarter , those are definitely there. And whether it will be a double -digit patient volume growth or not, I think that's something which we will have to come back to, I think that's an aspiration, but not really in the near future because there is a mix of two

things. One is the existing patient volume getting impacted because of more Swasthfit and also on the balancing factor, the outreach we are doing to newer geographies and newer Tier-3, Tier-4 towns and the activities we are doing there which is adding new patients, it's still under churn. When does it reach double digit is something which we will come back to that a bit later.

K Chellappa: My second question is, as far as competition is concerned, let's say, compare to your commentary a quarter ago or even two quarters ago, has there been any change either for the positive or negative, either in the form of pricing, discounting or number of players or any other attribute that you measure?

Dr. Om P Manchanda: I think our commentary is exactly very steady. First, I think you all know that our numbers should be viewed with the backdrop that we have not taken any price increase. Many of our competitors have taken price increase and we also have adverse impact of higher base as well. From that perspective, we always said that, we try and meet or

slightly beat our last year's performance, which I think at first half level, our growth is 10.4% compared to what we did last year full year. Last year 10.4% had an impact of nearly 3% of price increase. And this year, we have not had that and despite that not only the revenue, but we have also improved our margins a bit as well.

We continue to maintain that steady sort of commentary as to what we have been saying. I think if you really want to know about competitive intensity, I think the general perception in the market is that competition is less. I do not think that is true. Competitive intensity is still there. I think what is not there is irrational pricing, promotion, discounts, those are happening. But competitive intensity to my mind still continues to be there. I think many of these young new age players are looking for a path to profitability. My sense is they have stopped deep discounting their promotions, etc. But definitely one competition from hospital is on the horizon because I continue to see hospitals trying to move into retail pathology business, to my mind intensity is still there in the market. It's just that irrational competition on pricing is reduced.

K Chellappa: As far as the management succession is concerned, so once Dr. Om steps down in March '25, how is the succession going to be planned, will it be someone internally
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or are we going to look for somebody externally or could you give us some color or thought on that?

Dr. Om P Manchanda: If you look at we in any case have a full-time CEO who is Shankha Banerjee. We split this position a few years back and a large part of my role has already been transitioned to Shankha and he will spearhead as far as the internal management is concerned. And along with that, Ved is there as a group CFO as well as the international CEO. That's the way it is. As of now, there is no other plan as far as succession is concerned.

Moderator: Next question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: A couple of questions from my end. On Swasthfit, if you could give us some sense how much of this is a doctor recommended, how much is directly and is there a scope to increase this contribution in Tier-3, Tier-4 cities also? Given the work we have done over the last two years now in Suburban, so is it fair to assume now Suburban will continue to grow higher than the company average from here on? And lastly from a higher revenue growth perspective if I were to just guess the drivers in the medium term, will it be market share gain from smaller players, will it be higher number of patients or will it be more revenue from patients or you see Tier-3, Tier-4 contribution increasing, what could be the pecking order in terms of just trying to assess the potential revenue over the medium-term?

Shankha Banerjee: Prakash, let me try and answer them to the best possible degree. I think firstly on the Swasthfit contribution from direct patient v/s doctor prescription, we do not have accurate data on that, but one would assume that quite a bulk of it is also upgrading prescriptions, they may not directly be a Swasthfit prescription, but some prescriptions get upgraded, so quite a bulk of our Swasthfit kind of comes through that. We will not have exact numbers but that is one bit. Secondly, whether we are taking it to Tier-3 and Tier-4, I think that's something which I mentioned in my opening comment as well that we are actively taking Swasthfit packages into our Tier-3, Tier-4 markets and we are also seeing some traction build up in those markets on Swasthfit which is giving us the confidence that the growth trajectory we are seeing on Swasthfit still has the legs to run on.

I think on Suburban, yes, lot of work has been done in the last two years and I think we have now started seeing results on that. Independently, we definitely feel that, or we believe

ieve that Suburban growth rates are going to be sustained or better as we keep moving forward. Will it be always better than the company? Actually, my expectation would be even the other part of the company should grow faster. So, to me those two remain independent. But yes, Suburban growth, we should be able to now sustain double -digit and grow maybe even faster than that.

Your last question , in terms of how will future growth come and what are the driver? Yes, we continue to look at newer geographies, which is the Tier-3, Tier-4 expansion. I talked about new labs. We are opening more sales headquarters reaching more directly to more clients across more of geography, that gives wider coverage. And in South and West, we continue to focus on our core cities and core clusters and building our brand and writing on whatever we have built in the past, getting higher throughputs in our existing infrastructure. Those will be the key drivers and also the bundled test portfolio growth continuing.

Dr. Om P Manchanda: I just want to add one more thing about Suburban point that while we track Suburban performance separately, but to my mind we do not look at that number in isolation.

In fact as I mentioned that 60% of our business is coming from an inorganic in the West region. It is not only Suburban. It is also coming from that small subsidiary company that we have Pathlabs Unifiers. We actually look at this as a driving network

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effect of labs, collection center of all the brands put together in the region. We track first is West region, the entire region's growth first and then look at each component separately. As long as we are able to drive the region's growth, which is significantly better than company, I think we probably would have achieved our objective.

Prakash Kapadia: Lastly, Shankha, you mentioned about the growth drivers for a higher kind of revenue growth. Historically we had seen post -COVID lot of these new labs had sprung up. Any sense on these unorganized or these players getting marginalized because I guess over the mid -to-long term that also would aid growth and now should translate or show into higher growth, is that happening on the ground, not happening if you could give u s some sense that would be helpful.

Shankha Banerjee: It's a bit of mixed feedback there. The COVID period actually created a bit of a cushion for a lot of the single operating labs because lot of them need quite a bit of money. Therefore, some people have tried to expand, or they are sustaining their businesses and still continuing to run their operations , that is at one end. At another end, we also see people who may want to be exiting or wanting to close their businesses. But yes, it is a bit different than what we were seeing pre -COVID. There is still some bit of aggression that we see in the local lab ne tworks, but we have our eyes and ears open, I think the situation should improve in terms of consolidation as we move forward.

Moderator: Next question is from the line of Anshul Agrawal from Emkay Global. Please go ahead.

Anshul Agrawal: My first question is on network expansion. Is the target of adding 15 to 20 labs in the current year more back ended in nature? How many labs have we added in H1.

Shankha Banerjee: Although we do not maybe share the number, but one can say that we are on track, 50% of these labs have already been added in H1.

Anshul Agrawal: The reason why this is we have only incurred a CAPEX of about Rs. 10 crore in H1 if I am not mistaken. So, would that mean that our CAPEX numbers would be materially lower than what we had incurred last year?

Ved P Goel: I mean most of these CAPEX is happening in second half of the year. Though we have started the labs but maybe most of the real operation will start in the second half. That's why the CAPEX is low. And historically also if you see the CAPEX in first half is always low.

Anshul Agrawal: Our CAPEX guidance of roughly around Rs. 50-60-odd crore would stay as is. Secondly, the moment we sort of commission these labs, would our margins also get impacted in the second half of the year?

Ved P Goel: I think it's a good question. I mean that's why I always say that do not look at margins for QoQ basis, but on an annual basis, I think I mentioned on the last call as well that we are hoping that we can do slightly better than last year, but our margins will be sustained as last year. This Q2 always has a higher margin because of operating leverage and efficiency and due to most of the investments that come in the second half, that's why I think the overall margin for Q2 will be lower than H2.

Anshul Agrawal: Second question is on Swasthfit portfolio. If you may throw some light on this, what percentage of the Swasthfit portfolio would be illness testing and what percentage would be wellness?

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Shankha Banerjee: I think somebody in the past also asked this question how much is home v/s patient prescription upgraded. We do not have any accurate data to say that, but one can

look at it, either illness part is also the sizeable component.

Anshul Agrawal: What are our plans for the cash that we have on books? I mean, I am sure we do not have any CAPEX sort of requirement. Do we plan on giving out increasing dividend payouts or what is your thought process on the cash that we have on books ?

Dr. Om P Manchanda: See, obviously, our first priority is to use this cash for growth with purpose and growth consists of both organic and inorganic. Those efforts will continue. While organic we have better control on how we expand. Inorganic is very difficult to time it, but consistently we have been also paying out dividend, that will also continue. I think it's a combination of all 3 organic, inorganic and dividend and we will continue to progress as we have been doing in the past. Our effort on inorganic continues. If we cannot use this cash, then obviously a dividend will be paid out.

Moderator: Next question is from the line of Nancy Yadav from Allegro Capital Advisors. Please go ahead.

Nancy Yadav: I wanted to ask if we can give out any numbers on the IndAS adjustment that we have made for this quarter?

Ved P Goel: Nancy, IndAS adjustment is not new, I mean, it is implemented 3-4 years back and there is no exceptional in this quarter, I would say it's very much comparable as last year.

Nancy Yadav: And also, sir, if it's possible to give any sort of exact numbers for the Suburban labs like if we could get a percentage of exact growth from the previous quarters or any numbers that you will be able to provide?

Ved P Goel: Suburban growth this quarter is about 11.6% and EBITDA margin is about 20%.

Moderator: Next question is from the line of Saion Mukherjee from Nomura Securities . Please go ahead.

Saion Mukherjee: This strategy of not taking price increase unlike your competitors, I think, historically,

Dr. Lal used to take low single digit price increases. I am just wondering is this a strategy for a year or you think it should sustain this on a slightly longer -term perspective? And generally, we see inflation across everywhere. There's like 2 -3% price increase is something where the volume sensitivity is very high. How do you assess the strategy of keeping or not taking any price increase?

Dr. Om P Manchanda: We are not averse to taking price increase. I do not think it's our stated strategy that we will not take price increase as our effort is to go down to Tier-3, Tier-4 towns. As all of us are aware that those are the markets that require affordability and to achieve that if we continue to take price increases, then we do not achieve that objective. As long as we are able to achieve this without diluting our margins, that's the reason why we resist taking price increases, but it's not that if infla

tion is higher than what

we can absorb through this leverage, then we will obviously be forcing a price increase. But our intention is to make sure that our proposition appeals to wider footprint, otherwise, we can get narrowed down to large cities.

Saion Mukherjee: The second question I have on this employee expense in the quarter. It seems to have gone up almost 18% YoY, much higher than the revenue growth. If you can throw some light on the dynamics there?

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Ved P Goel: One is obviously, every year we give some increments and as Shankha mentioned, we are in the process of opening these 15 -20 labs. New additions, new manpower and so on and so forth , there is nothing which is exceptional here, but these are all where some increment every year we give to the employees and plus additional manpower which we are hiring.

Saion Mukherjee: And finally, now you have guided for more than 10.5% growth this year. Now, as you look forward with the lab expansion, Suburban picking up growth and possibly some price increase, are you expecting an acceleration in growth as we go forward, or you think low double digit is a level that we should sustain from a slightly medium -term perspective?

Shankha Banerjee: We have been saying equal to or better than last year growth, which is 10.4% and first half now we have delivered 10.5%. Therefore , our guidance still remains the same from that point of view. I think the point about Suburban growth coming through and price increase, like this financial year, definitely we may not be taking price increases like even Dr . Om mentioned. We continue to evaluate , if we feel that we need to take a price increase, we will take a price increase, but definitely not to drive revenue growth, that is not going to be a reason to take price increases because we like to be at broad base as possible. As of now we will stay with the revenue guidance that we have given in the past few quarters.

Saion Mukherjee: I was just looking at let's say for FY 26. Are you expecting growth to accelerate from let's say 10.5% this year or it would more or less remain at the current level?

Shankha Banerjee: See, our next year's financial plan we will start thinking once we are through with Q3

and maybe we will be in a better position to start talking about that a quarter or 4 -5 months from now.

Dr. Om P Manchanda: And also, please keep in mind that our base is much higher than many of our competitors.

Moderator: Next question is from the line of Prashant Nair from Ambit Capital. Please go ahead.

Prashant Nair: Related to how do you plan your network as you go into the future. 5-6 years back, you had a certain hub -and-spoke model with a certain ratio of collection centers to labs which has served you well so far. Now as you go into Tier-3 markets increasingly or into newer markets, would that change in any manner? So, I am talking more about number of collection centers in relation to number of labs. Would you be able to do with more collection centers relative to labs or would you need to set up more labs, how do you think about the network as you go into your next phase of development?

Shankha Banerjee: Over the last few years, we have been steadily increasing the number of collection centers per retail lab that we have, and we reached a reasonably good number of almost 28 -29 labs, in the last financial year we reported I think close to about 29 collection centers for retail labs. I think that kind of a ratio we should be able to sustain even with our expansion into Tier-3 and Tier-4 models. The broad hub -and-spoke model remains in place. It is just that we are now going into as closer to the consumer as possible even in the hinterland, but the overall model is not really changing.

Moderator: Next question is from the line of Bino Pathiparampil from Elara Capital. Please go ahead.

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B Pathiparampil: Looking at thi

s 15 -20 labs this year number, is that going to be a strategy going forward as well, so can we look at a similar number of lab additions for the next few years per year?

Shankha Banerjee: If you see our historic trend, typically we have added between maybe 10 -15 labs if you go back to pre -COVID that kind of run rate we usually had. In the last two or three years, there has been a bit of a slowdown in terms of the number of labs that we have added. This year is also a bit of a catch up in terms of doing 15 -20 labs. At best, this rate may be sustained for one more year, but chances are we will start going back to what our normal pre -COVID level of lab addition would be in a given year.

B Pathiparampil: Just one question to Ved. Depreciation expense has actually come down a little bit YoY. Is it going to be like this at the given level of CAPEX or will this change and start moving up again?

Ved P Goel: No. First half depreciation is about Rs. 70-odd crore, which is similar as last year. I mentioned that most of the CAPEX is usually in the second half. I think depreciation will be higher in the second half compared to the first half.

B Pathiparampil: Yes, but YoY comparison, would it be similar, or it will keep going up?

Ved P Goel: Yes, it is similar unless and until we are doing large CAPEX because there is a depreciation, there is addition, net-net you are finding that this is the impact.

B Pathiparampil: For the current level of CAPEX that you are doing, should depreciation be a kind of stable expenditure going forward?

Ved P Goel: Yes.

Moderator: Next question is from the line of Sumit Gupta from Centrum Broking. Please go ahead.

Sumit Gupta: Just want to understand on the margin guidance that you highlighted 27%. However, let's say for the second half then it comes out to be around 24.5 %-25%. I just want to understand where those investments will be largely since 50% of the lab addition has been done, just want to understand on the investment part? And what kind of returns that you can generate?

Ved P Goel: Sumit, as I mentioned, I mean, though we have started and opened few labs, but the real impact will start coming in second half, that is number one. Second, historically also if you see our Q3 particularly our margins are quite different than Q2. Overall, I am saying margin, we can do slightly better than last year, but it is not something which is like you have seen in Q2, which is not a representation of the full year.

Sumit Gupta: Just want to understand the majority of the breakup of the CAPEX that you would be investing in apart from the lab side?

Ved P Goel: There are investment in the technology, digital, there are higher in spending on the A&P, we are going deeper, we are going newer markets, there are investments, awareness and so and so forth. There are logistic expenses which will go up, when you added new infra you have to add newer routes and so on and so forth. There are expenses which are coming along with this new infra. It is not only the CAPEX, but the running expenses will also go up.

Sumit Gupta: What kind of margins can we target for the Suburban, let's say, over the next one year or so?

Ved P Goel: As Dr. Om mentioned, right now the focus is to drive top line rather than to see margins. Having said that and naturally whatever efficiency we can generate whether it is maintaining the efficiency or improving the efficiency. As Q2 is 20%, it is not representation of the full year as Q2 is always higher, but overall basis maybe you can take 16 %-17% kind of steady margins for the Suburban.

Shankha Banerjee: Also, I think it will be important or one thing would be to look at company total margin and that guidance. I think that will be a much more stable indicator because our key thing for Suburban will continue to be how to drive the top line. The margin profile of Suburban can f

luctuate. But I think the important thing would be to keep looking at the company total margin guidance.

Dr. Om P Manchanda: I think margins can fluctuate because we are also investing heavily into IT systems there. Some quarters those costs may come, A&P as well. I think my reading is that just let's focus on top line, right, because overall company margins are taking care of Suburban.

Sumit Gupta: On the geographical side, Delhi NCR still contributes around 30 %-32%?

Shankha Banerjee: Yes, Delhi NCR contribution is still at 31% for Q2.

Moderator: Next question is from the line of Siddhesh Raje from ICICI Prudential AMC Limited . Please go ahead.

Siddhesh Raje: I have two questions. One is we have seen this number of patient volume growth of 3.9% YoY and Swasthfit revenue is growing by more than 20%. Is it fair to assume that this new patient volume growth would have come largely because people are taking this Swasthfit packages and in that context where can the Swasthfit contribution settle down at company level? Second, in the last few calls you menti oned that the company will look to reinvest back margins into business so that even incremental competition gets very attractive, right. So, in that context when we say that we may do better than last year in terms of margin, can it incrementally attract more competition?

Shankha Banerjee: I think firstly to answer Swasthfit , where is it likely to settle down? I think in one of the previous calls also I had mentioned that we are seeing traction in terms of the newer geographies even in Tier-3 and Tier-4 as well as the source of revenue coming or these packages even coming from patients who come with prescription. So, therefore the scope of Swasthfit is much, much wider than only wellness. We still believe that there is a reasonable headroom for it to continue to grow at this level and improve contribution further. What that number would be is something which I do not think we are looking at putting a figure on right now. We will see how that evolves.

Also, in terms of overall market requirements, the incidence of non -communicable diseases keeps going up and that also is a driver for some of these health packages being utilized by patients , that's one. I think on the part about margins attracting competition, I think competition is there from multiple sides. I think Dr . Om mentioned in his comments, now we are seeing competition emerging from the hospital players wanting to get into retail pathology and they are driven by a very, very different thought process than just pure what is the retail or the margin profile that we see. As an industry I think again Dr . Om had made a remark in his opening comments that in the overall industry we are s eeing improvement in the margin profile , those things

are happening. Whether it will be attracting more investment? I am not too sure whether that's something I will be able to comment on right now.

Dr. Om P Manchanda: Obviously. I think it's a broad factor. If the margins improve, people would like to become aggressive in growth. Competitive intensity does grow if the industry has healthy margins. I think I will wait for this quarter results for other players. But my reading is that this year industry might just show a slightly better margin than last year mainly on account of higher contribution of bundle packages , that also augurs well with our strategy of not trying to unnecessarily take a price increase and be a loser on our proposition.

Moderator: Next question is from the line of Bhavya Sanghvi from PhillipCapital. Please go ahead.

Bhavya Sanghvi: I just have a single question. in our presentation and annual reports, we talk about offering close to 4,500 tests. I just wanted to get a sense as to how many of these tests are outsourced and how many of these are conducted in -house? And if you could just give a sense of the percentage c

contribution and revenues from these outsourced tests?

Shankha Banerjee: I do not think that's the data that is something we share, or we would like to share, but suffice to say that predominant tests are all in-house.

Bhavya Sanghvi: And over the last 5 years, the trend would be the same, right, the contribution from the outsourced or it has changed?

Ved P Goel : No. I think over the years we are in fact converted some of the tests in-house rather than sending out. And it's a very small number of tests which probably we are sending out of India and that's all.

Moderator: Next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

S Srinivasan: Just the first one is on the Suburban acquisition we announced three years back and now I think in this quarter you started giving us more granularity. Just some of the learnings is what I was looking not just generally from a Suburban perspective, but also just from an M&A perspective. What are some of the key learnings maybe even some color around the Mumbai market?

Dr. Om P Manchanda: It's a good question and we have actually done about I think 28 M&As of different sizes and scale. Obviously, Suburban is of a different scale. I have actually got many learnings out of this and fundamentally I think most important learning that I have is to really understand the business post-acquisition as to how in our lab it will look like. I think if you are able to assess restated financials and restated infrastructure, which is very important, because sometimes those challenges we tend to underestimate like say for example in Suburban itself, I would say that large part of Suburban infrastructure before acquisition was own infrastructure and that was one of the reasons why the margins also were lower because like LPL actually has grown out of entrepreneurial behavior of partners and franchising does help.

Now, the Suburban was not like that, this whole transition as we made, we ended up actually sharing revenue share with our partners that caused a little bit of a stress.

So, to me, I think I would sum it up our ability to visualize a scenario that post-acquisition what is the state of the business is one learning that I have out of this.

And that reason helps in all the negotiations, evaluations and operational challenges one would face post-acquisition.

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S Srinivasan: And the second question is just a data-keeping on the 11.6% growth. How is that split by the volume and price for Suburban again?

Dr. Om P Manchanda: That probably, Shyam, we have not really shared. That granular I would say as of now I think this data probably as the time comes, we will start sharing, but as of now I would say that we are not fully there yet because of the system integration.

Ved P Goel: There is no price increase in Suburban.

Dr. Om P Manchanda: This has not come out of price increases.

Moderator: Next question is from the line of Vaibhav Saboo from Nippon AIF. Please go ahead.

Vaibhav Saboo: If I look at the material cost and if I evaluate the gross margin for Q2 last year, the gross margin was around 79.7%, which was 80.2% in the previous quarter and 80.7% in the current quarter. Now considering we have not also taken any price increase, can you give any insight as to how we are able to increase the gross margin just on the material front, and whether this is something which is sustainable going forward or was it something like a one-off?

Dr. Om P Manchanda: Yes, I think I have an answer for that. I think again and again we are talking about, this is what bundling is leading to and if you study most of the bundle packages of various companies in the industry they consist of a lot of routine tests and relatively routine test cost is lower as a percentage of top line compared to higher end tests.

And Swasthfit is growing nearly about twice the rate of our company growth.

That

tends to reduce the consumption cost. Now, it will obviously stabilize at some point.

It cannot really keep going down again and again. I would say your question is, is it sustainable at this number? I get a sense, yes, right.

Ved P Goel: And in shorter term, yes.

Dr. Om P Manchanda: Yes, and obviously there is an impact of higher top line. In those quarters where top line is lower you might just see but on a yearly basis I would say they are sustainable.

Vaibhav Saboo: In terms of Swasthfit contribution, while we have been able to increase on a YoY basis but if I look at it, Q1 stood at 25% whereas for the current quarter stands at 24%. Like any particular insight from that?

Shankha Banerjee: Historically, the trend is that the Q2 contribution is lower than Q1 and it is because of the overall base number going up due to a lot of seasonal fever, etc., Even if you see QoQ, last year Q2 contribution was lower than last year Q1 contribution for Swasthfit.

Moderator: Ladies and gentlemen, that was the last question for the day. I would now like to

hand the conference over to the management for the closing comments.

Ved P Goel: Thank you, everyone for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries.

Please feel free to reach out to us in case you have any further questions or queries.

And once again thank you and wish you a very, very Happy Diwali and the rest of the festival. Thank you.

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Call: Feb 2025

Classification: Internal
February 03, 2025

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q3 FY25 Earnings Conference Call

Dear Sir/ Madam,
Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q3 FY25, Earnings Conference Call. We request you to please take the same on record.

Thanking You, Yours Faithfully,
For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer
Encl.: As above
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Classification: Confidential

Dr. Lal PathLabs
Q3 FY25 Earnings Conference Call Transcript
January 30 , 2025

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Dr. Om Prakash Manchanda - Managing Director
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO & CEO – International Business
Participants who asked questions • Saion Mukherjee - Nomura Securities
• Karthik Chellappa – Indus Capital Advisors Hong Kong Limited
• Rishi Modi - Marcellus Investment Managers
• Prakash Kapadia – Spark PMS
• Karan Vora - Goldman Sachs
• Saurabh Kapadia - Sundaram Mutual Fund
• Anshul Agrawal - Emkay Global
• Punit Pujara – Helios Capital
• Sumit Gupta – Centrum India
• Abdulkader Puranwala – ICICI Securities
• Yogesh Soni – InCred Capital
• Pradnya Ganar – UTI AMC
• Prashant Nair - Ambit Capital
• Rahul Jeewani – IIFL Capital

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Moderator: Ladies and gentlemen, good day and welcome to Dr. Lal PathLabs Q3 FY25 earnings conference call for investors and analysts. As a reminder, all participant lines will be in the listen-only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing "*" then "0" on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Nishid Solanki of CDR India. Thank you. And over to you.

Nishid Solanki: Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs Q3 FY25 earnings conference call. Today, we are joined by senior members of the management team, including Hony. Brig. Dr. Arvind Lal, Executive Chairman, Dr. Om Prakash Manchanda, Managing Director, Mr. Shankha Banerjee, CEO, and Mr. Ved Prakash Goel, Group CFO and CEO, International Business.

I would like to share a standard disclaimer. Some of the statements made on today's conference call could be forward-looking in nature and the actual results could vary from these forward-looking statements. A detailed statement in this regard is available in the results presentation, which has been circulated to you and also available on the stock exchange website.

I would now like to invite Dr. Arvind Lal to share his Perspectives. Thank you. And over to you, sir.

(Hony) Brig. Dr. Arvind Lal: Thank you very much. Good afternoon, everyone, and thank you for joining us on today's call to discuss our Q3 FY25 Results.

I would like to begin by sharing my thoughts on the dynamics of the diagnostic sector in India. And give you an overview of the key initiatives we are rolling out.

The Government of India has committed to increasing the public health spend to 2.5% of our GDP from the current approximately 1.8%. The need to strengthen primary and secondary healthcare infrastructure especially in rural areas and in cities classified as Tier-3 and beyond has never been more pressing. At the same time, we are also witnessing a rise in non-communicable diseases, also called NCDs or lifestyle diseases and these include diabetes, hypertension, cardiac diseases, strokes, cancers, kidney, liver and lung diseases and obesity. NCDs account for nearly 65% of deaths in the country and a financial loss of about 2% of GDP every year. Therefore, it is imperative that both the government and the private healthcare work together to control the rise of these NCD, starting with timely screening and preventive management, creating accessible testing capabilities, which often serve as the first steps in delivering health care has become increasingly crucial, as was also seen during the COVID pandemic.

In this context, Dr. Lal PathLabs network plays a key role in enhancing healthcare accessibility in an affordable and reliable manner, particularly in underserved regions. The awareness around preventive diagnostics is also growing. We are in a position to facilitate timely screening and early detection, enabling proactive healthcare management and improving overall health outcomes for all.

Our ongoing investments in digital infrastructure have given us the flexibility to efficiently deliver services to a wider customer base. We were pioneers in leveraging AI-driven diagnostic modules with introductions of our AI-enabled test for breast

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Classification: Confidential cancer and prostate cancer many years ago. Genomics, where we have made significant investments and established our Center for Excellence for 'Genevolve', serves as another platform that expands our diagnostic capabilities to have a laser focus on genetics and personalized medicine. It is these innovative and cutting edge diagnostic solutions that have made us the numero uno in diagnostics in India and the most

preferred testing partner for both patients and doctors.

Our operations continue to scale up according to our predefined strategy. That's by over 75-years of experience. As a leader in healthcare, it is our goal to provide timely, efficient quality care to everyone in the country.

With that, I would like to hand to Dr. Om Manchanda. Over to you, Om.

Dr. Om P Manchanda: Thank you, Dr. Lal and warm welcome to all the participants on this call today. The last time I spoke about bundle packaging and our strategic efforts in Western region.

This time I want to share about as to how we navigated our business without the price increase and still delivered better than last year revenue and EBITDA margin in the first nine months of the current financial year.

The first initiative that we took was a very sharp focus on geographical and test mix that delivered higher realization per patient as well as EBITDA margin. For example, Delhi NCR, a region with a better margin profile has delivered double digit growth in

line with the national average. This has happened after many years of single-digit growth in this region.

The second has been Swasthfit contribution continues to rise and has delivered higher realizations of patient. This is as a part of our stated strategy.

Second initiative has been that we are expanding lab and collection centers infrastructure in Tier-2 and Tier-3 towns.

Third initiative has been relentless focus on the productivity initiatives.

And lastly, a very sharp focus on franchisee collection centers that helps us to increase width as well as depth of the reach. The contribution from franchisee collection centers now has gone up to nearly 45%.

With that, I would now hand it over to Shankha for his thoughts. Thank you.

Shankha Banerjee: Thank you, Dr. Om. I extend a warm welcome to all participants joining us on call today. I will present the key business and operational achievements.

We maintained a strong performance trajectory in the third quarter of the fiscal year 2025, delivering 10.7% growth in revenues. The growth is delivered through sample volume growth and mix improvement and does not have any price increase benefit. I am pleased to share with you that Delhi NCR recorded double-digit revenue growth for the second successive quarter.

Let me share some of the key operating statistics. Sample volume stood at 20.6 million in Q3, a growth of 10.3% YoY, while patient volumes came in at 6.9 million, an increase of 3.8% over the same period last year. Revenue per patient for Q3 FY25 stands at Rs.861, a growth of 6.7% compared to Q3 FY24. This growth in

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Classification: Confidential revenue per patient is on account of mixed management initiatives on geography, channel and products.

We are accelerating our reach in the underserved markets with the planned opening of 15 to 20 new labs in this financial year. We continue to expand our reach into Tier-3 and Tier-4 markets and have started strengthening our presence in the core regions as well. We are now making investments in new infrastructure in metro and Tier-one cities.

Our bundle test program, Swasthfit, continues to contribute favorably to the overall performance with a robust growth of 27.2% in Q3 FY25. Building on positive results, we are developing a wider range of bundle packages and expanding our market penetration beyond the traditional Tier-one and two markets to address the growing demand. While it's still early days, the response thus far has been encouraging. Our future growth will be driven by a combination of strengthening our operations in core metro and Tier-one cities, penetration in Tier-3 and Tier-4 in North and East and building clusters in South and West. We are committed to maintaining the highest standards of diagnostic quality and accuracy while expanding our service offerings to include high end and super specialty test. Our robust digit

al infrastructure will

enhance the patient's experience by providing seamless access and maximizing solutions.

With that, I would like now to invite Ved, who will take you through the financial performance. Over to you, Ved.

Ved P Goel: Thank you, Shankha. Good evening, everyone and a warm welcome. I will be sharing the key financial highlights for Q3 FY25 and year till date.

- Revenue for Q3 FY25 came in at Rs.597 crore compared to Rs.539 crore in the same quarter last year, reflecting a growth of 10.7%.
- YTD FY25 revenue stands at Rs.1,859 crore against Rs.1,681 crore in the same period last year, a growth of 10.6%.
- Revenue per patient for Q3 FY25 is Rs.861 against Rs.807 in Q3 FY24 mainly due to test and geography mix.
- Test per patient for Q3 FY25 is 2.97 v/s 2.79 in Q3 last year, a growth of 6.3%.
- EBITDA for Q3 FY25 came in at Rs.154 crore compared to Rs.141 crore in Q3 FY24, registering a growth of 9.6% with the EBITDA margin of 25.8%.
- YTD FY25 EBITDA stands at Rs.527 crore v/s Rs.465 crore in last year same period with a growth of 13.3% and EBITDA margin of 28%.
- PBT for Q3 FY25 came in at Rs.138 crore v/s Rs.116 crore in last year same period, registering a growth of 19.4%.
- PAT for Q3 FY25 came in at Rs.98 crore compared to Rs.82 crore in the same period last quarter last year, registering a growth of 19.3% with a PAT margin of 16.4%. For YTD FY25, PAT stands at Rs.337 crore against Rs.277 crore in same period last year with a margin of 18.1%.
- EPS for Q3 FY25 is Rs.11.6 as against Rs.9.8 in the same period last year

and YTD FY25 EPS stand at Rs.39.9 against Rs.32.9 last year same period, registering a growth of 18.7% and 21.4% respectively. Net cash as on 31st December 2024 is Rs.1,123 crore.

- At last, I am pleased to share that the Board of Directors of the company have approved a Third Interim Dividend of Rs.6 per share, taking the total dividend for the current year till date to Rs.18 per share.

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Classification: Confidential With this I conclude my opening remarks, and I would now request the moderator to open the forum for Q&A.

Moderator: Thank you very much. We will now begin the question -and-answer session.

We will take our first question from the line of Saion Mukherjee from Nomura Securities. Please go ahead.

Saion Mukherjee: Yes, hi. Good evening and thanks for taking my question. Just some P&L numbers I just wanted to get some color. We have seen consistent improvement in gross margin despite no price increase. If you can take us through the dynamics in terms of volumes, Swasthfit mix and you have raw material prices, currency, etc., how should we think about gross margins going forward? And the second one was on employee expense. What I notice, it has been growing in mid -teens higher than the revenue growth for the past few quarters. What's driving this and how should this grow going forward in your view? Thank you.

Ved P Goel: I think just to correct, the gross margin in this quarter compared to last year same quarter has not improved. Basically, this gross margin is slightly lower than last year same quarter. Second, in terms of employee cost, yes, definitely, there is a increase because we have started investing in new infra and we hired about roughly 100 people in our sales and marketing team as well. So, that is the impact where we are investing in the business for future growth.

Dr. Om P Manchanda: I want to add what Ved just mentioned. I think in the last sort of 1-2 years post COVID, there has been a certain jump in gross margins and mainly because of the COVID testing decline, our margin improve and we also invested a lot behind technology initiatives, which helped us to improve productivity. But I would say that these margins one should assume more or less stable at this number rather than any improvement further going forward. As far as employee cost

is concerned, as Ved

mentioned, we are now relooking at our organic infra and not only just in terms of lab but also adding manpower in our business, which is combination of both these two.

Saion Mukherjee: If I can ask one more just on depreciation and tax rate, we see depreciation, amortization numbers kind of quite flattish or even declining. Any reason there? And you are still on the old tax rate and any plans to move into the new tax rate and what should we expect in terms of an effective tax rate in the years ahead?

Ved P Goel: Our applicable tax rate is 25%. If you see the standalone, the tax rate is similar to that effective rate, but in consol we are charging some depreciation on account of impairment of intangibles, where we are not getting any tax benefit right now, that's where the effective tax rate is about 29% in consol. But in future, I don't think there is any change we have on this applicable tax rate.

Saion Mukherjee: And any comment on depreciation, amortization number?

Ved P Goel: These are similar because there is not much CAPEX which is done in terms of newer investment. These will be coming in the future. Right now, depreciation is more or less in the same range.

Dr. Om P Manchanda: Getting offset.

Ved P Goel: Yes.

Saion Mukherjee: But you were adding labs, right, I understand your organic infrastructure you are adding.

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Classification: Confidential Ved P Goel: This is offsetting against the depreciation we are charging against the new addition.

Moderator: Next question is from the line of Karthik Chellappa from Indus Capital Advisors Hong Kong Limited. Please go ahead.

K Chellappa: Sir, two questions from my side. First, if I look at Swasthfit, that contribution to overall revenue has remained at about 23% -25% in the last four quarters. What is our medium -term aspiration in terms of, let's say, revenue composition and what needs to happen for us to take it to a higher level? That's my first question.

Shankha Banerjee: If you see every quarter v/s the same quarter last year, you will see that it's a 2.5% - 3% contribution over and that's been a trend that we have seen through the last two, three years period that the contribution remains kind of flat between Q4 to Q3 and then again in Q4 it jumps. All the actions that we are taking, you will likely see some benefit of that accruing to us in Q4 and the baseline will shift again for the next three quarters. The growth rate on Swasthfit, and if you also compare the contribution v/s

same quarter last year, you will see the delta.

K Chellappa: Got it. Which means that going into next year, this ratio, at least in a YoY basis, will continue to improve. Is that how I should interpret it?

Shankha Banerjee: Yes, that's really the plan.

K Chellappa: My second question sir, if I look at Suburban, this quarter at about 9.2%, that revenue was soft relative to both our overall revenue growth as well as Suburban's nine-month revenue growth as well. What would explain this relative weakness in Suburban this quarter and what is the plan or let's say aspiration for FY26?

Shankha Banerjee: I think we've been saying all along that in our business QoQ may not be the right metric to look at growth rates. I think what we are saying is that the growth trajectory on Suburban definitely is moving in the right direction. There is obviously some quarter there can be a slight up, some quarter there could be a slight down, but overall that trajectory will improve and obviously going forward our plans are to grow even faster than what you're seeing now.

Dr. Om P Manchanda: I also want to add more point. Actually, while the questions get asked on Suburban, but we are looking at West as a region for our total business. We have 3 verticals there. One is Suburban, the other is Dr. Lal PathLabs parent business and third also we have done a smaller acquisition under the vehicle calle

d PathLabs Unifiers. Our

front-end team now is common for all the three and we really don't evaluate them whether it has to be Suburban or parent as long as overall business grows. Because there's a huge sort of a network advantage we get one over the other and obviously there is a latent sort of brand equity for Dr. Lal PathLabs and I was looking at the number actually West region growth rate is even higher than our national average for not only for the quarter even on YTD basis. So, we've also seen some bit of uptick on the parent business as well in this geography. That network of Suburban is also helping us in a positive way to get our Dr. Lal PathLabs business growing as well.

Moderator: We will take our next question from the line of Rishi Modi from Marcellus Investment Managers. Please go ahead.

Rishi Modi: Om, I heard your comment on the gross margin where you said that large part of the gross margin expansion is done some with respect to COVID going away, the normal business having higher gross margins and the others the initiative that you all up taken at your end. One thing I wanted to understand right, today, on the other industry in the domestic reagents business, which is coming up, which is at a lower

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Classification: Confidential pricing than the international players. Like, are we not planning to increase that mix or like we going to pass on the benefits that we get from those and hence lower pricing or test pricing, like I am just trying to understand why wouldn't gross margin expansion take place if we incrementally keep using domestic players reagents?

Dr. Om P Manchanda: Ved can comment on this domestic part of it, but I also want to highlight because most of these regions are imported and dollar becoming expensive, while we are still holding on to this, I don't know what the response of our vendors going forward, but that pressure also is going to come not only for us, but for the entire industry, but as of now, we're just holding on to these prices. So, there are headwinds on the reagent cost. On the domestic v/s international, maybe Ved, you want to comment.

Ved P Goel: Yes, Rishi, I think as Om said, these are mostly reagents which we are directly not imported but a n imported material. However, having said that, we have started using some of the consumables, which is local Indian manufactured and there we are definitely getting some leverage and benefit. If you see last 2-3 years consumption cost, it has reduced substantially because of mix which we are changing from 22.5% to now roughly 20.9% this quarter. That is where wherever we have opportunity, we are doing that, but at the same time there is a inflationary pressure which is coming and that's where I don't think in future we should build any further improvement in the gross margin.

Rishi Modi: Dollar appreciation primarily in the near term will lead to pressure, sir. Second, I wanted to understand on the Swasthit , right. When I look at the contribution, from Q1, the contribution has come down to 23%, while YoY it's still up. I am just trying to understand, is there something which is missing here like this contribution has not historically dropped as a percentage of revenue ever since in the last 3 years at least that we have the data for, it's kept on decently well over a year upon growing on a QoQ basis as well. Is 25% the peak or like what's happening here if you can give some color?

Dr. Om P Manchanda: I think the data that I have in front of me, what I've seen is that Q3 generally is a soft quarter, 23% one see it as to what was the contribution last year in the same quarter, that number is 20%. And our sort of assessment is that Q4 actually peaks because of a lot of people actually go for health packages because there's reimbursement, I

think there's a tax advantage they get. It peaks in Q4 and then somewhere stabilizes after that in Q1 as well. But then it

softens in Q2 and Q3 and a gain it peaks in Q4. I

think 23% is not a representative figure for the year, but it's only a figure for the quarter.

Shankha Banerjee : I think the trend is similar last year. So, it's not as if every quarter last year was growing over the previous quarter. If you see the trend last year was exactly the same on contribution. Even last year, the QoQ contribution was highest in Q1, then it dropped in Q2, dropped in Q3, again peaked in Q4. I think that is the trend we have seen for the last 2 years and the same trend continuing this year.

Dr. Om P Manchanda: Or other way to look at is that peak contribution aside, Swasthfit this quarter has grown by 27% over last year same quarter.

Rishi Modi: And I think last time, Om, you mentioned like this can go much beyond the 25 %-30% mark. Is that still the case or you are still maintaining that, right?

Dr. Om P Manchanda: Directionally, I think industry is showing those trends and I haven't collated other companies data, but I think one should probably look at the trends in other companies as well, where the data is available publicly, but my own gut says that directionally it's moving towards bundling and we keep growing. Now I don't know what that number is, but definitely if 50 %-60% of disease incidence is coming out of

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Classification: Confidential NCDs, I think NCDs are, that segment is prone to bundling because they're all metabolic disorders, they are all club together and they are all falling in the domain of preventive testing, etc., It's only the infectious or communicable diseases you can't bundle it. You don't need to go for a malaria testing if you don't have fever, but you can still go for a lipid profile and kidney function test, etc., I think if you were to ask me as to where it can go, it is a reflective of total NCD burden in our society .

Rishi Modi: Finally, for Ved, just a couple of numbers I wanted your help with. Suburban EBITDA margin for the quarter v/s the same quarter last year if you could help me with? And on the tax piece I am seeing like a 28 %-29% tax rate for several quarters now. Just wanted to understand like what's the reason for this elevated tax rate? I maybe you have listed a few mentioned in the past.

Ved P Goel: Rishi, Tax as I mentioned earlier also that if you see standalone, there is a absolutely around 24 %-25% which is the applicable rate, but on consol there are some adjustment, maybe some depreciation, intangibles where we are not getting any tax and then dividend adjustment which is intercompany removal of those. For that, it is reflecting a higher tax rate, but normal tax rate is 25%.

Rishi Modi: Like this tax amount that we're showing in the P&L is actual amount that we are paying, not just the P&L amount?

Ved P Goel: Yes.

Rishi Modi: And if you could help me with the EBITDA margin for Suburban for the quarter v/s same quarter last year?

Ved P Goel : It is lower but again on an yearly basis, I think margins for Suburban will be in the range of 15 %-16% because in Q3 as revenue dropped as so as the margins.

Rishi Modi: If I take H1, we would have done about 17 %-18% of EBITDA margins, you're saying nine months is at 15 %-16%. This quarter, the EBITDA margin would be around 10 %-12%, is that right?

Ved P Goel: Yes, 12 %-13%.

Dr. Om P Manchanda: Yes, because higher fixed cost component is so high, it's very sensitive to revenue fluctuations.

Rishi Modi: You are saying it's declined v/s last quarter . So, last year we were in the 15% range. Last year, the margin profile would be the same, same quarter?

Ved P Goel: From last year, YoY, it has improved.

Dr. Om P Manchanda: Q3 FY24 EBITDA margin for Suburban was 11.5%, now it's 12.3%.

Moderator: We will take the next question from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: Two questions from my end. Any comments on how ar

e smaller players in the

industry doing, any specific trends you witnessed in terms of consolidation or regional players performance that would help? And secondly, any price increases on the anvil because some of the larger players are contemplating price increases?

Those are my questions.

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Classification: Confidential Dr. Om P Manchanda: Let me answer the second question first. Yes, I know that many players have been

thinking of taking price increase, some of them have already taken as well. I think,

internal, our view is that we want to get our volume growth going up and the good news is that this year, we have seen significant improvement v/s last year and we believe our strategy of holding on to the prices and still managing revenue growth in line with our internal budget as well as over last year, we think our strategy is working and without dilution of margins, we want to hold on to this for some time and see if we can get our volume growth up. That's our current view. We will probably review this going forward maybe 6-9 months later, but as of now, we don't plan to take any price increase.

Second is coming to the smaller players as to how it is, I think there is some sort of a stability and calmness in this whole industry right now because a couple of years back because of this deep discounting and aggressive pricing by some of the players which are really uneasy times for the industry. Overall, it is stabilizing. I don't see too much of a sort of stress in the smaller labs, but I also don't see too many new labs coming up. I think it's a stable time I would say. And if some people are taking price increase, it does help smaller labs as well, one must know that, because it gives them sort of leeway to take price increases even locally also. I think it is going to be a stable time for smaller labs going forward for some time.

Prakash Kapadia: And lastly, Dr., in this context of what you just mentioned, is it possible directionally we should grow faster than some of the other players, given that we are spreading geographically, we've been one of the earlier players in Tier 3-4 cities, so when do you know some of these initiatives fructify and lead to still higher sales growth for us?

Dr. Om P Manchanda: Yes, I think you're right, but the challenges also is our base is equally large because at such base, let's say give or take Rs. 2,500 crore base, if you have to grow over 10%, you technically have to add Rs. 250 crore of extra business every year. That's a kind of broad math one has. And growing it organically in the same market sometime becomes challenging and the newer markets are slow to respond. I probably would say what you are saying is right and we also want to make sure that it's a sustainable growth and my personal experience is, 1% or 2% slower growth is still fine, but as long as you are structurally building it up for long-term, slowly, slowly build your infrastructure. Because the reason for people to come back again and again is quality and trust. At no point in time, our expansion should compromise on that because at times in a hurry to get extra that 1%, one then compromise on those aspects. I think it's a more process-driven business, make sure that you build it brick-by-brick is what probably helps. And we are seeing that actually in the market. And one of the factors, in fact Delhi NCR, we've not talked about this. In the last two quarters, our growth rates are pretty good. And I actually would say some of the labs users which went to some of the new competition have started coming back to us so which is a sort of a proof to at least very positive. I don't know whether it's working that way, but it's just a hypothesis I have. I want to see for next 2-3 quarters, but I probably would say that it's a process-driven business. Go steady, slow, slow, build it up and it stays with you.

Moderator: We will take our next question

from the line of Karan Vora from Goldman Sachs.

Please go ahead.

Karan Vora: My first question is with respect to the patient service centers and lab ratio. So, when we mentioned that we are trying to improve productivity and efficiencies, what are the metrics we kind of look at, do we look at revenue per lab or revenue per patient service center or patient service center per lab, which are the right metrics, and what is the ideal metric you would like to have like over a medium term?

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Classification: Confidential Dr. Om P Manchanda: I think clearly this particular metric of ratio between lab and collection center is very

important. Because a throughput through a collection centre is definitely more efficient than throughput through a lab. Not only from a cost perspective, but also giving higher market access because you are spreading your infrastructure closer to the customer.

The second area has been area of overall fixed block that we have, how does it compare in terms of cost per test, basically that ratio. And we have seen simple things like printing a stationary, if you reduce the physical paper use, that itself gives you a lot of benefits. The second area could be if you franchise more collection center, your rental cost as a percentage of revenue actually is coming down. Because your own infra is high street infra, you end up paying more rent and now I think GST impact is also there, right on that?

Ved P Goel: Yes.

Dr. Om P Manchanda: And if it moves closer to collection center, it's really that kind of cost structure is not

there in their hands. So, some of these small, small initiatives, they all add it up and that's how we get the benefit.

Karan Vora: Right, now I think our patient service center to lab ratio is around 20 -21x. What is the ideal number you would look at and is it different between a Tier-1 and say a Tier-2 or Tier-3 city?

Dr. Om P Manchanda: Yes, Tier-1 definitely this number would be higher because you are operating in a small geography. If we go deeper, then obviously it will be much lower. Your average number, I don't know where it can stop. There was a time when we were 1:7 also and I've seen the journey from 7 to now 20 -21. I think this is technically competing with turnaround time as well because more collection centers you have, they are far away from lab, your turn around time actually gets sacrificed. I think it's a very dynamic number. I don't know whether we have a target in our mind, as long as overall sort of a cost overhead, we just keep an eye on that. I don't think we just decided target and go after that number because at some point in time like this year, we are trying to expand more collection centers. This ratio may go off as well. But it's a very dynamic number. I don't think we have a number in our mind to achieve. The higher the number better it is for us.

Karan Vora: My second question is with respect to the competition in Delhi NCR region from hospitals. So, are you seeing the competition kind of increase like on a YoY basis you think it is more or less in line with what you are expecting?

Dr. Om P Manchanda: More or less in line with what we expect, but I think last time also we spoke about, there are hospitals, especially the ones which have ambition to become bigger and get listed. They are the ones who are trying to look at retail pathology. But at least in our sales meetings, we don't really get to hear from them that hospital is a very big threat, but definitely it's a competition which is here to stay, there is no doubt about that and there's a large enough market. In fact, I personally always have believed that organized competition helps all of us because technically you are taking the market away from unorganized players and which is what we're seeing, the market has started responding now better and bundling is effect of aggressive competition activity around bundle packa

ges. Overall I have a feeling that these people will help market to grow on an organized play, shift from unorganized to organized will move further.

Karan Vora: Are they pricing like in line with what we are pricing or are they like more price competitive, do they price higher, how is their pricing?

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Classification: Confidential Dr. Om P Manchanda: They cannot afford to compete with us in pricing because they have to match their internal hospital pricing as well. I think hospital guys will never be aggressive on pricing. They can be aggressive on building infra, but my sense is they will not go to deep cut the pricing.

Karan Vora: My last question is with respect to the kind of region wise growth. I think we mentioned that West and Delhi NCR have actually grown faster, which kind of maybe implies that East or South have kind of grown below average. What has happened there? And on a normalized say medium term growth perspective, which regions do you think can be above corporate average and which regions would be below?

Dr. Om P Manchanda: I think see, the difference is not very stark. When I probably mentioned about West region growing faster, doesn't mean the East has grown very slow. I would actually say if I had to pick one region which probably requires focus is South for us. I think all other regions, in front of me I have the numbers, more or less in line with national average, few 1 or 2% higher or lower. But I think one region which probably requires focus for us is the South. This is going to be a focus area for us.

Karan Vora: Even from a medium -term standpoint, more or less growth should be split equally in across regions?

Dr. Om P Manchanda: Yes. It's a very uniform growth. It does help us to see that we are not dependent on one region for higher for the growth, it is uniform across India.

Karan Vora: And when we look at the margins like do you think or the margins across regions are similar or like if there's a rank order maybe Delhi NCR would be the highest, but like what others would be higher than corporate average and what others would be lower. Not asking for this particular quarter, but in general on a normalized basis?

Dr. Om P Manchanda: Ranking is I think it's no -brainer, Delhi NCR is the highest, then come rest of North and then East and West. Any concentrated geography will always have for us, not for the industry.

Karan Vora: And where would corporate average stand? So, corporate average would be better than rest of north ?

Dr. Om P Manchanda: Maybe we can chat offline.

Karan Vora: Okay.

Moderator: We will take our next question from the line of Saurabh Kapadia from Sundaram Mutual Fund. Please go ahead.

S Kapadia: You mentioned about making investment in infra in metro and Tier-1 cities. So, is it related to the lab investment or it would be more of a marketing infra?

Shankha Banerjee: So, we are looking at lab and marketing both.

S Kapadia: What could be a quantum of investment?

Shankha Banerjee: No. I think that's more in terms of number of labs that we look at. We are starting to look at new infrastructure in metro and Tier-1 in terms of labs.

S Kapadia: I am seeing now with the voluntary liquidation of Suburban, now we mentioned the production in the overhead cost. What kind of cost saving we could see post that?

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Classification: Confidential Ved P Goel: This is administrative and legal and compliance costs primarily, which can be saved because right now there are multiple entities, now it will be one. There are certain costs which we can save on account of this.

S Kapadia: Any number to it?

Ved P Goel: It's not very substantial, but yes, definitely there will be something.

Moderator: We will take our next question from the line of Anshul Agrawal from Emkay. Please go ahead.

Anshul Agrawal: If you could just help me with the number of labs you have already added in

the nine

months ended for this current year?

Shankha Banerjee: We have set 15 -20 labs. We are well on track to achieve that by the end of the year.

Anshul Agrawal: I have a follow -up question on that. Has the Delhi region grown on the back of network expansion or would this be a sort of an SSG kind of growth that we have witnessed over the last two quarters?

Shankha Banerjee: Primarily it is SSG kind of growth as of now. Yes. Whatever we are seeing in the last two quarters is still primarily at is.

Moderator: We will take our next question from the line of Punit Pujara from Helios Capital.

Please go ahead.

Punit Pujara: My first question is for Dr. Om. You mentioned that the new labs have stopped coming in, but has that resulted in let's say lower valuation in the existing labs? I am asking this because you have Rs. 1,100 crore of cash and I think it makes sense to follow the roadblock strategy . Yes, that would be my first question.

Dr. Om P Manchanda: The stable environment, I essentially meant that, one tends to see a lot of new labs being launched by smaller players. Especially in the unorganized space, I don't see a lot of new players coming and putting up new labs. That's what I meant by that.

But there is aggression , there is a competition from the organized because those players are putting up new labs, but I what I meant was unorganized sector.

Punit Pujara: I see. And the second is clearly you are focusing a lot on growth. Are there any adjacent areas that you are exploring to further accelerate the current rate of growth?

Shankha Banerjee: We are currently looking at a few options, but I think there are too immature right now to talk about. But yes, overall, a slightly longer term horizon there could be some adjacencies.

Dr. Om P Manchanda: For us essentially we are looking at higher end tests in the area of genomics and Dr.

Lal mentioned in his comments about 'Genevolve ' and it's in the area of autoimmune disorders. There is a lot of effort going behind promoting higher end tests. So, that actually grows on back of the network that we have expanded. I think that's the way we look at adjacencies. And the second area is in terms of basic radiology, especially ultrasound and X -ray and we have large number of labs where we have the services. That's another area of adjacency. So, that's the way we look at it.

Moderator: We take our next question from the line of Sumit Gupta from Centrum India . Please go ahead.

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Classification: Confidential Sumit Gupta: As we plan to expand geographically across the nation, so like, what is the patient volume growth that you expect going forward over the next 3-4 years?

Shankha Banerjee: Last year I think we ended the full year at about 2.7% and I think right now YTD we are about 4.4%. Give or take let's say we are in the range of 4.5% -5% is what I think we are expecting patient volume, and our endeavor is to improve that as we go forward. I think we are not taking any specific target on it, but the idea is with the new infrastructure, collection, network expansion, etc., we can reach and access larger patient pools and therefore grow better in terms of patient volumes going ahead.

Sumit Gupta: Okay. And then those geographies also you plan to expand through volume growth through Swasthfit ?

Shankha Banerjee: Swasthfit is just a one product which plays more prominently in maybe metro, Tier-1 and some Tier-2 cities. Infra growth and Swasthfit are two different access for growth.

They need not be overlapping. A certain level could be complementary to each other.

Dr. Om P Manchanda: I want to pick up this question on the volume. We first of all look at volume both these metrics together, it's not only just the patient growth but also sample growth. Because what we are seeing as a contribution of Swasthfit is moving up. There is a slight decline that we see on the patient growth

because actually technically these are patient visits, they are not unique patients. We are seeing the frequencies drop and that's why you see slightly slower or lower patient volume growth. We want to see both together and we want to see both growing as we move forward.

Sumit Gupta: Understood, sir. You also allocated revenue contribution from franchisee collection center, was 45%. Overall, how is the trend being and how do you expect it to go forward?

Dr. Om P Manchanda: I get a sense it's now stabilizing here because in addition to this, we have pickup points which is nearly about another 20 %-25%. The rest of it will remain as a walk - in and home collection, which we normally call retail business which is our own infra. I would say it is probably stabilizing at this number.

Sumit Gupta: And sir, lastly, how much CAPEX was there in nine months FY25 and what is your guidance for next year?

Ved P Goel: In nine months roughly we have made a CAPEX of around Rs. 30 crore. As Shankha mentioned, we are planning to open 15 -16 labs in this year. Maybe this is coming in the later part of this year, so Q3 and Q4. And going forward, on an ongoing basis, maintenance CAPEX is about Rs. 60 - Rs. 70 crore per year excluding any major investment in any other venture.

Moderator: We will take our next question from the line of Abdulkader Puranwala from ICICI Securities. Please go ahead.

A Puranwala: Just a follow up on what the previous participant was talking about. If I see your Swasthfit revenues, they are growing much faster and I think that is a broader trend for the last couple of quarters and we have been talking about network expansion into Tier-2, Tier-3 markets, that has worked quite well, but what in terms of the overall growth, that's end up to single. Just also wanted to understand on the volume front, how was the volume growth been in the Swasthfit v/s the residual part of your portfolio, is there any material difference as compared to how your revenues are getting shaped?

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Classification: Confidential Dr. Om P Manchanda: We don't have that data in front of us. We will come back to you offline. But YTD

overall volume growth for the business is about 4.4%.

A Puranwala: Second is on a similar line. While historically we have seen that bundling test is something which has always attracted a large competition say it from online or you know from other channels. If you could share your thought process about carefully selecting this as one of your growth engines for the near future despite the threat of competition any day coming into it?

Shankha Banerjee: I think Bundle Testing is a phenomena which is now across the industry, and I think to a certain extent the promotions done by the online competitors have even spurred this demand overall. It may not necessarily be one eating into the other, rather it helps building the category much faster.

Moderator: Ladies and gentlemen, to ask a question, please press "*" and "1" on your phone now. We will take a next question from the line of Yogesh Soni from InCred Capital . Please go ahead.

Yogesh Soni: Just wanted to understand , what has been the West region contribution in last year? And also wanted to understand your plans on expansion into West and Southern regions? Thank you.

Dr. Om P Manchanda: West region contribution is now nearly 15% all put together, which is similar to what we do in East. East and West are now easily sort of balanced now, gone up from 14 to 15 is what I see.

Yogesh Soni: Sir, what endeavors are we making to expand into these two regions, West and South?

Dr. Om P Manchanda: South, obviously, the region for us to focus . As we have always stated that these new regions, we look at both organic and inorganic and we will continue to put those efforts in these markets. And while on these calls we can talk about South, but South itself is very big cluster of Bang

alore, you have a cluster of Chennai, Hyderabad. We look at that cluster and then grow our business. We will definitely focus much more in South this year than what we've done in the past.

Moderator: We take a next question from the line of Pradnya Ganar from UTI AMC. Please go

ahead.

Pradnya Ganar: Sir, my question is on the M&A strategy. While you indicated that M&A you look at it to expand into new geography, but can you also look at M&A to add new capabilities, are there any kind of testing capabilities that you would want to add and for which you might want to evaluate M&A?

Dr. Om P Manchanda: Yes, that's a good question. Actually, when we look at some of these businesses and I would say there's a routine testing which I think every one of us has. Then there are a lot of these high-end tests that are done in India, and nobody has these test instruments, these instruments are actually available off-the-shelf. So, you don't need to go and acquire, if we Illumina is one machine for genetic testing which cost me about Rs. 8 - Rs. 10 crore. So, you just go and buy this machine, and you add these tests. You don't need to go and acquire any company. The effort really needs to be put on the sales and marketing side and then acquiring some of these because these technologies come from outside, available on the shelf and you just go and buy these equipment and you are adding those capabilities. I think the real effort is on the front end side to grow the business.

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Classification: Confidential Pradnya Ganar: Currently what would be our mix of specialized and semi-specialized set?

Shankha Banerjee: There is no industry standard definition of specialized, but the way we look at our business, the way we define specialized, that portfolio for us is about 22% right now.

Moderator: We will take our next question from the line of Prashant Nair from Ambit Capital.

Please go ahead.

Prashant Nair: Good evening, everyone. I just wanted to reconfirm the volume growth number for nine months. Did you mention 4.5% in that range?

Shankha Banerjee: Yes, YTD patient volume is 4.4%.

Prashant Nair: And is there a test volume number?

Dr. Om P Manchanda: 9.5%. This number that I am mentioning to you 4.4% volume and 9.5% sample growth is for nine months.

Prashant Nair: Dr. Om, when you mentioned you want to take volume growth higher. That's the internal thought process. Where you think you would want to settle, is it like 100 -200 bps higher or can you reach a higher number than that?

Dr. Om P Manchanda: Yes. Okay. Don't hold me for that number, but I would say that if you go back in time before COVID our average volume growth used to be in the range of about 8% -odd CAGR and sample growth was not much during that period. Then we went into a phase where the sample growth picked up and the volume growth declined and we were actually struggling to figure out what's really going on. And one of the variables we found was Swasthfit. Because bundling contribution was going up, sample growth was going up, but the volume was coming down. But then I look at volume growth as surrogate measure for our expansion efforts, I would want to see that more and more patients coming from newer markets, gives us that we're expanding. Sample growth at times tends to convey that you are actually upselling to the existing customer base. I think probably my message to the team would be at least go back to this number of 7 %-8% that we were doing in the past.

Prashant Nair: Right. And just one follow up question. Dr. Om, I think while the volume growth in terms of patients has come down, I think your test per patient has probably gone up over this period. Do you think you'll be able to retain that number where it is now, or maybe improve on it while you also step up on patient growth?

Dr. Om P Manchanda: I think directionally test per patient, there are lots

of tailwinds on that. One tailwind,

of course, is the bundling. I think you guys cover pharma as well. Maybe you may want to study some of these prescription behavior. I am noticing that now more and more tests get prescribed on the same prescription than before. That is also happening. I think there is a favorable sort of a situation for test per patient in future.

Moderator: We will take our next question from the line of Rahul Jeewani from IIFL Capital.

Please go ahead.

Rahul Jeewani: Sir, now you're talking about lab expansion in metro and Tier-one markets as well.

And as you rightly pointed out that we see a lot of these bundle tests on Swasthfit primarily in larger metro markets. With the expansion which we are targeting in metro markets, is there a potential that the sample volume growth, which has been around 9.5%-10% for us, that can accelerate to a low double digit kind of a number going forward?

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Classification: Confidential Dr. Om P Manchanda: But it'll get offset by newer markets going up for Swasthfit because it's not Delhi NCR

alone, you have entire UP, rest of north, these labs are also responding very well to Swasthfit kind of packages. I think if there is a headwind in bigger towns, then it'll

definitely get offset by smaller towns. That's my take away on this.

Shankha Banerjee: I think it is all just a slight shift to say that there is still quite a bit of headroom in the metro and Tier-one town, which we want to capture. And not only will it be captured through Swasthfit or bundle package, but also, the prescriptions which come out and where there is a lot of tag dependent delivery expected.

Rahul Jeewani: Sure, sir. I wasn't talking about any headwind. Actually, I was talking about this tailwind that over let's say past 2-3 years we have been expanding in Tier-2, Tier-3 markets, in North and East. Now, we are talking about expansion in metro markets as well. With that expansion in metro markets, is there a potential that we actually start seeing an acceleration in volume growth both on the patient volume as well as the sample volume growth?

Shankha Banerjee: These are in my view, it's a part of the overall mix. If you see the contribution of metro and Tier-1 today and we've been talking about our Tier-3 and Tier-4 kind of contribution, therefore, it's not as if metro and Tier-1 is a huge contributor, it's also a part contributor. It's just about ensuring that there are lot of tag dependent testing opportunities or patient opportunities that we sense in these markets as well which we would like to capture. Yes, the idea is to help us get newer patients or patients who may not be coming to us, and in a certain sense, it will help patient volume growth as well as sample growth.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to management for closing comments. Over to you, sir.

Ved P Goel: Thank you, everyone for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your queries and questions. Please feel free to reach out to us in case you have any further queries. Thank you once again. Now we can close the call.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: May 2025

Classification: Internal
May 01 , 202 5

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q4 & FY25 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Q4 FY25, Earnings Conference Call.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 14
Classification: Restricted

Dr. Lal PathLabs
Q4 & FY25 Earnings Conference Call Transcript
April 25 , 2025

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group C FO and CEO, International Business
Participants who asked questions • Prakash Kapadia – Spark PMS
• Karthik Chellappa – Indus Capital Advisors Hong Kong
• Anshul Agrawal - Emkay Global
• Shyam Srinivasan – Goldman Sachs
• Bharat Celly – Equirus Securities
• Raman Sivi – Sequent Investments
• Divyansh Gupta – Latent Advisors
• Amey Chalke – JM Financial
• Deven Kulkarni – Marcellus Investment Managers
• Sumit Gupta – Centrum Broking
• Harshal Patil – Mirae Asset

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Classification: Restricted Moderator : Ladies and gentlemen , good day and welcome to Dr . Lal PathLabs ' Q4 and FY25 Earnings Conference Call. As a reminder, all participant lines will be in the listen - only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference to Mr. Siddharth Rangnekar from CDR India. Thank you, and over to you, sir.

Siddharth Rangnekar: Thank you. Good afternoon , everyone, and welcome to Dr. Lal PathLabs ' Q4 & FY25 Earnings Conference Call. Today, we are joined by senior members of the management team , including (Hony) Brig. Dr. Arvind Lal , Executive Chairman, Mr. Shankha Banerjee , CEO and Mr. Ved Prakash Goel , Group CFO and CEO , International Business .

I would like to share our standard disclaimer : some of the statements made on today's call could be forward -looking and actual results could vary from these statements. A detailed note is available in the results presentation on this , and the presentation has been circulated to you and is also available on the stock exchange website.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to share his perspective. Thank you, and over to you, sir.

Dr. Arvind Lal: Thank you, Siddharth . Good evening, everyone, and thank you for joining us today on our annual earnings call.

I will share perspectives on diagnostics as an industry and the opportunities that it presents for Dr. Lal PathLabs . As we reflect on the strides made in India's healthcare sector during FY25 , it is evident that both Government and private entities have intensified their commitment to enhancing health infrastructure and services. The Union Budget for 2025 - 26 has increased healthcare allocation by nearly 11% to nearly Rs. 1 lakh crore. Flagship schemes like Ayushman Bharat , PMJAY received increased funding. Concurrently, private equity investments in healthcare have reached \$5 -6 billion in 2024, nearly doubling pre -endemic levels and expected to maintain at these levels concurrently. Both trends point towards breadth and depth of opportunity in this space.

Demographically, India stands at a pivotal juncture. The working age population , that is 15 - 59 years of age , is projected to constitute 65.2% of the total population in 2031, offering a substantial demographic dividend. The population of seniors in the same period is expected to stand at 13.1%. Some estimates project that India shall have 34 crore seniors by 2050, accounting for approximately 17% of world's elderly population.

This dual demographic trend underscores the pressing need for a healthcare system adept at addressing both preventive and geriatric care. In this evolving landscape, the diagnostics industry plays a crucial role. The diagnostic sector, growing in double digits annually, is witnessing steady expansion. However, it is noteworthy that the organized segment accounts for only a fraction of this, highlighting significant opportunities for consolidation and standardization.

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Classification: Restricted I would like to share a few important trends with you at this moment , given how the growth in testing is shaping -up, Tier-2 and Tier-3 towns and beyond are expected to account for a larger share of the diagnostic demand in the coming years.

Secondly, the burden of non -communicable diseases or NCDs or lifestyle diseases has significantly increased with cardiovascular conditions, diabetes and cancer estimated to impact about 23 crore people by 2030.

At Dr. Lal PathLabs, we are committed to serving this requirement for higher testing by leveraging technology, expanding our reach and ensuring that quality diagnostics are accessible a

cross the country. Our brand has introduced a new test and overseeing the establishment of new laboratories with a view to enhancing accessibility.

Investments have continued in digital technologies in order to streamline operations and improve patient experiences. This includes implementation of advanced laboratory information systems and usage of AI to enhance diagnostic accuracy. As we navigate these transformative times, our focus remains steadfast on innovation and excellence. Together, we can shape a healthcare ecosystem that not only meets the current demands, but it is resilient and responsive to future challenges.

Thank you for your continued trust and partnership. I now hand over to Shankha .
Over to you, Shankha .

Shankha Banerjee: Thank you, Dr. Lal. A warm welcome to everybody on our Q4 & FY25 earnings call.

On the operational front, we sustained a strong growth trajectory in Q4 driven by volume growth and mix improvement without taking any price hikes. Sample volumes grew by 9.5% to 20.9 million, while patient volumes increased by 3.8% to 6.8 million. Our bundle d test program, SwasthFit , also delivered robust growth of 22%, contributing meaningfully to the overall performance.

During the year, we added 18 new labs, expanding our presence in Tier-3 and Tier-4 markets, while strengthening our network in Metro and Tier-1 cities.

On the system side, we have successfully implemented Microsoft D365, Starlims and the full IT stack in Suburban , creating a unified digital infrastructure across both brands.

I also want to take this opportunity to share my perspective on the overall direction for Dr. Lal PathLabs . India remains vastly unorganized in terms of diagnostic testing. Thus, it is our primary objective to develop the market, expand our reach and enhance our impact across healthcare landscape of India.

We are strengthening our network in Metro and Tier-1 cities, ensuring we remain the trusted partner for diagnostics in places where we are already well regarded. We also continue to develop our presence across Tier-3 and Tier-4 cities, primarily in North and East. We have built a strong brand preference in the North and East, but we continue to work on building brand reach and strength in the West and South. Our vision also includes a relentless focus on expanding our test portfolio. We are strategically growing our capabilities in specialized verticals such as genomics, reproductive diagnostics, autoimmune disorders and other advanced tests. In line with this, we recently launched an advanced Amyloid Typing Test. We are proud to be the first diagnostic chain in South Asia to offer this highly specialized test under the guidance of the National Amyloidosis Centre, London , UK.

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Classification: Restricted Our bundle d test program, SwasthFit , is a potent instrument to gain scale while providing value to patients. We are extending the bundle d test philosophy into the illness segment, ensuring we provide comprehensive and cost -effective diagnostic solutions across the healthcare spectrum. As we progress strongly into the hinterland, here too we are strengthening SwasthFit .

Integral to our future success is digital transformation. We are leveraging technology across our value chain from enhancing patient experience through user -friendly digital platforms to streamlining our operational efficiencies through sophisticated logistics and data management systems. This digital backbone is a strategic tool for scalability, security and for delivering innovative services.

We believe that this clear strategic roadmap coupled with focused execution will lay a strong foundation for sustainable and scalable growth in the years to come. We are cognizant about the opportunities and challenges ahead and remain dedicated to our mission of providing high quality accessible diagnostics to all.

With that, I now hand over the ca

ll to V ed.

Ved Prakash Goel: Thank you , Shankha . Good evening , everyone , and a warm welcome.

I will now proceed to share the financial performance for the quarter and the full year 2025.

- Revenue for Q4 FY25 came in at Rs . 603 crore compared to Rs . 545 crore in the same quarter last year , reflecting a growth of 10.5%.
- Revenue for the full year stands at Rs . 2,461 crore against Rs . 2,227 crore in FY24, a growth of 10.5%.
- Revenue per patient for Q4 FY25 is Rs. 887 , up by 6.4% , compared to Rs. 833 in Q4 FY24. This is mainly due to improvement in test and geographic mix.
- Test per patient for Q4 FY25 stood at 3.07 compared to 2.91 in Q4 last year , and 2.97 for the full year FY25 against 2.83 in FY24 .
- EBITDA for Q4 FY25 came in at Rs. 169 crore compared to Rs. 145 crore in Q4 FY24 , registering a growth of 16.9 % with the EBITDA margin of 28.1%. For the full year, EBITDA stands at Rs . 696 crore compared to Rs . 609 crore in FY24, registering a growth of 14.2% with a margin of 28.3%.
- PBT for Q4 FY25 came in at Rs. 154 crore compared to Rs. 120 crore in the same period last year, registering a growth of 28.1% with a margin of 25.5%. Full year PBT stand at Rs. 625 crore against Rs . 505 crore in FY24, registering a growth of 23.6% with a margin of 25.4%.
- PAT for Q4 FY25 came in at Rs. 156 crore compared to Rs. 86 crore in Q4

FY24 , registering a growth of 81.4% with a PAT margin of 25.8%. Please note that in Q4 FY25 , we have a one -time deferred tax benefit of Rs. 40.8 crore on account of voluntary liquidation of Suburban Diagnostics . For the full year, PAT stands at Rs. 492 crore against Rs. 362 crore in FY24 , registering a growth of 35.9% with a margin of 20%.

- EPS for Q4 FY25 is Rs . 18.6 compared to Rs . 10.1 in Q4 FY24 . For the full year, EPS stands at Rs. 58.5 compared to Rs. 43 last year , registering a growth of 35.9%. This EPS has a one -time deferred tax benefit as explained earlier. Excluding this one -time benefit, normalized EPS for Q4 FY25 is Rs . 13.6 and for the full year, normalized EPS stand at Rs . 53.5.

- Net cash and equivalents as on 31st March 2025 is Rs. 1,229 crore.

Further, I am pleased to share that the Board of Directors of the Company have approved a final dividend of 60%, that is Rs . 6 per share, taking the total dividend for the year to 240%, that is Rs . 24 per share.

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Classification: Restricted I would also like to update that pursuant to the approved scheme of liquidation of Suburban, the entire business undertaking of Suburban has been transferred to Dr. Lal PathLabs Limited on a going concern basis in Q4 FY25 .

With this, I conclude my opening remarks , and I would now request the moderator to open the forum for question and answer. Thank you.

Moderator: Thank you very much. We will now begin the question -and-answer session . The first question is from the line of Prakash Kapadia from Spark PMS. Please go ahead.

Prakash Kapadia: Yes. Thanks . A couple of questions from my end. So, at a time when most estimates for the diagnostic industry are showing an 8 % - 9% growth, our growth has been 10%. So just wanted to understand, you know, what it will take at our end to grow at faster than the current rate. So, you know, because we have been expanding and derisking from North , we have expanded to East, to West is now almost 14 % - 15% of contribution. I guess with scale, you know, the low cost and the quality assurance and branding should ensure slightly offer better growth than this. This is obviously better than industry, but what will it take for us to grow still higher than the current growth rate? So is it going to be a specialized test? Is it going to be some specific region contribution where we are very low or weaker ? So, some ca rds on that will help.

Shankha Banerjee: Yes, so, the industry growth rate is obviously an estimate. And a s of now, you are

right. It could be anything between 8 % - 10%. I think, you know, this year, the year that has gone by, we had started the year saying that we would like to do slightly better than the previous year without having to take a price increase. And I think we have been able to do that. Effectively, it's a 3% better performance than last year because there is no price increase in FY25 v/s there was a price increase in FY24 . Directionally, I think we are moving in the right direction and some of the initiatives, it is unlikely to be any specific geography because as you have said, there are at least now three geographies which are reasonable sized, which is Delhi NCR, North, East as well as West. So, it is unlikely to be any specific geography driven.

But I think our investments in terms of new infrastructure, the whole expansion of our collection network that we have done a bit of aggressi ve on that in the last 12 - 18 months should start seeing benefits of access that we get to newer markets. And also we are looking at increasing our focus on rather the SwasthFit portfolio, like we said, is something which we are moving into the Tier-3 and Tier-4 towns as well. There is still juice left in that strategy . And that's something which we will continue to push for. And even in our North and East markets, the Tier-3, Tier-4, growth is still continuing. I think a mix of all of these is what we are aiming towards to see how we can maybe grow faster than what we did in FY25 .

Prakash Kapadia: Okay . And anything you want to call out on the contribution of specialized test ? Is that a focus area given our legacy and experience in the industry ? Can that be a big contribution going forward?

Shankha Banerjee: So, you know, again, I think we had mentioned in one of our previous calls, the specialized portfolio is something which each company would be defining differently. But the way we define our specialized portfolio; it is about one -fifth of our portfolio comes from the specialized test. But again, like I said, you know, specialized test is something, yes, we will focus on.

But what we realize is that any given geography when we go, we have to look at the whole range, right from routine , specialized, super specialized, bundle d test, you know, all of that put together is how the ecosystem moves forward and the growth

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Classification: Restricted trajectory builds up. So, it may not be right on my part to call out just one element,

but yes, there is focus on the specialized part of the portfolio as well.

Prakash Kapadia: Okay. Understood. And one question for Ved. Ved, given that Suburban is now part of Dr. Lal on an ongoing basis, so how does it impact the amortization and the balance sheet? Because we were writing part of the acquisition cost through amortizing it through P&L. So, what is left? How does it change the P&L and balance sheet impact?

Ved Prakash Goel: Prakash, there is, you know, on consolidated basis. I don't think there is any change because earlier also in consolidated financials we were impairing intangibles and the same will continue. It is just that it is coming in standalone now instead of consolidated.

Prakash Kapadia: Okay. And would you have the figure of amortization for this year and what would be pending?

Ved Prakash Goel: As I mentioned earlier also about Rs. 50 crore kind of, an amortization which is coming every year on account of intangibles.

Moderator: The next question is from the line of Karthik Chellappa from Indus Capital Advisors.

Please go ahead.

Karthik Chellappa: Thank you very much for the opportunity, sir. Congrats on the quarter. Two questions from my side. First, if we were to look at the sample volume growth, could you give us some color on how did NCR North and, let's say, the West and East did individually?

Ved Prakash Goel: Karthik, hi, this is

Ved. I think we are not splitting sample patient volume geographically. Right now, we have given the split in terms of revenue. So, maybe volume geographically we have not mentioned here.

Karthik Chellappa: Okay, or let me ask this question qualitatively. I think your endeavor was to improve your volume growth in NCR, which is like your biggest market, and that was probably growing lower than your overall volume growth. Are you starting to see traction there and in FY26 can we expect to see NCR volume growth at least better? At least in qualitative terms would you be able to comment on that?

Shankha Banerjee: Yes, I think, you know, what we were saying in maybe one quarter back that we were mostly talking about NCR revenue growth and since we are not taking any price increase, obviously it is driven by sample volume and mix. And we would like to, you know, we have done about a double-digit revenue growth this year for Delhi NCR. And going forward, we would like to maintain that trajectory.

Karthik Chellappa: Okay, great. My second question, is if I were to look at our patient service centers, this year, we have probably added more than what we added in the last 2 years, around ~845 or so, which is also a healthy 14 %-15% growth Y-o-Y. So, is this like a prelude that we can see to a better volume print, given that you have actually ramped up your infrastructure? And I know there is a lead and lag for sure. But just given the investments that you have made in the PSC count, is this a sign that we should start to see better volume growth?

Shankha Banerjee: You know, the whole reason for ramping up the infrastructure numbers is to ensure that we get access to newer micro markets and even newer geographies. And in some of those geographies and depending upon the geography that the access we

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Classification: Restricted are getting, the kind of time horizon on which the infrastructure, the collection network really matures will be different.

It could be anything between 18 -36 months that would take for the collection network or a new collection center and a lab network to really start maturing and contributing significantly to the volume and the revenue numbers. But directionally, yes, you know, ramping up of infrastructure should show better patient volume s and revenue in the future.

Moderator: We will move to the next question, which is from the line of Anshul Agrawal from Emkay. Please go ahead.

Anshul Agrawal: Hi, thank you for the opportunity. My question is on network expansion. Are we looking to aggressively add labs in the next 1 or 2 years or do we think that the current lab network should suffice our growth plans in this period?

Shankha Banerjee: We have added 18 new testing labs last financial year. And for this financial year and maybe one more, we definitely would like to keep up the tempo at this pace. We have been adding actually close to about 14 -15 labs pre -COVID and post -COVID there was a period when the lab addition was slightly slower. We need to maybe catch up in these few years and after that we will come back to our normal lab addition trajectory.

There is still a lot of, you know, geography space which is available for testing infrastructure. Therefore, you know, that lab addition will continue. And not only that, even existing infrastructure of labs, you know, there are improvements and

upgradation of instrumentation, etc., that we keep doing. And that is also a part of the normal annual operating cycle.

Anshul Agrawal: Just a follow up on this, sir. Would we see an increase in our PSCs as well commensurate to what we have seen in the current year? The reason why I ask is this, our lab utilizations surely seem to be going up. And would that continue in your view?

Shankha Banerjee: Yes, collection centers, so lab and collection centers or PSCs as we call them, go hand in hand. It is the

whole hub and spoke and the whole network approach that we have in every geography. It is not either or. It will happen simultaneously. And as I was mentioning to a previous question that it is that ecosystem of lab and collection center network, which once added, takes a bit of time to really mature and build up. Yes, you will see addition of collection center, PSC network as well.

Anshul Agrawal: Sir, my second question is on the West region. Assessing by the split that you have given, geography-wise, our West region portfolio seems to have just grown about 5% in FY25. Could you give us an update on how Suburban has panned out in the last quarter?

Shankha Banerjee: We did the whole IT stack changeover in Suburban in Q4 and also the voluntary liquidation process was carried out in Q4. So, because of that, there has been a bit of an impact in terms of the revenue for Suburban in Q4. And we have already started recovering and we believe that we should be able to recover quickly in the next 1-2 quarters.

Anshul Agrawal: Okay, so it's been mid-single digits. Would that be a right understanding?

Shankha Banerjee: Yes, it will be a mid-single digit.

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Classification: Restricted Anshul Agrawal: Great. I will fall back in queue. I have more questions. Thank you.

Moderator: Thank you. The next question is from the line of Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan: Thank you. The first question on FY26. What are we sharing in terms of our revenue guidance? Maybe I can start there.

Shankha Banerjee: Yes, Shyam, so this year we have ended at 10.5% in terms of revenue growth. So, next year I think we should be better than that, maybe between 11% - 12%.

Shyam Srinivasan: Okay. And just another point here, as you mentioned, not taken price increase couple of times in the FY25. Is there a contemplation of that in the near term or in the medium term?

Shankha Banerjee: Price increase is something which for us will be a very strategic decision. I think in some previous call also we had discussed that if there is, you know, if we see the need, whether it is in terms of costs or whether it is in terms of the whole pricing table in the industry moving in a certain direction where we are left behind, if any of those things happen, then we may consider, but it is not something which we will do for any tactical reason right now, because maintaining the price table also has its advantages in terms of overall growth that we are trying to achieve.

Shyam Srinivasan: Got it. Just the 10.5% going to 11%, let's assume. Are we expecting improvement in volume growth? Or is it still the SwasthFit contribution that keeps going up 26-28-30%? I am just making it up. But what is driving higher growth?

Shankha Banerjee: I think it will be both. You see, the RPP growth that we did this year was about 6%, right, and patient volume and then sample volume at 9.5%. The uptick, we definitely are trying to do the incremental growth over last year driven primarily through volume and try to maintain the RPP growth.

Shyam Srinivasan: Got it. Last question, just again on guidance on margins. We have reached 28.3%, so very healthy numbers. But I just want to understand how we should look at margin for FY26. Thank you.

Ved Prakash Goel: Shyam, margins, I think, we have close at 28.3%. Going forward, I think, since we have to make a lot of investment in newer geography, newer talent and some digital initiatives and so on and so forth, so around 27% kind of margins we are looking or we are confident to maintain.

Shyam Srinivasan: Ved, 100 bps lower, you are saying.

Ved Prakash Goel: Yes.

Shyam Srinivasan: Okay, and the Rs. 1,229 crore of cash, any thought process on how that will be likely allocated, Ved? Thank you.

Ved Prakash Goel: Again as a stated strategy, we are keeping this cash with us for any op

portunity

which will emerge and as earlier mentioned, I mean, obviously we are growing organically and we have started our own efforts in South, particularly South and West, but if there is any opportunity there, definitely we will be keen to do that

opportunity. And that's why we are keeping this cash. And in addition, obviously, we are paying dividends as well.

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Classification: Restricted Moderator: Thank you. The next question is from the line of Bharat Celly from Equirus Securities. Please go ahead.

Bharat Celly: Thanks for the opportunity. We have been actually seeing volume growth of around mid-single digit since last almost like 7-8 quarters. And despite the fact that we are moving to the hinterlands of India, can we see these numbers inching up to the high single digit?

Shankha Banerjee: Like I was answering in one of the previous participants, the infrastructure expansion push that we have taken in the last 12 - 18 months over the new labs and collection networks and incremental areas around that, those will obviously start maturing. It takes anything between 18 - 36 months for these networks to mature. And we believe that they will help us improve our patient volume number. For overall, like we have been saying earlier, for volume, look at sample volume and patient volume both as indicators of volume, not only the patient.

Bharat Celly: When we will start seeing this Hinterland started picking up probably Tier-2, Tier-3 towns, will it be a case where our patient volume growth will look higher, but the realizations will start taking a hit considering the prices there will be relatively lower?

Shankha Banerjee: So, our pricing approach in Tier-3, Tier-4 is not really different compared to whatever are the contiguous Tier-2 or Tier-1 markets. Our pricing is similar, it's not essentially lower pricing on an MRP basis. But the revenue per patient and those angles, we obviously will look at that, how it develops and emerges. As of now, we are not seeing any significant difference between the Tier-3, Tier-4 realizations v/s. what we are seeing in maybe a Tier-2 or a Tier-1 town.

Bharat Celly: And according to you how the pricing for the competitors will be in these markets?

Shankha Banerjee: The competition in these markets are primarily unorganized local labs. I mean, that is the main competition in the Tier-3 and Tier-4 markets. And pricing there is not very different. I mean, it could be slightly lower, but it is not as if it is very different from the kind of pricing we are able to work on in these markets.

Moderator: Thank you. The next question is from the line of Raman Sivi from Sequent Investments. Please go ahead.

Raman Sivi: Hello. I just have two questions. One is, I want to understand the competition in the industry, because from what I am hearing and what I have seen, that there is a competition from pharma players also, like pharma players entering the diagnostic industry. For example, recently Lupin announced its foray into diagnostic sector. Is the competition going to intensify in coming quarters or years?

Shankha Banerjee: You know, on the competition front, I think this industry has always been quite competitive. And there have been phases when newer kind of players have entered the industry. And I think as long as more and more organized players come into the industry, it is good. Because the overall industry benefits from more organized players. It becomes more level playing field. So, from that point of view, this competition with more organized players is not something which is very new and is likely to maybe benefit the industry at an overall level as well.

Ved Prakash Goel: See, the market is so big, this market has space for everyone. I mean, it's not that we are the only player remaining here.

Raman Sivi: Okay. And my second question is with respect to the CAPEX. How much CAPEX

are we planning for FY26?

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Classification: Restricted Ved Prakash Goel: Normally our maintenance CAPEX is about Rs. 60 – Rs. 70 crore a year. And I believe in next year, FY26, similar kind of CAPEX will be there.

Raman Sivi: You are saying the CAPEX will be around Rs. 70 crore and as of now there is no plan of any additional acquisition?

Ved Prakash Goel: No, that is something different. That's why I am saying maintenance CAPEX, which includes normal expansions where we are opening 15 -20 labs every year and some maintaining CAPEX on account of let's suppose investment in IT or software and all that stuff. But if there is any opportunity, obviously that will be different.

Raman Sivi: With respect to this follow-up question on CAPEX itself, how much usually is the CAPEX? If I have to set up a lab or pathology lab or a radiology lab, how much usually is the initial investment to set up a new lab or pathology center?

Ved Prakash Goel: It depends. I mean, we have different kinds of set up, reference lab. Then we have hub labs and then we have set labs. So, smallest is set labs where generally Rs. 80 lakhs to Rs. 1 crore kind of CAPEX is required. And if you go smaller lab, which is like Tier-3, Tier-4, even lesser amount is required. So, that's how the CAPEX is required in these labs. Obviously, we are not planning any reference lab further in

this FY26 , that is not required.

Raman Sivi: Okay. And one last question. So, in FY26 , what is the revenue from pathology and what is the revenue from radiology?

Shankha Banerjee: Our revenue from radiology is actually miniscule. It is very low single digits. I mean, it is all pathology revenue only.

Ved Prakash Goel: Less than 5%.

Raman Sivi: 4% - 5%.

Shankha Banerjee: Less than 5% .

Ved Prakash Goel: Yes, less than 5%.

Moderator: Thank you. The next question is from the line of Divyansh Gupta from Latent Advisors . Please go ahead.

Divyansh Gupta: Hi, a couple of questions. First is on , let's say, weaponization of chemicals by China. They stopped selling of Gadolinium , which is MRI agent, and today they stopped shipping, exporting of rare earth magnets , right. And I understand that a lot of chemicals come from China for the reagent purposes. Two questions in this , how much of it is coming from China as of today? Is there Indian capacity which can service in case China takes some similar steps in this domain? How are we thinking about that?

Dr. Arvind Lal: Yes, this is Dr. Lal here. The answer to your question is that we hardly get anything from China , miniscule it is, you know, would be even less than 1%. So, this actually does not involve us and since we are not in radiology, we would be able to answer that question, what you were asking is that there is some rare earth chemical goes into a dye which is injected into the patient. We wouldn't know that.

Divyansh Gupta: Sorry, I wasn't asking about radiology . I was saying just for the pathology diagnostics , our reagents .

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Classification: Restricted Dr. Arvind Lal: In pathology diagnostics, no reagent in our case comes from China. None.

Divyansh Gupta: Got it. That's good to hear. The second one was an earlier presentation you used to share this Tier-3 revenue footfall numbers. But in the current presentation I am not able to find. Would it be possible to share these numbers?

Shankha Banerjee: Yes, We have had a good growth in our Tier-3 revenues even in this year. We had a healthy, mid -teens kind of growth . So, it is definitely in the similar trajectory as what we would have seen earlier. That trend continues.

Divyansh Gupta: Got it. And just the last question . On a per patient basis in Tier-3 v/s. on Tier-1, what is the difference in realization purely because of mix of pricing difference? Or at an overall level.

Shankha Banerjee: As I was answering the

previous question, we don't have a differential pricing in a Tier-3, Tier-4 town , we have a geography approach in terms of pricing. Markets in that vicinity all have similar pricing. So, there is no differential in terms of pricing. If at all , it would be maybe a bit of number of tests on per prescription or on that basis or a mixed differential , that would be the only difference. It is not anything to do with pricing.

Divyansh Gupta: And so what would be that end result because of the mix change? Like 5% on a per footfall basis, we end up making 5% lower, 10% lower , and purely because of mix change.

Ved Prakash Goel: I think realization is lower than let's suppose Tier-1 in Tier-3, Tier-4, but that split obviously is varied because of many reasons as Shankha mentioned. I don't think we have exact split of that right now.

Moderator: Thank you. The next question is from the line of Amey Chalke from JM Financial. Please go ahead .

Amey Chalke: Yes, thank you for taking my question. Although most of my questions have been answered, I have one question on the expansion. So , this year we have seen something like 18 labs being added in a year and close to 1 ,000 PSC. Is it possible to tell how much would it be in the core areas of like North and how much would it be in all core areas?

Shankha Banerjee: So, we can look at that number and come back to you, but in terms of you asking only labs ?

Ved Prakash Goel: Mostly in our core geography.

Amey Chalke: I am saying the diagnostic paradigm, the way it is done largely, it is In Vitro Diagnostic at present, the dependence of it. So , going ahead, you see the importance of In vitro diagnostic to go down in place of, let's say, AI -powered image techniques, etc., if a lot of these ventures are using them. So , you expect the In Vitro Diagnostic testing as an importance to go down going ahead.

Dr. Arvind Lal: No, we don't think so. I think In Vitro Diagnostics will continue the way they are continuing.

Shankha Banerjee: You know, our view is, there is, if you look at imaging and pathology, firstly, there is a lot of AI deployment that is happening on the imaging side. In pathology, there is still AI work happening. And either ways, a lot of it is going to assist the current evaluation and reporting and maybe help enhance that rather than maybe replace it .

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Classification: Restricted Moderator: Thank you. The next question is from the line of Deven Kulkarni from Marcellus Investment Managers. Please go ahead.

Deven Kulkarni : Hi, Shankha . Hi, V ed. So , this quarter, I see that our fees to collection centers has grown by just 3 % odd YoY, which is like far better than our revenue growth. So , can you help me understand what's happened here? Have we cut channel margins or is it some mix change?

Ved Prakash Goel: There is no change in the margins. Deven, it's purely on the mix.

Deven Kulkarni : Okay, so then how to interpret that? Sales from collection centers has grown by just 3% odd Y-o-Y in Q4. And the rest of it has come from our own labs or direct channel, right?

Shankha Banerjee: No, that would not be the right way to read it. The sales from collection center has also grown in double digits. It is also, like he said, there is a test and a geography mix. You know, that is what will be impacting. But there is no change in terms of the revenue share that we are giving to the channel partners.

Deven Kulkarni : Okay, understood. And secondly, Ved, what was the EBITDA margin in Suburban in this quarter?

Ved Prakash Goel: So, as I mentioned this has been merged in the parent company in this quarter. Now we are looking overall margins, maybe it is the company margin. But as earlier mentioned, I mean couple of quarters back, this was having 15 %-16% kind of EBITDA margins. But really now after m erging with this, it's only one which is coming and not separately.

Mo

derator: Thank you. The next question is from the line of Sumit Gupta from Centrum B roking. Please go ahead.

Sumit Gupta: Hi, thank you for the opportunity. It's a follow -up to the previous participant's question. So, just to understand on the competitive scenario in the Mumbai market for Suburban and what are the other competitors , like how do you see this competition? For overall market, how is it shaping up?

Shankha Banerjee: If you see Mumbai market, you know, we are not the leaders. So, there is existing players who are the market leader there and number two strong player. For us obviously, since we are a challenger brand in that market, for us that competition has always been there and it remains similar.

Sumit Gupta: Okay, so how do you see Suburban pairing over the next 3 - 4 years? What is the plan for Suburban ?

Shankha Banerjee: The plan for Suburban is focused on these three geographic places, Mumbai, Pune and Goa. And we continue to work on those markets. These are large, you know, Mumbai as well as Pune have become large diagnostic hubs in their own right. And we believe both these markets have a huge headroom available for us to play both the brands and therefore we will continue with our two brand , dual brand strategy in these markets . And that's how we want to really move to becoming one of the top 2 brands in both of the geographies .

Moderator: Thank you. The next question is from the line of Harshal Patil from Mirae Asset Capital . Please go ahead.

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Classification: Restricted Harshal Patil: Good evening, and thank you for the opportunity. I just have two questions, probably from an understanding perspective. We did see that for FY26 , we are kind of expecting some 100 bps lower in terms of operating margins. And so I just wanted to also understand from the perspective that we have some, you know, we have kind of expanded our network and lab network and collection networks over the past 2-3 years. The benefits of leverage would definitely also accrue going ahead as these centers ramp up. And simultaneously, we are also expecting 100 bps lower margins. Is it that the initial start -up cost for the recently commissioned labs is higher or are we kind of talking of higher marketing expense?

Shankha Banerjee: You know, that is a very good question. I think the initial aggression in terms of opening of labs collection network is about 12 - 18 months old. And they are still in the maturing phase and even in this financial year, we are saying we want to open 15 - 20 new labs. And I also mentioned, you know, there will be upgradation in existing labs. It is not only about the newer labs, but upgradation in existing lab infrastructure that we will do. And also there is more frontline sales teams to work on in the newer geographies that we are going and intensifying our approach in the markets that we are already there and also advertising costs. Therefore, marketing

spend, it is a mix of all of that. Basically, you know, as that previous thing matures over a 18 - 36-month cycle, we should start maybe seeing those benefits, but then we may have newer areas to invest at that time.

Moderator: Thank you. The next question is from the line of Kunal Lakhani from CLSA. Please go ahead.

Kunal Lakhani: Hi, good evening. Just a follow-up on an earlier remark. You said that the radiology revenue mix is less than 5%. Is that a conscious strategy to keep the concentration lower or it could be a revenue driver for future? What's the thought process there?

Shankha Banerjee: You see currently, we have what we call as basic radiology in some of our core markets which is X-ray, ultrasound, maybe TMT and things like that. High-end radiology which is CT, MRI is something which we haven't really gone into on a big way. You know, as of now there is a lot of market opportunity in the pathology area itself and therefore

, being our core strength, we want to focus more energies in that direction where we have an advantage. But having said that, radiology as an option is always on the table and we continue to evaluate it. And as and when we find that to be something viable, fitting into our growth strategy, we will get into it.

Kunal Lakhani: Sure. I mean, the reason why I was asking that was because we do have plans to expand in the South and some of our Southern peers have a very strong radiology mix, which kind of in a way helps them in terms of branding and sort of brand recall.

You think that would not be an issue for us when it comes to expanding South India?

Shankha Banerjee: You know, in South India currently, our footprint is relatively quite weak and we are looking at both organic as well as inorganic opportunity in South. There could be a possibility that if there are inorganic acquisitions which are coming with the radiology business attached to it, then that becomes a part and parcel of our business also going forward. But organically trying to do radiology in the South is really not on the agenda right now.

Kunal Lakhani: Understood. Thank you so much. My second question was on the margin outlook again. This year you have guided for like 100 bps lower margin. But would it be more transient or you think this 27% margin is something that would go on for like, say, mid-to-long term?

Ved Prakash Goel: I think, Kunal, this is where we are investing and that is where that doesn't mean that operating leverage or efficiency is not coming. Whatever improvement or whatever

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Classification: Restricted increase in the margin, we are investing back into the business. And this is definitely for the FY26 we have narrated. Maybe for a longer term, how we will go, maybe it is difficult to tell at this point of time. But definitely for the next year, we have plans to invest in the business.

Kunal Lakhani: Sure. And lastly, on the pricing bit, right, I mean, we have seen a very prolonged period of new price hikes. And it's for any industry, right, price growth is like a litmus test or metrics to measure the health of the industry per se, right? Where do you think we are or how far away you think we are in terms of like the pricing power coming back to the industry, not just talking about Dr. Lal or any other operator, but for the industry per se, pricing power coming back. When do you think that can happen?

Shankha Banerjee: You know, I don't think this question of taking a price hike or not is something to do with the pricing power. It is more about a conscious choice and a strategic choice of not taking a price increase, given some of the strategies that we have deployed for growth, moving into Tier-3, Tier-4 towns and some of the other ways that we want to look at sample volume growth also being one of the major drivers, looking at how can we improve maybe even patient volume in the future.

It is more of a conscious reason. And since we are able to kind of drive efficiency through operating leverage, a lot of technology that we deploy, we are able to eat a lot of the inflation that is otherwise going to be in the system. In that scenario, it is a conscious choice of not taking a price increase.

But like I mentioned to a question in the past, you know, we obviously will keep evaluating if there is a cost pressure or if there is any other reason or if we see the pricing table in the industry is moving in a certain direction and we are kind of, you know, there is always a linkage in our industry of price and quality. We don't want to be caught on the wrong end of that. If those opportunities present to us, then we may consider a price increase.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I would now like to hand the conference over to the management for closing comments.

Ved Prakash Goel: T

Thank you , everyone , for being with us on this call today. We express our gratitude for your continuous trust and support. I hope we are able to answer all your questions. Please feel free to reach out to us in case you have any further queries. Thank you once again.

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Call: Aug 2025

Classification: Internal
August 05, 2025

National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001
Scrip Code: 539524

Sub: Transcript of Q1 & FY26 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q 1 & FY26.
We request you to please take the same on record.
Thanking You,
Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl. : As above
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Dr Lal PathLabs
Q1 FY26 Earnings Conference Call Transcript
July 31, 2025

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO and CEO, International Business
Participants who asked questions • Karthik Chellappa – Indus Capital Advisors Hong Kong Limited
• Anshul Agrawal - Emkay Global
• Prakash Kapadia – Spark PMS
• Surya Patra – Phillip Capital
• Karan Vora – Goldman Sachs
• Pranaya Jain – Banyan Tree Advisors
• Yogesh Soni – InCred Capital
• Aashita Jain – Nuvama Institutional Equities
• Harshal Patil – Mirae Asset Capital Markets (India)

Moderator: Ladies and gentlemen, good day, and welcome to Dr Lal PathLabs ' Q1 FY26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during this conference call, please signal an operator by pressing “*,” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand over the conference over to Mr. Nishid Solanki from CDR India.

Thank you. And over to you , sir.

Nishid Solanki: Thank you. Good afternoon, everyone, and welcome to Dr Lal PathLabs ' Q1 FY26 Earnings Conference Call.

Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal – Executive Chairman , Mr. Shankha Banerjee , CEO and Mr. Ved Prakash Goel , Group CFO and CEO, International Business.

I would like to share out standard disclaimer . Some of the statements made on today's conference call could be forward -looking in nature and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the results presentation , which has been circulated to you and also available on exchange website s.

I would now like to invite (Hony) Brig. Dr. Arvind Lal to Share his Perspectives.

Thank you , and over to you, sir.

Dr. Arvind Lal : Thank you very much. Good afternoon, ladies and gentlemen, and welcome to our 1st Quarter Earnings Call . I would first like to highlight the broader opportunity in healthcare and the role that we can play as a leading diagnostic brand.

Healthcare in India continues to grow at a healthy pace, characterized by digital transformation and shift towards tech -driven and holistic healthcare delivery. However, I would like to add that there is more than ample room for growth. When it comes to hospitals alone, it is said that India has a shortfall of 2.4 million beds to meet global standards. This is getting addressed with major hospital chains adding to capacity judiciously by upwards of 30% till FY27. Central Government flagship initiatives like Ayushman Bharat and Heal in India are propping up public and private investment in the healthcare system. One can be assured that this kind of growth will translate into a higher requirement for quality diagnostic services going forward.

On the tech side, we are witnessing increased integration of AI in healthcare services, including diagnostics. It is estimated that by the end of 2025, the Indian artificial intelligent healthcare market would have a size of US\$1.6 billion. The impact of telemedicine and digital health records is being felt given enhanced access in rural and semi -urban regions. The role of digital and tech within our industry is growing, where on the one hand, it appears to be operational efficiency and on the other, it enhances patient and clinical outcomes.

Let us look at the other side. The prevalence of chronic and lifestyle diseases in India continues to expand. We are the topmost country with incidence of diabetes, with 100 million patients afflicted. Rates of hypertension are also

climbing, with some surveys indicating that maybe 30% of adults having elevated blood pressure. Obesity is experiencing a surge in young adults and children, given linkages to higher consumption of processed foods and general inactivity. And these trends are coming up in the rural settings as well as lifestyles undergo change over there. The only drawback for rural populations has been access to regular screening and specialist care. Delayed diagnosis is also an important exacerbating factor here, especially in underserved populations.

Given this background, the role of diagnostics and management of these healthcare outcomes cannot be overstated. As a national brand, we remain committed to providing

quality diagnostic services to the country. Our operations are steadily spreading across hinterland markets, where demand for such services is rising. This is especially so in Tier-2 and beyond. We are building stronger presence in the West and South, to complement presence in the North and East. FY25 saw addition of 18 new labs to our network, and we expect to sustain a similar number in the present year too. Our patient service centers also saw a strong increase to match this, and again we expect this franchising trend to continue.

On the tech side, we are making the right investments to strengthen operations and create scale with flexibility.

Our journey will be marked by growth milestones as we seek to broaden the scope of services and geographic coverage. The model is scalable, and we have the intent to grow it to meet unserved healthcare requirements. With that, I would like to hand over to Shankha to continue. Over to you, Shankha. Shankha Banerjee: Thank you, Dr. Lal, and a very warm welcome to everyone. Let me share some insights into Dr Lal PathLabs performance in the 1st Quarter of Fiscal Year 2026.

As evident, our results are a testament to the enduring strength of our business model and the disciplined execution of our dedicated teams across the nation. The new financial year began on a strong footing, with 11.3% growth in revenues and 24.3% improvement in profit after tax. This was achieved by steady volume momentum and a favorable test mix. Our sample volumes grew by 10.7% to 23.4 million, while patient volumes increased by 5.3% to 7.6 million.

At an industry level, the competitive landscape continues to evolve. We have recently observed the entry of a new e-commerce player into the diagnostics arena. This will add to the existing online competitors that have been in the industry for nearly a decade. The foray from hospitals and pharma companies into the diagnostics sector continues to help in speeding up the unorganized to organized shift for the industry.

Strategically, we are expanding our capabilities in high-complexity testing. We have launched 58 new tests in the quarter, strengthening the genomics portfolio and introduced component-resolved diagnostics for allergy testing. We are also supporting antimicrobial stewardship with the launch of an in-house smart culture reporting algorithm for the right antibiotic recommendations for all types of culture reporting. We see a very strong future potential in high-end and specialized testing.

We are also seeing positive trends in bundle testing under "Swast hFit," which continues its strong growth trajectory. The bundle testing offer is being strengthened on the illness segment front as well. Further, we have added new

offerings in genomics, reproductive health, and autoimmune disorders to bring sharper focus to these areas.

On the operational front, our network continues to expand in line with our cluster-based strategy. We are strengthening our leadership in core urban markets in North and East, including Delhi NCR. We are also deepening our presence further in Tier-3 and 4 towns.

We are maintaining our calibrated pricing strategy and continue to hold our prices. The gains in realization are driven by premiumization of our offerings and driving a favorable test and geography mix.

On the digital front, we are investing more in automation and digital systems to improve patient experience, enhance cyber security, and drive operational efficiency.

Our outlook is defined by three strategic pillars: driving volume-led growth through aggressive market expansion, achieving operational excellence via comprehensive digital transformation, and steadfastly upholding our brand leadership through unwavering quality and widespread accessibility.

With that, I will now hand over the call to Ved.

Ved Prakash Goel: Thank you, Shankha. Good afternoon, everyone, and a very warm welcome once again.

Let me now walk you through the financial performance for the 1st Quarter of FY26:

- Revenue for Q1 FY26 stood at Rs. 670 crore compared to Rs. 602 crore in the same quarter last year, reflecting a strong growth of 11.3%.
- Revenue per patient rose to Rs. 880, up 5.7% from Rs. 833 in Q1 last year, driven by a favorable change in the test mix. Tests per patient increased to 3.07 compared to 2.92 in the same period last year, highlighting continued traction in bundled and preventive test adoption.
- Our Swast hFit portfolio contributed 27% of revenue this quarter, up from 25% in the same period last year, underscoring its growing relevance in preventive healthcare.
- EBITDA for the quarter came in at Rs. 192 crore v/s Rs. 170 crore in Q1 FY25, a growth of 13.1% with a stable and healthy EBITDA

margins of 28.7%.

- Profit before tax rose to Rs. 181 crore from Rs. 150 crore, up 20.8% with a PBT margin of 27%.
- Profit after tax stood at Rs. 134 crore compared to Rs. 108 crore in Q1 FY25, delivering a robust 24.3% growth and maintaining a PAT margin of 20%.
- Earnings per share for Q1 FY26 came in at Rs. 15.9, up 24.4% from Rs. 12.8 in the corresponding quarter last year.
- Our balance sheet continues to be strong and resilient, with net cash and equivalents of Rs. 1,389 crore as of June 30, 2025.

In recognition of this performance and to reward our shareholders, I am happy to share that the Board of Directors has approved an interim dividend of 60% i.e., Rs. 6 per share.

These results reaffirm the strength of our business model, the effectiveness of our execution and our disciplined financial stewardship. We are making strong progress across all strategic fronts, expanding in both core and emerging
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markets, scaling up our high-end and specialized test portfolio and accelerating our digital transformation journey. We remain confident in our ability to deliver sustainable, profitable growth while staying true to our purpose of delivering trusted diagnostics with care and precision.

With this, I conclude my opening remarks, and I would now request the moderator to open the forum for question-and-answer. Thank you.

Moderator: Thank you very much. We will now begin the question-and-answer session.

The first question is from the line of Karthik Chellappa from Indus Capital Advisors Limited. Please go ahead.

Karthik Chellappa: Yes, thank you very much for the opportunity, and congrats on the quarter. I have three questions. The first one is on the volume growth for the quarter, which is very healthy. The commentary that we heard from a lot of consumer companies is there were a lot of unseasonal rains and erratic weather this quarter. Did our volume growth benefit from any of these unseasonal trends, or is this like a very clean kind of unseasonal volume growth that we saw this quarter? That is my first question.

Shankha Banerjee: Yes, hi, Karthik. Thanks for your question. As of now, in the 1st Quarter numbers that are there, there is not too much of a favorable impact because of the unseasonal rain. It is more or less like-for-like compared to Q1 last year.

Karthik Chellappa: Excellent. The second question is, if I were to look at our realizations this quarter, they have not grown much, probably even less than one percentage or so. But despite that, we have seen a very strong gross margin expansion both YoY and QoQ. Is there anything specific on the cost front that we have done this quarter? And is this kind of improvement something that we can sustain, or are we hitting a normalized level of gross margin where we like to sustain it at an 80%-81% level?

Ved Prakash Goel: Hi, Karthik. I think realization has gone up from Rs. 833 to Rs. 880, which is not 1%. I mean, that is one. But there is no price increase as such we have taken in this. This is all due to test mix, or maybe hi

gh end test contribution

which has contributed to this realization. Margin specifically has increased, and we got the benefit because of this SwasthF it which has contributed 27% this time, which is highest ever contribution in this quarter. So, that is the reason we are getting some benefit out of this.

Karthik Chellappa: Excellent. Okay. Because the 1% I was referring to was the realization per test. But it is okay. I think I got the message or so. My last question, in last quarter, we indicated in our outlook for FY26 that we may see up to, let's say, 100 basis points margin compression because we are reinvesting into our brand as well as lab infrastructure, etc. This quarter, margins have actually been quite strong. So, should we see the impact of this margin compression on a lag basis, let's say, in the remaining quarters or was this quarter it just surprised us more than what we had anticipated?

Ved Prakash Goel: You are right. If you see first half, we generally have higher margins, because most of the investments we are getting into second half. I do not think these 28.7% margins are representative of the full year. Yes, we have improved, but let us see how the remaining quarters goes. But most of the investment will be in later quarter of the year.

Shankha Banerjee: But Karthik, just to add to that, I think there is internal view, although we are recalibrating that the overall annual margin also could be slightly better than

what we had projected at the beginning of the year.

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Moderator: The next question is from the line of Anshul Agrawal from Emkay . Please go ahead.

Anshul Agrawal: Hi, thank you for the opportunity. My first question is on volume growth. Could you provide some color, is this growth secular geographically? Is it some particular region which has led to this strong patient volume growth or sample growth?

Shankha Banerjee: If you look at the overall number, revenue at 11.3 % and let us say sample growth at 10.7 %, which is really very closely hugging the overall revenue. I think we are seeing similar trend of the sample growth being very close to our overall revenue growth across all our major geographies.

Anshul Agrawal: Okay. Core Delhi region is still growing at double -digits and same for West ?

Shankha Banerjee: Yes, Delhi has grown at double -digits for this quarter as well. West, like we said that post our changeover in Suburban of the whole IT stack, there has been a bit of a discontinuous impact in the market, which we had mentioned would take maybe 2 quarters to recover. We are still in that recovery phase. It is better than previous quarter, but I would not say we have still reached the level that we were 2 quarters back.

Anshul Agrawal: Got it. If at all that comes in after two quarters, we still have that lever to improve our volume trajectory.

Shankha Banerjee: Yes, that is what it would seem that we still have some headroom there to go back to our previous numbers there.

Anshul Agrawal: Great. Second question, is on the current quarter, which is so basically unseasonal monsoon is the question on that. Q2, we should see the beneficial impact of that . Would again that be correct?

Shankha Banerjee: You see Q2 has traditionally been the high quarter specifically driven by fever and seasonality. So, like if we are saying this year, the similar thing was there last year. I do not think there is an expectation of this year high being differential from others, because there is still quite a lot of the quarter still remaining to play out. It has been a seasonal high always, and we expect a seasonal high this year as well.

Moderator: The next question is from the line of Prakash Kapadia from Spark PMS . Please go ahead.

Prakash Kapadia: Yes. Thanks for the opportunity. A couple of questions from my end. In the last, I think, 3 odd years, we have launched the Bangalore Reference La

b as well

as the Mumbai Reference Lab. Historically, we have seen the sales growth is much higher in these markets when the reference lab is launched, be it east, be it north. Towards the latter half of the year, would we expect west and specifically south Bangalore side to have revenue growth faster than the company average growth, because that is what I think the trend shows and all the efforts which we typically put once the reference lab is there, they generally fructify and in addition to that, from a revenue side, also, I think last call you had alluded, Shankha, the patient service centers we have invested. So, if I see that last 2-3 years, they are up by 40 %, pickup points are up by, I think, 17%, 18%. When does the step up in revenue happen for us? Obviously, we are growing better than industry, but still the step up in revenue say 15 % revenue growth, when do some of these things fructify as we move forward?

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Shankha Banerjee: Right. Thanks, Prakash. Very interesting questions. I think firstly on the reference lab question that you asked. The reference lab setup is like, basically, we have a test menu, which is significantly larger than, let us say, a hub lab or a cluster lab. I mean, that is how the reference lab setup is and the higher test menu is typically a lot of not so frequently tested, but it is something we outsource from quite a lot of clients. Now, if you look at how this whole thing works is that, firstly, clients in that same geography who we were maybe sending the samples to Delhi or somewhere else, now start getting reports better and faster from the local lab or the local reference lab that we have, and then the conversion of the other clients for those high end tests will happen. It is a slightly long-term thing compared to a satellite lab. So, this is a slightly longer term horizon thing and it is a bit of a slow burn. So, to kind of directly link it only to opening a reference lab may not be appropriate because some of the other capacity of routine tests which get built in a brand strong market, the ability to leverage that is more quicker than in markets where brand strength is still under development. I would say that the reference lab impact

will be a slightly more longer-term one.

Coming back to your second question, which is on infrastructure and the front end, whether it is the collection network or the pickups that we are opening, now, as you would have also noted in our previous commentary, quite a bit of the infrastructure gets opened up in Tier-III, Tier-IV towns and some of the throughput per infrastructure may not be as high as the throughput per infrastructure in the larger urban towns. Just the number increase getting converted to a direct, same kind of a number increase on revenue side, may not always be appropriate. But, yes, there is also a buildup in terms of revenue that happens over a period of time. But it is a constant process. Although we have stepped up some of these additions, there has always been addition happening every year at certain level and that is a rolling benefit. It is unlikely to show you a serious step jump going forward. But, yes, it will all contribute because finally the whole network, the hub-and-spoke model of the collection network and the lab and the test menu in that lab is what will help us grow the revenues in the future.

Prakash Kapadia: And this, Shankha, you said would happen over a period of time. So, that would take more time in the Tier-III, Tier-IV markets where we are focusing since the last few years because there I think penetration, habits, awareness would take time. What would be that inflection point, which we have to monitor to see we have got there in terms of the faster revenue growth, which seems to be missing as of now? Obviously, it is better than what others or industry players are doing. But how does that step up happen

? Is it habit change? Is it awareness? Is it per capita?

Shankha Banerjee: Infrastructure is definitely one big contributory factor in terms of step up of revenue growth, but that is not the only factor which will determine. There are other things that need to be executed. Like I said, in terms of our test menu expansion, in terms of speed to market, in terms of the brand salience getting developed. There are a lot of other factors to play but yes, infrastructure is a large contributory factor. Again, directly linking to say that, in terms of numbers, 40% infrastructure means a 40% increase in revenue may not be the right correlation to build.

Prakash Kapadia: Sure. Understood. One question is for Ved. The amortization for Suburban would continue at Rs. 60 crore run rate and this should be over in the next 4 years, right, is that understanding, correct?

Ved Prakash Goel: Total is Rs. 50 crore, Prakash, for the year, Rs.12.5 crore per quarter, which will continue for some time.

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Moderator: The next question is from the line of Surya Patra from Philip Capital. Please go ahead.

Surya Patra: Yes, thank you for this opportunity, and congratulations for the great set of numbers. My first question is on the number of tests per patient. If I see there is a kind of a very steady secular rise that we are witnessing and possibly that is helping you deliver growth without taking even any price hike, although the industry in following. Although there is kind of a consistent rise that we are seeing in the number of sample per patient or the test per patient, is there any benchmark that one can think, see, the number could be, let us say or based on your any matured sub-segment or sub-market, the number of a test per patient could go as high as 4 or 5 or what number that one can kind of target and achieve and over what time period, if you can give some sense on that?

Shankha Banerjee: I think that is a very interesting question. And, at the outset, let me say we do not have a target that we are following on a test per patient basis. But one thing that we definitely see is that there are a lot of factors which are contributing to this test per patient increase, and some of those factors definitely in the near-term would continue.

First and foremost is this whole SwasthFit acceptability and growth that we see, and I think I mentioned that sometime before as well, we see traction in terms of more people wanting to use it not only in urban areas, but we are also seeing some of our smaller towns, the acceptability is rising, we are also seeing some traction coming from the prescription channel. So, that is one driver for test per patient. And obviously, there is another driver, which is in terms of prescribing habits of clinicians where there could be, more tests being prescribed on a single prescription now than previous. Yes, there are certain factors which are contributing, we are not running after a target on this one. But you are right. This is definitely contributing to our growth and obviously

helping us to maintain our price stance the way we have taken.

Surya P atra: Sure . My second point was, let us say, what is the growth over last year that we have seen in terms of the number of test panels and whether it is fair to believe that with the rising trend of the test panels that we offer, that will lead to kind of a faster growth in either the Swast hFit growth or the overall growth for us ?

Shankha Banerjee: I assume by panels you are implying Swast hFit. So, Swast hFit portfolio Q1 this year has grown about 22 % over the same quarter last year and obviously that is ahead of our overall revenue growth and therefore contributes positively to our overall revenue growth as well as to our profitability mix.

Surya P atra: But it is not the number of panels that also contribute to the growth of Swast hFit portfolio

o ?

Shankha Banerjee: I am not too sure what you mean by number of panels. We have got some set predefined Swast hFit bundles. There are 1-2 new that we add bundles, but these are all predefined bundles.

Surya P atra: Okay. Currently what 385 test panels that we are having , that number would not be very frequently changing , is that understanding right ?

Shankha Banerjee: Yes, that understanding is right.

Surya P atra: Okay. Then my last point was, do you believe likely a significant boom in the usage of the GLP -1 medication , is likely to have a kind of a big potential boost

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to the overall test volume numbers for us because of our pan India presence that we are having?

Shankha Banerjee: No, I think that is a very interesting question. We have not really maybe figured out the connection between GLP -1 usage and diagnostic testing or direct connection to that. But let us as a team maybe we will look into this slightly more deeply and try and see if there is a direct connection that could be there between this and what impact it could possibly be.

Moderator: The next question is from the line of Karan Vora from Goldman Sachs. Please go ahead.

Karan Vora: Yes, thank you for taking my question. My first question is with respect to Suburban. While we have been one of the best -run diagnostic companies, for a long time now. Like for Suburban, we have not been able to work it out like probably the way we would have wanted when we had acquired the asset. Any color if you could provide on what are the things where we have struggled or what went wrong and what were the learnings from it , would help ? And also a sub-part to that is this IT restack, which we are doing on Suburban , any particular reason we are doing it right now v/s. say do it when we acquired or immediately after we acquired it , that is my first question.

Shankha Banerjee: I think the Suburban question has been asked quite a few times in the past and I think on the investor call, we have tried to answer that quite a few times. But if you reflect back and see what are the challenges, I think we said that there was one big change that we had wanted to make with the collection network, which was there , when the COVID test went down, a lot of our existing collection network within Suburban actually started becoming unviable and closing the franchise part and then, we also wanted to change the whole structure from an owned collection network to a franchise collection network model. So, that changeover, I think, took longer, specifically because of the slowdown of the existing franchise network, which happened post the COVID reduction. I think that was obviously one core reason.

I think the other thing is that , starting itself we said that there are quite a few parts of the business that we felt were non -strategic and non -core, which we started de -emphasizing and that obviously went on a slow decline. And those kind of non -core geographies as well as non -core business lines , on the top line side, one would not see the benefit , although internally, there would be segments that we wanted to grow, were starting to see those results. Coming back to the point on the IT stack changeover , we had done quite a lot of work , we have to prepare the teams. The operations part had to be aligned to a certain level before the IT stack changeover could be done. I think the acceptability within the system, the team, as well as the preparation that would have gone into it and we felt that now is the right time for us to really start looking at more backend synergies. And that is why the timing was chosen to do the IT stack changeover to align fully to the Dr. Lal PathLabs IT stack, which we did in February this year.

Karan Vora: Got it. And is this the last step or, post which all the measures like conversion

of own centers to franchisee centers and all the other things

are done and we

should start seeing recovery , would that be a fair understanding , like maybe after a quarter or two?

Shankha Banerjee: I would not say all is done because there are still certain geographies there, we have actually now even transitioned the business to Dr. Lal PathLabs system. There will still be some transition which will continue because, we do not want to do everything in one shot , it really does not work in an ongoing business , we have to obviously handle it carefully.

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Karan Vora: Got it. And my second question is with respect to the inorganic opportunities and the cash allocation, which we have in the balance sheet. So, like while we have always mentioned that we want to explore for assets in the South and in untapped geographies for inorganic acquisition. But just to get a sense on, what are the challenges when you go and approach the labs? First of all, are there enough labs for you to target like potential targets? And then if they are, then what are the issues you are facing other than the valuation part, which might always probably be expensive in India? Any color on that would be helpful.

Ved Prakash Goel: Yes, Karan . I think, we already stated that inorganic is the idea and especially in South. Right now, there is nothing which is we can share right now. But the exercise is on in terms of what should be the value , what is the strategy and all. I think the foremost thing we want a platform where we can ride and, make that platform big. We do not want to just acquire for the sake of adding turnover. But wherever we feel there is a strategic fit into our whole long -term strategy, I think that is where we require. But yes, there are opportunities . At the right time maybe it will come.

Moderator: Thank you. The next question is from the line of Pranaya Jain from Banyan Tree Advisors. Please go ahead.

Pranaya Jain: Hi, thank you for taking my question. Actually, I wanted to understand our gross margins outlook. When we look at gross margins over the last few years, they have been constantly expanding. As we scale up further, should not gross margins go up even beyond, say, 82 %-83% over a 3-4 year horizon?

Ved Prakash Goel: I think your observation is right and the gross margin is increasing. Obviously , a couple of things contributing. One is, I alluded on my opening remarks because of contribution of Swa sthFit where the test mix is very different. Second is scale, obviously and third, geography mix also. I can not comment on where it can go but our efforts is always to create efficiency where we can still get some benefits, whether it is volume, whether it is test mix and all. But right now it is hovering around 79 %-80%. And I would say near -term, I do not think it will go substantially, from here. So, that is where you should consider that it should be around at the current levels.

Pranaya Jain: Got it. And my second question is in our non -core geographies, what are some of the strategic initiatives that we have taken to fasten organic growth?

Shankha Banerjee: Non-core geographies, obviously , there is a slightly slower turnaround in terms of building the organic business. And it will follow a quite simple mechanism, obviously building the testing infrastructure , collection network and then put feet on the ground, who can then meet up with the prescriber community and be able to promote our services and how we can give good quality services compared to maybe a lot of other local labs and maybe some of the smaller regional labs that may be operating in that geography.

Moderator: Thank you. The next question is from the line of Yogesh Soni from In Cred Capital . Please go ahead.

Yogesh Soni : Yes. Thanks for the opportunity. My first question is on the Delhi NCR region. We have seen consistently double -digit growth for the last couple of quarters. I want

to understand, I mean, what has changed in the said region - whether the market competitiveness has changed or whether we have made certain efforts that is resulting in double -digit growth?

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Shankha Banerjee: Yes, I think firstly, there is a slightly benign market competitiveness in terms of the online players and pricing , that is a contributing factor. But over and above that, there are quite a lot of service enhancement initiatives that we have taken in the market, whether it is on logistics, test menu, people on ground. A lot of those and even looking at our channel partners and kind of motivating the

channel partners performance, etc. So, a lot of those initiatives have gone into play, which is resulting in what we are seeing as the consistent double-digit growth in NCR.

Yogesh Soni : Okay. That helps me to understand. Another question is on the genomics. I just wanted to discuss about the genomics market, what kind of market size is it now and the kind of growth rate that we are seeing in the market ? Also, I mean, if you could let us know what is the overall contribution of genomics in our revenues and how we can see genomics becoming affordable often in the coming time ?

Shankha Banerjee: Like our overall industry, I think, having a sub-sector market sizing is something which is not available at an industry level. I may not be able to comment on the market size currently but one thing which is sure is that genomics as a segment is definitely, getting more acceptability amongst the clinicians and is also seen as a way to kind of pursue or go ahead on the whole personalized care and personalized medicine, which is evolving, globally and also in India. Also, I think the whole cancer or the onco area that is getting more focus in our country, also uses genomic testing. We believe that in the future, the requirement for genomic testing is going to grow from wherever it is today, and we definitely would like to get a sizable share of that growth going forward and we are therefore investing in that basis. As of now, I do not think I am in a position to share, sectoral breakup of what percentage of our business is genomic.

Yogesh Soni : Okay, that helps. Just to get a further clarity, if you will be able to provide on the affordability part, I mean, how will that be achieved?

Shankha Banerjee: I think affordability of genomic testing is improving day-by-day. It is a combination of volume and as well as the type of testing equipment that one is going to use, and technology improvement that we are seeing from time-to-time. So, affordability and the testing costs are getting competitive. So, I think if that trend continues the way it is, the user acceptability will grow even further.

Moderator: The next question is from the line of Aashita Jain from Nuvama Institutional Equities. Please go ahead.

Aashita Jain: Good day, everyone. My first question is on your West India strategy. I understand you are in the last leg of integration, but with the dual branch that we have now with Suburban and Dr. Lal, how should we see your West India region performing going forward? Could you give us more color around your organic expansion, maybe focus cities, or how are you dividing the local areas, and also the white spaces that you are seeing in the West India region? That is my first question.

Shankha Banerjee: Right. West India for us is, Maharashtra, MPCG, Gujarat and Goa, that is what West India is. And amongst this whole wide geographic array, the dual branch strategy is operational in Mumbai, Pune, and Goa primarily. The rest of the parts of West India is still fully Dr. Lal PathLabs organic and a few inorganic acquisitions that we have done in the past. In terms of our strategy, the core medical hubs are Mumbai and Pune and that is the area that we really want to build a strong base in. I mean, the other parts of

West India in MPCG, MP and

Chhattisgarh, there is quite a lot of organic Dr. Lal PathLabs presence and quite a lot of those markets we are the number one brand and we continue to

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expand our footprint as well as our business there, whereas in markets like Maharashtra and Gujarat, we are not the leaders and could be quite far behind some of the incumbent brands there. Those are the geographies that we are trying to build for the long term and working on that to see how we can now grow these two brands organically in Mumbai and Pune.

Aashita Jain: Is it fair to assume that the majority of the expansion would be franchisee collection centers going forward rather than the lab addition in these markets or how should we see it from say, 3-5 years down the line?

Shankha Banerjee: In a 3-5 year scenario, definitely we will have lab additions as well. So, it is not that lab additions are not going to happen. But, since we are first focusing on a very compact geography of Mumbai, Pune, I think the lab infrastructure for Mumbai, Pune is already established. In these markets, it could translate into more of a collection network expansion. But when you look at West as a whole, which includes other parts of Maharashtra, Gujarat, there is also a possibility of adding lab infrastructure which we continue to do on a steady basis.

Aashita Jain: Understood. That is helpful. My second question is on the radiology side. I think in the last earnings call, you made a remark that radiology is also on the table.

How should we think radiology as a growth lever for you? And also, one of your competitors have also started ECG at home and they are doing a couple of other basic radiology as well as advanced. How should we see this for Dr Lal PathLabs going forward?

Shankha Banerjee: Radiology has a gamut of tests, there is something called basic radiology which would be X-ray, ECG, Ultrasound, TMT tests, etc., and then there is high-end radiology which would be CT, MRI, maybe even let us say a PET / CT scan, scanners, and things like that. Basic radiology, we do in some of our urban markets we do basic radiology, although it is still a very low contributor. But we were running some pilots which we continue to do on high-end radiology and we are still working on a plan as to how we will expand into high-end radiology, which is CT, MRI. That business plan is still not fully frozen. But yes, that is an opportunity that we are aware. And at the appropriate time, we will look at that business model as well as a part of our growth strategy.

Aashita Jain: Sure, that is helpful. And last couple of quick bookkeeping questions. What was the growth in the Suburban this quarter as well as the margin for this quarter?

Ved Prakash Goel: Aashita, as we have merged now, separately it is not Suburban but west as a whole which we have already answered, Shankha has given that. Due to this IT stack change now maybe going forward it will come back what we used to do 2 quarters earlier. But right now, this quarter also is impacted because of this changeover. And obviously margins also is now whole as a Company, not separately. But this quarter and the last quarter was impacted due to this IT stack change.

Aashita Jain: Okay. And this quarter your tax rate is 26%. Is there been any change or 26% is something we should build in for the full year and the coming years as well?

Ved Prakash Goel: Yes. 25%-26% is the normal tax rate. If you are comparing with the last year if you see there was some deferred tax reversal because of Suburban and that was the one-time impact, we have explained last time. But going forward, I think this is the standard rate, 25%-26%.

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Aashita Jain: Understood. And I think lastly on the guidance you said you are expecting better than the earlier guidance of 27% EBITDA margin. But are you also revising up your gr

owth guidance revenue of 11 %-12%?

Shankha Banerjee: No. We will stay with our 11 %-12% revenue growth projection.

Moderator: Thank you. The next question is from the line of Harshal Patil from MIRA e Asset Capital Markets (India). Please go ahead.

Harshal Patil: Thank you for the opportunity. Most of the questions are answered. Just need one understanding from you. You alluded to quite a lot of reasons for a very good volume growth that we have achieved. I just also wanted to kind of have your qualitative views on, we are seeing a good volume growth, also the competitive intensity as you kind of rightly said is benign. In this whole thing, how do you see the mix from unorganized to organized moving around across markets? Maybe has that picked up or probably how should we see that going ahead just in the light of the volume growth that we have got?

Shankha Banerjee: The caveat always is that we do not have a very clear industry level number. All that we are saying is obviously some extrapolation from what we observe from the other listed players and what kind of results they are declaring and some of the maybe quasi-listed players and what kind of results they are declaring. I think at an overall level, one can see that the quasi-listed space definitely seeing a slightly better revenue volume growth, and to that extent one can imagine that maybe the shift towards organized is getting slightly more accelerated, but these are all directional statements, I think the actual percentages or rate of change, contributions, etc., are very difficult to pinpoint.

Moderator: Thank you. Ladies and gentlemen, that was the last question for today. I now hand over the conference to management for closing comments.

Ved Prakash Goel: Thank you, everyone for joining us today. We truly appreciate your continued trust and support. We hope we have been able to address all your questions.

Please do not hesitate to reach out to us if you have any further queries.

Thank you once again and my best wishes to all of you.

This is a transcription and may contain transcription errors. The Company or sender takes no responsibility for such errors, although an effort has been made to ensure a high level of accuracy.

Call: Nov 2025

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Call: Feb 2026

Classification: Internal
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National Stock Exchange of India Limited
Exchange Plaza,
Plot No. C/1, G Block,
Bandra Kurla Complex, Bandra (E)
Mumbai – 400 051

Symbol: LALPATHLAB BSE Limited
Corporate Relationship Department
Phiroze Jeejeebhoy Towers
Dalal Street
Mumbai – 400 001

Scrip Code: 539524

Sub: Transcript of Q3 & 9M FY26 Earnings Conference Call

Dear Sir/ Madam,

Pursuant to Regulation 30 of SEBI (Listing Obligations and Disclosure Requirements) Regulations, 2015, please find attached herewith Transcript of Earnings Conference Call on Financial Results for Q3 & 9M FY26.

We request you to please take the same on record.

Thanking You,

Yours Faithfully,

For Dr. Lal PathLabs Limited

Vinay Gujral
Company Secretary & Compliance Officer

Encl.: As above
Page 1 of 19

Dr. Lal PathLabs Limited
Q3 FY26 Earnings Conference Call Transcript
January 30, 2026

Call Duration • 1 hour
Management Speakers • (Hony) Brig. Dr. Arvind Lal – Executive Chairman
• Mr. Shankha Banerjee - Chief Executive Officer
• Mr. Ved Prakash Goel – Group CFO and CEO, International Business
Participants who asked questions • Prakash Kapadia – Kapadia Financial Services
• Vivek Agrawal - Citigroup
• Anshul Agrawal - Emkay Global
• Raman KV - Sequent Investments
• Bino Pathiparampil - Elara Capital
• Saion Mukherjee - Nomura
• Shyam Srinivasan - Goldman Sachs
• Surya Patra – Phillip Capital
• Vamsi Hota - ASK Investment Managers
• Reet Jain - Niveshaay
• Lokesh Manik - Vallum Capital

Moderator: Ladies and gentlemen, good day, and welcome to the Dr. Lal PathLabs Q3 FY26 Earnings Conference Call.

As a reminder, all participant lines will be in the listen -only mode and there will be an opportunity for you to ask questions after the presentation concludes. Should you need assistance during the conference call, please signal an operator by pressing “*” then “0” on your touchtone phone. Please note that this conference is being recorded.

I now hand the conference over to Mr. Nishid Solanki from CDR India. Thank you, and over to you, sir.

Nishid Solanki : Thank you. Good afternoon, everyone, and welcome to Dr. Lal PathLabs Q3 FY26 Earnings Conference Call.

Today, we are joined by senior members of the management team, including (Hony) Brig. Dr. Arvind Lal – Executive Chairman ; Mr. Shankha Banerjee, CEO , and Mr. Ved Prakash Goel , Group CFO and CEO International Business.

I would like to share our standard disclaimer. Some of the statements made on today's conference call could be forward -looking in nature, and the actual results could vary from these forward -looking statements. A detailed statement in this regard is available in the results presentation, which has been circulated to you and also available on stock exchange website.

I would now like to invite Dr. Lal to share his perspectives. Thank you, and over to you, sir.

Dr. Arvind Lal : Thank you, Nishid, and good afternoon, everyone, and thank you for joining us for the Q3 and 9M earnings call for fiscal 2026.

Healthcare in India is undergoing a profound transformation. We are witnessing a steady rise in lifestyle -related and chronic diseases, an aging population with evolving care needs and a growing awareness among consumers about the importance of early detection and preventive health. These shifts are redefining diagnostics from being episodic and illness driven to becoming an essential continuous part of long -term health management.

At the same time, access to high -quality diagnostics remains uneven across the country, reinforcing the relevance of organized technology -enabled players who can deliver reliable and standardized services at scale. This is where our purpose at Dr.

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Lal PathLabs remains deeply aligned. We are committed to building an accessible, dependable and clinically rigorous diagnostics ecosystem for every Indian.

This quarter, we made meaningful progress on several strategic priorities :

- First one was launch of Sovaaka, our personalized preventive healthcare platform : A major milestone in this direction was the launch of Sovaaka operating under the philosophy of science behind wellness. Sovaaka represents a strategic pivot from traditional disease detection towards science -led disease prevention. By integrating high -end diagnostics and AI -supported imaging with curated programs, we are directly addressing the rising demand for personalized preventive care in India.

- Next is the expansion of specialized and advanced testing capabilities : We continue strengthening our offering in high -end diagnostics, including oncology, genomics, advanced infectious disease testing, and investments in our reference labs and collaboration with global technology partners remain key enablers of this strategy .

- Next is deepening our presence in underpenetrated markets : Our scalable franchisee -led model continues to help us expand into Tier 3 and Tier 4 markets. Improving diagnostic access in semi -urban and rural India remains a core long -term priority.

Together these initiatives reflect our integrated approach that is combining a robust physical network, deep clinical capability and digital innovation to serve India's evolving healthcare needs while driving disciplined and sustainable growth.

With that, I would now like to invite our Chief Executive Officer, Mr. Shankha Banerjee, to share the key performance highlights

for the quarter. Thank you, and over to you, Shankha.

Shankha Banerjee : Thank you, Dr. Lal, and good afternoon to you all. I am glad to connect with everyone today to walk through our Q3 FY26 performance.

Our Q3 FY26 results reflect sustained growth and operational excellence. We delivered strong financial performance this quarter with revenue rising 10.6% YoY, resulting in 10.8% growth for the 9 -month period. Patient volume growth was lower

than our current trend at 2.7% in Q3 due to unexpected decline in our "seasonal fever portfolio". The YTD patient volume growth is 4.4%. Our YTD sample growth remains robust at 9.6%.

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In Q3, we continued the journey of significant clinical advancement with introduction of more than 15 new tests, including more than 5 which are "First in India". These additions were across multiple portfolios, further strengthening our high-end complex testing capabilities. Our technical team had 5 international journal publications with 1 award-winning paper in the area of "using ML for rapid acute leukemia diagnosis". We have upgraded our enterprise IT infrastructure to next-generation technology, which has an AI-enabled environment at compute and network layer. We have rolled out a new agentic bot for patients at multiple touch points and a clinician-facing digital tool in pilot mode. We have enhanced our patient experience at walk-in labs and home collection with faster reporting standards in Delhi NCR. All these initiatives and more in the pipeline are aimed to enhance the patient and clinician experience with the brand. Our recent launch of "Sovaaka" preventive wellness program model also reimagines the patient experience with diagnostics. Together these developments solidify our position, not just as a laboratory service provider, but as a comprehensive high-tech partner in our patients' long-term health journey.

During the quarter, Swasthfit contributed 26% to total revenue and continues to play a primary role in our B2C market growth. We are scaling this affordable preventive health model to Tier 2 and below towns to capture an untapped preventive testing market.

The transition from unorganized to organized diagnostics in India is sustained as patient priorities shift towards clinical reliability. Our competitive edge lies in stringent quality protocols and superior service architecture. Beyond the test itself, our differentiation is defined by the end-to-end patient experience from seamless sample collection to digital report delivery. Ultimately, these superior service levels are the primary drivers of patient acquisition and brand loyalty.

We are scaling through expansion in newer markets, innovative offerings and digital enablement. This approach cements our diagnostic leadership and enables long-term profitable growth.

With that, I will now hand over the call to our Group CFO and CEO International Business, Ved. Over to you.

Ved Goel : Thank you, Shankha. Good evening, everyone, and a warm welcome. Thank you for joining us today.

I will take you through the key operational and financial highlights for Q3 and 9 M of FY26, followed by our year-to-date performance.

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Let me begin with some key operating metrics.

- Revenue per patient for Q3 FY26 stood at Rs. 927, representing a 7.7% YoY increase over Rs. 861 in Q3 FY25. This improvement was driven primarily by favourable test mix and geographic mix rather than pricing.
- Test per patient increased to Rs. 3.11 in Q3 FY26 compared to Rs. 2.97 in the corresponding quarter last year, reflecting our continued focus on menu expansion and preventive healthcare offerings.
- As part of our commitment to strong governance and customer-centric practices, the company has gladly passed on the benefits arising from reduction in GST on reagents and chemicals from 12% to 5% through price reduction for patients. While this had a modest

impact on realization, we

believe it strengthens our long-term trust and demand sustainability.

Overall, our growth continues to be volume led with realization improvements coming largely from mix enrichment, which we believe are structurally sustainable drivers.

- Revenue for Q3 FY26 came in at Rs. 660 crore compared to Rs. 597 crore in the same quarter last year, reflecting a YoY growth of 10.6%.
- For the 9 months ended December 2025, revenue stood at Rs. 2,060 crore versus Rs. 1,859 crore in YTD FY25, registering a growth of 10.8%, driven by steady volumes and operating leverage.
- During Q3 FY26, we incurred a onetime cost of Rs. 30 crore related to implementation of new labour code, which has been reported as an exceptional item. This is a non-recurring adjustment and does not impact the underlying operating performance or steady-state cost structure of the business.
- EBITDA before exceptional item for Q3 FY26 stood at Rs. 179 crore compared to Rs. 154 crore in Q3 FY25, reflecting a growth of 16.3% and

EBITDA margin of 27.2%.

- For the 9 months ended December 2025, EBITDA before exceptional items came in at Rs. 596 crore compared to Rs. 527 crore in the same period last year, registering a growth of 13.1% with EBITDA margin of 28.9%.

- Profit before tax for Q3 FY26 after taking the hit of labour code was Rs. 124 crore compared to Rs. 138 crore in Q3 FY25 with a PBT margin of 18.8%.

For the 9 months ended December 2025, PBT stood at Rs. 509 crore v/s Rs. 471 crore in YTD FY25 with a margin of 24.7%.

- Profit after tax for Q3 FY26 after taking the hit of labour code stood at Rs. 91 crore compared to Rs. 98 crore last year, resulting a PAT margin of 13.9%.

For the 9 months period, PAT was Rs. 378 crore compared to Rs. 337 crore in YTD FY25 with a margin of 18.3%.

- Earnings per share for Q3 FY26 was Rs. 5.4 compared to Rs. 5.8 in Q3 FY25. For the 9 months ended December 2025, EPS stood at Rs. 22.4 compared to Rs. 20 in the previous year.

- Our balance sheet remains robust with net cash and cash equivalent of Rs. 1,411 crore as on December 31, 2025, providing us significant flexibility to support organic growth, calibrated inorganic opportunities and shareholder returns.

- I am pleased to share that the Board of Directors has approved an interim dividend of 35% amounting to Rs. 3.5 per share on enhanced equity share capital post 1:1 bonus issue.

Our capital allocation priorities remain unchanged, investing in growth, pursuing M&A opportunities and returning surplus capital to shareholders while maintaining financial prudence.

Overall, we remain focused on delivering sustainable growth and healthy profitability while continuing to invest in quality, compliance and customer experience.

With this, I conclude my opening remarks, and I now request the moderator to open the forum for question -and-answer. Thank you.

Moderator: Thank you very much. The first question is from Prakash Kapadia from Kapadia Financial Services. Please go ahead.

Prakash Kapadia : Thanks for the opportunity. A couple of questions from my end. Shankha, at a time when industry revenues have been growing in the 7% -8% range, our growth, if I look at last 11 -12 quarters, has been 10.5% on an average. So what am I missing? We have brand, we have technology, we have franchisee network, we have reach, we have doctor empanelments, sales force, low cost of test, on-time delivery. What is the missing factor which is leading to this kind of a growth? Because if I look at some of the other listed players, they have been reporting higher growth. So that is the first question on revenue.

And secondly, on Swasthfit, it has been growing almost 2x of our company revenue.

How is the journey been? Is it now a Tier 2, Tier 3 phenomenon also? Is that also contributing there? What is the journey in Tier 2, Tier 3 cities? Is it movement from

ent from

basic diabetes and thyroid to some of these packages? And is there any possibility that Swasthfit is cannibalizing some repeat usage or revenues at the company level?

Those are my 2 questions.

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Shankha Banerjee : Thanks, Prakash. I think on the first one in terms of revenue, I think we have been consistent in trying to say that right now we are focusing only on organic revenue growth. If you remember, about 2 years back, we had slipped to about a 7% organic revenue. And so we were quite clear that we want to build the organic momentum back across the organization. And that is what the team has been working on.

Simultaneously, the building blocks of how to take the next jump forward are being put in place. Revenue growth in the future, we are obviously working towards making this better than whatever we have now. But the stability that you see is kind of designed to at least bring the organic growth trajectory at a level that we are comfortable.

I think we have been saying 11% to 12% organic growth for this year. And even as of now, we are confident that we should be able to deliver between 11% to 12% for the year. And once the whole base kind of comes to that level, I think other initiatives which are already being laid out should help us step up going forward.

On the second question on Swasthfit, yes, I think even in some of the previous calls I had mentioned that Swasthfit now with our expanding reach we ourselves are able to now put Swasthfit into more geographies, including the smaller towns, and we are definitely seeing offtake or acceptability of this in the smaller towns.

Also, even in the urban centers, our penetration or availability of this is , we are seeing not only direct consumer or patient pickup, but like I mentioned, in some cases, we are even seeing doctor recommendations coming for the portfolio. Those levers are still in play, and that is what is helping us maintain the growth momentum on Swasthfit.

Prakash Kapadia : Okay. And in some of these initiatives which you have been calling out over the last few quarters, be it distribution, be it new product development, be it enhancing the menu, I think increasing the sales force. Where will that trajectory come in? Now is FY27 going to get us there? Because I think last call, you had hinted suburban has stabilized, the IT issues are done, Mumbai reference lab has stabilized.

Will West and South contribute more meaningfully to that, then we will see that delta or incremental revenue coming through us? What is the tipping point or what as analysts and fund managers we should focus on, this is what it is and we are there?

Shankha Banerjee : I think for a size of organization and the scale that we are running, I do not think one single city, geography will really help in kind of taking the overall number higher.

What has to work for us is slightly different strokes in different geographies, but all
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geographies will contribute towards a higher growth momentum. I would say, starting Delhi NCR, North, East, West, even South, international as well, all of these geographies put together will contribute to the higher growth trajectory that we are aiming for. It is unlikely that one geography will help the overall number of the organization to go to the level that we want it to move towards.

Prakash Kapadia : Okay. Understood. And from next year, we should see some better growth? Is that the path we should expect?

Shankha Banerjee : Yes, that is the trajectory that all of us expect, including me.

Moderator: Thank you. Next question is from Vivek Agrawal from Citigroup. Please go ahead.

Vivek Agrawal : Thanks for the opportunity. In the opening remarks , you talked about that the quarter has been also impacted or the volumes have been impacted by adverse season. So is it possible for you to break it down how the seasonal volumes or the fever -related volumes

have grown in this quarter or how the specialty or semi -specialty test -related volumes have grown in the quarter? Thank you.

Shankha Banerjee : Yes. I do not think we will get into that micro detail, Q3 is usually not a thing where we discuss season or fever or things like that. But it has been an unexpected quarter in the sense that we have seen an impact due to our seasonal fever portfolio has got negatively impacted. We have declined on that portfolio. And yes, that has resulted in lower patient volume numbers and even to a certain extent the lower sample volume numbers.

Vivek Agrawal : Understood. Basically, it has declined, right? And approximately, what is the share of this portfolio in the overall patient volume, approximately? Can you give any ballpark number?

Shankha Banerjee : You see that share is different by quarter, so I am not too sure, and that is not something which we focus upon. I am not too sure that is the data we would want even any of you to be focusing upon.

Vivek Agrawal : Yes. No problem at all. Another question is if you look at the patient volume growth, so although this quarter has been impacted, but even if you look at the 9 months, it is around 4 %-5% kind of growth. And this is despite that you are pushing significant growth as far as in the Tier 2, Tier 3 town, expanding the network and all. Is it the right way to look at that? Your core markets, like, for example, Delhi NCR or in the Tier 1 markets, is there any kind of a structural slowdown as far as the growth is concerned? Or how to put it.

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Shankha Banerjee : I think the patient volume numbers are seeing a steady growth. I think quarter -to-quarter fluctuations are there. But if you at least see in the last 2 -3 year movement, from an annual low of almost 2.7% that we saw 2 years back, we are going upwards. I think expecting a jump at the scale of patient volumes that we do, expecting this number to suddenly go up by 3 %-4% in one calendar year or one financial year may not be the right expectation.

But yes, there is a slow and steady improvement that we are trying to build on the patient volume number. And that you will see in the YTD number this year also. We have obviously this impact right now in this quarter, but we are seeing some increase in the patient volume numbers this financial year as well.

Moderator: Thank you. The next question is from Anshul Agrawal from Emkay Global . Please go ahead.

Anshul Agrawal : Thank you for the opportunity. Sir, my first question is on margins. I think generally, we see a dip in margins in the current quarter, and we had previously indicated that

H2 margins would also be lower than H1 owing to network investments and other expansion projects. Are the current margins, I am talking about 9-month ended margins, sustainable at around 29% going forward?

Ved Goel: I think if you see H1 margin was higher this quarter. If we take out the impact of labour code, it is 27.2%. And later part of the year we said it will be lower than first half of the year. Overall, earlier, we said a similar number, around 28% kind of margin. And we are confident that we can achieve in this financial year between 27%-28% even after this impact of labour code.

Anshul Agrawal: Got it. Going forward, the margin trajectory would remain firmly around this 28% is what I am trying to understand despite us passing on those GST-related benefits, et cetera, and all that stuff.

Ved Goel: Yes. I think we are hopeful that we can maintain the trajectory between 27%-28%.

Anshul Agrawal: Got it. Second question I had was, can you provide us with an update with the advanced radiology pilot project that we had undertaken in Delhi? Is there any update that you can share with us with regards to traction or any plans to sort of expand on that?

Shankha Banerjee: The traction on the advanced radiology is that we were running a one center pilot in

Delhi NCR, and we are looking at it and it is kind of going as per the planned trajectory that we have. We are now starting or just started another center in Delhi

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NCR on advanced radiology because we want to now see how replicable is our first pilot in another location which is geographically apart. So that is the status as of now.

Anshul Agrawal: Got it. This would be an advanced center only, sir, right? It will include MRI and CT scan machines.

Shankha Banerjee: Yes, this is another center with the MRI and CT scan.

Anshul Agrawal: Just one last bookkeeping question from my end. Can you help us with the CAPEX number for the current quarter? Any full year guidance incorporating these high investments in radiology and even Sovaaka preventive wellness program?

Ved Goel: Anshul, this year, we have made 2 investments, one, we bought one property which is for our precision diagnostic center, and, of course, the Sovaaka, which we have just started. Roughly, we will be spending about Rs. 150 crore to Rs. 160 crore kind of CAPEX in this year. Normally, we spend about Rs. 50 crore - Rs. 60 crore - Rs. 70 crore on an operating kind of capex.

But these radiology center, let us suppose if we take one center, it takes away about Rs. 15 crore - Rs. 16 crore kind of investment. And if we take, let us suppose, going forward 1 or 2 more, then accordingly someone can factor in 2 more investments.

Anshul Agrawal: Right. So sustainably, capex could settle at around, say, around Rs. 60 crore - Rs. 70 crore of operational capex plus 2 center capex on this figure. About Rs. 100 odd crore could be the right number?

Ved Goel: Plus some new initiative or new investments. So near to time, maybe you can see between Rs. 100 crore - Rs. 150 crore kind of CAPEX someone should assume.

Moderator: Thank you. The next question is from Raman KV from Sequent Investments. Please go ahead.

Raman KV: Sir, I have 2 questions. One is, sir, can I get the total test volume for the quarter?

Shankha Banerjee: Total test volume?

Raman KV: Yes, number of tests conducted, not the sample collected?

Shankha Banerjee: Number of tests.

Ved Goel: We have done 22.2 million samples in this quarter.

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Raman KV: Yes, that is the sample collected, right? I just want to understand the number of tests performed on those samples?

Ved Goel: Raman, we generally give sample numbers. What you are talking about the parameter where, let us suppose, if we do one test, which is TFT, LFT, let us suppose, and within that, there are so many parameters, which we do not report directly. So these are not reported. So we generally give sample numbers.

Raman KV: Okay. Understood, sir. And sir, my second question is with respect to Swasthfit. I just want to understand the margin difference between Swasthfit and the rest of the portfolio. Where I am coming from, I just want to understand if once your contribution of Swasthfit improves further, will it aid margins or will it be a pressure on margins or the margins will remain flat?

Ved Goel: So realization is, of course, higher in Swasthfit because of bundle and packages, but same time, we provide discount because these packages are at discounted rate.

Obviously, we get operating leverage. And overall, net-net, it is not like very different, but yes, realization is higher and operating leverage is always coming, be it consumption, be it other operating overheads. But somebody should not factor in

that higher contribution from Swasthfit will give very high margins. That one should not factor in.

Shankha Banerjee : It is not margin dilutive.

Raman KV: Okay. Understood, sir. Sir, my third question is with respect to the newly launched Sovaaka . Can you just elaborate what are the tests the company will be performing under this?

Shankha Banerjee : Sovaaka is a very new conceptual setup. It is focused on p

reventive wellness testing

only, and if you want more details, you can go on to Sovaaka.com, which is the website dedicated to Sovaaka. You can type S -O-V-A-A-K-A, Sovaaka.com, and you will get details. But it is all preventive health checkups .

These are packages , there is a pre -consultation with the doctor, and depending upon individual conditions, family history, etcetera, packages are suggested and finalized, and then the testing happens and report consultation and diet consultation post that.

So it is a wellness and preventive focused testing only.

Raman KV: Understood, sir. And sir, B2C share during the quarter, is it still 75% or has it increased?

Shankha Banerjee : B2C share is still around 75%.

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Moderator: Thank you. Next question is from Bino Pathiparampil from Elara Capital. Please go ahead.

Bino Pathiparampil: A couple of questions from my side. On one side, we have seen a sharp depreciation in Rupee s. and at the same time, we have seen this European Union trade deal which cuts tariffs. Does either of this favourably or unfavourably affect our cost base and can it impact margins?

Ved Goel: I think dollar is moving up, we all know. And most of our reagents and chemicals are imported. Indirectly, we are also impacted. But we are able to absorb that inflationary cost because of our volume. Right now, we do not have any which is linked to - directly linked to dollar because we buy from Indian companies. On the other side, this EU trade deal is very early to comment on. Definitely, there may be some opportunity in future which can come. But right now, it is too ea rly to comment on that.

Bino Pathiparampil: Okay. And in case these reagent prices actually go up, do you think you can pass them on without impacting volume growth?

Ved Goel: We have long -term contracts with most of our large suppliers. Right now, we are not having that. But yes, of course, if dollar trajectory is upwards like this, then definitely we have to review our pricing again.

Bino Pathiparampil: Understood. And one question on the radiology plan. So you mentioned that it is going as per plan. In the normal course if it goes as per your plans, when can we see a significant ramp -up on that strategy? Would it be in a couple of years or would it be in 5 years?

Shankha Banerjee : Radiology contribution to our business is less than 5%, and even a significant ramp - up may or may not immediately impact the overall fortunes of the company. But having said that, we are definitely understanding the business model around high - end radiology in brand strong urban markets, and then we will also maybe try and understand in some other markets and then figure out our long-term strategy. Having said that, in terms of time horizon, like you said, it is unlikely to be a significant contributor to top line growth at least not in the next 2 -3 year scenario.

Moderator: Thank you. Next question is from Saion Mukherjee from Nomura. Please go ahead.

Saion Mukherjee : Good evening. Thanks for taking my question. Sir, you mentioned about the patient volume growth at 4% -5%, which you think can increase going forward. It may not be a step-up, but it can go up. If you can share your thoughts as to what we should look

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at, what would drive that? And also acquisition, small, big task, is that a possibility you see in the current environment? And can that emerge as a driver for you?

Shankha Banerjee : Right. For the organic patient volume growth there are 1 or 2 obviously quite clear levers that I think we have spoken about earlier, and the most important one is the spread and the access that we are increasing for patients. You see the number of new testing labs and the collection network expansion that we have stepped up over the last 2 years is aimed at gettin g more wider geographic spread, and therefore, ability to access a larger population and give more access

to population to come and

use our brand for their diagnostic needs. I think that is number one.

And then primarily over and above that, even through our existing network, there are patient acquisition activities that we keep doing from time to time. And our Tier 3,

Tier 4 and rural push that we have been undergoing for over, I would say, maybe 4 - 5 year journey that we have had on that is also going to help us get the organic volume.

On the inorganic side, definitely, there is appetite for us to be looking at inorganic opportunities and we are on the lookout for suitable inorganic opportunities which can fit into our portfolio and our strategic growth areas.

Saion Mukherjee : And sir, I mean, are you looking at, how should I say? Like is it like midsized chains, big chains or individual labs? What kind of acquisitions are most likely?

Shankha Banerjee : It is not that we look at all kinds of options because it is geography specific. I think the South market is something where we would definitely like to have something which is reasonably large, at least maybe a top 3, top 5 player in a given city with a slightly larger platform play, which we can then adopt and grow.

Whereas in some of our other Northeast, maybe even some parts of West, we are open to doing standalone lab acquisitions, which gives us maybe access to some micro market or some other benefit which we are maybe not able to get through our normal organic business.

Saion Mukherjee : Understood. Sir, on the pricing front, I understand like it has been like 3 years there has not been any price increase. In fact, you mentioned there has been the GST cut on inputs were passed on. So what is your view on pricing? When do you plan to take some price increase? Can we expect this year or next year? Any thoughts around that?

Shankha Banerjee : Like Ved mentioned, we have just taken a decision to do a price cut to pass on the GST benefit. So that cycle has to run for some time before we consider a price

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increase. So therefore, I definitely do not see a price increase on the horizon, at least not for the next 2 -3 quarters.

Moderator: Thank you. The next question is from Shyam Srinivasan from Goldman Sachs. Please go ahead.

Shyam Srinivasan : Just one on the realization part. Again, I am just calculating revenue per patient from your revenue numbers and the patient numbers. So that comes up by 7.5%. You mentioned I think the GST cuts have not come in Q3. So I am assuming it is in Q4. What would be some of the enablers of this? Would it be higher Swasthit? Can you just explain that? Or has the environment from a competitive standpoint been a lot more easier, do you think?

Shankha Banerjee : The GST price cut was taken in Q3, but it was maybe towards the end of Q3. So obviously, the full impact is not reflecting in the RPP of Q3. To that extent, you are right. It is a combination of test and geography mix that we are seeing in terms of RPP. As we said, Delhi NCR continues to grow double digits and we all understand that Delhi NCR is a higher RPP market for us compared to the rest of the country, and we also have some test mix benefits that are there. So those are the major contributing factors for the geography and the test mix for the RPP increase that we see.

Shyam Srinivasan : That is helpful, sir. Just the second question is you alluded to higher growth over time. Are we talking about not the 11% to 12%, but are we talking about higher than that? We think, and at some point of time, if it is 3-4-5 quarters later, we are looking at price increases. Could we then go beyond the 12% on a medium-term basis? I am not holding you to a specific year, but just want to understand are we in the trajectory looking up at any point of time?

Shankha Banerjee : Yes. I think the thought process that you are saying is reasonable. We are looking at 11% to 12% is what we have been saying for this

financial year. So going forward, if that becomes our baseline, we obviously would like to be slightly better than that. And maybe 3 -4 quarters down the line, the price increase window may open up again.

Shyam Srinivasan : Yes. And what would be a trigger for a price increase? I am just curious. We have not done it for many, anything that you think will then say this prompts us to take prices? Is there something that we monitor closely? Is it inflation? I am just curious. What would trigger the price increase.

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Shankha Banerjee : I think there are two triggers we look at. One is, obviously, internal cost is one thing which we continue to look at, how much we can absorb or what is it that cannot get absorbed, et cetera. And second would also be the external pricing table, because we do not want to get into a situation where we are not or we seem to be falling off the pricing premium table because there are quality connotations associated with price in our country still.

Moderator: Thank you. Next question is from Surya Narayan Patra from Phillip Capital. Please

go ahead.

Surya Patra : Thanks for the opportunity. My first question is on the Sovaaka initiative . So I just wanted to understand, sir, how is this different from the Swasthfit and whether this will be complementing Swasthfit growth. And another point here is that, can the Sovaaka initiative be scalable and scalable beyond the NCR?

Shankha Banerjee : Sovaaka and Swasthfit are very different in their idea and execution. Swasthfit is a pathology only and widely distributed and accessible testing portfolio, which we said not only do we do in urban centers, it also goes into Tier 2, Tier 3, Tier 4 towns across the country. Sovaaka is a personalized, customized wellness , preventive wellness setup which integrates pathology and imaging, including high -end imaging and is a specific center -driven touch and feel experience for people who are looking at really in-depth understanding of their individual health status. So this is kind of center driven. As of now, it is the first center that we have launched. Again, this will be on a mode where we will see how we are scaling this, the patient footfall, the walk -ins, etcetera, and the buildup of this center. I think only on the basis of that will we decide how do we scale it. But there could be possibly multiple modes or models of scaling. But yes, the scalability is way beyond Delhi NCR once we get the model right.

Surya Patra : Okay. Is it fair to believe Swasthfit is a kind of annual testing mechanism, while Sovaaka would be a once in lifetime or once in 10-year kind of thing?

Shankha Banerjee : Both are annual. It is just that the type of testing is different and the offering is very different. Both are annual in nature from a preventive side.

Ved Goel : Surya, I mean, just on the Swasthfit, it is not only preventive wellness, but in our case, most of the time it is upselling. Maybe patients who are coming for individual tests, they are trying to take this package because it makes sense and avoid multiple visits where they can get a lot more tests in a single visit. It is not always preventive wellness. While Sovaaka is purely preventive, preventive and wellness kind of thing. But Swasthfit is always where we have patients who comes in need also.

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Surya Patra : This is helpful. Just one more. Whether we do get any benefit out of this Ayushman Bharat Digital Mission success? What we have seen?

Shankha Banerjee : As of now, we are enabled to incorporate ABHA IDs into our system. So whosoever is giving us their ABHA IDs, we put it into their records the ABHA IDs. But primarily, ABDM is still focused on the hospital care. Diagnostics as of now we have not yet seen any reasonable thing to believe that this can become a key driver of growth for us right now.

Moderator: Thank you. The next question is from Vamsi Hota from ASK In

vestment Managers.

Please go ahead.

Vamsi Hota : My first question is on the extent of utilization of both the Mumbai and Bangalore reference labs. Given that we continue to open about 600 to 800 collection centers annually with greater focus on the high -growth North markets, except of Delhi, how do you plan to ramp up the utilization of these 2 specific reference labs?

Shankha Banerjee : The reference lab capacity by design is very high because it depends on what type of machine modularity that is currently deployed, and there is obviously ability for us to keep deploying more. A lab is designed with a life which says , we are looking at maybe a 10 -15-20 year outlook on a reference lab kind of a setup. So I do not think that kind of correlates with the structure of network collection that is going in other parts of the country.

The reference lab actually helps in building the tier -wise hub -and-spoke model that we work on. We have got reference labs and hub labs and cluster labs and satellite labs, and then front end, we have the collection network.

It is about putting a certain test menu in the market and then our ability to really build that test menu with the local prescribing community in the catchment area and our ability to deliver that timely reports and the quality report from the reference lab that we have for that geography. So that is an independent task and does not necessarily correlate with saying that more collection network will certainly mean more capacity utilization at a reference lab.

Vamsi Hota : That is helpful, sir. I will just ask one more question. Given that we follow a revenue sharing arrangement with our franchise network and that in markets, except of Delhi, we end up sharing more than 25% of our revenue with the network, do you foresee incremental margin dilution given franchisee -led growth? If not, how much more bandwidth do we have on expanding our EBITDA margins via process efficiencies

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and leverage, given that all of our growth has been volume and price led at least over the last 2 years?

Shankha Banerjee : I think Ved will add to whatever I say. I think one must understand that when a

franchisee is given a margin, there are certain tasks of the whole process which also get outsourced. And there is a certain benefit that we also get because of accumulation of samples, and on top of that, we get an independent entrepreneur who is also kind of really running after generating business for us.

So, there are multiple reasons why the franchisee footprint works for us, and we have been able to really develop a very strong mechanism of franchisee management. If you see, besides Delhi NCR, the model of franchisee revenue share is different in certain other geographies, depending upon how the competition, layout, etc. is. We have been able to manage it quite well till today in terms of our overall portfolio margin and we do not see any reason why that cannot be managed going forward as well. So Ved, do you want to add anything to that?

Ved Goel: No. So I do not think margin, where it is dilutive because, first of all, this model of having a franchisee is because of scalability and not putting a fixed overhead on our head because there is an entrepreneur who always drives, and it is a variable model where you share out of your revenue share instead of incurring on your own infra cost.

In spite, you see our contribution has increased so much. We are not diluting our margins. In fact, we are getting operating leverage because we already have testing outlets. We have already capacity which is available, and we can leverage on that.

So I do not think this will dilute our margins.

Vamsi Hota : Sure, sir. Incrementally, how much more scope or rather another way to ask this is, what is the dream state EBITDA margin that you kind of look at?

Shankha Banerjee : I think we are very near the dream state, if you ask me, be

cause we would rather

see if we feel that we are in a position to generate more margin, I would rather see how can it be deployed back to generate growth, sustainable growth, over the long term. I think the margin profile that we run, 27% -28% EBITDA, we are maybe in the quote what you say, "dream state".

Moderator: Thank you. Next question is from Reet Jain from Niveshaay. Please go ahead.

Reet Jain: Sir, basically my question was the molecular diagnostic players like 3B BlackBio or Mylab, Molbio, they are coming into the industry, like Molbio filed their IPO, DRHP.

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Basically, are they a competition or are they a supplier to us for kits? And how is that molecular diagnostic industry doing? Can you give a color on that?

Shankha Banerjee : Molbio is a vendor, is a supplier. They are not really into front -end retail pathology as far as I am aware. And so that is one question. And molecular testing, obviously, has seen increase over time and PCR testing, et cetera. Yes, I think that part of the portfolio should increase. I think maybe more traction will come on that portfolio.

Also because of the oncology testing, et cetera, happening, I think that portfolio will see, and I think the second thing which Dr. Lal is also mentioning is that we see Genetics, again, another area where quite a bit of traction we foresee will come in the market.

Reet Jain: Basically, can you give the reasons why that side of portfolio is doing well and such extensive tests are taking market share from this simple testing?

Shankha Banerjee : I do not think it replaces simple testing. Please understand what is happening is when you get into Genetic testing or molecular testing, these are more personalized, customized and more deeper testing. But that does not take the need away of routine or regular testing. Actually, on the contrary, as people get diagnosed with more difficult or complex conditions, for therapy and the patient management the routine testing is required and that need may actually go up.

I do not see it to be a replacement. Rather, it is more about building and helping build your clinical superiority as well as the trust with the clinicians of our ability to really solve the complex diagnostic problems. I think that is where these portfolios also play a role in our business.

Dr. Arvind Lal : We are also acutely aware of the increase in testing for all these noncommunicable diseases, NCDs, which are basically basic diseases in our jargon. But at the same time, the numbers are humongous, led by diabetes, high blood pressure and lipid profiles. And of course, cancer is catching up, liver, kidney. All these are a very large set of diseases which we club under NCDs. So there is a remarkable improvement for performing because 65% of the Indians are dying from these diseases every day, noncommunicable diseases, lifestyle diseases. Of course, infectious diseases have not left us. They are about 35%. But right now, NCDs are catching up.

Moderator: Thank you. Next question is from Lokesh Manik from Vallum Capital. Please go ahead.

Lokesh Manik : Sir, just one clarification I needed. The new initiative of Sovaaka, would you say this is more like a DTC foray, direct -to-customer foray, v/s. our established model where

the doctor and the lab setup may not exactly be in the same premises v/s a Sovaaka, which would be in the similar premises with obviously more advanced testing and advanced services product portfolio? Would that be a correct assessment?

Shankha Banerjee : Sovaaka , like I was mentioning earlier, is a preventive testing and a wellness testing - driven platform. So to that extent, it can see more self -generated test, but it does not mean that there is not going to be clinician -referred testing here as well. I think even annual health checks which people do, they may end up showing it to a clinician after that. Therefore, it is not

either/or. But yes, compared to the standard diagnostic lab testing where primarily much more prescription -driven testing is happening, in this situation, we might see slightly more self -driven testing as well.

Lokesh Manik : Right. So this would be company owned, right? This would not be franchisee or channel partner.

Shankha Banerjee : Yes, this is company owned, fully company owned.

Lokesh Manik : And even when you plan to scale, it will be a company -driven model. It will not be outsourced to franchisees.

Shankha Banerjee : The scaling model we will enumerate later, because once we have the experience of running the center, obviously, many more details will be with us for us to then decide. But the flagship store is company owned.

Moderator: Thank you very much. We will take that as the last question. I would now like to hand the conference back to the management team for closing comments.

Ved Goel : Thank you all for joining the call today and for your continued trust and support. We hope we have been able to address your questions satisfactorily. Should you have any further queries, please feel free to reach out to us. Thank you once again, and have a good evening.

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Call: May 2026

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